TRANSFORMING AMERICAN HEALTHCARE

A Six-Pillar Framework for System Transformation

With Vermont as the National Proving Ground

Health Transformation Review

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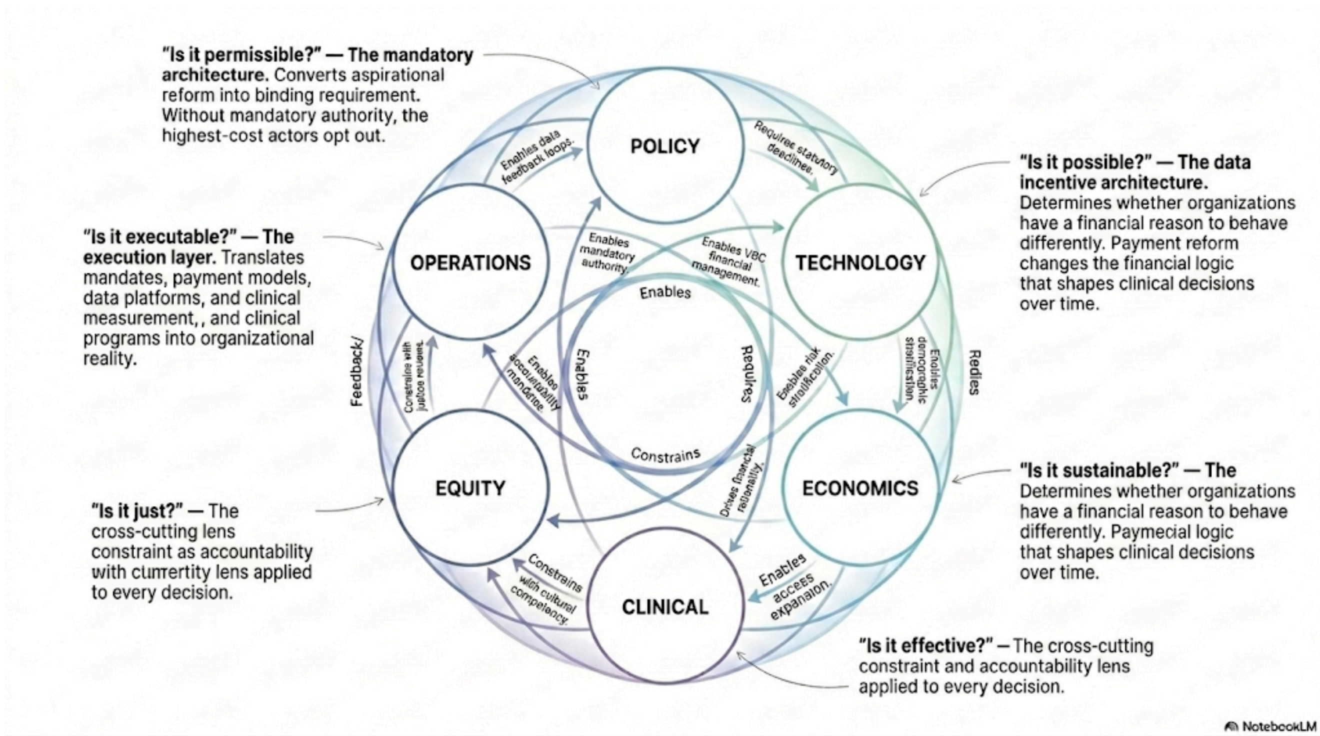

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PREFACE

This book began with a room full of people who already knew something was wrong.

In September 2024, the Green Mountain Care Board of Vermont convened a public meeting at which the consulting firm Oliver Wyman presented the findings of a year-long, $1 million statewide hospital systems analysis. Hospital CEOs sat next to state legislators. The GMCB chair sat across from the head of the Vermont Hospital Association. AHS officials, health economists, community advocates, and clinicians filled every seat and lined the walls. They had come because they suspected what was about to be confirmed, and because some part of them still hoped they were wrong.

They were not wrong. Nine of Vermont's fourteen hospitals were already operating at a loss. Thirteen of fourteen would be in the red by 2028, absent structural intervention. The cumulative five-year system deficit ranged from $700 million to $2.4 billion. Premiums had risen 108% in six years while household income grew 22%. The working-age population that funds the cross-subsidy keeping rural hospitals alive was declining. The over-65 population was growing toward 30% of the state.

That room is the origin of this book. Not because Vermont is uniquely troubled — every American state is facing some version of the same forces on a time delay — but because Vermont is early. Early in its demographic aging. Early in its hospital financial fragility. Early in its recognition that thirty years of voluntary payment reform have not changed the fundamental incentives of the system. And early, therefore, in its willingness to do something that no state has done with this level of statutory specificity and capital commitment: mandate structural change.

This book is the analytical framework for that change. It is not a policy brief for or against any particular agenda. The five goals of Vermont's Act 167 — reduce inefficiencies, lower costs, improve health outcomes, reduce health inequities, and increase access to essential services — are goals that virtually every person across the political spectrum would endorse. The means for achieving them are legitimately contested. This book engages that contestation honestly and presents the evidence on both sides where the evidence genuinely runs in two directions.

It is organized around the six-pillar framework: Policy, Technology, Economics, Clinical, Equity, and Operations. This framework emerged from the conviction that transformation fails when it addresses any single pillar in isolation. Payment reform without clinical redesign produces financial savings that evaporate when care models do not change. Clinical quality improvement without payment reform produces better care that the financial system immediately discourages. Technology investment without operational readiness produces platforms that no one uses. Equity goals without structural investment produce aspirations without outcomes. The six pillars must move together — which means the people responsible for each pillar must understand all the others.

Vermont runs through this book as its primary teaching case. Vermont is in this book not because it is large or powerful, but because it is transparent. The combination of a small state's intimacy, a strong public-sector analytical tradition, and an active regulatory body in the Green Mountain Care Board has produced a documentary record — the Oliver Wyman analysis, the GMCB's monthly hospital data, the AHS transformation reports, the AHEAD State Agreement — that gives this book a foundation most states cannot match. Vermont's transformation is happening in public, with detailed data, and the lessons transfer.

This book is also not a prediction. The transformations described here are underway but unfinished. Whether Vermont's hospital system stabilizes or contracts, whether global budgets reduce costs or constrain access, whether the Agency of Human Services can execute the restructuring its mandate requires — these are genuinely uncertain. What this book provides is the analytical framework for understanding what is happening and why, and for contributing to better decisions as the story unfolds. The decisions, and their consequences, belong to the people who make them.

The reader profiles and recommended reading paths are in the Introduction that follows. Use them. This material rewards a reading strategy calibrated to your role and your decisions, not a linear march through twenty chapters.

April 2026

INTRODUCTION

September 18, 2024

The room was full before the presentation started.

The Green Mountain Care Board of Vermont had convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group would present findings from a year-long, $1 million statewide hospital systems analysis commissioned under Act 167 of 2022. Hospital CEOs sat next to state legislators. The GMCB chair sat across from the head of the Vermont Hospital Association. AHS officials, health economists, community advocates, and clinicians filled every seat and lined the walls. They had come because they already suspected what was about to be confirmed, and because some part of them still hoped they were wrong.

They were not wrong.

Oliver Wyman's lead consultant opened with the financial trajectory. Nine of Vermont's fourteen hospitals were already reporting operating losses, with the worst reaching negative 8.9%. If current trends continued — the same trends that had been building for a decade, the trends that voluntary reform had been addressing for years without reversing — thirteen of the fourteen would be in operating losses by 2028. The cumulative five-year system deficit ranged from $700 million in the optimistic scenario to $2.4 billion in the realistic one.

The next slide showed premiums. The average silver exchange plan premium had risen from $456 per month in 2018 to $948 in 2024 — a 108% increase. Median household income had grown 22% over the same period. Out-of-pocket maximums had more than doubled. The University of Vermont Medical Center, with approximately 50% of the state's hospital market share, had charged approximately 417% of Medicare rates for outpatient services in 2022. Those charges flowed directly into the premiums that Vermont families were paying, with no regulatory brake.

The final slide showed the demographics. Vermont's working-age population was projected to decline 13% by 2040 — from 367,000 to approximately 318,000 — while the over-65 population rose from 21.7% toward 30% of the state's total. This shift would progressively move patients from commercial insurance, which pays above cost and funds the cross-subsidy that sustains hospitals, into Medicare and Medicaid, both of which reimburse below cost. The slide did not say the words "system collapse." It did not need to.

Vermont is not an outlier. It is a preview.

No one in that room disputed the numbers. No one argued that the trend would reverse itself. The question that followed — the question this book is an attempt to answer — was whether structural change was possible. Not aspirational change. Not incremental improvement. But actual change to the fundamental incentives, relationships, and capabilities of the system. Whether a small rural state could force a healthcare transformation that the national system, with all its resources and political energy, had been deferring for thirty years.

Vermont's answer, enacted in Acts 167, 51, and 68 and funded by the $195 million Rural Health Transformation award, is the subject of this book. Whether it constitutes successful transformation will be determined over the next five to ten years. What is already clear is that Vermont has committed to structural change with more specificity, more statutory force, and more capital investment than any comparably sized system in recent American health policy history.

This book is written in real time, from inside that transformation. Not as a retrospective account of a completed success, but as an analytical framework for a transformation in progress — with all the uncertainty, difficulty, and instructiveness that implies.

What Transformation Actually Means

The word "transformation" is used so frequently in healthcare that it has nearly lost its meaning. Press releases describe routine process improvements as transformational. Strategic plans invoke transformation as aspiration rather than commitment.

This book uses the word in its precise sense: transformation is structural change that alters the fundamental incentives, relationships, and capabilities of a system, producing different behaviors and different outcomes rather than incremental improvements within the existing structure.

By this definition, most of what has been called healthcare transformation over the past fifteen years has not been transformational. Voluntary accountable care organization programs that high-cost providers can decline to join are not transformational. Shared savings models that reward hospitals for reductions in utilization they would have achieved anyway are not transformation. Electronic health record adoption that digitizes paper processes without changing care delivery is not transformation. Quality measurement initiatives that produce reports without affecting payment are not transformation.

Transformation, in the sense this book uses the term, looks like what Vermont is attempting: mandatory reference-based pricing that eliminates the ability of dominant hospitals to charge commercial payers three to four times Medicare rates; statutory global hospital budgets that change every hospital's financial incentive from maximizing volume to managing population health; a Statewide Health Care Delivery Strategic Plan that commits Vermont's Agency of Human Services to specific, measurable outcomes by specific dates; and capital investment through the Rural Health Transformation Program that builds the community infrastructure that payment reform requires to produce population health improvement rather than just cost reduction.

Vermont Is Not an Outlier. It Is a Preview.

Readers from outside Vermont sometimes ask whether Vermont's experience translates to their state, their system, their community. Vermont is small. Its politics are unusual. Its hospital sector — fourteen nonprofit community hospitals, no investor-owned chains, a regulator with real rate-setting authority — is not typical.

The answer is not found in Vermont's specific characteristics. It is found in the forces driving Vermont's crisis. The demographic aging that is collapsing Vermont's commercial cross-subsidy is present in every Northeastern state now, and will arrive in the rural Midwest, the Mountain West, and the South on a ten-to-twenty-year lag. The failure of voluntary payment reform to reach the tipping point at which fee-for-service becomes the minority mode of payment is a national finding. The administrative complexity exhausting physicians and consuming resources that should go to patients is a national emergency. The pricing opacity that allows dominant hospitals to charge two, three, and four times Medicare rates while rural neighbors close — that is national too.

Vermont faces these forces earlier and more acutely because of its demographics and economic structure. But the same forces are building in every state. The question Vermont is answering — whether mandatory structural reform can actually work — is the question American healthcare will have to answer nationally within a generation.

How to Read This Book If You Don't Work in Vermont

This book uses Vermont as its primary teaching case for the reasons explained above: transparency, documentation, and an early position on a curve every state is on. But the subject of this book is the six-pillar framework — Policy, Technology, Economics, Clinical, Equity, and Operations — and the dependency logic that links them. Vermont is the evidence, not the scope. If you do not work in Vermont, here is how to get the most out of the chapters that follow.

Translate the institutions, not just the policies. Every named Vermont statute or institution in this book is a specific instance of a general category that exists, in some form, in every state. The Green Mountain Care Board is Vermont's version of a hospital rate-setting or cost-growth-benchmark authority — Massachusetts, Rhode Island, and several other states have analogous bodies under different names. Acts 167 and 68 are Vermont's versions of a diagnostic mandate and an operational mandate — the categories, not the bill numbers, are what transfer. AHS is Vermont's version of a state health and human services agency acting as a system operator. The AHEAD State Agreement is Vermont's version of a federal model agreement with

CMMI. As you read, ask: what is the equivalent institution, statute, or agreement in my state — even if it is smaller, newer, less formal, or does not yet exist?

Look for the framework statements, not only the Vermont data. Each chapter makes a claim about how the pillars depend on one another that does not require Vermont to be true — for example, that technology investment must precede payment reform because payment reform without data infrastructure cannot be measured or trusted. Vermont's experience is the proof, presented in detail because detail is what makes an argument verifiable rather than asserted. But the claim itself is the point. Where a chapter's argument seems to depend on a fact that is true only in Vermont, that is worth flagging — it likely means the claim needs to be either generalized or explicitly scoped, and feedback on those instances is genuinely useful for future editions.

Use the "Implications for You" sections as your entry point. Most chapters close by addressing different reader roles directly — hospital executives, payers, state officials, clinicians. These sections are written to apply regardless of which state you work in, and are often the fastest way to find the actionable content inside a Vermont-heavy chapter. If you read nothing else in a chapter, read that section, then go back for the supporting detail you need.

In short: treat Vermont the way a medical textbook treats a case study. The case is specific so that the lesson does not have to be. Vermont is not the audience for this book. Vermont is the evidence.

The Strongest Case Against This Book's Argument

This book argues that structural, mandatory reform can succeed where voluntary reform has failed. That is a contested claim, and intellectual honesty requires engaging the strongest version of the opposition before asking the reader to proceed on that premise.

The Price Regulation Argument

Economists and hospital advocates argue that mandatory reference-based pricing — tying hospital payment to a percentage of Medicare rates — will reduce access rather than improve affordability. If hospitals cannot cover their costs at regulated prices, they will close service lines, exit markets, or reduce Medicaid volumes. Vermont, by this reading, is not pioneering a solution. It is forcing a reckoning that will harm the rural communities it claims to protect.

This is a serious argument. Maryland's experience with hospital global budgets is the primary counterevidence: Maryland has operated all-payer rate regulation since 1977 and maintains rural hospital access comparable to or better than non-regulated states, while holding per-capita cost growth below the national average. But Maryland's model is decades old, built during a different regulatory and demographic environment. Vermont's experiment is newer, its hospitals more financially fragile, and its rural population more dispersed. The access-reduction risk is real and this book does not dismiss it. Chapter 1 (Political Sustainability) analyzes it directly as the primary failure mode to monitor.

The Global Budget Argument

Critics of hospital global budgets argue that fixed revenue envelopes create perverse incentives: hospitals that reduce utilization under a global budget model may shift costs rather than eliminate them, delay necessary care to manage within budget, or game quality metrics rather than improve quality. The literature on budget caps and quality outcomes is genuinely mixed.

This book's response is not that global budgets are without risk. It is that the alternative — fee-for-service payment that makes population health management financially irrational — has a documented record of failure that is longer and more extensive than the record of global budget problems. The choice is not between a perfect payment model and a risky one. It is between a payment model that systematically rewards volume and one that, with adequate safeguards, can reward outcomes. Chapter 1 (Economics) develops the specific safeguards Vermont's model requires.

The AHEAD Skepticism Argument

CMMI has operated innovation models for fifteen years. Most have not produced the savings they projected. AHEAD's predecessor — the Vermont All-Payer ACO Model — produced modest results over nine years despite significant federal and state investment. Why will AHEAD be different?

This is perhaps the strongest challenge in the book. The answer requires acknowledging the critique honestly: CMMI models have generally underperformed their projections, and there is no guarantee AHEAD will be different. What is different about Vermont's approach is the mandatory statutory architecture that AHEAD did not have in its predecessor: Acts 167 and 68 require hospitals to participate in global budgets and reference-based pricing regardless of whether AHEAD succeeds. Vermont's transformation does not depend on AHEAD — AHEAD amplifies it. If AHEAD underperforms, Vermont's mandatory state framework remains. Chapter 3 (The Future) returns to this directly.

The Oliver Wyman Independence Question

This book relies heavily on Oliver Wyman's Act 167 analysis. Oliver Wyman was paid $1 million by the Vermont state government to produce a report that would support the case for reform. That is not independence. Readers should note that the Oliver Wyman financial projections — hospital deficit trajectories, administrative cost benchmarks, workforce shortage estimates — cannot all be independently verified from public sources, and the firm had a client relationship that may have shaped how it framed its conclusions.

This book treats Oliver Wyman's quantitative findings as estimates, not certainties, and notes throughout where specific figures cannot be independently confirmed. The structural argument — that Vermont's system faces a genuine financial crisis requiring structural response — is supported by public GMCB data, independent KFF analysis, and GMCB's own enforcement actions that do not depend on Oliver Wyman's projections.

The Vermont-Specificity Argument

The most common version of this objection does not concern any single policy. It is a reaction to the book as a whole: this is a book about one small, unusually homogeneous, unusually transparent state with an unusual regulatory body — does any of it actually apply to a large, diverse, politically fragmented state with a dozen competing health systems and no Green Mountain Care Board?

This objection deserves to be taken seriously rather than answered with a slogan, because the honest answer has two parts, and the second part is a real limitation of this book rather than a rebuttal.

The first part is that the six-pillar framework itself — the dependency logic that determines sequence, the argument that payment reform without clinical redesign evaporates, that technology investment without operational readiness produces unused platforms, that equity gaps require targeted rather than averaged interventions — was not derived from anything unique to Vermont. It is derived from how health systems, payment, technology, and care delivery relate to one another as a matter of structure. A large, fragmented state with a dozen competing systems does not escape this logic; it experiences it in a more fragmented way, with more actors who can defect, more negotiation required to establish anything "mandatory," and a longer, messier sequence. The logic is the same. The execution is harder.

The second part is the real limitation: this book cannot, in one volume, also provide the equivalent depth of documentary detail for every state's version of each institution. Vermont's GMCB, AHS, and AHEAD agreement are described in the detail they are because that detail exists publicly and because Vermont is far enough along its sequence that the consequences of each decision are now visible. A reader in a state without an analog to the GMCB will not find a chapter-length treatment of their own state's rate-setting politics here. What they will find is a description, in Vermont's case, of what that function does, why it has to exist in some form for the rest of the sequence to work, and what tends to happen when it does not exist or lacks enforcement authority — which is itself the diagnostic question worth asking about their own state.

Put plainly: a reader who finishes this book having learned a great deal about Vermont but nothing transferable about sequencing, payment design, technology architecture, equity accountability, or operational execution has found a real gap between this book's intent and its execution — not a correct reading of a book that was only ever about Vermont. The "How to

Read This Book If You Don't Work in Vermont" section earlier in this Introduction, and the "Implications for You" sections that close most chapters, exist specifically to close that gap, and later editions of this book will continue to expand the explicit bridges between Vermont's case and other states' starting points.

The Transition-Window Argument

Reference-based pricing and global hospital budgets shift a hospital from a system where it can grow revenue by growing volume to one where revenue is fixed or capped. For hospitals already operating below break-even — which, per Oliver Wyman's own numbers, describes most of Vermont's system today — the most dangerous financial period may not be the destination (FY2030 statewide global budgets) but the transition itself: the years in which RBP is phasing in, global budgets are not yet operational statewide, and the underlying losses documented in Chapter 6 have not yet been addressed by anything. This book documents both the destination and the starting crisis in detail. It is more cautious about the specific financial bridge — the combination of Rural Health Transformation Program funds, AHEAD EAST Fund payments, and the sequencing of Medicaid versus commercial global budgets — that determines whether a given hospital remains solvent in fiscal years 2027 and 2028 specifically. A hospital CFO's most pressing question is rarely "what does the system look like in 2030?" It is "what keeps us open in 2027?" Appendix H addresses this transition window directly.

The Enforcement Question

This book repeatedly emphasizes that Vermont's approach is mandatory rather than voluntary, and treats this as the central correction to OneCare's design flaw. But a mandatory regime is only as real as its enforcement, and a skeptical reader is entitled to ask: what actually happens to a hospital that does not comply — that refuses a global budget, or continues billing above the reference-based-pricing ceiling? Act 68 gives GMCB subpoena authority and data-sharing power with the Department of Financial Regulation, and the answer is no longer hypothetical. GMCB's FY23 enforcement actions against UVMMC ($80.3M overage) and RRMC ($11.1M overage) were the first such enforcement actions in Vermont's regulatory history, and UVMMC's subsequent court challenge to that enforcement — Vermont's largest, most powerful hospital system testing GMCB's authority directly — was decided against UVMMC. That outcome is the strongest available evidence that "mandatory" in this book's framework describes a regime that has been tested against its most resourced potential opponent and held. Appendix H develops the enforcement mechanics and this case in more detail.

The AHEAD Durability Argument

AHEAD is an eleven-year CMMI model running from 2024 through December 2035, under a single innovation-center framework that has already been modified once (the January 2026 changes extending the end date and adding new accountability requirements) and will span at least one more presidential transition before Vermont's Cohort 2 implementation even begins in

January 2028. CMMI's own track record — acknowledged elsewhere in this book — includes models redesigned or discontinued mid-course. The book's response to AHEAD skepticism is that Vermont's mandatory state framework under Acts 167 and 68 remains in force even if AHEAD underperforms or is altered. That is true for the commercial and Medicaid components of Vermont's reform, which Act 68 governs directly. It is not fully true for the Medicare component specifically: Medicare global budgets exist in Vermont's framework only because AHEAD provides them, and no state statute can substitute for a federal payment model on Medicare claims. A reader assessing AHEAD durability should therefore distinguish between "Vermont's reform survives if AHEAD changes" (true, for the commercial and Medicaid pillars) and "Vermont's Medicare global budget survives if AHEAD changes" (not guaranteed, and not within any state's control).

These are not the only critiques of Vermont's approach. They are the most serious ones, and this book engages them throughout rather than assuming the policy case is settled. It is not settled. Vermont is the experiment. This book is the analytical framework for watching it clearly.

The Six-Pillar Framework: Why All Six Must Move Together

The central analytical contribution of this book is the six-pillar framework for healthcare transformation: Policy, Technology, Economics, Clinical, Equity, and Operations. The framework is not a checklist of things that should be done. It is an integrated analytical architecture for understanding why transformation attempts fail when they address any pillar in isolation.

The most common failure mode in healthcare transformation is single-pillar thinking. A payer that focuses exclusively on the Economics pillar — redesigning payment models — while leaving the Technology, Clinical, and Operations pillars unchanged produces payment reform that providers cannot operationalize. Hospitals without population health analytics infrastructure cannot manage global budgets. Clinicians without team-based care models cannot deliver the primary care access that makes reduced hospitalization possible. Administrators without administrative simplification cannot absorb the management complexity of value-based contracts on top of fee-for-service billing requirements.

The six pillars interact through a logic that Chapter 1 develops in detail. The short version: payment reform (Economics) changes what behaviors the financial system rewards; clinical redesign (Clinical) changes how care is actually delivered; technology investment (Technology) provides the data infrastructure that makes new care models manageable at scale; policy change (Policy) creates the mandatory architecture that ensures voluntary actors cannot opt out of structural reform; equity investment (Equity) ensures that transformation benefits reach the populations that need them most; and operational execution (Operations) converts strategy into implemented programs, trained staff, and measured outcomes.

Remove any one pillar and the transformation fails. Vermont's reform agenda addresses all six simultaneously — imperfectly and incompletely, as any real-world policy effort does, but with a comprehensiveness that distinguishes Vermont's approach from the single-pillar interventions that have failed repeatedly at the national level.

How to Use This Book

This book has four primary reader profiles. Each has a different relationship to the material and a different way of getting maximum value from it.

The Healthcare Policy Professional

You work in a government agency, a legislative office, a think tank, or a foundation focused on healthcare policy. Recommended path: Read the Introduction and Chapter 1 (six-pillar framework) in full, then Chapter 1 (The Execution Sequence). For the pillar you care most about, read the corresponding chapter in depth. Read Chapter 1 (AHS Restructuring) carefully — it is the most direct bridge between policy mandate and organizational design. Read Chapter 3 (The Future) for the national context.

The Healthcare Executive or Administrator

You lead a hospital, health system, health plan, ACO, or healthcare services organization. You are making decisions — about contracts, capital investments, workforce, technology — that the transformation environment is complicating. Recommended path: Read the Introduction for the overarching argument, then go directly to the chapters most relevant to your immediate decisions. Chapter 3 (Economics) for VBC contracts. Chapter 3 (Clinical) for care management programs. Chapters 6-7 (Technology) for infrastructure investment. Chapters 14-15 (Operations) for hospital transformation planning. Return to Chapters 1-2 for the integrating logic.

The Vermont Practitioner

You work for AHS, GMCB, DVHA, the Vermont legislature, a Vermont hospital, a Blueprint practice, or a Vermont community organization. Recommended path: Read Appendix A (Vermont System Portrait) first, then Chapter 1 (The Execution Sequence) and Chapter 1 (Policy — Acts 167 and 68). Then Chapter 1 (AHS Restructuring) as the operational framework for your work. Use the implementation timeline tables throughout as a working reference guide.

The Healthcare Student or Researcher

You are building foundational knowledge in a graduate program or as an independent researcher. Recommended path: Read the entire book in order. Chapter 1 establishes the framework; Chapters 4-15 develop each pillar; Chapters 16-20 provide future context and the AHS restructuring roadmap. The Key Concepts sections at the end of each chapter are the glossary. The source notes are the bibliography. Use Vermont as the case study while building transferable analytical skills.

What This Book Cannot Do

This book cannot predict whether Vermont's transformation will succeed. The outcome depends on thousands of decisions that will be made by hospital executives, primary care physicians, community health workers, legislators, regulators, and patients over the next five to ten years. This book provides the analytical framework for those decisions; it cannot make them.

This book cannot substitute for the operational expertise required to execute transformation. The frameworks here are accurate and the evidence base is solid, but implementing a global budget requires actuarial expertise, regulatory knowledge, and operational experience that a book cannot provide.

This book cannot resolve the political contests that surround healthcare transformation. Reference-based pricing is opposed by hospital systems that benefit from high prices. Global budgets are feared by hospitals that worry about access restrictions. These are legitimate conflicts of interest that will be resolved by political processes — legislative deliberation, regulatory proceedings, stakeholder negotiations — that this book can inform but cannot replace.

What this book can do: provide the analytical architecture for understanding what is happening and why; supply the evidence base for evaluating what is working and what is not; develop the conceptual vocabulary for communicating across the disciplines — clinical, financial, policy, technological, operational — that transformation requires; and give Vermont practitioners the framework to see their specific work in the context of a coherent transformation agenda. That is enough to be worth writing, and worth reading.

Sources: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024); Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application (November 2025); GMCB VHCURES Data and Analytics documentation; KFF State Health Facts (Vermont); Vermont Department of Health; Act 68 of 2025; Act 167 of 2022; AHEAD State Agreement (January 2025).

Chapter 1: The Six-Pillar Framework and the Execution Sequence

> *Healthcare transformation is a system problem: six pillars — Policy, Technology, Economics, Clinical, Equity, and Operations — connected by fifteen dependency relationships that determine whether reform succeeds or fails. This chapter defines the six pillars, maps their interdependencies, shows what cascades when any one is missing, and then turns to the question those dependencies force: in what order must the pillars be built? Vermont provides both the proof of concept and the canonical failure case — OneCare Vermont's collapse is the most instructive sequencing autopsy in recent American health policy.*

1.1 The Problem with Single-Pillar Thinking

In 2010, the Affordable Care Act created the Center for Medicare and Medicaid Innovation with a mandate to test new payment and delivery models and scale what worked. Over the following fifteen years, CMMI launched more than fifty models, investing billions of dollars and the attention of thousands of healthcare organizations. The results were modest. A 2023 evaluation found that the vast majority of CMMI models had not achieved statistically significant reductions in spending or improvements in quality. Most were discontinued. A handful showed promise and were expanded.

50+	<10%	15 Yrs	6
CMMI Models Launched	Models With Significant Results	Span of Experience	Pillars Required

The pattern of failure across CMMI models is instructive. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar — redesigning care coordination or care management programs — produced clinical improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment — electronic health records, population health platforms, data exchange — found that providers lacked the operational capacity to use the tools effectively.

The lesson is not that payment reform, clinical redesign, or technology investment is wrong. It is that each intervention, pursued in isolation, runs into the constraints created by the pillars it ignores. This is the core analytical argument of the six-pillar framework: healthcare transformation is a system problem, and system problems require system solutions.

Every major healthcare transformation framework in the past thirty years has identified the same list of necessary conditions. Pay differently. Redesign care delivery. Build data infrastructure. Address social determinants. Ensure equity. Execute operationally. The list is right. The reasoning behind it is right. And yet the transformation that the list is supposed to produce has not arrived — not in most states, not at national scale, not with the speed and completeness the evidence has long indicated is possible.

The problem is not the list. The problem is treating it as a list.

A checklist is a set of independent items. You can check them in any order. Checking one does not affect the others. Failing to check one does not prevent you from checking the rest. A checklist of healthcare transformation requirements implies that you can make progress on any one of these independently, and that partial completion is partial progress. This is wrong. The six pillars are not independent items on a checklist. They are a system of interdependent components in which each pillar's effectiveness depends on the others, in which the failure of any single pillar cascades through the system, and in which the order and timing of interventions matters as much as the content of those interventions. Treating the six pillars as a checklist is precisely how decades of technically correct healthcare reform recommendations have produced inadequate results.

"A policy intervention that passes question one but fails questions three, four, five, or six is not a solution — it is a well-intentioned mistake waiting to be made."

— HTR Framework Documentation

1.2 The Six Pillars Defined

The six pillars are not equal in their causal priority — enabling legislation (Policy) and payment reform (Economics) are preconditions that the other pillars depend on — but they are equal in their necessity. A transformed healthcare system requires all six to be functional.

Policy — The legislative and regulatory environment that creates the mandatory architecture for transformation. Policy establishes what actors must do, what they are prohibited from doing, and what governance structures ensure accountability. Voluntary reform operates within the Policy pillar's constraints; structural transformation requires it to change. *Vermont application: Acts 167 and 68, the AHEAD Model State Agreement, and the Statewide Strategic Plan mandate are Vermont's Policy pillar interventions. Each converts voluntary reform into statutory obligation.*

Technology — The data infrastructure, health information exchange, analytical platforms, and clinical technologies that make population health management possible at scale. The Technology pillar is the information substrate on which all other pillar functions depend — you cannot manage what you cannot measure. *Vermont application: VHCURES (all-payer claims* *database), VITL/VHIE (health information exchange), the AHS-GMCB analytics vendor, AI scribe grants, and Vermont's Clinically Integrated Network are Technology pillar investments.*

Economics — The payment and financial architecture that determines what behaviors the system rewards. The Economics pillar encompasses payment reform (from fee-for-service to value-based and global payment), financial modeling, total cost of care management, and the hospital financial sustainability that transformation requires. *Vermont application: Reference-based pricing (FY2027), global hospital budgets (FY2028–2030), and the AHEAD Model's total cost of care accountability are Vermont's Economics pillar interventions.*

Clinical — The care delivery models, quality measurement frameworks, clinical workforce design, and patient care programs that determine how care is actually delivered. The Clinical pillar is where payment reform becomes patient outcomes — or fails to. *Vermont application: The Blueprint for Health (all-payer PCMH program), Collaborative Care Model behavioral health integration, CCBHC expansion, and the primary care investment strategy are Vermont's Clinical pillar interventions.*

Equity — The programs, investments, and accountability structures that ensure transformation benefits reach all populations — and specifically those most harmed by the current system's failures. Equity is not a soft afterthought; it is an analytical requirement. A transformation that reduces average costs while widening disparities has failed. *Vermont application: Geographic equity across Vermont's 14 HSAs, SDOH investment through the EAST Fund and Blueprint HRSN screening, rural access preservation in the RBP and global budget design, and health equity metrics in Act 167/68 reporting are Vermont's Equity pillar interventions.*

Operations — The organizational capacity, management infrastructure, administrative simplification, and implementation discipline that converts strategy into executed programs. The Operations pillar is where transformation plans become real or remain documents — the graveyard of well-designed reforms that could not be implemented. *Vermont application: The 14-hospital transformation planning process, RHRC technical assistance, the AHS HSA Coordinator model, the Division of Planning and Effectiveness, and EMS regionalization are Vermont's Operations pillar investments.*

The table below characterizes each pillar's diagnostic function and structural role in the system — not as a repetition of the definitions above, but as a specification of what each pillar produces when functioning and what question it answers in the transformation logic.

Pillar	Diagnostic question	Structural role in the system	What it produces when functioning
Policy	Is it permissible?	The mandatory architecture. Converts aspirational reform into binding requirements. Without mandatory authority, the highest-cost actors opt out and voluntary reform achieves marginal results.	Statutory mandates that eliminate voluntary opt-out; accountability timelines that cannot be deferred; federal-state agreements that commit both parties to specific outcomes.
Technology	Is it possible?	The data substrate. Makes population health management, VBC contract execution, equity measurement, and strategic planning analytically feasible. Without data infrastructure, the other pillars are managing blind.	Attribution lists, TCOC measurement, HEDIS stratification, risk stratification, care-gap identification, AI-assisted clinical workflows — the analytical functions that VBC and population health management require.
Economics	Is it sustainable?	The incentive architecture. Determines whether organizations have a financial reason to behave differently. Payment reform does not change clinical behavior directly — it changes the financial logic that shapes clinical decisions over time.	Payment models in which population health management is financially rational; price structures that break the commercial cross-subsidy chain; accountability for total cost of care rather than encounter volume.
Clinical	Is it effective?	The mechanism of change. Payment reform changes incentives; clinical redesign changes behavior. The financial case for preventive investment only materializes when clinical programs actually deliver the utilization reduction that produces savings.	Primary care transformation that prevents hospitalizations; behavioral health integration that reduces crisis presentations; care management that closes gaps before they become acute episodes.
Equity	Is it just?	The cross-cutting constraint and accountability lens. Not a separate program — a dimension applied to every decision in every other pillar. Transformation that improves averages while widening disparities has failed by the framework's own standards.	Stratified measurement that reveals disparities hidden in aggregates; payment designs that do not penalize providers serving high-SDOH populations; clinical programs that reach disadvantaged populations; technology infrastructure that produces disaggregated data.

Pillar	Diagnostic question	Structural role in the system	What it produces when functioning
Operations	Is it executable?	The execution layer. Translates statutory mandates, payment models, data platforms, and clinical programs into organizational reality. The most analytically sound framework fails if the implementing organization lacks capacity, infrastructure, and management discipline.	Hospital transformation plans that are implemented, not filed; AHS organizational capacity sufficient to manage 14 simultaneous transformation processes; project-management infrastructure that tracks progress against statutory deadlines.

Figure 1.A — The six pillars: diagnostic questions and structural roles. Source: six-pillar framework documentation; Acts 167 and 68 of 2025.

1.3 The Architecture of Interdependency: The Fifteen Dependency Relationships

The six pillars do not operate independently. They interact through a set of dependencies and feedback loops that determine whether transformation produces the outcomes it promises. Understanding these interactions is the core analytical skill this book develops.

The foundational dependency is between Policy and Economics. Payment reform (Economics) requires policy authority — either statutory mandate or regulatory power — to be effective at scale. Vermont learned this lesson through a decade of voluntary payment reform efforts: the Vermont All-Payer ACO Model, OneCare's voluntary hospital participation, VITL's voluntary HIE connectivity. Each produced partial results because voluntary participation allowed the highest-cost actors to opt out. Act 68's mandatory RBP and global budget requirements are a Policy pillar response to the failure of Economics pillar interventions without Policy pillar backing.

The second critical dependency is between Economics and Clinical. Payment reform changes the financial incentive structure; clinical redesign changes the care delivery model to respond to the new incentives. A hospital operating under a global budget has a financial incentive to prevent hospitalizations — but it can only act on that incentive if it has the primary care network, care coordination infrastructure, and behavioral health integration to actually keep patients out of the hospital. Without the Clinical pillar investment, the Economics pillar incentive produces financial pressure without clinical improvement.

The third dependency is between Technology and all other pillars. You cannot attribute patients to hospitals for global budget purposes without accurate patient matching across providers. You cannot identify care gaps for quality improvement without population-level claims and clinical data. You cannot model the financial impact of service line closures without analytics infrastructure. You cannot monitor equity metrics without data stratified by race, income, and geography. Technology is the information substrate on which every other pillar's management functions depend.

The Equity pillar has a different kind of dependency: it is simultaneously a constraint on every other pillar and a beneficiary of all of them. Every payment reform intervention, clinical redesign, and technology investment must be evaluated for its equity implications — does it reduce disparities or exacerbate them? Does it reach the populations that need it most or concentrate benefits among those already well-served? Act 68's goals explicitly include reducing health inequities alongside reducing costs and improving outcomes, recognizing that transformation that achieves the first two without the third is incomplete.

The Operations pillar is the translation layer — the organizational capacity that converts policy intent, payment incentives, technology tools, clinical models, and equity goals into implemented programs with trained staff and measured outcomes. A transformation strategy that is analytically sound, financially compelling, technically feasible, and clinically validated will still fail if the organization responsible for executing it lacks the project-management infrastructure, the change-management capability, and the sustained leadership attention that execution requires.

These five relationships are the framework's structural core. What follows develops the complete architecture: all fifteen directed dependency relationships between and among the six pillars, the specific mechanisms by which each operates, the failure modes that emerge when dependencies are ignored, and the Vermont evidence that confirms the structure with empirical specificity. Figure 1.B renders all fifteen relationships as a single matrix; the six-pillar framework map — available as an interactive visualization on Health Transformation Review — renders them as a navigable diagram.

From ↓ / To →	Policy	Technology	Economics	Clinical	Equity	Operations
Policy	—	—	Enables mandatory authority	—	Enables accountability mandates	Requires statutory deadlines
Technology	—	—	Enables VBC financial management	Enables risk stratification	Enables demographic stratification	—
Economics	—	Requires data infrastructure	—	Drives financial rationality	Requires social risk adjustment	—

From ↓ / To →	Policy	Technology	Economics	Clinical	Equity	Operations
Clinical	—	—	—	—	Enables access expansion	Requires execution infrastructure
Equity	Constrains with justice reviews	—	—	Constrains with cultural competency	—	—
Operations	Enables data feedback loops	Requires workforce to run infrastructure	—	—	—	—

Figure 1.B — The architecture of interdependency: the fifteen dependency relationships, read From-row to To-column. Source: six-pillar framework documentation; Vermont Health Transformation / HTR (2026).

1.3.1 Policy Dependencies: Three Relationships That Establish the Mandatory Architecture

Policy is the pillar from which mandatory authority flows. Its three outbound dependencies — to Economics, Equity, and Operations — are among the most critical in the framework, because they are the relationships that convert every other pillar from optional to required.

Policy → Economics: Mandatory authority enables structural payment reform · *ENABLES (critical path)*

The most important dependency in the entire framework. Payment reform that is voluntary produces voluntary results. Vermont spent a decade under OneCare Vermont — a voluntary ACO model — and produced modest results among participating hospitals while non-participating hospitals continued operating under fee-for-service without consequence. Act 68 of 2025 changed the architecture: reference-based pricing is mandatory for all Vermont hospitals beginning FY2027, and global budgets are mandatory beginning FY2028. This is not an incremental improvement on the voluntary model. It is a structural change that eliminates the opt-out that made the voluntary model ineffective.

VERMONT EVIDENCE

Oliver Wyman's Act 167 diagnosis explicitly found that voluntary reform was insufficient given the financial trajectory. The $700 million to $2.4 billion five-year deficit projection assumes the continuation of the commercial pricing structure that mandatory RBP is designed to dismantle. Without the Policy pillar's mandatory authority, the Economics pillar remains aspirational.

Policy → Operations: Statutory deadlines force execution capacity · *REQUIRES (critical path)*

One of the most underappreciated dependencies in healthcare transformation. Organizations develop the capacity to execute complex initiatives when they have non-negotiable deadlines, not when they have good intentions. Act 68's December 2028 Statewide Strategic Plan deadline, the FY2027 RBP mandate, and the FY2028 global budget mandate are not just policy milestones — they are organizational forcing functions that require AHS to build the project-management infrastructure, analytics capability, and staffing depth that the transformation requires. Without the Policy pillar imposing external accountability, the Operations pillar faces the chronic problem of healthcare administration: transformation planning that is perpetually in progress but never complete. Vermont's statutory deadlines are the mechanism that prevents this. They are deliberately irreversible.

Policy → Equity: Statutory mandates embed equity accountability · *ENABLES*

Acts 167 and 68 both contain explicit equity requirements — they are not aspirational statements about desired outcomes but operational mandates: equity metrics must be tracked, disparities must be reported, and the Statewide Strategic Plan must address geographic and demographic equity gaps. This is the Policy pillar enabling the Equity pillar's accountability function. Without statutory embedding, equity monitoring is a discretionary activity that competes with operational priorities. With it, equity measurement is a compliance requirement.

> **VERMONT EVIDENCE**
> The Health Care Delivery Advisory Committee established under Act 68 is required to consult with the Health Equity Advisory Commission and to incorporate equity considerations into the Statewide Strategic Plan. This institutional design is the Policy pillar building the Equity pillar's accountability structure into the statutory framework.

1.3.2 Economics Dependencies: Three Relationships That Transmit Incentive Change

Economics is the pillar through which policy mandates are converted into behavioral change at the organizational and clinical level. Payment models do not change behavior directly — they change the financial logic that shapes decision-making. The Economics pillar's three outbound dependencies — to Clinical, Equity, and Technology — describe the mechanisms through which this conversion happens.

Economics → Clinical: Payment incentives drive care delivery redesign · *DRIVES (critical path)*

The most important economics dependency. Under fee-for-service, preventing a hospitalization costs the hospital revenue. A primary care practice that successfully manages a diabetic patient's condition and prevents a $15,000 hospitalization has generated no billable encounter for the prevented admission. The clinical logic and the financial logic point in opposite directions. Under global budgets, the calculus reverses: every avoidable hospitalization is a cost against a fixed revenue envelope, and every prevented admission is a margin improvement. The payment model makes population health management financially rational for the first time.

> **VERMONT EVIDENCE**
> Vermont's Blueprint for Health demonstrates this dependency in reverse. The Blueprint's 5.8:1 return on investment — $5.8 million in reduced medical expenditure per $1 million invested in primary care — is achievable because the Blueprint's all-payer PMPM payments to PCMHs create a financial structure that rewards the clinical investment. Sustain the Economics pillar through AHEAD's global budgets and the Clinical pillar's population health logic becomes fully incentive-aligned.

Economics → Technology: VBC contracts require data infrastructure · *REQUIRES*

Value-based care contracts are agreements to share financial accountability for a population's total cost of care. They cannot be executed without the data infrastructure to measure that total cost. Attribution lists, TCOC benchmarks, shared savings calculations, risk adjustment factors, quality measure reporting — all of these require the all-payer claims data, the population health analytics platform, and the FHIR-based data exchange that the Technology pillar provides. An organization that signs an APM contract without adequate data infrastructure is making a financial commitment it cannot monitor, manage, or dispute.

> **VERMONT EVIDENCE**
> Vermont's AHEAD global budgets for hospitals, beginning January 2028, require GMCB to set prospective annual budgets based on historical Medicare spending adjusted for population risk. This calculation requires accurate VHCURES data, validated risk adjustment methodology, and an analytics platform capable of producing hospital-specific TCOC projections. The AHS-GMCB analytics vendor procurement underway in 2025–2026 is the Technology pillar investment that makes the Economics pillar's AHEAD implementation analytically executable.

Economics → Equity: VBC design must include social risk adjustment · *REQUIRES*

Value-based care contracts that do not adjust for social risk systematically penalize providers serving high-SDOH populations. A hospital in Essex County, Vermont — serving a population with higher poverty rates, housing instability, food insecurity, and transportation barriers — faces higher care management costs and lower quality measure performance potential than a hospital in Burlington serving a more economically stable population. If their global budgets are set at the same percentage of Medicare rates without social risk adjustment, Essex County's hospital is being held to a standard that does not reflect its population's actual cost burden.

> **VERMONT EVIDENCE**

> Vermont's AHEAD global budget design must address this explicitly. The methodology for social risk adjustment in Vermont's hospital global budgets is an active design question as of early 2026. Getting it wrong — designing an Economics pillar instrument that ignores the Equity pillar's requirement — would produce a payment system that penalizes the hospitals serving the most vulnerable Vermonters.

1.3.3 Technology Dependencies: Three Relationships That Enable Analytical Management

Technology is the data substrate on which every other pillar's management functions depend. It has the broadest set of outbound dependencies in the framework — three enabling relationships to Economics, Clinical, and Equity that describe what becomes possible when data infrastructure is adequate, and what remains impossible when it is not.

Technology → Economics: Analytics makes VBC financial management possible · *ENABLES*

This dependency runs in both directions between Technology and Economics: the Economics pillar requires data infrastructure, and the Technology pillar enables the financial management capability that VBC contracts require. The distinction is between existence and effectiveness. An organization can enter an APM contract without adequate analytics — the contract still exists, the financial obligations are still real. But the organization cannot manage its financial performance under that contract, cannot identify where its costs are above benchmark, cannot target care management investment toward the highest-return opportunities, and cannot dispute errors in the payer's shared savings calculations.

> **VERMONT EVIDENCE**
>
> VHCURES population health analytics make the APM financial modeling described in Chapter 7 possible. The APM Shared Savings Calculator, the Hospital Financial Stress Test, and the VBC Readiness Assessment — all tools in the HTR Research Lab — are built on VHCURES data logic. Vermont hospitals that invest in VHCURES analytics capability before AHEAD launches in January 2028 will be able to manage their global budgets proactively rather than reactively.

Technology → Equity: Disaggregated data reveals disparities hidden in averages · *ENABLES (critical)*

The Equity pillar's entire analytical function depends on the Technology pillar's ability to produce demographic stratification. You cannot measure a disparity you cannot see. Vermont's 91% primary care access rate — four points above the national benchmark — is a genuine achievement. It also hides an 11-point gap between white Vermont adults and BIPOC Vermont adults. That gap is invisible in aggregate data. It becomes visible only when VHCURES data is stratified by race/ethnicity, and that stratification is only possible when the data contains adequate demographic information.

> **VERMONT EVIDENCE**
> Vermont's race/ethnicity data in VHCURES is incomplete — commercial claims frequently lack demographic fields. This is not a minor data quality issue; it is a Technology pillar gap that directly limits the Equity pillar's analytical capability. The Vermont Department of Health Health Equity Data Report (January 2025) documents this limitation explicitly. Closing the Technology gap is prerequisite to the Equity pillar's disparity measurement function.

Technology → Clinical: Data infrastructure enables population health management · *ENABLES*

The clinical programs developed later in this book — Blueprint PCMH, CoCM, CCBHC, CHT deployment — can all be designed and implemented without data infrastructure. What they cannot do without it is function as population health programs rather than individual encounter programs. Risk stratification identifies which patients are most likely to deteriorate; without it, care management deploys reactively, after hospitalization rather than before. Care-gap identification triggers outreach for overdue preventive services; without it, gaps close only when patients happen to schedule appointments. SDOH screening tools flag patients for CHT navigation; without the data platform to connect the flag to the navigation workflow, the flag is generated and ignored.

> **VERMONT EVIDENCE**
> Blueprint's clinical registry is the Technology pillar product that enables Blueprint's Clinical pillar outcomes. The 5.8:1 ROI documented for Blueprint is achievable precisely because Blueprint combines the Clinical pillar's care team model with the Technology pillar's population health analytics. The AI scribe grants in the RHT Program are the Technology pillar's contribution to the Clinical pillar's workforce capacity problem.

1.3.4 Clinical Dependencies: Two Relationships That Connect Care Models to System Requirements

Clinical is the pillar that converts payment incentives into actual health outcomes. Its two outbound dependencies — to Equity and Operations — describe the operational requirements of clinical programs and their equity implications.

Clinical → Operations: Care models require execution infrastructure · *REQUIRES*

The most consistently underestimated dependency in healthcare transformation. A care model is a diagram until it has workforce, credentialing, administrative infrastructure, and operational management. The Collaborative Care Model requires a behavioral health care manager employed by a primary care practice. The CCBHC model requires a certified entity with the organizational capacity to serve all patients regardless of insurance status. Blueprint PCMHs require care coordinators, health information exchange connectivity, and NCQA recognition processes. None of these are available without Operations pillar investment.

> **VERMONT EVIDENCE**
> Oliver Wyman's Act 167 analysis found that Vermont's administrative cost per adjusted discharge runs far above the national benchmark — UVMMC at $3,826 against a $1,427 benchmark. This is not merely an inefficiency problem — it is a signal that Vermont's Operations pillar infrastructure is absorbing cost without producing the execution capability that clinical transformation requires. The RHRC engagement with all 14 Vermont hospitals is specifically designed to build the Operations pillar capacity that Clinical pillar programs need.

Clinical → Equity: Care delivery expansion is the primary access gap mechanism · *ENABLES*

Geographic and demographic disparities in healthcare access are primarily clinical access problems — not enough primary care providers, not enough behavioral health capacity, not enough community health workers in underserved communities. The Equity pillar's access gap goals cannot be achieved without the Clinical pillar's care model expansion. Blueprint's PCMH expansion into Northeast Kingdom communities, the CCBHC network's geographic reach, and the CHT deployment in high-SDOH communities are Clinical pillar investments that directly serve Equity pillar objectives.

> **VERMONT EVIDENCE**
> Essex County's elevated uninsurance rate and limited primary care access are equity problems with a clinical solution — expanding Blueprint-participating practices in underserved HSAs, deploying telehealth through the RHT Program to reduce geographic access barriers, and certifying additional CCBHCs in Northeast Kingdom communities. Remove the Clinical pillar investment and the Equity pillar's access gap remains unmeasured but unaddressed.

1.3.5 Operations Dependencies: Two Feedback Relationships That Close the Loop

Operations is the pillar that executes all other pillars' intentions. Its two outbound dependencies run back to Technology and Policy — feedback relationships that make the system self-correcting rather than purely top-down.

Operations → Policy: Implementation data feeds back into policy design · *ENABLES*

This is the feedback loop that allows the policy architecture to be self-correcting. AHS's monthly transformation reports to the Vermont Legislature — required under Act 68 — document whether transformation is proceeding on the statutory timeline, where gaps have emerged, and what adjustments are required. The GMCB's RBP methodology for FY2027 is being refined based on hospital financial data from the operations layer. The December 2028 Statewide Strategic Plan will be drafted based on what the Operations pillar has learned from implementing the preceding three years of transformation. This is not passive reporting — it is active policy learning.

Operations → Technology: Workforce runs the data infrastructure · *REQUIRES*

The Technology pillar's infrastructure — VHCURES analytics, the CIN data platform, FHIR compliance, AI governance programs — does not operate itself. It requires IT staff to maintain systems, data analysts to generate the reports that clinical and financial management teams use, privacy officers to manage HIPAA compliance, and cybersecurity staff to protect the infrastructure from the ransomware threats that have disabled rural health systems nationally. The Operations pillar's workforce is the human substrate on which the Technology pillar's data infrastructure runs.

> **VERMONT EVIDENCE**
> Vermont's rural CAHs — Grace Cottage, Gifford Medical Center, North Country Hospital — have limited IT staff. This Operations pillar constraint directly limits their Technology pillar capability. The Vermont CIN's shared services model addresses this: a shared analytics and IT function within the CIN provides all 14 hospitals access to Technology pillar capability they cannot individually afford. The CIN is an Operations pillar solution to a Technology pillar gap.

1.3.6 Equity Constraints: Two Relationships That Apply the Justice Lens to the Whole System

Equity's two outbound dependencies are constraining rather than enabling — they impose requirements on Clinical and Policy that must be designed in from the beginning rather than retrofitted. Equity is not the last pillar in the sequence; it is the lens applied to every other pillar's decisions throughout the design and implementation process.

Equity → Policy: Equity accountability constrains every policy decision · *CONSTRAINS*

Every policy decision in Vermont's transformation — the RBP rate methodology, the global budget design, the COE designation framework, the Statewide Strategic Plan structure — must be evaluated for equity impact before it is finalized. This is not post-hoc equity review; it is an equity constraint embedded in the policy design process. A RBP methodology that sets rates uniformly without adjusting for the market conditions faced by hospitals in underserved communities is technically correct and practically inequitable.

Equity → Clinical: Equity constrains how care is delivered, not just to whom · *CONSTRAINS*

Implicit bias in clinical settings, cultural incompetence, language access barriers, and differential treatment quality by race/ethnicity are clinical practice problems with equity implications. They are not solved by expanding access — expanding access to biased care widens some disparities while narrowing others. The Equity pillar constrains the Clinical pillar's design by requiring that care models address cultural competence, language access, and practice variation as design requirements, not implementation afterthoughts.

1.4 The Failure Cascade: What Happens When a Pillar Is Missing

6	15	6	1
Pillars, All Required	Dependency Relationships	Failure Cascade Patterns	Pillar Missing = System Fails

The most rigorous test of a dependency framework is not whether it describes how things work when all components are present — it is whether it accurately predicts what fails when a component is removed. The fifteen dependencies described above generate specific, testable failure-cascade predictions. The following analysis applies this test to each pillar.

Pillar removed	Immediate failure	Second-order failure	System-level outcome	Historical example
Policy	Payment reform remains voluntary; highest-cost actors opt out; rate benchmarks have no enforcement.	Clinical programs continue without the financial incentive alignment that makes population health investment rational; technology is built but not used for financial management.	Incremental improvement among willing participants; no system-level change; ongoing premium inflation; hospital financial crisis continues.	Vermont 2010–2022: OneCare voluntary ACO. Modest results; 9/14 hospitals in losses by 2023.
Technology	VBC contracts signed but not managed; attribution errors undetected; TCOC measurement unreliable; quality measures cannot be stratified.	Economics contracts produce unexpected losses; Equity cannot measure disparities it cannot see; Clinical cannot identify high-risk patients before they hospitalize.	VBC produces losses for providers who cannot manage it; organizations exit VBC; payment reform stalls; equity gaps widen invisibly.	Most early-CMMI ACO failures: organizations entered models without analytics and produced unexpected losses.
Economics	Clinical programs continue under fee-for-service incentives; hospitals remain rewarded for volume, not value.	Primary care investment produces outcomes but cannot capture the financial return — the 5.8:1 ROI accrues to payers, not providers.	Clinical quality improves at the margin; costs keep rising; the financial case for transformation cannot be made; fragility deepens.	U.S. national experience: 20+ years of quality improvement without payment reform. Quality rose; cost growth continued.
Clinical	Payment reform changes incentives but has nothing to incentivize; global budgets create pressure	Operations manages financial pressure through service cuts rather than care redesign; Equity access	Hospital financial crisis accelerates under	Maryland HSCRC early period: global budgets without adequate primary care produced pressure

Pillar removed	Immediate failure	Second-order failure	System-level outcome	Historical example
	without the programs that reduce cost.	gaps widen as capacity is cut to meet budgets.	global-budget pressure; rural hospitals close.	without sustainable improvement.
Equity	Transformation proceeds but disparities widen; average outcomes improve while geographic and demographic gaps grow.	Technically optimal policy produces inequitable distributions; technology is built but not used for stratified measurement; clinical programs improve averages without closing gaps.	Transformation that increases efficiency while reducing equity — technically successful, ethically failed.	National VBC experience: most APMs improved averages while widening disparities; safety-net providers penalized.
Operations	Statutory mandates exist on paper; clinical programs are designed; data platforms are procured; nothing is implemented.	Policy accountability fails without capacity to meet deadlines; platforms deployed but unused for management; contracts signed but not managed.	Transformation documents accumulate; deadlines slip; the 2028 Strategic Plan is descriptive rather than committal; the deficit trajectory continues.	Most state reform efforts of the 2010s: correct policy, adequate financing, sophisticated data — capacity insufficient to execute.

Figure 1.C — Failure-cascade analysis: what breaks when each pillar is absent. Source: HTR analysis; CMMI evaluation literature; Oliver Wyman Act 167 Report; Vermont experience.

1.5 Using the Dependency Map as an Analytical Tool

The dependency map is not merely a theoretical framework — it is a practical analytical tool for three management functions: transformation sequencing, risk identification, and investment prioritization.

Transformation sequencing — what must come first. The dependency structure generates a partial ordering of interventions. Some investments cannot produce their expected results until enabling investments are in place. Getting the sequence wrong is not merely inefficient — it produces failed interventions that generate political resistance to subsequent correct interventions. The correct sequencing logic: Policy before Economics (mandatory authority must precede payment reform); Technology before Economics in its analytics function (the infrastructure for VBC contract management should be in place before full-risk contracts are signed); Clinical capacity before Operations scale; and Equity throughout, not at the end — it is a constraint, not a final review.

Risk identification. The dependency map identifies Vermont's vulnerabilities with precision. The critical path runs through the Technology pillar's analytics capability: AHEAD global budgets require VHCURES analytics, equity measurement requires demographic stratification, and clinical risk stratification requires the population health platform. The AHS-GMCB analytics vendor procurement — underway in 2025–2026 but not yet complete — is the single highest-risk gap in Vermont's current architecture. A delay does not merely delay one program; it delays the financial management capability for AHEAD, the equity measurement capability for the Statewide Strategic Plan, and the risk stratification capability for Blueprint's CCBHC expansion. Three dependencies converge on this single investment.

Investment prioritization. Not all pillar investments have equal leverage. An investment in a pillar with many inbound dependencies has higher leverage than one required by only a single pillar. The Technology pillar has the highest inbound dependency count in Vermont's current architecture (Economics, Clinical, and Operations all require it); Operations is second (Clinical and Policy require it). The highest-leverage investments in Vermont's current phase are therefore analytics vendor deployment, AHS organizational capacity building, and HCC gap closure at AHEAD-participating hospitals.

1.6 The Vermont Thread: Six Pillars in One System

Vermont's healthcare transformation is the closest thing to a controlled experiment in simultaneous six-pillar intervention that American health policy has produced. The following table maps Vermont's primary reform vehicles onto the six-pillar framework, establishing the connective tissue that runs through every subsequent chapter.

Pillar	Primary Vermont intervention	Secondary interventions	Developed in
Policy	Acts 167 (2022), 51 (2023), 68 (2025): the legislative reform cascade; AHEAD Model State Agreement (January 2025)	Act 55 (drug pricing); Act 49 (insurer solvency); Act 62 (HIE governance); federal RHT Program award	Chapters 2–3
Technology	VHCURES all-payer claims database; Vermont CIN (RHT Program); AHS-GMCB analytics vendor; AI scribe and RPM grants	VITL/VHIE HIE; DVHA HIT Plan (Act 62); statewide EHR feasibility assessment; FHIR compliance investments	Chapters 4–5
Economics	Reference-based pricing (mandatory, FY2027); global hospital budgets (FY2028–2030); AHEAD total cost of care accountability	Act 68 hospital spending reduction (2.5%, FY2026); GMCB FY27 budget guidance (−1% commercial rate); Act 55 drug cap ($135M savings)	Chapters 6–7
Clinical	Blueprint for Health (all-payer PCMH + CHT); CoCM and MHI behavioral health	Hub-and-Spoke SUD model; PACE program development; COE	Chapters 8–9

Pillar	Primary Vermont intervention	Secondary interventions	Developed in
	integration; CCBHC expansion (5 entities by July 2026)	regionalization; dementia care infrastructure	
Equity	Geographic equity monitoring (HSA-level); SDOH integration through AHS HSA Coordinators; rural access preservation in RBP/global budget design	HRSN universal screening (Blueprint); CCBHC access for all populations; Northeast Kingdom focus; GLP-1 access advocacy	Chapter 10
Operations	14-hospital transformation planning (RHRC + AHS); Vermont CIN shared services; Act 68 transformation grants ($2M); PMO establishment	EMS regionalization; administrative simplification (CON, prior auth); AHS restructuring; Division of Planning and Effectiveness	Chapters 11, 16

Figure 1.D — Vermont's six-pillar transformation map. Sources: Acts 167, 51, 55, 62, 68 of 2022–2025; AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report.

This table is not a comprehensive inventory of Vermont's healthcare reform. It is a navigation map — a way of locating Vermont's specific interventions within the analytical framework that the subsequent chapters develop. The framework gives every Vermont policy intervention a structural home and every analytical concept a Vermont application.

1.7 From Framework to Sequence: Why Order Is Not Optional

The fifteen dependencies establish that the six pillars are interdependent. They also establish something sharper: that the pillars cannot be built in any order. A dependency is, by definition, a constraint on sequence — if Economics requires Technology, then Technology must come first. The rest of this chapter develops that argument, using Vermont's OneCare failure as the case that proves what happens when the sequence is violated.

1.8 The Core Argument: Dependency Logic Determines Sequence

Strip away the state names and the claim is this: any organization or system attempting to move all six pillars — Policy, Technology, Economics, Clinical, Equity, and Operations — will fail if it deploys them against their dependency order, and the failure will be structural rather than a matter of effort, funding, or political will. This is true whether the actor is a state government, a health system, or a payer, and regardless of which pillar feels most urgent at the time. Vermont is the case where both halves of this claim — the failure when the order is wrong, and the correction once the order is fixed — are fully documented and publicly visible, which is why it carries the argument in this chapter.

Vermont did not choose careful sequencing because it was disciplined. It was forced into it because it had run out of time to get it wrong.

When Oliver Wyman presented Vermont's hospital financial trajectory in September 2024 — nine of fourteen hospitals in operating losses, thirteen of fourteen projecting losses by 2028, a cumulative five-year system deficit between $700 million and $2.4 billion — the room understood something that policy documents rarely state directly: the cost of getting the sequencing wrong was now larger than the state's ability to recover. A decade of OneCare Vermont had demonstrated what the wrong sequence produces. Acts 167, 51, and 68 are the statutory attempt to build the right one.

The six pillars of transformation — Policy, Technology, Economics, Clinical, Equity, and Operations — are often presented as simultaneous imperatives. Each is necessary. Each depends on the others. Remove any one and the system fails. This is true. But it obscures the practical question every reform architect must answer before a dollar is spent: in what order do you actually build this?

The answer is not arbitrary, and it is not a matter of institutional preference or political opportunity. The execution sequence is determined by dependency logic — the structural reality that some things cannot produce value until other things are already in place. Getting the order wrong does not merely slow progress. It produces structural failures that are expensive to reverse, politically difficult to explain, and — as OneCare's experience shows — sometimes impossible to correct from within the organization that made the error.

The correct sequence is: Policy → Technology → Economics → Clinical → Equity → Operations. The critical and counterintuitive placement is Technology before Economics. Most reform architects reverse this, for understandable reasons: payment reform feels like the lever, technology feels like the support function. OneCare's failure is the proof that this instinct is wrong.

BEYOND VERMONT

The Policy → Technology → Economics → Clinical → Equity → Operations sequence does not depend on a state having an all-payer model, a Green Mountain Care Board, or a single dominant ACO. Substitute any state's reform vehicle for OneCare and the same diagnostic question applies: did payment reform get layered onto a data infrastructure that did not yet exist? If so, the same cascade — unmeasurable risk, unmanageable contracts, eventual collapse — is the predictable result, not a Vermont-specific accident.

1.9 The OneCare Failure: A Sequencing Autopsy

Before presenting the sequence, it is worth examining OneCare Vermont's failure in structural terms, because the failure was not random and it was not primarily the result of bad intentions or poor execution. OneCare failed because of sequencing errors baked into its design from the beginning. Identifying those errors precisely is more useful than simply noting that the ACO wound down after a decade of operation.

OneCare was Vermont's primary Accountable Care Organization vehicle under a Total Cost of Care model — holding OneCare financially responsible for the total healthcare spending of an attributed Vermont population. On paper, sound design. In practice, three sequencing failures that compounded into a cascade the organization could not survive.

1.9.1 Sequencing Failure 1: Economics Without Policy Foundation

The most fundamental problem: Vermont attempted to operate a sophisticated payment reform model — one requiring system-wide accountability — without the policy foundation to enforce participation. Provider participation in OneCare was voluntary.

This matters structurally, not just administratively. The Total Cost of Care model holds the ACO financially accountable for the total spending of an attributed population. But when high-cost, high-revenue providers remain outside the ACO — because participation is optional — the ACO is held responsible for costs it cannot influence. A hospital generating significant revenue from inpatient admissions has a rational financial incentive to stay outside a model that rewards reducing those admissions. When it does, the ACO's ability to manage total cost is structurally undermined from day one.

> **WORKED EXAMPLE — What optional participation means for a hospital CFO**
> Imagine you are the CFO of a Vermont hospital. Your hospital generates $40 million annually from inpatient admissions. OneCare is asking you to join an ACO that will hold you financially accountable for reducing those admissions — reducing your revenue. Participation is voluntary. You have three choices: join and accept the risk; join and manage costs without affecting revenue; or don't join. For many Vermont hospitals, the rational choice was the third. This is not a failure of values. It is a predictable response to an incentive structure that the policy framework failed to close.

Act 68's mandatory architecture is the direct statutory response to this failure. Reference-based pricing and global budgets are not optional. Every Vermont hospital is in.

1.9.2 Sequencing Failure 2: Economics Without Technology — The "Managing Blind" Failure Mode

OneCare attempted to manage a complex, population-level payment model involving real financial risk without the technology infrastructure required to do so. Specifically: no integrated system combining timely clinical data with claims data at meaningful scale.

Claims data is the financial record of healthcare — every bill submitted to an insurer. It tells you what was done, when, and at what cost. Clinical data is the medical record — diagnoses, lab results, medications, care plans. It tells you why something was done and what the patient's health status actually is. Neither dataset alone is sufficient for managing value-based payment. You need both, integrated, in near-real time.

OneCare operated without this. Care managers were working from claims data arriving 60 to 90 days after care was delivered — meaning they were identifying patients who had already been hospitalized rather than preventing hospitalization in the first place. This is the "Managing Blind" failure mode: assuming downside financial risk under a Total Cost of Care model without the analytics to track total cost of care in real time. The result is unexpected, unexplainable financial losses — not because the model is wrong, but because the organization cannot see what it is supposed to be managing.

WORKED EXAMPLE — Why timeliness is as critical as integration

A care manager responsible for 500 high-risk patients under a Total Cost of Care model needs to identify patients who are deteriorating before they are hospitalized. If her data arrives 60 days after the fact, she learns in March that a patient was hospitalized in January. The intervention opportunity is gone. If her data arrives within 24 to 48 hours — integrated clinical and claims feeds updated daily — she can see that a patient filled a new prescription last week, had an abnormal lab result three days ago, and has not seen their primary care provider in six months. She can call today, before the hospitalization.

Vermont's AHEAD technology infrastructure addresses this directly: VHCURES provides longitudinal population data; VITL/VHIE provides clinical data exchange; and the AHS-GMCB Analytics Vendor provides the modeling layer for global budget management and equity monitoring. This is the substrate OneCare never built.

1.9.3 Sequencing Failure 3: The Cascade

The two failures above did not operate independently. They compounded. Because participation was optional, OneCare could not achieve the provider network coverage required to manage total cost of care. Because it could not manage total cost of care, it could not demonstrate savings. Because it could not demonstrate savings, it could not attract the capital to build the technology infrastructure that might have allowed it to manage costs. Because it lacked that technology, care management remained limited, financial reporting remained inadequate, and clinical value remained undemonstrated. Because clinical value was undemonstrated, skeptical providers had no reason to join. Which returned the cascade to its starting point.

Sequencing Gap	Structural Error	Direct Consequence	Cascade Effect
Policy (Stage 1)	Voluntary participation — no mandate	High-revenue hospitals opt out; ACO accountable for costs it cannot influence	Total cost of care cannot be managed → savings targets missed
Technology (Stage 2)	No integrated real-time data; 60–90 day lag	"Managing Blind" — care managers identify hospitalized patients after the fact	Proactive intervention impossible → high-cost events not prevented → financial exposure accelerates
Economics (Stage 3)	Payment model assumed risk before technology was ready	Unexpected losses → cannot demonstrate savings to payers	Capital unavailable to build technology → cycle repeats
System-level	Three gaps mutually reinforcing	Organizational wind-down after ~10 years	Failure was architectural — no management decision could reverse it

Figure 1.1 — The OneCare Failure Cascade. Source: HTR Analysis (2026). Based on GMCB records, AHS November 2025 Transformation Report, and OneCare Vermont public filings.

BEYOND VERMONT

The three sequencing failures above — economics without a policy foundation, payment reform without data infrastructure, and clinical redesign without measurement — are not OneCare-specific defects. They are the three failure modes any value-based care vehicle exhibits when its launch order is wrong, whether that vehicle is a Medicaid ACO in a large fragmented state, a commercial risk contract for an independent physician group, or a CMMI model anywhere in the country. The autopsy is transferable even where the entity, the state, and the payer mix are not.

1.10 The Critics: Why Sequencing Logic Has Real Opponents

The sequencing argument presented in this chapter has critics, and they deserve direct engagement rather than dismissal.

1.10.1 "Sequencing Is a Post-Hoc Rationalization"

The most sophisticated critique: sequencing frameworks are analytical stories told after the fact to explain failures that had multiple causes. OneCare failed for many reasons — not just sequencing errors, but inadequate provider engagement, the complexity of Vermont's payer landscape, and the inherent difficulty of achieving savings under a CMMI model with multiple confounders. Calling it a "sequencing failure" imposes a cleaner narrative than the evidence warrants.

This critique has merit. OneCare's failure was overdetermined — it had more causes than sequencing alone. The sequencing analysis is useful not because it identifies the single cause but because it identifies the structural errors that made the failure architecturally inevitable regardless of operational quality. A car without brakes will crash regardless of how skilled the driver is. The sequencing failures in OneCare's design were the missing brakes.

1.10.2 "Mandatory Architecture Creates Provider Opposition That Delays Reform"

A second serious critique: by making participation mandatory, Vermont's approach creates an adversarial relationship with hospital systems that could be productive reform partners under voluntary models. Hospital opposition to mandatory reference-based pricing has been fierce and legally aggressive. UVMMC challenged GMCB budget enforcement in court. A voluntary approach would face less resistance, move faster initially, and produce more durable buy-in.

The counterevidence is OneCare itself: a decade of voluntary reform produced modest results and left the financial crisis unaddressed. The choice is not between a smooth voluntary process and a contentious mandatory one. It is between a process that produces structural change — contentiously — and one that produces durable accommodation of the status quo — smoothly. Vermont's legislature chose structural change. Chapter 14 (Political Sustainability) analyzes what it takes to maintain that choice across election cycles.

1.10.3 "Technology-First Is a Recipe for Expensive Shelfware"

A third critique targets the Technology-before-Economics sequencing specifically: building analytics infrastructure before the payment model that will use it often produces sophisticated technology that no one has the financial incentive or organizational capacity to deploy. Vermont's VITL health information exchange is cited in this chapter as an example of technology deployed without the surrounding incentive structure — exactly the outcome the technology-first argument is supposed to prevent.

This critique identifies a real failure mode. The response is a distinction the framework makes explicitly: technology deployment must be coordinated with payment architecture design. Building VHCURES analytics capability in 2025–2026 while designing the AHEAD global budget methodology in parallel — so that the technology is built to serve a specific and known payment model — is different from building technology without a defined use case. The failure mode to avoid is not "technology before economics" but "technology without economics."

1.11 The Six Stages: Resolved

1.11.1 Stage 1: Policy — Mandate and Capital

Policy is the absolute precondition for everything that follows. Its two functions: it creates the mandatory architecture that prevents high-cost actors from opting out of reform, and it secures the transformation capital that funds the technology build.

Vermont's legislative progression illustrates both. Act 167 established the diagnostic foundation. Act 51 defined the transformation architecture. Act 68 operationalized the mandate — converting voluntary reform into binding statutory requirements for global budgets and reference-based pricing. The AHEAD Model federal agreement layered in the federal funding that made the technology build financially feasible. The $195 million Rural Health Transformation award is a Policy-pillar outcome, not an Economics-pillar outcome. Transformation capital comes from policy and regulatory relationships, not from the payment model itself.

1.11.2 Stage 2: Technology — Build the Substrate Before Assuming Risk

Technology is the operating system on which every subsequent stage runs. It is not a layer added on top of a functioning reform. It is the foundation that makes reform functional. The required components: integrated clinical and claims data; data updated in near-real time; and shared infrastructure for organizations that cannot independently support sophisticated analytics.

Vermont's VHCURES provides longitudinal population data. VITL/VHIE provides the clinical exchange. The AHS-GMCB Analytics Vendor provides the modeling for global budget design and equity monitoring. The Vermont CIN provides shared capability to rural hospitals that individually lack the scale to build these functions.

The payment architecture design — Economics-as-design — happens in parallel during Stage 2. This is the work of specifying what the payment model needs to measure, which determines what the technology needs to do. But Economics-as-management — the operational work of running global budgets — does not go live until the technology substrate is functional.

1.11.3 Stage 3: Economics — Incentive Architecture on a Visible System

Economics is third because its management function requires the technology substrate to already be operational. Vermont's economic architecture has two structural components. Reference-based pricing, targeted at 200% of Medicare rates or below, addresses the unit price problem: Vermont's commercial insurance rates run far above Medicare, creating a cross-subsidy that is neither sustainable nor equitable. Global hospital budgets address the volume dimension: setting a fixed total revenue envelope for each hospital makes volume expansion financially irrational.

Together, reference-based pricing and global budgets break the fee-for-service trap that makes population health management economically punishing for providers. The AHEAD EAST Fund — up to $150 million annually in Medicare funds — operates alongside the incentive architecture as transformation capital for community health investment. This is the distinction between Incentive Architecture and Transformation Capital in practice: global budgets and reference-based pricing permanently restructure the payment environment; the EAST Fund provides time-limited bridge capital for the workforce development, telehealth infrastructure, and community health worker programs the transformed system requires.

The equity constraint becomes operationally urgent at Stage 3. Global budgets designed without social risk adjustment inadvertently penalize safety-net providers serving high-SDOH populations. Vermont's Northeast Kingdom is the clearest example: a global budget benchmark without social risk adjustment generates financial pressure that reflects social disadvantage, not operational inefficiency. Social risk adjustment must be embedded in the payment design, not retrofitted afterward.

1.11.4 Stage 4: Clinical — Redesigning Care on Aligned Incentives

Clinical redesign is where the economic return on the entire investment is realized. Patient-Centered Medical Homes transform primary care from acute episodic treatment into proactive population management. The Collaborative Care Model integrates behavioral health into primary care. Vermont's Blueprint for Health generated a 5.8-to-1 return on investment from its primary care investment — among the most robust ROI findings in the value-based care literature.

Clinical redesign comes fourth because clinical transformation without economic alignment reverts when the incentives revert. Clinicians redesign their practice in response to the incentives they face and the data available to them. Without a payment model that rewards the redesigned care — already live at Stage 3 — and without the technology to identify which patients need which interventions — already built at Stage 2 — clinical transformation is isolated, unsustainable, and unable to demonstrate the population-level outcomes that justify its cost.

1.11.5 Stage 5: Equity — Design Constraint and System Calibration

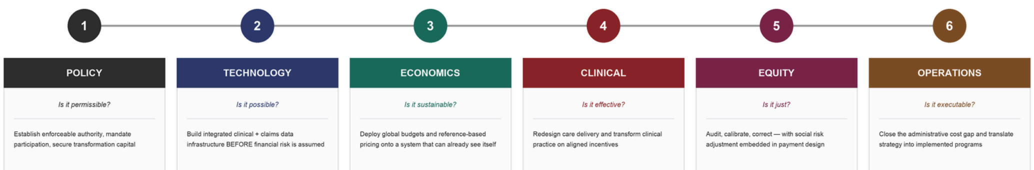
Equity is fifth in execution sequence but must be understood as a constraining dependency that runs through every preceding stage — not a final add-on applied to a finished system. Vermont's 11-point primary care access gap between white and BIPOC residents is not closed by a Stage 5 audit. It is closed by Stages 3, 4, and 5 working in sequence: payment design that does not penalize safety-net providers, clinical redesign deploying Community Health Teams in underserved areas, and Stage 5 audit that measures whether those interventions are actually closing the gap.

What Stage 5 adds is systematic, population-level measurement of whether the equity commitments embedded in earlier stages have produced equitable outcomes — and the corrective action, informed by real data from a functioning system, when they have not.

1.11.6 Stage 6: Operations — Closing the Administrative Cost Gap

Operations is the execution layer that converts all prior-stage investments into organizational reality. Vermont's critical-access and rural hospitals spend $2,730 per discharge on administration, compared to a national benchmark of $1,427. This $1,303 per-discharge gap represents the primary source of the $100 million or more in projected direct savings from shared services and administrative simplification.

Operations is last not because it is least important, but because operational redesign requires the technology infrastructure, payment incentives, and clinical models of the preceding stages to already be in place. A hospital that redesigns its administrative operations before its payment model is live will optimize for a fee-for-service world that the reform is designed to replace.



The six-stage execution sequence: numbered Policy → Technology → Economics → Clinical → Equity → Operations, each with its diagnostic question and key action.

Figure 1.2a — The six-stage execution sequence at a glance: each pillar's diagnostic question and the key action that defines its stage. The table below gives the full sequencing rationale and Vermont anchors.

#	Pillar	Key action	Why this sequence	Vermont anchor
1	Policy	Establish enforceable authority, mandate participation, secure transformation capital	Nothing can be authorized, built, or funded without statutory permission. Mandatory architecture prevents high-cost actors from opting out.	Acts 167, 51, 68; AHEAD State Agreement; $195M RHT award
2	Technology	Build integrated clinical + claims data infrastructure — real-time — before financial risk is assumed	Technology is the operating system. Must be operational before Economics goes live. "Managing Blind" is the failure mode.	VHCURES; VITL/VHIE; AHS-GMCB Analytics Vendor; Vermont CIN
3	Econo	Deploy global budgets and RBP onto a system that can already see itself	Economics-as-management requires the technology substrate. Payment	Global hospital budgets; RBP at ≤200% Medicare; EAST Fund up to $150M annually

#	Pillar	Key action	Why this sequence	Vermont anchor
	mics		design (Economics-as-design) happens in parallel during Stage 2.	
4	Clinical	Redesign care on aligned incentives	Clinical transformation is where ROI is realized — but only when payment incentives are live and technology identifies which patients need which interventions.	PCMH transformation; CoCM; CCBHC expansion; Blueprint 5.8:1 ROI
5	Equity	Audit, calibrate, correct — with social risk adjustment embedded in payment design	Equity is a constraining dependency throughout. Without social risk adjustment, global budgets penalize safety-net providers.	HRSN screening; EAST Fund for SDOH; SRA in global budgets; REH model
6	Operations	Close the administrative cost gap and translate strategy into implemented programs	Operations is the execution layer. Vermont's $1,303 per-discharge administrative gap vs. national benchmark is the primary source of projected direct savings.	RHRC methodology; shared services; 14-hospital transformation planning

Figure 1.2 — The Six-Stage Execution Sequence. Source: HTR Analysis (2026).

1.12 Three Sequencing Principles in Practice

The six-stage sequence is the *what*. Applying it to real investment decisions requires three operating principles — the rules a reform architect uses to decide what to fund now, what to fund in parallel, and what to hold until an upstream gate opens.

1.12.1 Principle 1: The Critical Path Is Non-Negotiable

The six-pillar framework identifies three critical-path dependencies: Policy enables Economics, Economics drives Clinical, and Technology enables Economics-as-management. These are not preferences or best practices — they are structural requirements. Violating them does not produce suboptimal results; it produces failed interventions that generate political resistance to the correct interventions that follow.

Critical-path dependencies cannot be shortcut by allocating more resources to the downstream intervention. A hospital that signs a full-risk AHEAD global budget contract before its VHCURES analytics capability is operational has not accelerated its transformation — it has exposed itself to financial losses it cannot explain, cannot dispute, and cannot manage. The analytics capability cannot be retroactively built while the contract is running. The sequence cannot be reversed after it has been violated.

In practice, reading the critical path means identifying which investments are *gates* — prerequisites that must be in place before the next investment can be productive. Vermont's critical-path sequence for AHEAD implementation: the Policy gate closed in June 2025 when Act 68 established mandatory RBP (FY2027) and global budgets (FY2028). All Vermont hospitals are now in the mandatory architecture. The Technology gate — VHCURES analytics, HCC gap closure, attribution modeling — must be operational before the January 2028 AHEAD performance year begins. This is the current critical bottleneck. The Economics gate (global budget contracts set by GMCB for FY2028) opens after the Technology gate is complete. The Clinical gate (clinical programs generating utilization reduction at scale) must be operating before the full financial accountability of global budgets takes effect.

1.12.2 Principle 2: Parallel Work Is Possible — But Not on Critical-Path Dependencies

The critical path is sequential by definition. But the framework does not require that *all* work be sequential. Many investments can proceed in parallel, as long as none of the parallel tracks violates a critical-path dependency.

Vermont's current architecture illustrates this: while the Technology gate is being opened (analytics vendor deployment, VHCURES integration), parallel work proceeds on Equity design (social risk adjustment methodology for global budgets), Operations capacity building (RHRC hospital transformation planning, AHS PMO establishment), and Clinical infrastructure (Blueprint PCMH expansion, CCBHC certifications). None of this parallel work depends on the Technology gate being open — it can proceed simultaneously without violating any critical-path dependency.

The diagnostic question for any proposed parallel investment: *does this investment have an unmet upstream dependency?* If yes — if its effectiveness depends on a gate that is not yet open — it is not parallel work. It is premature work that will produce either wasted results or results that cannot be used until the upstream dependency is met.

1.12.3 Principle 3: Equity Is a Design Constraint, Not a Downstream Filter

The most common sequencing error organizations make with the Equity pillar is treating equity review as the last step before a decision is finalized rather than as a constraint embedded in the design process. Equity-as-last-step produces expensive redesigns at the approval stage. Equity-as-design-constraint asks the equity-implication question at the *beginning* of the design process — shaping the instrument rather than filtering it.

In practice, embedding equity as a design constraint requires two organizational changes: adding equity review to the *initiation* criteria for policy and program design (not only the approval criteria), and ensuring that Health Equity Advisory Commission consultation under Act 68 happens early in the design cycle rather than at the comment stage. Vermont's statutory framework requires equity consultation — it does not specify when in the process that consultation must occur. Organizations that treat it as early design input rather than late validation produce more equitable outcomes with less redesign cost.

TRY THIS — Test a critical-path violation.

Open the **HTR Simulator** (/htr-simulator) and model an organization that funds a downstream pillar before its upstream gate is open — for example, high Economics ambition with a weak Technology score. Watch the composite readiness collapse. Then open the **Transformation Friction Index** (/transformation-friction-index) to see which pillar is your binding constraint. The principle is simpler to feel in the tool than to argue on the page: you cannot buy your way past a closed gate.

GO DEEPER — HTR Academy

The dependency mechanics behind these three principles — how payment incentives drive care redesign, why VBC contracts require data infrastructure first — are developed hands-on in the Academy's **Value-Based Care** course. Readers new to APM risk and total-cost-of-care accountability should start there before applying the sequencing framework to a live contract.

1.13 Five Sequencing Decisions That Organizations Get Wrong

1.13.1 Failure 1: Economics Without Policy Readiness

Pattern: A payment reform program launches without mandatory policy architecture requiring all relevant actors to participate. Early warning signals: voluntary participation below 80%; persistent non-participation by highest-cost providers; contract redesigns driven by adverse selection rather than clinical results. Vermont evidence: OneCare Vermont. Correction: Act 68's mandatory RBP and global budgets. Irreversibility threshold: After a decade, the voluntary architecture had normalized — providers had built business models around non-participation that made the transition to mandatory reform more disruptive than it would have been at the outset.

1.13.2 Failure 2: Technology Deployment Without Operations Readiness

Pattern: A technology platform is deployed to an organization lacking the IT staff, data governance, clinical workflow integration, and change management capacity to use it effectively. Early warning signals: adoption rates below 30% six months post-deployment; continued reliance on manual processes despite platform availability; clinical staff workarounds that bypass the platform. Vermont evidence: VITL has been operational for years; adoption among smaller Vermont providers remains incomplete not because VITL is poorly designed but because the Operations pillar infrastructure to connect, train, and support smaller providers is insufficient. Correction: Vermont CIN's shared services model — providing Technology pillar capability to organizations whose Operations pillar cannot support it independently.

1.13.3 Failure 3: Clinical Programs Without Financial Alignment

Pattern: A high-quality clinical program is implemented under fee-for-service payment. Clinical results are produced but not financial results. The organization cannot sustain it. Early warning signals: program operating at a financial loss absorbed through cross-subsidy; leadership support contingent on continued favorable cross-subsidy; program perpetually at risk of budget cuts. Vermont evidence: Vermont's Blueprint for Health has operated under this partial misalignment. Blueprint PCMHs produce the documented 5.8-to-1 ROI — but under fee-for-service, most of that ROI accrues to payers rather than to the practices generating it. Correction: AHEAD global budgets align financial incentives with clinical investment.

1.13.4 Failure 4: Equity Review at the End Rather Than the Beginning

Pattern: A payment reform methodology is completed and then submitted for equity review. The equity review identifies significant disparate impact. Redesign is expensive, politically complicated, and often incomplete. Early warning signals: equity staff consistently identifying the same structural problems in finalized designs; high rate of equity-related modifications at approval stage. Vermont evidence: Vermont's RBP methodology design process is the current test case. The design question — how to set reference prices in a way that preserves access for rural communities and does not defund hospitals serving high-Medicaid populations — is an equity question that must be answered in the methodology, not added as a correction after it is set. The Health Equity Advisory Commission consultation required by Act 68 must happen at the methodology design stage, not the comment stage.

1.13.5 Failure 5: Operations Execution Without Adequate Capacity

Pattern: A statutory mandate requires organizational execution at a pace the organization's current Operations pillar capacity cannot support. Plans are produced. Deadlines are missed. Deliverables are partial. Early warning signals: statutory deadlines being extended or waived; deliverables consistently described as "in progress" at reporting dates; transformation governance structures that convene but do not decide. Vermont evidence: AHS must simultaneously manage 14 hospital transformation planning processes, AHEAD implementation support, RBP methodology finalization, Statewide Strategic Plan development, and ongoing program administration — with organizational capacity the Oliver Wyman analysis identified as insufficient. The RHRC engagement with all 14 Vermont hospitals is the correct sequencing: building hospital-level transformation capacity before AHEAD's clinical accountability requirements take full effect.

1.14 Vermont's Implementation Timeline: Reading the Sequence in Statutory Deadlines

Vermont's transformation is unusual in one critical respect: it has a legislatively mandated implementation timeline that is legally binding, not aspirationally stated. Reading this timeline through the sequencing framework reveals both Vermont's correct sequencing decisions and its current critical vulnerabilities.

Pillar	Status (timeframe)	Where it stands
Policy	Completed (2022–2025)	Acts 167, 51, and 68 establish the mandatory architecture. Federal RHT award provides capital. AHEAD State Agreement signed January 2025. The Policy gate is fully open.
Technology	In progress (2025–2026)	AHS-GMCB analytics vendor procurement underway. Vermont CIN under development. VHCURES attribution modeling in progress. AI scribe and RPM grants deployed. The current critical bottleneck — the Technology gate is partially open but not fully operational.
Economics	Staged (2027–2028)	RBP mandatory beginning FY2027, global budgets mandatory beginning FY2028. The sequence is correct: mandatory payment reform follows mandatory policy authority and follows the technology build in design.
Clinical	Parallel (2024–2028)	Blueprint PCMH expansion, CCBHC certifications, CoCM deployment proceed in parallel with Technology work — correctly, because they do not depend on the Technology or Economics gates being open to begin.
Operations	Building (2025–2028)	RHRC engagement, AHS HSA Coordinator model, Division of Planning and Effectiveness, PMO structure — correctly sequenced to precede the peak execution demand of FY2027–2028.
Equity	Throughout (2025–2028)	Social risk adjustment methodology, HEROI scoring, geographic equity monitoring — correctly treated as a parallel design constraint rather than a final review.

Figure 1.4 — Vermont's transformation timeline, by pillar. Source: HTR Analysis (2026); Act 68 of 2025; AHEAD State Agreement.

1.14.1 The Current Vulnerability: The Technology-Economics Race

Vermont's most significant current sequencing risk is the gap between AHEAD's performance year start and the analytics vendor deployment timeline. Vermont hospitals will begin accumulating AHEAD financial accountability in January 2028 — updated per CMS's September 2025 announcement moving the Cohort 2 start from 2027 to 2028. If the AHS-GMCB analytics vendor is not fully operational by that date, hospitals will be making financial management decisions based on incomplete or lagged data for the first year of full AHEAD accountability.

This is a manageable but costly risk. Vermont hospitals that invest in their own VHCURES analytics capability — particularly HCC gap closure, the highest-ROI pre-launch analytics investment — can partially compensate for the analytics vendor gap. A delay in analytics vendor selection and deployment does not merely delay one program: it delays the financial management capability for AHEAD, the equity measurement capability for the Statewide Strategic Plan, and the risk stratification capability for Blueprint's CCBHC expansion. Three dependencies converge on this single Technology pillar investment.

1.15 Work This Chapter on the Platform

The execution-sequence argument is best understood by manipulating it, not just reading it. The HTR platform turns this chapter's three core claims — that the pillars are interdependent, that order is non-negotiable, and that getting the order wrong produces a measurable failure cascade — into tools you can run against your own organization or state.

Do this	On this tool	What to look for
Trace the 15 dependency relationships between the six pillars and see which downstream pillars a given gap disables	**Six-Pillar Map** — /about/framework	Click any pillar to highlight what it enables and what it depends on. Confirm visually why Technology sits upstream of Economics.
Score a real organization or state across all six pillars and watch the composite readiness number respond	**HTR Simulator** — /htr-simulator	Drop one pillar's score to zero and observe the cascade — the simulator penalizes downstream readiness, reproducing the OneCare logic in Figure 1.1.
Quantify where reform is losing momentum and which pillar is the binding constraint	**Transformation Friction Index** — /transformation-friction-index	Identify your own "Technology gate" bottleneck — the chapter's central Vermont vulnerability — before it becomes a financial one.

Figure 1.3 — Hands-on platform tools for Chapter 1: working the execution-sequence argument on the HTR platform.

TRY THIS — Reproduce the OneCare cascade.

> Open the **HTR Simulator** and build a profile with strong Economics ambition (high) but weak Policy mandate (voluntary) and weak Technology substrate (no integrated real-time data). The composite score should collapse — not because the Economics inputs are bad, but because the simulator enforces the same dependency logic that sank OneCare. Then fix the order: raise Policy and Technology first, and watch the same Economics inputs finally produce a viable score. That reversal *is* the argument of this chapter.

1.16 Implications for You

If you are a Vermont hospital executive: The most important action you can take before AHEAD's performance year begins in January 2028 is completing HCC gap closure for your full Medicare-attributed population. This single Technology pillar investment improves risk adjustment accuracy, produces a more defensible global budget benchmark, and prevents the "Managing Blind" failure mode in your first AHEAD performance year. Do not wait for the AHS-GMCB analytics vendor to complete this work. Start with VHCURES direct access and your current analytics resources now.

If you are an AHS or GMCB official: The Technology gate is your critical bottleneck, and it has a hard deadline — analytics capability must be operational before January 2028 AHEAD financial accountability begins. The three-dependency convergence — AHEAD management, Statewide Strategic Plan measurement, CCBHC risk stratification — means that a delay in the analytics vendor is a compound failure across multiple statutory obligations. This deserves the urgency of a critical-path emergency, not the pace of a standard procurement.

If you are a Vermont legislator: The sequencing framework translates directly into the right oversight questions to ask AHS and GMCB at every reporting interval: Is the Technology gate open before the Economics gate goes live? Is the RBP methodology being designed with equity constraints from the beginning, or will the equity review happen at the approval stage? Is the RHRC engagement building hospital-level Operations capacity ahead of AHEAD's clinical accountability requirements? These three questions will determine whether Act 68's mandates produce the transformation they were designed for.

If you are a national health policy professional: Vermont's OneCare failure is the most precisely documented sequencing failure in recent American health policy. Before designing or evaluating any state total-cost-of-care model, ask three questions: Is participation mandatory or voluntary? Is the technology substrate operational before financial risk is assumed? Is the equity constraint embedded in the payment design or appended at the review stage? The answers predict outcome more reliably than any other design feature of the model.

1.17 Key Concepts in This Chapter

Dependency logic

The principle that some pillar investments cannot produce results until upstream enabling investments are in place. Determines the execution sequence.

Execution sequence

The order in which the six pillars must be built: Policy, Technology, Economics, Clinical, Equity, Operations. Determined by structural dependency, not by the relative importance of each pillar.

Critical path gate

A prerequisite investment that must be in place before a downstream pillar investment can be productive. Cannot be bypassed by allocating more resources downstream.

"Managing Blind" failure mode

Assuming downside financial risk under a value-based payment model without the analytics infrastructure to track total cost of care in real time. Produces unexpected, unexplainable losses.

Sequencing failure

Deploying a downstream pillar before the upstream enabling investment is in place. OneCare Vermont is canonical: Economics (voluntary ACO) deployed without Policy mandate and Technology substrate.

Voluntary vs. mandatory architecture

The distinction between reform programs actors can opt out of and those they cannot. Act 68 establishes mandatory RBP and global budgets — eliminating voluntary opt-out.

Economics-as-design vs. Economics-as-management

Design (defining the payment architecture) can proceed in parallel with the technology build. Management (operating the model) requires the technology substrate to be operational first.

Transformation capital vs. incentive architecture

Transformation capital (RHT award, EAST Fund) is time-limited bridge funding for infrastructure. Incentive architecture (global budgets, RBP) is permanent structural change to the payment environment. Confusing the two produces unsustainable programs.

Reform cascade

A legislatively structured sequence where each intervention creates capacity and legitimacy for the next. Vermont's Act 167 → Act 51 → Act 68 is the prototype.

HCC gap closure

The highest-ROI pre-AHEAD-launch analytics investment: completing hierarchical condition category documentation for all Medicare-attributed patients before the performance year begins. Improves risk adjustment accuracy and produces a more defensible global budget benchmark.

Parallel work

Investment that proceeds simultaneously with critical path work because it does not depend on any upstream gate opening. Test: could this investment produce its full intended value today, with upstream pillars in their current state?

Premature investment

An investment in a downstream pillar that proceeds before the upstream enabling investment is in place. Most common forms: Technology deployment without Operations readiness; Economics contracts without Technology analytics readiness.

1.18 Sources

Vermont Agency of Human Services, Health Care System Transformation Report (November 2025); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024); Vermont AHEAD State Agreement (January 2025); CMS AHEAD Model documentation (September 2025 update); Vermont Blueprint for Health Annual Report 2024; Act 68 of 2025; OneCare Vermont ACO annual reports (2014–2023); Maryland HSCRC Total Cost of Care Model data; GMCB FY25 and FY26 Hospital Budget Orders.

Chapter 2: The Policy Pillar — Legislative Architecture for Structural Reform

> *Vermont's Acts 167, 51, and 68 constitute the most comprehensive state-level healthcare reform mandate in the United States since Maryland's all-payer model in the 1970s.*

Every state's reform effort eventually confronts the same architectural question: which provisions are aspirational and which are binding? A reform agenda that depends on voluntary participation by the actors with the most to lose from change will, with near certainty, be defeated by exactly those actors. This is not a Vermont-specific risk — it is the central design problem of structural healthcare reform anywhere, and it is the reason the Policy pillar has to come first in the six-pillar sequence. Vermont's Acts 167, 51, and 68 are presented in detail in this chapter because they show, with unusual legislative specificity, what it actually takes to convert that aspiration into something binding — the diagnostic mandate, the operational mandate, and the enforcement architecture that connects them.

2.1 The Anatomy of a System-Level Reform Mandate

Year	Milestone	Pillar	Significance
2022	Act 167 of 2022 enacted	Policy	Commissioned Oliver Wyman analysis; mandated system-wide diagnostic
Aug 2024	Oliver Wyman Act 167 Report	Policy / Economics	Documented 9/14 hospitals in losses; $2.4B 5-year deficit projection; triggered legislative action
Sept 2024	Oliver Wyman presentation to GMCB	Policy	Room full of stakeholders; financial trajectory confirmed publicly; political inflection point
2023	Act 51 of 2023 enacted	Policy	Defined the six-pillar transformation architecture; established AHS restructuring mandate
2025	Act 68 of 2025 enacted	Policy / Economics	Mandatory RBP (FY2027), global budgets (FY2028), Dec 2028 Strategic Plan deadline

Year	Milestone	Pillar	Significance
Jan 2025	Vermont AHEAD State Agreement signed	Policy / Economics	Committed Vermont to Cohort 2; up to $150M/year EAST Fund; 8-year performance period
Nov 2025	Vermont RHT Program Application submitted	Policy / Operations	$195M award application; 19 HTR tools; 370-FTE workforce plan
Dec 2025	CMS BALANCE Model announced	Economics / Clinical	GLP-1 coverage at negotiated prices; Vermont DVHA opt-in decision pending
Dec 29, 2025	Vermont receives $195M RHT award	Policy / Technology	Largest per-capita rural transformation award; funds CIN, telehealth, workforce
Jul 4, 2025	H.R. 1 signed (One Big Beautiful Bill Act)	Policy	$911B Medicaid cuts over 10 years; work requirements; 10M additional uninsured by 2034
FY2027 (Oct 2026)	RBP mandatory for all Vermont hospitals	Economics	First mandatory all-payer price cap in modern Vermont history; ≤200% Medicare target
FY2028 (Oct 2027)	Global budgets mandatory (non-CAH hospitals)	Economics	Fixed revenue envelopes; transforms financial incentive from volume to population health
Jan 2028	AHEAD Cohort 2 performance year begins	Economics / Technology	Updated per CMS Sept 2025; Vermont hospitals begin AHEAD financial accountability
Dec 2028	Statewide Strategic Plan due to legislature	Operations / All pillars	Act 68 binding deadline; must commit to specific targets, timelines, accountable owners
FY2030 (Oct 2029)	Global budgets mandatory — all 14 hospitals	Economics	Full all-payer global budget coverage; complete transition from fee-for-service signal
2030-2035	AHEAD performance period continues	Economics / Clinical	CMS evaluation of Vermont total cost of care; evidence base for national replication

Figure T.1 — Vermont Healthcare Transformation Timeline: 2022–2035. Sources: Vermont Legislature, CMS, AHS, GMCB.

Most healthcare reform unfolds incrementally — a new demonstration model here, a revised billing rule there — accumulating over years into changes that are difficult to attribute to any single decision. Vermont's reform trajectory between 2022 and 2025 is different in kind, not degree. Within three years, the Vermont General Assembly enacted a sequence of legislation that collectively mandated the most comprehensive statewide healthcare restructuring attempted by any U.S. state since Maryland's all-payer model in the 1970s. Understanding this sequence — its logic, its mechanisms, and its unresolved tensions — is essential preparation for any healthcare leader operating in an environment of payment and delivery system transformation.

The sequence has a clear internal logic. Act 167 of 2022 was a diagnostic mandate: commission a rigorous, public, community-grounded diagnosis of what is actually wrong. Act 51 of 2023 was a planning mandate: begin doing something about it. Act 68 of 2025 was an operational mandate: specific tools — reference-based pricing, global hospital budgets, a Statewide Health Care Delivery Strategic Plan — were authorized and given statutory deadlines with real consequences for non-compliance. Each act built on the evidence and institutional capacity developed by its predecessor. The whole sequence constitutes what policy scholars call a reform cascade: a legislatively structured process in which each intervention creates the conditions for the next.

This chapter section examines each act, the analytical infrastructure it created, and the six-pillar implications for organizations navigating comparable reform environments. It draws heavily on the Oliver Wyman Act 167 Community Engagement: Recommendations report — the most comprehensive independent public diagnosis of a state hospital system in recent American health policy history — which must be read as the intellectual foundation for everything Vermont's legislature did in 2025 and everything AHS is mandated to deliver by 2028.

2.2 The Three Imperatives: Oliver Wyman's Central Finding

9 of 14	$2.4B	108%	57%
Hospitals in Loss FY2023	5-Year Deficit Projection	Premium Increase 2018-2024	Growth in 65+ by 2040

Before examining the legislative sequence, it is worth stating plainly what the Oliver Wyman analysis concluded at the highest level of abstraction. After 230 meetings across Vermont's 14 Hospital Service Areas, engagement with more than 3,100 participants representing over 100 organizations, and systematic analysis of financial, demographic, and quality data spanning every hospital in the state, Oliver Wyman's executive summary distilled Vermont's challenge to three imperatives:

OLIVER WYMAN'S THREE IMPERATIVES

1. Fix the upstream drivers — build housing and other facilities and fix transportation.

2. Fix the price and budget architecture — pay PPS hospitals with reference-based pricing and move to global budgets/capitation for all when conditions for success are met.

3. Shift the locus of care — move all care possible out of hospitals.

These three imperatives are not a wish list. They are the analytically derived conclusion of a year-long process that examined every dimension of Vermont's system. Each imperative maps directly onto the six-pillar framework: the housing and transportation imperative is an Equity and Operations pillar challenge; the pricing and global budget imperative is an Economics and Policy pillar challenge; and the shift of care out of hospitals is a Clinical pillar challenge that requires Technology and Operations pillar execution. The entire Vermont reform agenda can be read as a simultaneous, coordinated intervention across all six pillars — which is precisely why it is analytically instructive for any organization facing comparable pressures.

Oliver Wyman was equally direct about what will not work: more funding without structural change. Their analysis showed that simply increasing funding to attract more providers would drive up prices without improving access given Vermont's labor market constraints. Raising taxes or commercial premiums further risks driving residents and businesses out of the state. And the rural nature and shrinking population of most Vermont communities means that many areas simply cannot sustain a hospital with a full-time, full-spectrum of specialties. The implication is unambiguous: easy solutions — pour in money, add providers, maintain the status quo — will not resolve Vermont's healthcare crisis. The system must be structurally redesigned.

BEYOND VERMONT

"More funding without structural change" is the default policy instinct in every state because it is the path of least political resistance — it asks no incumbent actor to accept less. Oliver Wyman's finding for Vermont generalizes directly: in any market where the underlying incentive structure rewards volume over value, additional money flows into the existing structure rather than changing it, producing higher prices and the same access problems. The three imperatives — addressing upstream drivers of cost, fixing the price and budget architecture, and shifting the locus of care — are a sequencing-neutral checklist for diagnosing whether a state's reform agenda is structural or merely additive, regardless of which consulting firm or analytical process produced it.

2.3 The Platform for Change: Vermont's System in Crisis

The Oliver Wyman report documented five interconnected challenge domains that constitute what it called the Platform for Change — the evidence base that makes transformation not just desirable but unavoidable. Each domain has precise, quantified data that the book's previous draft understated. The full picture is considerably more alarming than most public discussion of Vermont healthcare acknowledges.

2.3.1 Decreasing Affordability

Vermont's healthcare cost trajectory has been disconnecting from household income for at least six years, with no natural correction mechanism in sight. Oliver Wyman documented that average premiums for silver exchange plans reached $985 in 2024 — a 108% price increase in six years. Individual and small group plan premiums increased 60-80% in the same period. Out-of-pocket maximums more than doubled in five years. Meanwhile, Vermont median household income grew only 22% between 2018 and 2022. Hospital approved charge increases grew 38% over the same period, compared to income growth of 22%. The gap is structural: hospital price growth has been running at two to three times income growth, and there is no market mechanism that closes this gap without regulatory intervention.

The premium data from Vermont Blue Cross Blue Shield confirms and extends this picture. Average annual per-member-per-month medical and pharmacy costs rose from $481 in 2020 to $812 in 2024 for local claims — a 69% increase in four years. Out-of-pocket maximums doubled in five years. A 2025 state survey found that many Vermonters, even those with insurance, delay care due to fears of medical debt. The affordability crisis is no longer a risk on the horizon; it is a current condition affecting clinical behavior.

2.3.2 Deteriorating Hospital Financial Sustainability

The financial projections in the Oliver Wyman report are the most important data in the entire Vermont reform story, and they need to be stated with the precision Oliver Wyman provided, not rounded down to reassuring generalities.

In FY2023, nine of Vermont's fourteen hospitals reported operating losses, with the worst reaching -8.9%. Oliver Wyman modeled two scenarios for the trajectory through 2028. Under the conservative scenario — 3.5% annual non-340B revenue growth and 5% annual expense growth — 13 of 14 hospitals are projected to report losses by 2028, with a cumulative 5-year system-wide deficit of $700 million against break-even ($1.4 billion against a 3% target operating margin). This is before accounting for the fact that Vermont hospitals collectively requested $285 million in budget increases for FY25 versus FY24 alone, suggesting the conservative scenario may already be understated.

Under the more realistic scenario — accounting for 10% annual expense growth on non-physician labor, 7-8% growth on other operating expenses, and constrained revenue — the 5-year cumulative deficit reaches $2.4 billion against break-even, or $3.1 billion against a 3% operating margin. This is approximately $3,700 to $4,800 per Vermont resident over five years. The UVM Health Network, which accounts for more than half the state's hospital capacity, faces $1.5 billion in cumulative deficits over five years in the high-expense scenario.

These projections assume no structural change. They are not predictions of what will happen — they are the analytical case for why structural change is necessary. Without the interventions mandated by Acts 167 and 68, Vermont's hospital system does not gradually decline; it collapses, with hospitals closing unexpectedly rather than through orderly transformation.

VERMONT IN PRACTICE — The Hospital Financial Crisis by the Numbers

Oliver Wyman's financial modeling established the quantitative case for urgent intervention. The key data points every healthcare leader and policymaker should know:

- 9 of 14 Vermont hospitals reported operating losses in FY2023, with losses reaching -8.9%
- Under conservative assumptions (3.5% revenue growth, 5% expense growth): 13 of 14 hospitals report losses by 2028; $700M cumulative 5-year system deficit
- Under realistic assumptions (7-8% expense growth): $2.4B cumulative 5-year system deficit against break-even; $3.1B against 3% operating margin
- Vermont hospitals requested $285M in budget increases for FY25 vs. FY24 — suggesting the conservative scenario is already understated
- UVM Health Network alone faces $1.5B cumulative deficit over 5 years under the high-expense scenario
- Average silver exchange plan premium reached $985 in 2024 — a 108% increase in 6 years
- Out-of-pocket maximums more than doubled in 5 years
- Hospital charges grew 38% since 2018; median household income grew only 22%

These are not projections of modest stress. They are projections of systemic collapse without intervention. The entire Vermont legislative reform agenda from 2022 to 2025 is a response to this specific data.

2.3.3 The Demographic Trap

Vermont's population dynamics create what Oliver Wyman called a demographic trap — a self-reinforcing cycle from which conventional market responses provide no escape. Vermont's population is aging and shrinking simultaneously: the 65+ population is projected to increase 57% by 2040, rising from 21.7% in 2020 to exceed 30% of total population. The working-age population (20-64) will decline 13% by 2040, from 367,000 to 318,000. By the mid-2030s, Vermont will have nearly as many residents over 65 as under 20.

The consequences cascade in multiple directions at once. Aging drives higher utilization of complex, expensive services: cancer, heart disease, stroke, dementia, and frailty-related hospitalizations all increase with age. A rising 65+ population means a rising proportion of Medicare and Medicaid patients — payers that reimburse below cost — at the expense of the commercially insured working-age population that cross-subsidizes hospital operations. The shrinking commercial base makes the cross-subsidy less viable precisely as the demand for it grows. Oliver Wyman was direct: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift.

This dynamic operates at the hospital service area level, not just statewide. Oliver Wyman's HSA-level projections showed that every service area except Burlington will experience population decline and aging — with the Northeast Kingdom (Caledonia, Essex, Orleans counties) facing the most severe trajectory. Newport's 65+ population, already at higher proportions than statewide averages, will represent a majority of the HSA's healthcare demand by 2040.

2.3.4 The Interconnected Failure System

Perhaps the most analytically powerful contribution of the Oliver Wyman report is its systems map of how Vermont's healthcare failures interconnect and amplify each other. The map traces how a housing shortage creates a staffing crisis (workers cannot afford to live near the hospitals that need them), how staffing shortages create capacity constraints, how capacity constraints produce ED crowding and delayed discharges, how delayed discharges require expensive travel nursing that drives up costs, how cost increases drive premium increases, how premium increases reduce affordability, how reduced affordability produces avoidable ED visits from patients who delayed primary care, completing the cycle.

External forces compound these internal dynamics: inflation drives labor and supply costs beyond what hospital budgets can absorb; an aging population shifts the payer mix toward lower-reimbursing Medicare and Medicaid; the cumbersome Certificate of Need process slows capital investment; licensure complexity impedes provider recruitment; and out-of-state care migration reduces procedure volumes, further undermining the clinical volume needed to sustain specialist expertise.

The systems map has a direct implication for policy design: interventions that address only one node of this system will be absorbed by the others. A workforce recruitment program without affordable housing produces recruits who cannot stay. A primary care access expansion without transportation infrastructure does not reach rural patients. Reference-based pricing without global budgets controls unit prices but allows volume to expand. Oliver Wyman's three imperatives are formulated precisely to address the highest-leverage nodes simultaneously, not sequentially.

2.3.5 Healthcare Equity Gaps

Oliver Wyman documented equity challenges across three categories that map directly onto the equity pillar's disparity root cause taxonomy. Gaps in culturally competent care were the most frequently cited equity challenge in community meetings — particularly for patients who do not speak English as a first language, LGBTQ+ patients, patients with disabilities, and patients in rural communities with cultural mistrust of mainstream healthcare institutions. Vermont's foreign-born population, while lower than the national average at 4.5%, is concentrated in Burlington and surrounding communities where language access remains inadequate relative to need.

Gaps in workforce culture and psychological safety were the second category — not just about patients but about providers. Community meeting participants described weight bias, stigma, and a lack of diversity in clinical leadership that affects both care quality and staff retention. Keeping healthcare staff is a function of cost of living, inflation, and affordable housing, multiple stakeholders noted — connecting the workforce equity problem directly to the SDOH infrastructure problem.

Lack of coordination between healthcare and community services was the third category — the failure to close referral loops between clinical care and housing, transportation, food security, and behavioral health. Multiple meeting participants noted that EMR systems do not talk to each other, that patients must navigate 31 EMS agencies for transfers, and that no centralized system guides patients through the coverage and financial assistance they are entitled to.

2.4 The Perception vs. Reality Problem

One of Oliver Wyman's most valuable contributions is a systematic correction of widely held misconceptions about Vermont's healthcare infrastructure — misconceptions that, if left uncorrected, produce policy responses calibrated to a system that does not exist. The following table reproduces Oliver Wyman's analysis:

Common perception	What the data actually shows
Vermont has an 'All Payer' model	The 'All Payer' system does not include FFS Medicare or approximately 40% of commercially insured Vermonters. It covers a minority of total healthcare spending.
Vermont has a statewide ACO (OneCare)	OneCare participation is voluntary and does not include several hospitals or many community-based providers. OneCare has been unable to provide the information needed to guide patient care and anticipate needs.
Vermont has a functioning State HIE (VITL)	Participation in VITL is voluntary and many community-based providers are not included. VITL lacks pharmacy claims data and is not viewed as user-friendly by providers.
Vermont does not have enough primary care providers	If primary care providers were supported to see 3 patients per hour, there are enough for well into the future. HRSA recognizes no Health Profession

Common perception	What the data actually shows
	Shortage Areas in Vermont. The problem is productivity and practice model, not pure headcount.

Table 2.1 — Common perceptions of Vermont's health system versus the data. Source: Oliver Wyman Act 167 Report; GMCB.

These corrections matter for the book's argument because they explain why Vermont's reform agenda is more complex than it appears from the outside. A state that appears to have all-payer payment reform, an ACO, and a health information exchange actually has voluntary, partial implementations of each — generating the appearance of a transformed system while the underlying fee-for-service, fragmented, data-poor reality persists. Acts 167 and 68 are, in part, a legislative response to this gap between perception and reality.

2.5 Act 167 (2022): The Diagnostic Mandate

Vermont passed Act 167 in the immediate aftermath of the COVID-19 pandemic, when the financial fragility of Vermont's hospital system had become impossible to ignore. The act had three core directives: commission a comprehensive statewide hospital systems analysis; direct AHS and GMCB to develop a federal multi-payer payment model proposal; and mandate design work on hospital global budgets. The three directives are analytically linked — you cannot design a credible global budget without understanding each hospital's actual cost structure and community role, and you cannot negotiate a federal model without a clear picture of total cost of care and where it diverges from benchmarks.

2.5.1 The Oliver Wyman Process: A Template for System-Level Reform

GMCB contracted with Oliver Wyman at a fixed price of $1,050,000 for work running from August 2023 through September 2024. The scale of the engagement was unusual for a state of Vermont's size: over 230 meetings across all 14 Hospital Service Areas, involving more than 3,100 participants from over 100 organizations. Meeting types included 57 sessions with hospital leadership and boards, 45 sessions with state partners, 50 public HSA-level community meetings averaging 68 participants each, 35 provider meetings, and 15 sessions specifically designed to reach diverse populations whose voices are typically absent from healthcare planning processes.

The political economy of this process deserves study because it is a template for system-level reform in any context. Publishing recommendations to close or restructure named services at named community hospitals is not analytically difficult — it is politically treacherous. Communities organize around their local hospitals. Hospital closures generate intense local resistance. The Vermont process managed this by front-loading community engagement before recommendations, establishing the community meetings not as forums for defending the status quo but as inputs to a shared diagnosis. This sequencing — listen first, diagnose second, recommend third — produced findings with enough public legitimacy to survive the inevitable political backlash from hospitals whose specific services were named for restructuring or closure.

The methodology also reflects a design philosophy worth naming explicitly: the report moved from hospital-centered point solutions to systematic redesign; from individual stakeholder consultation to community-driven process; and from post-crisis remedy to pre-planned transformation. These are not rhetorical preferences — they reflect Oliver Wyman's learned lesson from failed hospital reform processes elsewhere, where solutions designed around individual institutional interests rather than community needs predictably failed to achieve systemic change.

2.5.2 The Five Act 167 Goals as an Accountability Framework

Act 167 established five statutory goals for Vermont's healthcare transformation that have since become the accountability framework for everything that followed: reduce inefficiencies; lower costs; improve health outcomes; reduce health inequities; and increase access to essential services.

These goals matter not as aspirational language but as operational accountability criteria embedded in subsequent reporting and regulatory requirements. AHS's monthly reports to the legislature track progress on these five dimensions. The GMCB's hospital budget review process references them. The Rural Health Transformation Program application explicitly maps each proposed initiative to them. When Act 68's Health Care Delivery Advisory Committee develops the Statewide Strategic Plan, it will be measured against them. This is how statutory goal-setting works when properly designed: not as a preamble, but as a persistent accountability structure that outlives the bill that created it.

VERMONT IN PRACTICE — Oliver Wyman's Priority Action Lists for 2025

The Oliver Wyman report concluded with explicit priority action lists for each actor in Vermont's governance system — one of the most operationally specific outputs of any state healthcare consulting engagement on record. These lists were published in September 2024; their implementation status one year later is a direct measure of Vermont's reform momentum.

Priority legislative changes for the Vermont General Assembly:

- Remove barriers to building affordable housing for Vermont residents and newcomers
- Approve funding for EMS transformation
- Expand broadband coverage to rural areas
- Review existing AHS agency structure and program list to identify overlaps and efficiency opportunities

- Develop state regulations for Rural Emergency Hospital and free-standing birthing center designations
- Expand professional licensure and practice scope for nurses, EMTs, and pharmacists
- Regionalize specialty care services across hospitals
- Professionalize and regionalize EMS
- Improve care coordination for heavy utilizers — elderly, mental health, neuro-divergent, and foster care populations
- Develop dual-eligible targeting, care planning, and coordination
- Coordinate and optimize statewide electronic medical record systems
- Permit no further increases in commercial subsidization for hospital financial shortfalls
- Refrain from licensing any further hospital-based outpatient department units
- Simplify and shorten the Certificate of Need process
- Encourage free-standing diagnostic, ambulatory surgical, and birthing centers
- Begin movement to reference-based pricing, ideally at 200% of Medicare or less for PPS hospitals
- Require all hospitals to use the same accounting agency and method for budget submissions

The alignment between this list and what Act 68 subsequently mandated is not coincidental. The Oliver Wyman recommendations are the intellectual source material for Act 68's operative provisions.

2.6 The Oliver Wyman System Redesign Blueprint

The Oliver Wyman report's most detailed section — running from pages 32 through 114 of the full document — provides a three-tier system redesign framework that any healthcare system undertaking structural transformation can apply. The framework organizes recommendations at the state level, the system level, and the individual hospital level, recognizing that each tier requires different interventions executed by different actors on different timelines.

2.6.1 State-Level Recommendations: Infrastructure and Governance

At the state level, Oliver Wyman identified four domains of foundational infrastructure that hospital transformation cannot succeed without, regardless of payment model or clinical redesign. These recommendations are addressed primarily to AHS and the Vermont General Assembly, not to hospitals themselves.

The first domain is governance clarity. Oliver Wyman found that AHS and GMCB are under-resourced for the transformation work they are being asked to lead, that roles between the two agencies need clarification, and that the process requires a dedicated project management office and facilitation support. The specific recommendation to add a Division of Planning and Effectiveness at GMCB — to calculate the impact of service site changes on hospital budgets, monitor access and affordability of community providers, and track quality, access, and equity metrics — is a significant organizational design recommendation that has direct implications for AHS's own restructuring mandate under Act 68.

The second domain is AHS structural alignment. Oliver Wyman recommended that AHS align its agency sub-units with Hospital Service Areas and meet regularly with hospitals and providers in each HSA. It also recommended that AHS fully computerize and integrate its sub-units — an implicit diagnosis that AHS's current organizational structure impedes the kind of coordinated population health management that a transformed system requires. This recommendation is the foundation of the AHS restructuring chapter developed later in this book.

The third domain is state-level investments in Social Determinants of Health: specifically housing, transportation, and broadband. Oliver Wyman was unusually direct about including these as healthcare system recommendations rather than separate social policy recommendations — reflecting the systems analysis that showed housing shortage driving staffing crisis driving cost escalation. The implication is that AHS's healthcare reform mandate cannot be separated from its housing, transportation, and economic development responsibilities. This is a governance challenge as much as a policy challenge.

The fourth domain is administrative simplification: streamlining prior authorization, shortening the CON process, simplifying Medicaid billing, and upgrading IT infrastructure and interoperability. Oliver Wyman documented the direct cost of administrative complexity — a cumbersome CON process prevents cost-effective contracts from being secured in time, and fragmented IT systems mean a provider may need to contact 31 separate EMS agencies to arrange a patient transfer.

2.6.2 System-Level Recommendations: Regionalization

At the system level, the Oliver Wyman blueprint calls for a fundamental redesign of how Vermont's 14 hospitals relate to each other and to the communities they serve. The core concept is regionalization: rather than each hospital attempting to provide a full spectrum of services to its local population — a model that is increasingly unsustainable given population density and physician shortage — Vermont should develop regional Centers of Excellence (COEs) in specific specialties, concentrating volume where it can sustain clinical expertise, while ensuring access through improved transportation and telehealth.

Oliver Wyman developed specific COE assignments for 10 of Vermont's 13 current Hospital Service Areas across 24 specialty categories. The University of Vermont Medical Center received 16 COE designations, covering all major specialties except acute general surgery, mental health emergency department, pediatric psychiatry, and skilled nursing. Southwestern Vermont Medical Center received 9 COE designations. Most community hospitals received between 1 and 6 designations in specialties where they have sufficient volume and expertise to sustain clinical excellence.

Hospital	COE Count	Key Specialties
UVM Medical Center (Burlington)	16	Cancer (complex), neurology, women’s health, rehabilitation, most surgical specialties
Southwestern VT Medical Center (Bennington)	9	Cancer surgery (non-complex), geriatric care, orthopedics, psychiatry (adult/adolescent), radiation therapy
Central VT Medical Center (Barre)	5	Geriatric care, infusion therapy, neurology, psychiatry (adult), radiation therapy
Brattleboro Memorial Hospital	6	Acute general surgery, cancer surgery, geriatric care, orthopedics, robotic surgery
Northwestern Medical Center (St. Albans)	4	Cancer surgery (non-complex), infusion therapy, neurology, radiation therapy
Northeastern VT Regional Hospital (St. Johnsbury)	4	Cancer surgery (non-complex), infusion therapy, minimally invasive surgery, radiation therapy
Rutland Regional Medical Center	5	Acute general surgery, geriatric care, minimally invasive surgery, neurology, psychiatry (adult)
Springfield Hospital	2	Memory care, psychiatry (adult)
Copley Hospital (Morrisville)	2	Orthopedics, rheumatology
Mt. Ascutney Hospital (White River Junction)	1	Rehabilitation
Grace Cottage, Gifford, North Country, Porter	TBD	Further discussion required as part of regionalization plan

Table 2.3 — Proposed Centers of Excellence assignments by hospital. Source: Oliver Wyman regionalization blueprint.

The COE framework serves multiple goals simultaneously. Concentrating surgical and specialty volumes creates the patient throughput needed to maintain clinical excellence — a surgeon performing 200 procedures a year maintains skills that one performing 20 cannot.

Concentrating volumes in financially viable hospitals reduces the total number of facilities that need to sustain high-cost infrastructure. And freeing smaller hospitals from the expectation that they must provide full-spectrum care allows them to redesign their role — becoming hubs for primary care, mental health, geriatric services, and community health rather than trying to maintain services they cannot staff or afford.

The system redesign blueprint also calls for a fundamental shift in the geography of care: move all care possible out of hospitals. Community-based care, primary care, mental health care, and housing capacity should all increase to divert care to lower-cost settings. Home-based care and mobile care programs should expand. Ambulatory surgical units and free-standing diagnostic centers should replace hospital-based equivalents for low-risk procedures. The savings from this shift — estimated at over $300 million in indirect savings over five years — come from delivering the same clinical value at a fraction of the cost of hospital-based provision.

2.6.3 Hospital-Level Recommendations: Repurposing or Closure

At the individual hospital level, Oliver Wyman provided the most politically sensitive recommendations in the report: several hospitals are at risk of closing their inpatient beds and should consider repurposing their facilities and clinical staff through one of several designated models. The three primary repurposing options are the Rural Emergency Hospital designation (providing 24/7 emergency services and observation without inpatient beds), the Community Ambulatory Care Center (outpatient and primary care services without inpatient capacity), and the Care at Home support program (community-based services without a hospital facility).

Oliver Wyman was explicit that these recommendations are not about abandoning communities — they are about matching the level and configuration of facility to actual community health need and financial sustainability. A community that cannot sustain a full inpatient hospital but has reliable access to a Regional Specialty Center via improved EMS and transportation is better served than one that maintains an understaffed, financially distressed inpatient facility that cannot reliably staff its emergency department or maintain physician expertise across specialties.

The report also identified specific operational improvements for hospitals that will remain inpatient facilities: UVMMC was specifically called out to examine its overhead and administrative costs, particularly the proportion of providers supporting non-patient-care activities. Reducing exceedingly high administrative costs at some hospitals was identified as one of the three primary sources of direct savings — contributing to the more than $100 million in direct savings projected from administrative consolidation and shared services.

VERMONT IN PRACTICE — The $400M Transformation Savings Estimate

Oliver Wyman projected that full implementation of the hospital transformation recommendations would yield more than $400 million in direct savings over five years — independent of savings from payment reform activities like AHEAD. The savings sources break down as follows:

- Close inpatient facilities with unsustainable financials, whose deficits would otherwise require escalating cash infusions
- Reduce hospital administrative costs, which are exceedingly high at some facilities and have been driving budget increases system-wide
- Reduce costs through synergies — shared services and staff across individual hospitals
- Deliver same procedures in ambulatory surgical units and outpatient settings, where low-risk procedures can be performed at substantially lower cost
- Expand access to primary care settings to manage patient needs, reducing preventable emergency department visits
- Address social determinants of health, reducing inpatient stays and ED visits that are avoidable if housing and transportation needs are met upstream

These savings can be reallocated to improve Vermont's healthcare system outside of hospital-based care: investing in community-based care (primary care, home health), investing in social needs programs (housing, mental health), and bending the trend of rising health insurance premiums.

The $400M figure is significant in context: Vermont's Rural Health Transformation Program award of $195M over five years represents less than half of the projected savings from structural transformation alone. The capital investment is smaller than the structural problem it is meant to help solve — which is why AHS has been clear that the RHT funding is a tool for executing reform, not a substitute for reform.

2.7 Act 68 (2025): The Operational Mandate

Act 68 Key Statutory Deadlines

When	Milestone
FY2027 (Oct 2026)	Reference-based pricing mandatory for all commercial payers
January 2028	AHEAD Model Cohort 2 launches; Medicare FFS hospital global budgets begin
FY2028 (Oct 2027)	Global hospital budgets for non-Critical Access Hospitals
December 2028	Statewide Health Care Delivery Strategic Plan delivered to Vermont Legislature
FY2030 (Oct 2029)	Global hospital budgets for ALL Vermont hospitals including CAHs

Figure 2.1 — Act 68 statutory implementation timeline. Source: Act 68 of 2025.

Signed by Governor Scott on June 12, 2025, Act 68 is the legislative operationalization of the Oliver Wyman diagnosis. The alignment between Oliver Wyman's priority recommendations and Act 68's operative provisions is direct and traceable: Oliver Wyman recommended beginning movement to reference-based pricing at 200% of Medicare or less; Act 68 mandated it by hospital fiscal year 2027. Oliver Wyman recommended moving to global budgets when conditions for success are met; Act 68 required them for non-critical access hospitals by FY2028 and all hospitals by FY2030. Oliver Wyman recommended that GMCB add a Division of Planning and Effectiveness; Act 68 authorized three new permanent classified positions at GMCB. Oliver Wyman recommended reviewing AHS agency structure and program list; Act 68 directed AHS to develop a Statewide Health Care Delivery Strategic Plan by December 2028.

The act's purpose statement is worth reading in full because it encodes the Oliver Wyman framework into law. The General Assembly declared the purpose of Act 68 to be transformation of and structural changes to Vermont's health care system, advancing five goals: improvements in health outcomes and reducing access disparities; an integrated care system with robust coordination and increased investment in primary care, home health, and long-term care; stabilizing providers and controlling costs beginning with reference-based pricing and continuing to global hospital budgets; evaluating progress through standardized accountability metrics; and establishing a system that attracts and retains high-quality health care professionals. Every one of these goals is a direct response to a finding in the Oliver Wyman report.

2.7.1 The Reference-Based Pricing Architecture

Act 68's RBP provisions represent a significant departure from the voluntary, incentive-based payment reform approach that dominated American health policy for the previous fifteen years. The act does not invite hospitals to experiment with alternative pricing — it directs GMCB to establish, by rule, maximum amounts that hospitals shall accept as payment in full, not later than hospital fiscal year 2027. The language is mandatory and statewide.

The underlying price problem that RBP addresses is more severe than most national discussions of hospital pricing acknowledge. GMCB's February 2026 report documented that Vermont hospitals charge commercial payers approximately 250-300% of Medicare rates on average — but this average conceals extraordinary variation. Outpatient imaging at some Vermont hospitals reaches 715-944% of Medicare rates. Inpatient mental health services at the same hospitals range from 123% to 409% of Medicare. UVMMC outpatient charges specifically averaged 417% of Medicare rates in 2022 — among the highest in the nation. Hospitals are not clustered near a rational break-even point of 136% of Medicare; they are distributed across a range from roughly profitable to wildly extractive, with no relationship between price and quality.

The design principles embedded in Act 68's RBP statute reflect the cross-state evidence base. Prices must be based on a percentage of Medicare reimbursement — establishing a transparent, nationally comparable anchor. Consideration must be given to each hospital's specific community context: payer mix, labor costs, social risk factors, and role in Vermont's system. And hospitals are explicitly prohibited from charging patients or insurers more than the established reference-based price, closing billing workarounds that undermined earlier price transparency experiments.

State / Program	Key design features and outcomes
Vermont Act 68	Mandatory statewide maximum allowable payments through GMCB rate-setting authority. Methodology set by rule FY2027, effective FY2028. RBP then reviewed annually as part of hospital budget process.
Oregon (2019)	State employee plan only. 200% in-network / 185% out-of-network cap. Exemptions for rural and critical access hospitals. Savings exceeded $107.5M in first 27 months. All 24 affected hospitals remained in-network.
Montana (2016)	State employee plan. Direct contracting at percentage of Medicare. Initial $48M in savings but politically fragile without statutory mandate — illustrating Vermont's choice to legislate rather than demonstrate.
Washington (2021)	Public option ('Cascade Care'). Hospital reimbursement capped at 160% of Medicare for participating networks. Voluntary participation, expanding to state employee plans.
Maryland (2014–present)	All-payer system with fixed annual revenue for hospitals based on standardized prices and volume expectations. Includes Medicare waiver. Strong incentives for population health management. The closest model to Vermont's Act 68 trajectory.

Table 2.2 — State reference-based pricing and rate-setting programs compared. Sources: state program documentation; RAND; GMCB.

2.7.2 The Global Budget Mandate

Act 68 requires GMCB to establish global hospital budgets for non-critical access hospitals by hospital fiscal year 2028, and for all Vermont hospitals by 2030. This statutory obligation makes Vermont the first U.S. state outside Maryland on a legislatively mandated path to a comprehensive all-payer global budget system.

Understanding why Vermont needs global budgets in addition to reference-based pricing requires understanding the balloon squeeze problem that Oliver Wyman identified. Reference-based pricing controls unit prices but does not control volume. A hospital facing lower per-service revenue can compensate by delivering more services — scheduling more imaging, performing more procedures, admitting patients who could be managed in outpatient settings. Total hospital spending equals price times volume; controlling only price leaves volume as a release valve. Global budgets close this loophole by capping total hospital revenue regardless of service volume, fundamentally changing the hospital's financial incentive from maximizing throughput to maximizing population health per dollar of revenue.

Vermont has attempted global budget design four times in the past decade: the GMCB net patient revenue cap in 2012, the Rutland Regional pilot proposed in 2014 (never implemented), the Vermont All-Payer ACO Model from 2017 to 2025, and the current Act 167/Act 68 mandate. Each prior attempt foundered on the same obstacle: voluntary participation. When payers can opt out of a global budget arrangement, the highest-cost, most commercially valuable services migrate out of the capped system, leaving lower-reimbursing services behind and making the global budget financially unworkable. Act 68 eliminates the voluntary option. This is the critical design difference between Vermont's current trajectory and everything that preceded it.

2.7.3 The Statewide Health Care Delivery Strategic Plan and AHS Restructuring

The most consequential long-term provision of Act 68 is the requirement that AHS develop a Statewide Health Care Delivery Strategic Plan, to be delivered to the legislature by December 2028 and updated every three years thereafter. This plan is the statutory vehicle for answering the foundational question that underlies all of Vermont's reform work: what should Vermont's healthcare delivery system actually look like?

Act 68 established the Health Care Delivery Advisory Committee to support the plan's development, with responsibilities including setting affordability benchmarks, monitoring system performance, advising on the continuous evaluation process for the healthcare delivery system, and working with AHS to develop and maintain the strategic plan. The committee's mandate also requires incorporating recommendations from the Vermont Steering Committee for Comprehensive Primary Health Care — ensuring that primary care's central role in the redesigned system is structurally embedded in the planning process.

Oliver Wyman's state-level recommendations are the most detailed public blueprint available for what this strategic plan should address. The four domains Oliver Wyman identified — governance clarity, AHS structural alignment with HSAs, state-level SDOH investments, and administrative simplification — constitute the operational dimensions of the strategic plan. The AHS restructuring chapter later in this book develops a full six-pillar framework for how AHS should approach this planning mandate, drawing directly on both the Oliver Wyman recommendations and the Act 68 statutory requirements.

VERMONT IN PRACTICE — The Reform Cascade Timeline — Statutory Deadlines

For healthcare leaders, policymakers, and organizations doing business with Vermont's system, the following statutory deadlines define the transformation calendar:

- GMCB hospital budget guidance includes negative 1% commercial rate growth benchmark — the first negative benchmark in Vermont history
- AHS hospital transformation grants of $2M under Act 68; all 14 hospitals developing transformation plans with RHRC support
- Vermont Medicaid global budget for hospitals begins January 2026 under AHEAD Cohort 2 State Agreement
- Act 55 outpatient drug cap at 120% of ASP effective April 2025, generating estimated $135M in annual savings
- Reference-based pricing for hospital services implemented — maximum amounts hospitals can charge commercial payers, set by GMCB rule
- GMCB publishes hospital price transparency dashboard
- RBP vendor selection and methodology finalized; rulemaking completed
- Global hospital budgets established for non-critical access hospitals
- AHS Statewide Health Care Delivery Strategic Plan delivered to legislature by December 2028
- AHEAD Model Cohort 2 full operation; Medicare FFS hospital global budgets active
- CMMI transformation grants of $1.2M available through AHEAD Cooperative Agreement Year 3
- Global hospital budgets for all Vermont hospitals, including critical access hospitals

These deadlines are not aspirational. They are statutory obligations. Organizations planning capital investments, contract negotiations, workforce strategies, or technology deployments that interact with Vermont's hospital system must treat these dates as certainties, not contingencies.

2.8 The Rural Health Transformation Program: Capital for an Operational Strategy

In November 2025, AHS submitted Vermont's application to the federal Rural Health Transformation Program — a $50 billion national fund created by the One Big Beautiful Bill Act of 2025, with $10 billion available annually from 2026 through 2030. Vermont was awarded $195 million in December 2025, nearly double what the state had expected to receive and among the highest per-capita awards nationally.

The RHT application is itself a teaching document for healthcare transformation practice, because it is structured entirely around the Act 167 and Act 68 goals. Vermont's vision, stated explicitly in the application, is to ensure that rural residents have access to the right care, at the right time, in the right place, at an affordable cost — a direct echo of the AHS health care reform vision statement. The application's three strategic goals map onto Oliver Wyman's three imperatives: strengthening hospital sustainability and primary care (Move all care possible out of hospitals); building workforce pipeline and retention capacity (Build housing and fix the conditions that drive workforce shortage); and leveraging innovative strategies to reduce costs (Pay with reference-based pricing and move to global budgets).

AHS Secretary Samuelson's framing of the award is the correct analytical frame: the $195 million is not Vermont's healthcare reform plan. It is capital to execute a plan that already exists. The funding is explicitly one-time money, and AHS has stated publicly that it must be treated as a supplement to ongoing reform work, not a lifeline. The distinction between investment capital and reform strategy is one of the most persistently confused in health policy. States that treat federal grants as reform strategies — spending the money on programs without addressing underlying structural problems — predictably return to crisis when the grant period ends. The Vermont approach, which explicitly aligns every RHT initiative to the statutory Act 167 and Act 68 goals, is a model of how capital investments should be positioned within a structural reform agenda.

The specific initiatives funded through Vermont's RHT award include telehealth expansion for specialty care access in rural hospitals, workforce recruitment and retention programs with five-year service obligations, hospital analytics infrastructure to support regional transformation planning, dialysis capacity development for nursing home residents, and CCBHC expansion to certify five additional community behavioral health clinic entities by July 2026. Each of these investments addresses a specific node in Oliver Wyman's systems failure map — and each is designed to reduce ongoing structural costs rather than subsidize their continuation.

2.9 Cross-State Lessons: What Vermont's Model Generalizes

Vermont's reform architecture is instructive because it addresses structural problems present, in varying degrees of severity, across rural America. The analytical framework Oliver Wyman applied, and the legislative mechanisms Vermont enacted in response, generalize well to any state or regional system facing demographic aging, hospital financial fragility, workforce shortage, and the failure of voluntary payment reform to achieve systemic change.

Five generalizable lessons emerge from the Vermont case.

First, diagnosis precedes legislation. Vermont's Act 68 is as effective as it is because it was preceded by Act 167's comprehensive, public, community-grounded diagnostic process. Legislatures that skip the diagnostic phase and proceed directly to structural mandates produce policies that are technically sound but politically unsustainable. Vermont's sequence — diagnose publicly, build shared understanding, then mandate — is a process design that other states should study.

Second, voluntary reform achieves voluntary compliance. Vermont's decade-long experience with voluntary global budget arrangements — the All-Payer ACO Model, OneCare participation, VITL connectivity — produced partial, fragile outcomes precisely because voluntary participation allows the highest-cost actors to opt out. The Oliver Wyman report documented this plainly in its perception-versus-reality analysis. Act 68's mandatory architecture is a direct response to that experience.

Third, price control without volume control is insufficient. Reference-based pricing is necessary but not sufficient for controlling total hospital spending. The Act 68 sequence — RBP first, then global budgets — reflects the analytical understanding that price and volume must both be addressed. Oregon's RBP experience, Montana's experience, and Maryland's global budget model all confirm this.

Fourth, housing and transportation are healthcare infrastructure. Oliver Wyman's finding that housing shortage drives staffing crisis drives cost escalation is not a soft social observation — it is a hard systems analysis. Healthcare reform strategies that treat housing and transportation as separate social policy domains rather than healthcare infrastructure will fail to achieve the workforce stability and access improvements they require.

2.10 Work This Chapter on the Platform

Chapter 2's legislative architecture becomes concrete when you model it. The platform lets you stress-test the policy choices Vermont made — mandatory vs. voluntary participation, waiver design, and the budget-neutrality math CMS requires.

Do this	On this tool	What to look for
Model an 1115 waiver or all-payer global budget design against a pre-loaded state scenario	**Policy Simulator** — /research-lab/policy-quality?tab =policy	Compare a "more funding, same structure" path against a structural-reform path — the Oliver Wyman three-imperatives test.
Trace how a statutory mandate cascades into operational deadlines	**Vermont Act 167** — /vermont-act-167 and **Act 68** — /vermont-act-68	Map each statutory deadline to the pillar it gates.

Figure 2.H — Hands-on platform tools for the Policy Pillar.

2.11 Implications for You

If you are a Vermont hospital executive: The single most important thing Act 68 requires of you is not compliance — it is preparation. RBP at 200% of Medicare is not a theoretical target; it is a statutory mandate effective FY2027. The question is not whether it happens but whether your organization has modeled what it means for your revenue, your payer mix, and your cost structure before it happens. If you have not run an Act 68 financial scenario analysis — what your net revenue looks like under mandatory RBP across all commercial lines — that analysis is overdue. The hospitals that run it early have time to respond. The hospitals that do not will be responding to a financial reality they did not see coming.

If you are an AHS or GMCB official: The RBP methodology is being written now, and the equity constraint must be embedded in the methodology — not reviewed at the approval stage. The Health Equity Advisory Commission consultation required by Act 68 should be happening alongside the methodology design, not after it. A methodology that passes equity review at the approval stage is better than one that fails it. But a methodology designed with equity constraints from the beginning is better than both, because it does not produce the redesign costs and delays that equity review at the approval stage inevitably generates.

If you are a Vermont legislator: The mandatory architecture of Act 68 — RBP, global budgets, the December 2028 Strategic Plan — is only as durable as the political will to enforce it. UVMMC challenged GMCB enforcement in court and lost. Future challenges are certain. The legislature's role is to make clear that enforcement is not discretionary, that the GMCB has the resources and authority to defend its orders, and that the December 2028 deadline for the Strategic Plan requires a binding plan — not a descriptive one. The difference between a transformation that succeeds and one that produces documents is often determined by the legislature's willingness to accept only the real thing.

If you are a national health policy professional: Vermont's Act 68 is the most detailed mandatory all-payer pricing statute enacted by any state in the modern regulatory environment. Before designing comparable legislation for other states, study what Act 68 got right — the mandatory RBP, the global budget timeline, the equity review requirements — and what remains untested: the December 2028 plan specificity requirement, the enforcement mechanism for plan non-compliance, and the interaction between federal Medicaid cuts and the state's mandatory pricing timeline. Vermont will have answers to these questions by 2030 that no model projection can provide.

Fifth, the AHS restructuring is the strategic plan. Oliver Wyman's explicit recommendation that AHS align its sub-units with Hospital Service Areas, add a Division of Planning and Effectiveness, and restructure to better coordinate health and social service needs at the community level is the organizational design specification for what Act 68's Statewide Strategic Plan must produce. The book develops this specification fully in the AHS Restructuring chapter that follows.

2.12 Key Concepts Introduced or Expanded in This Chapter

Platform for Change

Oliver Wyman's term for the five interconnected challenge domains — decreasing affordability, deteriorating sustainability, demographic trap, interconnected failure system, and equity gaps — that collectively make structural transformation unavoidable for Vermont.

The three imperatives

Oliver Wyman's highest-level summary of what Vermont must do: build housing and fix transportation; pay PPS hospitals with reference-based pricing and move to global budgets; move all care possible out of hospitals.

Center of Excellence (COE)

A hospital or facility designated to concentrate volume and expertise in a specific specialty, receiving referrals from other hospitals and ensuring that patient volumes are sufficient to maintain clinical quality. Vermont's regionalization plan designates COEs across 24 specialty categories at 10 of 14 current Hospital Service Areas.

Balloon squeeze problem

The phenomenon in which controlling hospital prices without controlling volume allows hospitals to compensate for lower per-service revenue by delivering more services. Reference-based pricing controls price but not volume; global budgets are required to address both simultaneously.

Statewide Health Care Delivery Strategic Plan

The plan AHS is required to deliver to the Vermont legislature by December 2028 and update every three years, establishing the vision, goals, affordability benchmarks, and accountability metrics for Vermont's transformed health care delivery system. Established by Act 68.

Hospital Service Area (HSA)

A geographic area defined around a hospital and the population it primarily serves. Vermont has 14 HSAs corresponding to its 14 hospitals. The HSA is the fundamental unit of analysis for Vermont's regionalization and transformation planning.

Rural Emergency Hospital (REH)

A federal designation allowing a hospital to provide 24/7 emergency services and observation care without maintaining inpatient beds — an option for Vermont hospitals that cannot sustain full inpatient capacity but need to maintain emergency access for their communities.

Sources: Act 167 of 2022 (Vermont); Act 51 of 2023 (Vermont); Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations, September 2024 (revised October 2024 and January 2025); Green Mountain Care Board, Act 68 Update on Hospital Reference-Based Pricing and Global Budgets, February 17, 2026; Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application, November 3, 2025; Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, presented to Vermont House Health Care Committee, January 31, 2025; Green Mountain Care Board, 2024 Annual Report, January 15, 2025.

Chapter 3: The Policy Pillar in Practice — CMMI Models, Waiver Strategy, and the Federal-State Interface

> *The implementation of Vermont's mandatory policy architecture reveals how legislative intent becomes operational reality — and where the gaps between statute and execution create the most consequential risks.*

A state statute is not the same thing as a federal policy environment, and the gap between the two is where most reform agendas lose momentum. CMMI model design, Section 1115 waiver negotiation, and prior authorization reform are the three federal-facing tools through which a state's domestic mandate either gets amplified by federal authority or runs into its limits. Every state pursuing structural reform has to navigate this same federal-state interface — the specific models and waiver numbers differ, but the negotiation dynamics, the budget-neutrality constraints, and the administrative-burden problem are common across states. Vermont's AHEAD agreement and waiver history are this chapter's worked example of a negotiation every reforming state eventually has.

Understanding what CMMI models work and why, how 1115 waivers are negotiated, and how H.R. 1 changes the Medicaid landscape — the practitioner framework for navigating federal policy.

3.1 From Policy Architecture to Policy Practice

Chapter 3 established Vermont's legislative reform architecture — the Acts, the mandates, the timelines, and the analytical foundation of the Oliver Wyman diagnosis. This chapter turns from architecture to practice: how do healthcare organizations, state agencies, and policymakers actually navigate the federal policy landscape that shapes every transformation decision?

The federal policy environment is not a monolithic force that organizations respond to passively. It is a complex, evolving, and frequently internally contradictory set of programs, rules, incentives, and administrative interpretations that skilled policy practitioners can navigate strategically. Understanding the CMMI model landscape, the mechanics of Medicaid section 1115 waivers, the structure of APM contracts, and the federal-state relationship that determines whether state-level reform efforts are supported or blocked by federal requirements is not academic knowledge — it is operational necessity for any organization or state agency working on healthcare transformation in 2026.

3.2 The CMMI Landscape — What Exists, What Works, and What Has Been Discontinued

The Center for Medicare and Medicaid Innovation has been the primary federal vehicle for payment reform experimentation since its creation under the ACA. Over fifteen years, CMMI has launched more than fifty models, invested billions of dollars, and engaged thousands of healthcare organizations across the country. The results have been mixed in ways that carry specific lessons for organizations designing their own value-based care strategies.

3.2.1 The CMMI Track Record — An Honest Assessment

A 2023 independent evaluation of CMMI models found that the vast majority had not achieved statistically significant reductions in Medicare spending or improvements in quality. Most models were discontinued. The models that showed the most promise — ACO REACH (formerly Direct Contracting), AHEAD, the Making Care Primary model — share a set of characteristics that distinguish successful from unsuccessful CMMI designs.

Characteristic	What it means	Models with this characteristic	Models without it
Mandatory or near-mandatory participation	When high-cost, high-volume providers cannot opt out, the model's financial logic applies to the full population. Voluntary models allow opt-out by the actors who benefit most from the status quo.	AHEAD (for participating states, all hospitals must participate). Maryland All-Payer Model (mandatory for all Maryland hospitals). Vermont Act 68 (mandatory for all Vermont hospitals).	Most CMMI ACO models. MSSP Track 1. Bundled Payments for Care Improvement (voluntary). Most demonstration projects.
Two-sided risk that is real	Financial downside for underperformance is sufficient to change organizational behavior. One-sided shared savings with minimal upside and no downside does not change hospital or health system decision-making at scale.	ACO REACH (mandatory downside risk). Global budgets (hospitals bear population health risk directly). AHEAD (state-level accountability with meaningful consequences).	MSSP Track 1 (upside only for most participants). Most early CMMI bundled payment models with modest downside. Primary Care First (limited financial exposure).

Characteristic	What it means	Models with this characteristic	Models without it
Adequate implementation support	Organizations need technical assistance, analytical tools, and sufficient runway to redesign care delivery before being held financially accountable for outcomes they cannot yet control.	AHEAD (multi-year pre-performance period with substantial cooperative agreement funds). Vermont's RHRC engagement. Making Care Primary (practice transformation support).	Many early CMMI models that launched with insufficient operational support and were discontinued when providers could not implement effectively.
Alignment with state regulatory environment	Federal models that conflict with or duplicate state regulatory structures create compliance complexity without adding value. Models designed in partnership with state agencies produce better results.	AHEAD (designed explicitly as federal-state partnership; requires state agreement and state regulatory alignment). Vermont-specific APM (state-federal co-design). Maryland TCOC (built on Maryland's existing HSCRC regulatory authority).	Federal models imposed on states without coordination with state Medicaid agencies, insurance regulators, or hospital budget review processes.

Figure 3.1 — CMMI model success factors: what distinguishes models that produce results from those that don't. Source: HTR analysis of CMMI evaluation literature; CMS model documentation.

3.2.2 The Current Active CMMI Model Landscape (2026)

As of 2026, the CMMI model portfolio has been significantly rationalized from its peak breadth under prior administrations. The models with the highest relevance to Vermont-style transformation are:

Model	Key features and relevance
AHEAD (Achieving Healthcare Efficiency through Accountable Design)	Six-state voluntary total cost of care model. Vermont in Cohort 2 (begins January 2028). Hospital global budgets for Medicare FFS. Enhanced primary care payments. $150M/year additional Medicare funds for Vermont. The most relevant model for state-level transformation.

Model	Key features and relevance
ACO REACH (Realizing Equity, Access, and Community Health)	Medicare FFS ACO model with mandatory two-sided risk. Full-risk capitation option. Equity focus including SDOH requirements. For organizations managing attributed Medicare populations outside of a state-level model, ACO REACH is the highest-accountability current federal ACO option.
Making Care Primary (MCP)	Multi-payer primary care transformation model in 8-state initial cohort. 10-year model. Three-stage progression from basic to advanced primary care capabilities. States not participating in AHEAD may find MCP a viable primary care transformation vehicle.
BALANCE Model	GLP-1 obesity drug coverage for Medicare and Medicaid enrollees at negotiated prices. Voluntary for manufacturers, states, and plans. Medicaid launch May 2026; Medicare Part D January 2028. Critical equity and chronic disease management model.
Bundled Payments for Care Improvement Advanced (BPCI-A)	Episode-based payment for acute and post-acute care episodes. Voluntary. For health systems and post-acute providers, BPCI-A remains a viable two-sided risk VBC entry point for specific service lines (joint replacement, cardiac, etc.).

Table 3.1 — CMMI models most relevant to Vermont's transformation. Source: CMS Innovation Center.

3.3 Section 1115 Waivers — The State's Primary Federal Policy Tool

Section 1115 of the Social Security Act gives CMS authority to waive certain Medicaid requirements and provide federal matching funds for experimental programs that promote program objectives. For states attempting comprehensive healthcare reform, the 1115 waiver is the most powerful federal policy tool available — enabling states to design Medicaid programs that would not be permitted under standard federal rules, to test new benefits and delivery models, and to demonstrate approaches that, if successful, can become national policy.

3.3.1 What 1115 Waivers Can and Cannot Do

Understanding the scope and limits of 1115 waiver authority is essential for any state or organization involved in Medicaid reform. The following framework organizes the most common 1115 waiver applications:

3.3.2 1115 Waiver Authorities: What States Can Accomplish

Coverage expansions beyond standard Medicaid categories — including populations not otherwise eligible, non-traditional benefit structures, and premium assistance programs that allow states to use Medicaid funds toward employer-sponsored insurance.

Vermont's Global Commitment to Health demonstration extends coverage and benefits beyond standard Medicaid state plan requirements, enabling HRSN (health-related social needs) services, expanded developmental disability benefits, and substance use disorder treatment models not available under the standard state plan.

Delivery system and payment reform — enabling states to implement global budgets, ACO arrangements, and alternative payment models for Medicaid that are not permitted under standard Medicaid FFS or managed care rules.

Vermont's alignment of Medicaid global budgets with AHEAD is executed through the Global Commitment to Health demonstration, which provides CMS authority to implement the Medicaid component of Vermont's hospital global budget design.

SDOH investments — since 2021, CMS has permitted states to use 1115 waiver funds for health-related social needs services, including housing supports, food assistance, and transportation, for Medicaid enrollees at risk of institutionalization or with complex social needs.

Vermont's recent 1115 amendment approval authorizes HRSN services — enabling Blueprint's universal SDOH screening to connect to Medicaid-funded services, not just community resources.

Work requirement programs — subject to ongoing legal and policy uncertainty. H.R. 1 (2025) explicitly authorizes work requirements for Medicaid expansion enrollees, resolving the prior legal uncertainty in favor of state authority. Vermont has not implemented work requirements and has not signaled intent to do so.

3.3.3 The Vermont 1115 Waiver Portfolio

Vermont operates several 1115 demonstrations that together constitute its Medicaid transformation infrastructure. The Global Commitment to Health demonstration — Vermont's primary 1115 waiver — was originally approved in 2005 and has been renewed and amended repeatedly. The current authorization runs through December 2027, with the AHEAD alignment provisions approved in 2024. Vermont's AHEAD Medicaid global budgets for hospitals, beginning January 2026, operate under this demonstration authority.

Vermont's 1115 portfolio illustrates a principle worth stating explicitly: 1115 waivers are negotiated instruments. They are not applications that CMS approves or denies based on a checklist; they are the product of sustained federal-state negotiation in which the state advocates for the policy design it wants and CMS negotiates conditions, monitoring requirements, and budget neutrality standards that protect the federal government's financial interests. Vermont's ability to implement its current AHEAD-aligned Medicaid policy is the direct product of years of negotiation that produced a federal-state agreement on methodology, accountability, and withdrawal conditions. States that engage with CMS as partners in designing innovative demonstrations get better waiver terms than states that submit standard applications.

3.3.4 Budget Neutrality — The Binding Constraint

Every 1115 waiver is subject to a budget neutrality requirement: the federal government will not spend more under the demonstration than it would have spent under the standard program. Budget neutrality is calculated against a 'without-waiver' baseline — what Medicaid spending would have been if the state had not implemented the demonstration — and must be maintained over the demonstration period.

Budget neutrality is simultaneously the most important financial discipline in 1115 waiver design and the most technically complex. The without-waiver baseline is inherently uncertain — it requires projecting what would have happened counterfactually. States and CMS frequently negotiate over baseline methodology. Vermont's experience with AHEAD budget neutrality negotiations — where the methodology for calculating the Medicare global budget baseline was a specific condition of the state's AHEAD participation — illustrates that these technical negotiations have major financial consequences. Getting the baseline right (or wrong) can determine whether a waiver is financially sustainable for the state or produces unexpected federal financial pressure.

BEYOND VERMONT

Every state pursuing a 1115 waiver or CMMI model faces the same budget-neutrality negotiation Vermont faced for AHEAD — CMS will not approve a state policy whose federal cost is uncertain or one-sided, and the counterfactual baseline is always contestable. A state with no all-payer claims database (Vermont's VHCURES) will face this negotiation with a weaker evidentiary position, not a different negotiation. The lesson that travels is not the specific AHEAD numbers but the sequencing implication: building the data infrastructure to support a credible baseline argument is itself a prerequisite for favorable waiver terms, which is one more reason the Technology pillar precedes the Economics pillar in the six-pillar sequence.

3.4 Prior Authorization Reform — The Administrative Pillar of Policy Strategy

Prior authorization — the requirement that providers obtain health plan approval before delivering certain services — is one of the most significant drivers of administrative burden, care delay, and provider frustration in American healthcare. The average physician practice spends 14.9 hours per week on prior authorization tasks. Prior authorization denials delay care for 33% of patients and cause some to abandon treatment entirely. The administrative cost of prior authorization nationally is estimated in the tens of billions of dollars annually.

Prior authorization reform is a Policy pillar issue because it requires either federal rulemaking or state legislation to change meaningfully. Individual payers reforming their own prior authorization processes voluntarily have limited impact; the burden arises from the aggregate of PA requirements across dozens of payers, each with different criteria, forms, and approval timelines.

3.4.1 Federal Prior Authorization Reform: What H.R. 1 and CMS Rules Require

The Improving Seniors' Timely Access to Care Act, passed in 2022, required Medicare Advantage plans to implement electronic prior authorization, transparency reporting, and expedited review timelines. CMS implemented rules in 2024 requiring payers to respond to expedited PA requests within 72 hours and standard PA requests within 7 days — a significant improvement from previous norms where standard reviews could take weeks.

H.R. 1 (2025) included additional prior authorization reform provisions: public reporting of PA denial rates by service type, required disclosure of clinical criteria used in PA decisions, and stronger external appeal rights for denied requests. These are incremental improvements, not transformational changes. Full elimination of PA for evidence-based, guideline-concordant care — the policy that would most reduce administrative burden — remains aspirational.

3.4.2 Vermont's Prior Authorization Reform — Act 167's Provisions

Act 167 explicitly included prior authorization reform as one of its covered domains, directing AHS to develop simplified prior authorization processes and directing GMCB to report on prior authorization burden and denial rates. Vermont's Health Care Advocate has documented PA denial rates and challenged insurance rate filings that include excessive PA complexity costs. Act 68 builds on this by requiring insurers to demonstrate that administrative cost structures, including PA programs, are consistent with medical loss ratio requirements.

Oliver Wyman's community engagement findings are explicit about the real-world impact: providers described the CON process and prior authorization as major barriers to efficient care delivery, with cost-effective contracts unable to be secured in time due to administrative friction. Vermont's transformation agenda treats administrative simplification — including prior authorization reform — as an Operations pillar requirement that enables every other transformation initiative.

3.5 The State-Federal Policy Interface — Navigating the Relationship

For state health agencies and the organizations that work with them, the federal-state policy relationship is simultaneously a resource and a constraint. Federal funding through Medicaid, CMMI models, HRSA grants, and the RHT Program provides the capital that makes transformation possible. Federal requirements — budget neutrality, CMMI model conditions, Medicaid state plan requirements — constrain the policy designs states can pursue. Federal policy uncertainty — created by administrative transitions, legislative changes like H.R. 1, and rulemaking processes that can reverse course — creates planning challenges for states with multi-year transformation timelines.

3.5.1 Managing Federal Policy Risk — Vermont's Approach

Vermont's AHEAD State Agreement exemplifies sophisticated federal policy risk management. The agreement includes specific conditions under which Vermont can unilaterally withdraw from AHEAD — before launch, with 30-day notice; after launch, with 6-month notice. The agreement includes conditions that protect GMCB's regulatory independence. It includes explicit provisions about methodology acceptance and dispute resolution. Vermont negotiated these withdrawal rights precisely because a nine-year commitment to a federal model involves significant policy risk — the model could change under a future administration, the methodology could produce unexpected financial outcomes, or federal policy changes could make AHEAD participation financially unsustainable.

Vermont's approach — engage the federal government as a partner, negotiate specific protections against adverse policy changes, maintain clear withdrawal rights, and design state-level statutory authority (Act 68) that does not depend on federal model continuation — is the right model for any state undertaking long-term federal-state policy commitments. States that design transformation strategies that are entirely dependent on continued federal program participation are strategically exposed; states that use federal models to accelerate and fund state-designed transformation that has independent statutory authority are more resilient.

3.5.2 The Policy Strategy Framework for Healthcare Organizations

For healthcare organizations navigating the policy environment — hospitals, health systems, payers, physician groups — the Policy pillar generates three distinct strategic functions: regulatory compliance (understanding and meeting mandatory requirements), opportunity identification (finding federal and state programs that can fund or accelerate transformation goals), and advocacy (shaping policy in directions favorable to the organization's mission and the populations it serves).

Function	Key activities	Vermont-specific examples	HTR Advisory service
Regulatory compliance	Monitor rulemaking; assess financial and operational impact of new requirements; develop compliance roadmaps; manage comment letter processes	Act 68 RBP compliance planning for FY2027; AHEAD hospital global budget participation; H.R. 1 Medicaid work requirement implementation timeline	Policy Strategy service line: Phase 1 Landscape and Exposure Assessment
Opportunity identification	Track CMMI model launches and eligibility; identify state waiver opportunities; evaluate RHT Program grant eligibility; assess AHEAD primary care participation options	Vermont hospitals: Act 68 transformation grants ($2M); AHEAD EAST Fund; RHT Program subrecipient grants; BALANCE Model GLP-1 coverage	Policy Strategy service line: federal program opportunity assessment
Policy advocacy	Participate in public comment on proposed rules; engage state legislature on transformation legislation; provide testimony on hospital financial sustainability; participate in GMCB public meetings	Vermont hospital leaders: engagement with AHS transformation planning; community input on COE designations; participation in Health Care Delivery Advisory Committee public comment	Policy Strategy service line: Phase 3 Implementation Support — legislative testimony and comment letter drafting

Figure 3.2 — Policy strategy functions for healthcare organizations. Source: HTR Advisory framework.

3.6 The Medicaid Policy Landscape — What H.R. 1 Changes and What It Does Not

H.R. 1 (One Big Beautiful Bill Act, July 2025) — Key Healthcare Provisions

- $911 billion in Medicaid cuts over 10 years — the largest reduction in federal health spending in American history.
- 10 million additional Americans projected uninsured by 2034 (CBO estimate).
- Work requirements (80 hours/month) for Medicaid expansion enrollees beginning late 2026.
- Eligibility redetermination every 6 months instead of annually — increased coverage churn.

RHT funds ($50B) expire 2030; Medicaid cuts accelerate after 2030. The math is unforgiving.

The 2025 reconciliation law has changed the Medicaid landscape in ways that every healthcare organization and state agency must understand. The following framework distinguishes between provisions already in effect, provisions taking effect in late 2026 and 2027, and provisions whose implementation timeline remains uncertain.

H.R. 1 Medicaid provision	When it takes effect	Financial impact	Vermont-specific implications
Provider tax restrictions: ban on new or higher provider taxes in expansion states; phase-down of existing taxes	In effect for new taxes; phase-down beginning 2027	Reduces states' ability to finance their Medicaid matching share; intensifies pressure on state Medicaid budgets	Vermont's financing mechanisms will need review; state budget implications require modeling through 2031
State-directed payment caps at 100% of Medicare (expansion states)	2027	Reduces enhanced Medicaid payments that many states use to supplement provider rates above standard Medicaid levels; significant for safety-net hospitals	Vermont hospitals receiving state-directed supplemental payments face revenue reduction; GMCB global budget design must account for this
Work and community engagement requirements (80 hours/month)	Late 2026 (states must implement by December 31, 2026)	10 million projected uninsured nationally by 2034; variable state impact depending on implementation;	Vermont has not expressed intent to implement work requirements; federal law now clearly authorizes them. Coverage implications

H.R. 1 Medicaid provision	When it takes effect	Financial impact	Vermont-specific implications
		administrative burden on states and enrollees	for Vermont's expansion population require monitoring.
Eligibility redetermination every 6 months (vs. annual)	2026	Increases administrative burden; produces coverage churn as enrollees who remain eligible lose coverage due to administrative failures; disproportionate impact on working families	Vermont DVHA will face increased redetermination workload; outreach to ensure eligible Vermonters maintain coverage is an equity priority
Medicaid reimbursement reductions (DSH cuts, payment rate changes)	2026-2031 (phased)	$911B over 10 years nationally; $137B in rural areas; hospitals will experience revenue reductions	All 14 Vermont hospitals face reduced Medicaid revenue; most severe impact on CAHs with high Medicaid payer mix; integrated into hospital financial sustainability modeling

Figure 3.3 — H.R. 1 Medicaid provisions: timeline, financial impact, and Vermont implications. Sources: KFF analysis; CBO estimates; Pew Charitable Trusts (January 2026).

For Vermont specifically, the Medicaid revenue reductions create a financial stress that compounds the hospital financial crisis Oliver Wyman documented before H.R. 1 passed. Vermont's hospitals were already projecting 5-year cumulative deficits of $700 million to $2.4 billion under the pre-H.R. 1 baseline. The additional Medicaid revenue reductions accelerate the financial timeline for hospitals that were already financially distressed, and reduce the financial cushion for hospitals that were marginally sustainable. This is the fiscal environment in which Vermont's transformation must succeed — which makes the urgency of the transformation not just a policy preference but a fiscal survival requirement.

3.7 Policy Strategy for Healthcare Leaders — The Practitioner Framework

Policy strategy for healthcare leaders is not primarily about lobbying or advocacy — though those functions matter. It is primarily about anticipation: understanding what the policy environment will require of organizations 18-36 months from now, positioning the organization to comply at manageable cost, and identifying the policy opportunities that can be converted into strategic advantage.

3.7.1 The Policy Monitoring Framework

Every healthcare organization needs a policy monitoring infrastructure proportionate to its size and regulatory exposure. The minimum viable policy monitoring framework includes four functions:

- Federal rulemaking surveillance: Tracking proposed rules from CMS, ONC, and other relevant agencies through the Federal Register notice and comment process. Final rules typically take effect 60 days after publication, but proposed rules give 60-90 days of advance notice that organizations can use for impact assessment and comment letter preparation.
- State legislative monitoring: Tracking legislation that affects the organization's operating environment through the state legislative session. Vermont's pattern of annual healthcare legislation — each session producing significant new requirements or opportunities — means that organizations not engaged in the legislative process are consistently surprised by its outputs.
- CMMI model pipeline monitoring: Tracking CMMI model announcements, application deadlines, and eligibility criteria. New CMMI models typically have narrow application windows; organizations that are not monitoring for them miss the opportunity to participate on favorable terms.
- Regulatory intelligence: Monitoring GMCB decisions, CMS guidance documents, and state agency program updates that affect payment and operating requirements. Vermont's GMCB publishes hospital budget guidance, RBP methodology updates, and program decisions that directly affect every Vermont hospital's financial planning.

3.7.2 The Policy Impact Assessment Framework

When a new policy is identified — a proposed rule, a new CMMI model, a state legislative development — the organization needs a structured impact assessment process. The Policy Strategy service line uses a four-phase engagement structure (described in Chapter 3), but the core analytical logic can be applied internally:

- Exposure mapping: What specifically does this policy change require of our organization? What are the financial, operational, and compliance implications? What is the timeline?
- Scenario analysis: Under what range of outcomes (final rule differs from proposed rule, implementation delayed, model parameters change) does our strategic position change?
- Response strategy: What is the optimal response — compliance roadmap, comment letter participation, CMMI model application, advocacy engagement?

3.8 Work This Chapter on the Platform

Chapter 3 moves from policy architecture to federal-state practice. These tools quantify the waiver and Medicaid-policy decisions the chapter analyzes.

Do this	On this tool	What to look for
Estimate Medicaid coverage loss under H.R. 1's work-requirement and eligibility provisions	**Work Requirements Calculator —** /research-lab/policy-quality?tab=medicaid-wr	The gap between nominal exemptions and real-world coverage loss from administrative churn.
Model the H.R. 1 funding cliff for a state's Medicaid program	**H.R. 1 Cliff Scenario —** /research-lab/policy-quality?tab=hr1-cliff	How federal policy shifts the financial assumptions every downstream pillar depends on.
Test eligibility outcomes for a specific household	**Medicaid Eligibility Simulator —** /medicaid-eligibility-simulator	Where coverage transitions create gaps the clinical and equity pillars must absorb.

Figure 3.H — Hands-on platform tools for the Policy Pillar in practice.

3.9 Implications for You

If you are a Vermont hospital executive: Vermont's participation in AHEAD as a Cohort 2 state means your organization will be operating under a CMMI total-cost-of-care model beginning January 2028. The single most consequential preparation step is HCC documentation accuracy. Under AHEAD's risk adjustment methodology, your global budget benchmark will be set based on your attributed population's risk scores. A population with incomplete HCC documentation will receive a benchmark that underestimates its true risk — and your organization will be held financially accountable for costs the benchmark does not reflect. Start your HCC gap closure program now, not in 2027.

If you are an AHS or GMCB official: Vermont's AHEAD implementation plan must address the gap between CMS's performance start date (January 2028) and the AHS-GMCB analytics vendor deployment timeline. AHEAD's predecessor — OneCare Vermont — failed partly because organizations assumed financial risk without the analytics to manage it. Vermont cannot repeat that failure at the system level. The analytics vendor selection should be treated as a critical-path decision with a hard deadline, not a standard procurement timeline.

If you are a Vermont legislator: Vermont's AHEAD agreement creates federal financial accountability beginning January 2028. The legislature's oversight role includes ensuring AHS has the resources to implement AHEAD support infrastructure — especially the analytics tools and the HSA Coordinator positions — before performance accountability begins. Appropriating for these positions is not optional; it is the enabling condition for the federal investment Vermont has committed to.

If you are a national health policy professional: Vermont's AHEAD experience will be the most detailed real-world test of whether a mandatory state payment architecture can improve outcomes under a CMMI total-cost-of-care model. The prior evidence is mixed — most CMMI models have underperformed projections. What is different about Vermont is the mandatory participation, the global budget layer beneath AHEAD, and the RBP price constraint that prevents high-cost providers from offsetting AHEAD risk through commercial rate increases. Watch Vermont's first AHEAD performance year results carefully. They will be more informative than any model projection.

Stakeholder communication: Who needs to know about this policy change, and what decisions does it require from the board, executive team, clinical leadership, and financial team?

3.10 Key Concepts

Section 1115 waiver

Authority under the Social Security Act allowing CMS to waive standard Medicaid requirements and approve experimental programs that promote program objectives. Vermont's Global Commitment to Health demonstration is its primary 1115 vehicle.

Budget neutrality

The federal requirement that 1115 waiver spending not exceed what Medicaid spending would have been without the waiver, calculated against a negotiated without-waiver baseline.

CMMI (Center for Medicare and Medicaid Innovation)

Federal entity created by the ACA to test new payment and delivery models. Has launched 50+ models since 2010; AHEAD is the most relevant current model for state-level transformation.

Two-sided risk

A payment arrangement in which the organization shares in both savings (upside) and losses (downside) relative to a financial target. Mandatory downside risk changes organizational behavior more effectively than one-sided shared savings.

Prior authorization

Health plan requirement for pre-approval before delivering specified services. Oliver Wyman documented PA complexity as a major administrative burden for Vermont providers; Act 167 and Act 68 both address PA simplification.

Making Care Primary (MCP)

CMMI's 8-state multi-payer primary care transformation model. A potential vehicle for states not participating in AHEAD seeking federal support for primary care investment.

Federal Register

The official journal of the U.S. federal government, where CMS and other agencies publish proposed rules (open for public comment) and final rules (legally binding). Essential monitoring source for healthcare policy practitioners.

Sources: CMS CMMI model documentation; CMS 1115 waiver framework; KFF Medicaid policy analyses; H.R. 1 One Big Beautiful Bill Act (July 2025); Vermont Global Commitment to Health demonstration (CMS approval January 2025); Act 167 of 2022; Act 68 of 2025; AHEAD State Agreement (January 2025); American Medical Association prior authorization burden survey; Georgetown Center for Children and Families.

Chapter 4: The Technology Pillar — Data Infrastructure for a Transformed Health System

> *Vermont's data infrastructure investment — VHCURES, VITL, and the AHS-GMCB analytics platform — is building the operating system that mandatory payment reform requires to function.*

Three data assets recur in every state's transformation infrastructure, whatever they happen to be called locally: a claims database that lets regulators and payers see total cost of care across the system, a health information exchange that lets clinicians see a patient's record across organizations, and an analytics capability that turns both into something a hospital or payer can act on. Without these three, global budgets cannot be monitored, care coordination cannot be measured, and equity gaps cannot be tracked — which is why Technology precedes Economics, Clinical, and Equity in the six-pillar sequence. Vermont's VHCURES, VITL, and AHS-GMCB analytics platform are this chapter's worked example of what building (and struggling to fully integrate) these three assets looks like in practice.

> *You cannot manage what you cannot measure. Vermont's VHCURES, VITL, and the AHS-GMCB analytics vendor are the data substrate on which every other pillar's management functions depend.*

4.1 The Technology Pillar: Why Data Infrastructure Is the Foundation of Everything Else

4.1.1 Vermont's Health Data Infrastructure — Three Layers

Layer	Role in the data stack
Layer 1 — VHCURES (All-Payer Claims Database)	Foundation for total-cost-of-care measurement and HEDIS stratification. Gap: race/ethnicity data incomplete in commercial claims.
Layer 2 — VITL / VHIE (Health Information Exchange)	Clinical data substrate — diagnoses, labs, medications, care plans. Voluntary participation means not all providers are connected.
Layer 3 — AHS-GMCB Analytics Vendor (procurement underway)	Modeling layer for global budget design, TCOC tracking, risk stratification, and equity monitoring. AHEAD launches January 2028.

Figure 4.1 — Vermont's three-layer health data infrastructure. Source: GMCB VHCURES documentation; Vermont HIE Strategic Plan.

There is a precise sense in which the Technology pillar is the precondition for every other pillar. Reference-based pricing requires knowing what each hospital actually charges for each service. Global budgets require attributing patients to hospitals and tracking total cost of care across the year. Clinical quality improvement requires measuring the gap between current performance and evidence-based targets. Health equity requires stratifying outcomes by race, income, and geography. Operational transformation requires modeling the financial and access impact of structural changes before implementing them.

All of these functions require data — specifically, data that is timely, complete, accurate, linked across clinical and claims sources, accessible to the organizations that need it, and governed in ways that protect patient privacy while enabling population-level analysis. Vermont's current data infrastructure — anchored by VHCURES (the all-payer claims database), VITL (the health information exchange), and the Blueprint clinical data registry — is sophisticated by national standards for a state of its size. It is also, in its current form, insufficient for the management demands of a system operating under global budgets and a mandatory Statewide Strategic Plan accountability framework.

This chapter develops Vermont's technology pillar comprehensively: what the current infrastructure does and does not do, where the critical gaps are, what the RHT Program is funding to close them, and what the national technology landscape — FHIR interoperability, ambient clinical intelligence, remote patient monitoring, AI diagnostic tools — means for Vermont's specific circumstances. It concludes with a technology architecture framework that Vermont's Strategic Plan should adopt and that any state-level transformation can use as a reference model.

4.2 VHCURES — Vermont's All-Payer Claims Database

The Vermont Health Care Uniform Reporting and Evaluation System (VHCURES) is Vermont's all-payer claims database, managed by the Green Mountain Care Board with Onpoint Health Data as the data aggregation vendor. VHCURES has been operational in its current form since 2013 (when GMCB assumed responsibility from the Department of Financial Regulation) and is the primary analytical foundation for Vermont's healthcare regulatory functions.

4.2.1 What VHCURES Contains and What It Can Do

VHCURES is a substantial data asset that gives Vermont analytical capabilities most states lack. Understanding both its capabilities and its limitations is essential for anyone designing Vermont's technology infrastructure.

Commercial population covered 60% Of commercially insured Vermonters — partial due to ERISA self-insured employer exemption	Government payer coverage 100% Of Medicaid and Medicare enrollees in Vermont
Data types Claims + Rx Medical and pharmacy claims, plus eligibility and demographic data	Years available per extract Up to 5 Eligibility and paid claims; data lag typically 12-18+ months

Figure 4.1 — VHCURES data coverage summary. Source: GMCB VHCURES documentation.

VHCURES enables Vermont to conduct analyses that directly support the transformation agenda: population-level total cost of care measurement (the GMCB publishes all-payer TCOC dashboards derived from VHCURES); potentially avoidable utilization analysis (the 32.3% avoidable ED visit rate in AHS's transformation reports is calculated from VHCURES via VUHDDS); hospital price transparency analysis (the GMCB's 2026 RBP report used VHCURES-derived 2024 claims data to show hospital prices ranging from 279% to 697% of Medicare for outpatient services); and prescription drug affordability analysis (GMCB's 2025 report on drug pricing used VHCURES to model the impact of IRA drug negotiations on Vermont spending). This is a powerful analytical platform that has produced some of the most credible and consequential health policy analysis in recent Vermont history.

4.2.2 VHCURES Limitations — Where the Gaps Are

VHCURES has four structural limitations that matter directly for Vermont's transformation management needs. Oliver Wyman identified the data infrastructure as one of the barriers to coordinated transformation, and the specific limitations below explain why.

Limitation	What it means for transformation	Required improvement
Incomplete commercial coverage (60%)	The 40% of commercially insured Vermonters in ERISA self-insured employer plans — many of them higher-income, higher-resource patients — are not in VHCURES unless their employer voluntarily submits. Total cost of care calculations and population health analyses are systematically missing a significant portion of the commercially insured population. Under federal law, self-insured	Voluntary submission incentives; federal ERISA reform advocacy; use of commercial insurer aggregate data for the covered portion; flag analyses as partial in all public reporting

Limitation	What it means for transformation	Required improvement
	employers cannot be required to submit data to state APCDs.	
Data lag of 12-18+ months	The most recent VHCURES data typically reflects conditions 12-18 months prior to analysis. The November 2025 AHS transformation report explicitly acknowledged that most available measures reflect 2023 or earlier data. Under global budgets, where hospitals need to understand their population health performance in near-real-time to manage within their budget cap, this lag makes VHCURES insufficient as a management tool.	AHEAD Model requires more timely data reporting; RHT Program analytics investments; GMCB FY27 hospital budget guidance requires hospitals to report more frequently; push for 60-90 day claims lag instead of 12-18 months
No behavioral health or SDOH data integration	VHCURES contains medical and pharmacy claims but not mental health and SUD clinical records (governed by 42 CFR Part 2 and state confidentiality laws), social service utilization data, housing status, or other SDOH indicators. The HIE Steering Committee has a statutory directive to create one health record per person integrating claims, clinical, and SDOH data — but this integration has not been implemented.	VHCURES-VHIE integration per HIE Steering Committee statutory mandate; Act 167 HIE provisions; SDOH data collection through Blueprint HRSN screening linked to VHCURES identifiers with appropriate consent framework
Voluntary provider participation in HIE	VITL's VHIE receives clinical data from hospitals but participation by community-based providers, mental health agencies, skilled nursing facilities, and other post-acute providers is voluntary and incomplete. Oliver Wyman specifically cited VITL as lacking	Act 68 hospital HIE connectivity requirement is a start; AHEAD requires primary care participation; RHT Program telehealth investment includes interoperability requirements;

Limitation	What it means for transformation	Required improvement
	pharmacy claims data and not being viewed as user-friendly. Act 68 requires hospitals to maintain HIE connectivity but does not mandate all provider types.	progressive connectivity incentives for non-hospital providers

Figure 4.2 — VHCURES structural limitations and required improvements. Sources: GMCB VHCURES data documentation; Oliver Wyman Act 167 Report; AHS Transformation Reports; Act 68 of 2025.

VHCURES Perception vs. Reality — Oliver Wyman's Finding

Oliver Wyman's perception-versus-reality analysis identified Vermont's data infrastructure as one of the areas where the gap between apparent capability and actual capability is most consequential. The relevant finding:

- Perception: Vermont has a State Health Information Exchange (VITL).
- Reality: Participation in VITL is voluntary and many community-based providers are not included. VITL lacks pharmacy claims data and is not viewed as 'user friendly' by providers.

This matters because Vermont's transformation strategy depends on care coordination between hospitals and community providers — and care coordination requires data sharing. A care coordinator trying to manage a patient transitioning from a hospital inpatient stay to a community mental health program cannot access the clinical data they need if the mental health agency is not connected to VITL. A primary care practice trying to close chronic disease management gaps cannot see the specialty care their patient received at a different facility if that facility's data is not in the HIE.

Vermont's technology investments — the VHCURES-VHIE integration mandate, the RHT Program interoperability grants, the AHEAD data sharing requirements — are all aimed at closing this gap between VITL's nominal existence and the functional interoperability Vermont's care coordination model requires.

BEYOND VERMONT

The gap Vermont is closing — between a data asset existing on paper and being functionally usable for population health management — is the single most common technology-pillar failure nationally. Many states have an all-payer claims database, a health information exchange, or both, that are technically operational but functionally underused because of data lag, incomplete participation, or lack of analytics capacity layered on top. The diagnostic question for any organization is not "do these systems exist?" but "can a hospital or payer act on this data inside the decision window that matters?"

> — and Vermont's VITL integration timeline is a useful benchmark for how long that gap typically takes to close even with statutory mandates and dedicated funding behind it.

4.3 The Vermont HIE — VITL, the Governance Shift, and the Integration Imperative

Vermont's Health Information Exchange (VHIE) is operated by Vermont Information Technology Leaders (VITL), an independent nonprofit designated by statute as the exclusive operator of the statewide HIE network. Every hospital in Vermont contributes records to the VHIE. The VHIE operates a provider portal (VITLAccess) and a query service that allows providers to access patient records from other facilities. Vermont has been building this infrastructure since 2009, when VITL was designated as the Regional Extension Center to help primary care providers adopt electronic health records.

4.3.1 The 2025 Governance Shift: Act 62 Transfers HIE Authority to DVHA

Effective July 1, 2025, Act 62 of 2025 made a significant governance change: responsibility for Vermont's statewide Health Information Technology Plan was transferred from the Green Mountain Care Board to the Department of Vermont Health Access (DVHA). Prior to this, GMCB had authority to review and approve the HIE Strategic Plan, VHIE Connectivity Criteria, and VITL's budget. Under Act 62, DVHA now coordinates Vermont's statewide HIT Plan in consultation with the HIE Steering Committee, with annual revisions submitted by November 1 and comprehensive updates every five years.

The governance rationale for this shift is significant: DVHA has direct operational relationships with healthcare providers through its Medicaid contracts, Blueprint for Health administration, and AHEAD Model management. Moving HIT plan responsibility to DVHA aligns the technology strategy with the healthcare delivery transformation authority. It also means that Vermont's AHEAD implementation — which requires robust data exchange for population health management and global budget monitoring — is now coordinated by the same agency managing the data infrastructure.

4.3.2 The Integration Mandate: One Health Record Per Person

The Vermont legislature gave the HIE Steering Committee an explicit statutory mandate that defines the technology pillar's central ambition for the next five years: create one health record for each person that integrates claims data, clinical data, mental health and substance use disorder services data, and social determinants of health data. This mandate is documented in state law and in AHS's own letters to the legislature. It is the most ambitious data integration goal any state has formally committed to, and its achievement would give Vermont a population health management capability that no other state currently possesses.

The practical path to this integration has three components that must be executed in sequence:

Step	What it requires	Timeline	Current status
1. VHCURES-VHIE claims-clinical integration	Merge VHCURES claims data with VHIE clinical data using a privacy-compliant patient matching methodology. This gives every provider record access to both the claims history (what care was delivered, at what cost, through what payers) and the clinical record (diagnoses, medications, lab results, care plans).	DVHA leading; AHEAD Model requires this for population health management; target 2026-2027	HIE Steering Committee has a statutory directive to include data integration strategy in HIE plans; VHCURES-VITL linkage already used for Blueprint HEDIS measurement; full bidirectional integration not yet operational
2. Mental health and SUD data integration	Add mental health and substance use disorder data to the integrated record, with appropriate consent frameworks that comply with 42 CFR Part 2 (federal SUD confidentiality rules) and Vermont's own privacy statutes. This is technically complex — SUD data has a separate consent regime from medical data — but clinically essential.	2027-2028; requires federal 42 CFR Part 2 compliance pathway	DVHA working with VITL on consent policy; 2025 federal regulatory changes to 42 CFR Part 2 may simplify compliance pathway; behavioral health CCBHC expansion creates urgency
3. SDOH data integration	Add social service utilization data — housing program participation, food assistance, transportation use, domestic violence services — linked to the health record with appropriate privacy protections. This is the	2028 and beyond; requires cross-agency data governance agreement	Blueprint HRSN screening generates SDOH data but it is not yet linked to VHCURES or VHIE at scale; AHS HSA Coordinator model (Chapter 3) provides the cross-agency governance vehicle

Step	What it requires	Timeline	Current status
	most politically sensitive and technically complex integration because it involves non-healthcare agencies and data that carries significant stigma risk.		

Figure 4.3 — Vermont's three-step path to integrated health records. Sources: Vermont HIE Steering Committee statutory mandate; AHS January 2025 Legislative Presentation; DVHA HIE governance documents.

4.3.3 The Statewide EHR Question — Vermont's Active Feasibility Assessment

Oliver Wyman's community meetings generated a clear finding about EHR fragmentation: providers cannot efficiently coordinate care across facilities when different hospitals run different EHR systems that do not share data effectively. Vermont's 14 hospitals currently use four different EHR vendors — Epic (dominant at UVMMC), Oracle Health (formerly Cerner), TruBridge (formerly CPSI, used by smaller community hospitals), and Meditech — plus various additional software for laboratory management, pharmacy, and other functions.

AHS has commissioned a feasibility assessment to evaluate the cost-benefit of implementing a statewide electronic health record — a single EHR platform that all Vermont hospitals and primary care practices would use, enabling seamless data exchange as a byproduct of shared infrastructure rather than through complex interface work. Vermont's HIE Strategic Plan describes the assessment as evaluating 'what a statewide EHR would look like in line with the Act 167 Report.'

Case for a statewide EHR	Case against (or caution)
Eliminates interface complexity — data is naturally shared across providers on the same platform	Capital cost is substantial — major EHR implementations run $100-300M for large health systems; statewide scope amplifies this
Community meeting participants specifically cited EMR fragmentation as a barrier: 'EMR systems don't talk to each other and it's difficult to transfer from one center to another'	Disruption risk — forced EHR migrations have caused significant clinical workflow disruption and temporary quality problems at major health systems
Reduces per-provider IT infrastructure cost — smaller hospitals cannot afford full IT	FHIR-based interoperability (see Section 4) may make data exchange manageable without a single

Case for a statewide EHR	Case against (or caution)
departments; shared infrastructure is more efficient	EHR, reducing the case for the disruption of migration
Enables real-time population health data that VHCURES's 12-18 month lag cannot provide	Vendor lock-in — a single-vendor statewide EHR creates significant dependence on that vendor's pricing, contract terms, and long-term viability
Maryland's experience: common data infrastructure enabled the HSCRC to manage global budgets effectively	ERISA self-insured employers — outside state authority — would still not be in the system even with a statewide EHR
RHT Program IT advance funds can support the capital investment — this is an explicitly authorized use of RHT funds	Vermont's diverse provider landscape (hospitals, FQHCs, private practices, mental health agencies) may not all be able to adopt a single platform

Figure 4.4 — Vermont statewide EHR: case for and against. The feasibility assessment underway will weigh these factors against Vermont's specific cost, disruption tolerance, and FHIR interoperability capacity.

The feasibility assessment's most important analytical question is whether FHIR-based interoperability — the national standard for health data exchange that is rapidly maturing — can deliver the data sharing Vermont needs without the cost and disruption of a statewide EHR migration. If FHIR can deliver real-time, bi-directional data exchange across Vermont's four EHR platforms at an acceptable implementation cost, the case for a single statewide EHR weakens significantly. Section 4 develops the FHIR picture. AHS's feasibility assessment should produce a clear answer on this question by 2026-2027 — in time to inform the 2028 Strategic Plan.

4.4 The Vermont Clinically Integrated Network — A New Technology Institution

Vermont's Rural Health Transformation Program application includes one of the most significant technology investments in the entire $195M award: the creation of a Clinically Integrated Network (CIN) linking Vermont's hospitals and community providers into a coordinated clinical and data-sharing infrastructure. Vermont is one of four states (along with Arkansas, Nevada, and Oregon) that explicitly included CIN creation in their RHT applications, reflecting a national trend toward CINs as the organizational vehicle for rural health system integration.

4.4.1 What a Clinically Integrated Network Does

A CIN is a formal network of healthcare providers — hospitals, primary care practices, specialists, post-acute providers — that share clinical protocols, performance data, and care coordination infrastructure while maintaining independent ownership and governance. CINs occupy the space between full health system merger (which requires consolidation of ownership) and loose referral relationships (which have no formal data sharing or accountability). For Vermont's 14 independent, non-profit hospitals — which cannot and should not be merged into a single system — a CIN provides the coordination benefits of integration without the legal and political costs of ownership consolidation.

4.4.2 What the Vermont CIN Will Provide

Vermont's RHT Program application describes the Shared Services initiative — which includes the CIN infrastructure — as providing five specific functions for participating hospitals and providers:

- Analytics support: A shared analytics platform available to all hospitals and non-hospital providers for transformation and regionalization planning — identifying areas most in need, modeling the impact of service line changes, and tracking quality and cost metrics. This is the analytics vendor that AHS and GMCB are jointly procuring.
- Shared clinical protocols: Standardized clinical pathways for high-volume conditions — sepsis, heart failure, COPD, mental health crisis — that all participating providers adopt, enabling consistent quality regardless of which facility a patient uses.
- Group purchasing: Coordinated supply chain and group purchasing for medical supplies, equipment, and technology — capturing the scale economies that individual small hospitals cannot achieve independently. Oliver Wyman identified this as one of the highest-leverage near-term cost reduction opportunities.
- Administrative shared services: Shared infrastructure for billing, coding, credentialing, HR, and IT support — reducing the administrative overhead that drives Vermont's $1,303 per-discharge administrative cost premium over national benchmarks.
- Referral network formalization: Structured referral agreements between RSC hospitals and Tier 2/3 facilities for each COE specialty — ensuring that the regionalization blueprint operates as a reliable clinical network rather than a set of informal relationships.

The CIN also serves as the governance vehicle for the technology investments Vermont's transformation requires. Individual small hospitals cannot justify the capital investment in AI scribe technology, remote patient monitoring equipment, telehealth infrastructure, or advanced analytics platforms. A CIN can negotiate enterprise-wide technology contracts that make these investments affordable for the entire network, deploy shared technology infrastructure that individual providers access as a service, and establish common data standards that make the integrated health record vision achievable.

4.5 FHIR Interoperability — The National Standard and Vermont's Compliance Path

FHIR (Fast Healthcare Interoperability Resources) is the HL7 standard for healthcare data exchange that CMS mandated for payers and providers beginning in 2021 and 2022. FHIR defines a common data format and API structure that allows different health IT systems to exchange information without custom interfaces. It is the closest thing to a universal language for healthcare data that the industry has adopted, and its maturation over the past five years has fundamentally changed what is technically possible in health data exchange.

4.5.1 What FHIR Enables That Vermont Currently Cannot Do

Vermont's current health data infrastructure predates widespread FHIR adoption and was built around older data exchange standards. The operational implications are concrete:

- Patient matching across facilities: Without FHIR-based master patient index integration, Vermont still lacks reliable patient matching across all providers. Community meeting participants described needing to contact 31 separate EMS agencies for patient transfers — a fragmentation problem that a FHIR-enabled patient matching system would substantially reduce.
- Real-time care gap identification: FHIR-enabled EHR systems can query a patient's record at the point of care, identifying care gaps (missing preventive services, chronic disease management gaps) in real time during the clinical encounter. Vermont's current Blueprint data profile process delivers care gap information to practices, but with significant lag. FHIR enables the same information at the point of care.
- Attribution accuracy for global budgets: AHEAD's hospital global budgets require accurate attribution of patients to hospitals for Medicare FFS. FHIR-based data exchange enables more precise attribution by tracking care delivery across providers in near-real-time, reducing the attribution disputes that complicate global budget performance measurement.
- Population health management at scale: Vermont's Blueprint CHTs currently manage populations using a combination of claims data, clinical registry data, and provider-reported information. FHIR-based population health platforms can aggregate this data automatically, maintaining continuous patient registries that trigger care coordinator outreach when patients are overdue for care or when clinical indicators suggest increased risk.

4.5.2 Vermont's FHIR Compliance Status

Vermont's larger hospitals — particularly UVMMC with its Epic EHR — are FHIR-compliant for the CMS-mandated payer-to-patient and payer-to-payer API requirements. The critical gap is at smaller community hospitals (on TruBridge and Meditech platforms) and at community-based providers (mental health agencies, skilled nursing facilities, home health agencies) that lack FHIR-capable systems entirely.

Vermont's RHT Program IT advance investments address this gap directly. The program includes grants for technology upgrades at smaller and more rural facilities that cannot afford FHIR-capable EHR migrations independently. The CIN infrastructure also supports FHIR compliance by providing shared technical assistance, vendor negotiations for favorable pricing on FHIR-capable platforms, and shared technical staff who can support implementation at facilities that lack internal IT capacity.

FHIR compliance level	Vermont providers and current status
Full FHIR R4 compliance	UVMMC (Epic); Vermont hospitals on Oracle Health (Cerner) post-2022 upgrade; major commercial payers (BCBS VT, MVP)
Partial FHIR implementation	Central Vermont Medical Center; hospitals on Meditech Expanse (FHIR-capable but may require configuration)
Limited / no FHIR	Smaller community hospitals on TruBridge legacy; most Designated Mental Health Agencies; many skilled nursing facilities; independent primary care practices on older EHR versions
FHIR investment planned (RHT Program)	Rural and independent practices; smaller CAHs; targeted technical assistance through Vermont CIN shared services

Figure 4.5 — Vermont FHIR compliance landscape (estimated as of 2026). Sources: AHS Legislative Presentation January 2025; Vermont RHT Program Application; GMCB HIE connectivity criteria.

4.6 AI and Digital Health Technologies — Vermont's Targeted Investment Strategy

Vermont's RHT Program application is notable among the 47 state applications submitted in late 2025 for its specificity about technology investments. Rather than generic technology grants, Vermont's technology investment strategy is structured around five specific technology categories, each directly linked to an identified gap in Vermont's current system. This is the correct approach to one-time capital investment in technology: identify the specific operational problems, fund the specific technologies that solve them, and measure whether the problems are resolved.

4.6.1 AI Scribe Technology: Addressing the Documentation Burden Crisis

Vermont's RHT Program application explicitly includes grants for providers to purchase AI scribe technology, noting that smaller and more rural and independent practices are struggling with access to the same technology that larger practices can afford. The application cites that AI scribes automate an average of over two hours of daily administrative work by transcribing and processing clinician-patient conversations.

This is a direct response to a Vermont workforce problem: physician and clinician burnout driven by documentation burden is a primary contributor to provider attrition and is reducing Vermont's effective primary care capacity. A provider spending two hours daily on documentation has approximately 25% less time for patient care than one whose documentation is automated. At Vermont's scale — where the primary care FTE shortage is already projected at 370 by 2030 — recovering two hours per provider per day through AI documentation support is equivalent to adding significant additional clinical capacity without hiring new providers.

The evidence base for AI scribe technology is strong and growing. A 2024 quality improvement study published in JAMA Network Open found that ambient scribe tools were associated with improved documentation efficiency and reduced documentation burden. Clinicians used the technology spent less time writing notes during visits and reported less after-hours charting. Community providers at national rural health conferences have independently identified ambient AI as the single technology investment most likely to address workforce shortages while supporting billing processes and clinical decision-making simultaneously.

4.6.2 Remote Patient Monitoring: Managing Chronic Disease Outside the Hospital

Vermont's RHT Program application includes grants for remote patient monitoring (RPM) equipment for use in home health, primary care/specialty, and community paramedicine settings. RPM enables earlier interventions and ongoing monitoring for chronic conditions — congestive heart failure, COPD, diabetes, hypertension — in the home to prevent hospital admissions or readmissions.

The economics of RPM under global budgets are compelling. Under fee-for-service, RPM generates separate billable encounters and competes with in-person visits for reimbursement. Under global budgets, RPM is a cost-reduction tool: every hospital admission or readmission prevented by early RPM-triggered intervention saves revenue under the fixed budget. Vermont's aging population — with high rates of CHF, COPD, and diabetes among the 65+ cohort — is precisely the population where RPM produces the largest clinical and financial benefit.

Vermont's rural geography amplifies RPM's value. A patient in the Northeast Kingdom managing heart failure who needs twice-weekly monitoring faces a two-hour round trip to a clinic. RPM provides the same monitoring without the trip — better for the patient (who avoids travel costs and time), better for the provider (who can monitor more patients with existing staff), and better for the system (which reduces transportation-related access barriers that drive missed appointments and deteriorating chronic disease management).

4.6.3 Telehealth: Specialty Access for Rural Vermont

Vermont's RHT Program application includes specific telehealth investments to expand specialty care access in rural hospitals — allowing specialists at RSC facilities to provide consultation to patients at Tier 2 and Tier 3 facilities without requiring patient transfer. This is the technology enabler for Oliver Wyman's regionalization blueprint: rather than requiring every COE specialty to maintain physical presence at every community hospital, telehealth enables COE specialists to provide consultation across the network while patients remain near their homes.

Vermont's broadband landscape is a constraint: the RHT application notes that 80% of rural Vermonters have broadband access, compared to 85% in non-rural areas. The gap, while relatively small by national rural standards, means that home-based telehealth cannot reach all Vermont patients. Vermont's proposed broadband expansion (Oliver Wyman's third priority legislative change recommendation) is the infrastructure prerequisite for telehealth reaching the communities that need it most. The RHT Program's explicit inclusion of broadband infrastructure as an allowable use reflects this reality.

Vermont's telehealth strategy from the RHT application has four specific applications: specialty consultations (neurology, psychiatry, oncology) at rural hospitals; tele-ICU support for rural hospitals managing critical patients before transfer; tele-behavioral health for community mental health and CCBHC services; and community paramedicine — enabling EMTs and paramedics to provide extended care in patients' homes with real-time physician supervision via telehealth, reducing 911 calls and ED visits for non-emergency conditions.

4.6.4 Diagnostic AI: The Emerging Standard of Care

The book's Part V discussion of diagnostic AI maturation — FDA-cleared AI diagnostic devices for radiology, pathology, ophthalmology, and cardiology becoming standard of care in high-volume settings — applies directly to Vermont's context with one important distinction: Vermont's smaller rural hospitals may be the settings that benefit most from diagnostic AI, precisely because they lack the specialist volume to maintain expert human diagnostic capacity in every specialty.

A rural Vermont hospital with one radiologist who handles all imaging across multiple modalities cannot maintain the same expert pattern recognition as a high-volume academic center reading 50 chest CTs daily. AI-assisted radiology reading — which has demonstrated accuracy equal to or exceeding specialist readers for specific modalities like chest X-ray, mammography, and retinal imaging — can partially close this quality gap without requiring additional specialist recruitment. This is not a replacement for radiologists; it is a tool that makes existing radiologists more accurate and more efficient, particularly in the high-volume, lower-complexity reads that constitute the majority of rural hospital imaging.

Vermont's RHT IT advance funding can support diagnostic AI implementation at smaller hospitals that cannot fund these investments independently. The CIN's shared service model is an appropriate delivery mechanism: a network-wide diagnostic AI contract that all participating hospitals access, with shared quality monitoring and governance, would provide far better economics and oversight than 14 individual hospital technology decisions.

4.6.5 Price Transparency Technology

Vermont's RHT Program application includes a price transparency and insurance competition initiative — building the technology infrastructure to make hospital pricing legible to patients, payers, and regulators. GMCB was already developing a hospital price transparency dashboard as of February 2026 (first release forthcoming), providing the public with access to 2024 claims data showing hospital prices relative to Medicare. The RHT investment extends this infrastructure to enable patient-facing price comparison tools that allow Vermonters to see what a procedure costs at different facilities before choosing where to receive care.

Price transparency technology is a complement to reference-based pricing, not a substitute for it. RBP directly limits what hospitals can charge; price transparency helps patients and payers identify and act on pricing differences within the RBP framework. The combination — mandatory price caps plus transparent pricing information — creates the closest approximation to a functioning market for hospital services that any regulatory framework has achieved.

4.7 AI Governance — Vermont's Responsibility Before the Risk Arrives

The book's Part V discussion of AI governance as a clinical requirement — moving from a quality initiative to a regulatory requirement as FDA, ONC, and CMS develop AI governance frameworks — applies to Vermont with specific urgency. Vermont is making substantial AI investments through the RHT Program: AI scribe grants, diagnostic AI potential, RPM algorithms. Each of these involves clinical AI that has the potential to produce biased outputs, performance degradation over time, or harms to specific patient populations if not properly monitored.

Vermont does not currently have a statewide AI governance framework for clinical AI. Individual hospital systems have varying levels of AI governance maturity — UVMMC, with its Epic deployment and academic medical center sophistication, is likely more advanced than a smaller community hospital purchasing AI scribe software for the first time. The CIN infrastructure Vermont is building through the RHT Program creates the opportunity to establish a shared AI governance framework that applies consistently across all Vermont providers.

4.7.1 Vermont AI Governance Framework: Minimum Requirements

Drawing on the AI Governance Checklist and Vermont's specific clinical AI deployment context, Vermont's Statewide Strategic Plan should establish minimum AI governance requirements for clinical AI deployed in Vermont healthcare settings. At minimum, these should include:

- Pre-deployment bias testing: Any AI model used for clinical decision support, diagnostic assistance, or care gap identification must be tested for differential performance across race/ethnicity, age, sex, income, and geographic subgroups before deployment. Vermont's equity goals require that AI tools do not introduce or amplify health disparities.
- Performance monitoring: Deployed AI tools must have ongoing performance monitoring with predefined thresholds that trigger review or suspension. An AI model trained on national data may perform differently in Vermont's specific patient population — particularly in rural areas and among dual-eligible patients with complex social needs.
- Transparency requirements: Patients must be informed when AI tools are involved in their care. Clinicians must understand the basis and limitations of AI outputs. 'Black box' AI with no explainability is inappropriate for clinical settings regardless of its overall accuracy.
- Vendor accountability: Technology vendors providing clinical AI to Vermont providers must agree to performance monitoring, bias testing disclosure, and incident reporting. Contract terms must include performance standards and remedies for underperformance.
- Equity monitoring: AI performance must be specifically monitored for Vermont's highest-risk equity populations — Northeast Kingdom rural residents, Medicaid patients, elderly patients with complex needs, and patients whose primary language is not English.

Vermont's RHT Program AI scribe grants should include AI governance requirements as a condition of funding. The CIN's shared services function is the natural organizational home for a shared AI governance committee that monitors all AI deployments across Vermont's hospital network.

4.8 Vermont's Target Technology Architecture — A Framework for the Strategic Plan

Vermont's Statewide Health Care Delivery Strategic Plan must specify a technology architecture — the integrated set of data systems, exchange standards, governance structures, and analytical platforms that enable the transformed delivery system to be managed, monitored, and improved. The following architecture organizes Vermont's technology investments across five layers, from foundational infrastructure to advanced analytical applications.

Layer	Components
Layer 5 (Top) — Advanced Analytics & AI Applications	• Population health analytics platform (joint AHS-GMCB vendor): models transformation scenarios, evaluates hospital proposals, tracks outcome metrics • AI scribe & documentation automation: RHT grants; CIN enterprise contract • Diagnostic AI: network-wide deployment via CIN shared services with shared AI governance • Remote patient monitoring: RPM for CHF, COPD, diabetes; community paramedicine integration • Price transparency dashboard: GMCB public tool; patient-facing comparison via RHT
Layer 4 — Population Health Management	• Blueprint clinical data registry: HEDIS measurement, care-gap reporting, population risk stratification • Attribution management: patient-to-hospital attribution for AHEAD global budgets; PC AHEAD attribution • SDOH screening data: Blueprint HRSN results linked to health records • Care coordination registry: CHT patient registries; CCBHC care-management data
Layer 3 — Integrated Health Record (Target State)	• VHCURES–VHIE integration: claims merged with clinical data per HIE Steering Committee mandate • Mental health & SUD data integration: 42 CFR Part 2 compliant consent; CCBHC and DA data

Layer	Components
	• SDOH data integration: social-service utilization linked to the health record with privacy protections • Single patient identity: master patient index across all 14 hospitals and community providers
Layer 2 — Data Exchange Infrastructure	• Vermont Health Information Exchange (VHIE/VITL): clinical data exchange; connectivity mandated by Act 68 • FHIR R4 APIs: CMS-mandated payer and provider APIs; expanding to community providers via RHT grants • Vermont Clinically Integrated Network: shared data standards, protocols, exchange agreements across 14 hospitals • CommonWell / Carequality: national network connectivity for out-of-state care records
Layer 1 (Foundation) — Core Data Assets	• VHCURES (all-payer claims database): 100% Medicaid/Medicare; 60% commercial; GMCB-managed; Onpoint vendor • VUHDDS (hospital discharge database): all 14 hospitals; all inpatient, outpatient, ED discharges • EHR systems at 14 hospitals: Epic (UVMMC), Oracle Health (CVMC, others), TruBridge (CAHs), Meditech (others) • Blueprint clinical registry: quality-measure data linked to VHCURES for population-level HEDIS • GMCB regulatory data: hospital budgets, rate filings, CON decisions, ACO budget data

Figure 4.6 — Vermont target technology architecture: five-layer model, top (advanced analytics) to foundation (core data assets). Sources: GMCB VHCURES documentation; Vermont HIE Strategic Plan; Oliver Wyman Act 167 Report; Vermont RHT Program Application; Act 62 and Act 68 of 2025.

Two observations about this architecture are important for Vermont's technology planning. First, layers 3 and above represent target states, not current states. Vermont's current technology infrastructure is solid at Layers 1 and 2, developing at Layer 4, and nascent at Layers 3 and 5. The RHT Program, AHEAD requirements, and the AHS-GMCB analytics vendor procurement are all investments in accelerating the build-out from Layer 1-2 completeness toward Layer 3-5 capability. The Statewide Strategic Plan's technology section should be explicit about which layers are complete, which are under development, and what the completion timeline is.

4.9 Work This Chapter on the Platform

Chapter 4's data-infrastructure argument is best understood by building on the actual standards. The FHIR Lab turns the interoperability discussion into working resources.

Do this	On this tool	What to look for
Build FHIR R4 resources and map terminology across ICD-10, SNOMED, LOINC, RxNorm	**FHIR Interoperability Lab** — /research-lab/interoperability?tab=fhir	How a working data exchange differs from a data asset that merely "exists."
Stratify a population by CMS-HCC risk and see the attribution that global budgets require	**Risk Stratification Engine** — /research-lab/interoperability?tab=risk	Why technology must precede economics: you cannot manage a budget for a population you cannot see.

Figure 4.H — Hands-on platform tools for the Technology Pillar.

4.10 Implications for You

If you are a Vermont hospital executive: Your VHCURES attribution file already exists. If you have not analyzed it — understood which patients are attributed to your hospital, what their HCC profiles look like, what their utilization patterns are — you are operating without the most important information your AHEAD performance year will depend on. The hospitals that invest in VHCURES analytics capability now will enter the January 2028 performance year with a 12-to-18-month advantage in understanding their population. Those that wait for the AHS analytics vendor will be learning in their first performance year what they could have understood in advance.

If you are an AHS or GMCB official: Vermont's technology infrastructure has three critical components that must be operational before January 2028: VHCURES attribution and HCC modeling, the VITL/VHIE clinical data exchange with adequate provider participation, and the

AHS-GMCB analytics vendor for population health management and global budget performance tracking. None of these is a separate initiative. They are the same system, built in layers. The risk is that each layer is procured and implemented independently, without the integration architecture that makes the layers work together. The analytics vendor selection should specify integration requirements for VHCURES and VITL/VHIE, not leave integration as a post-deployment activity.

If you are a Vermont legislator: Vermont's $195M RHT award includes capital for technology infrastructure. The legislature's oversight role is to ensure this capital builds sustainable infrastructure — systems that operate after the RHT award expires — rather than time-limited programs. Ask specifically: what technology investments made with RHT capital will still be in operation in 2032? If the answer is unclear, the capital allocation needs clarification.

If you are a national health policy professional: Vermont's technology infrastructure challenge — building integrated population health analytics for a 14-hospital system with 6,000+ providers on a limited budget and a hard deadline — is the technology challenge every small and mid-size state faces. Vermont's CIN model, VHCURES attribution approach, and AHS-GMCB analytics vendor structure are worth studying as a replicable architecture for states that cannot afford enterprise-scale health IT but cannot manage value-based payment without it.

Second, the technology architecture has a governance counterpart that is equally important. Each layer requires governance: who owns the data, who can access it under what conditions, what privacy protections apply, how disputes are resolved, and how the infrastructure is funded sustainably. Vermont's technology investments will only produce their intended value if the governance layer — the policies, agreements, and oversight structures — keeps pace with the technical infrastructure. The DVHA-led HIE governance, the GMCB's data stewardship program, and the CIN's shared governance model are all components of this governance architecture.

4.11 Key Concepts Introduced or Expanded in This Chapter

VHCURES

Vermont Health Care Uniform Reporting and Evaluation System — Vermont's all-payer claims database, managed by GMCB with Onpoint Health Data. Contains 100% of Medicaid and Medicare claims and approximately 60% of commercial claims; used for total cost of care measurement, hospital price analysis, and population health analytics.

VITL / VHIE

Vermont Information Technology Leaders / Vermont Health Information Exchange — the nonprofit designated by statute to operate Vermont's statewide HIE network, providing clinical data exchange between hospitals and other providers. Participation is voluntary for non-hospital providers.

FHIR (Fast Healthcare Interoperability Resources)

The HL7 standard for healthcare data exchange, mandated by CMS for payers and providers beginning 2021-2022. FHIR enables different health IT systems to exchange information using common data formats and APIs, without custom interfaces.

Clinically Integrated Network (CIN)

A formal network of healthcare providers that share clinical protocols, performance data, and care coordination infrastructure while maintaining independent ownership. Vermont is creating a CIN through its RHT Program to provide shared analytics, group purchasing, administrative services, and referral formalization across its 14-hospital network.

AI scribe / ambient clinical intelligence

AI technology that listens to clinical encounters and automatically generates structured clinical documentation, reducing the documentation burden that contributes to clinician burnout. Vermont's RHT Program includes grants for AI scribe technology for rural and independent practices.

Remote patient monitoring (RPM)

Technology enabling continuous or periodic monitoring of patients' clinical parameters — heart rate, blood oxygen, blood pressure, glucose — in their homes. Enables earlier intervention for chronic conditions (CHF, COPD, diabetes) to prevent hospital admissions. Vermont's RHT Program includes RPM grants for home health, primary care, and community paramedicine settings.

42 CFR Part 2

Federal regulations governing confidentiality of substance use disorder treatment records. Separate and more restrictive than HIPAA; creates the primary legal barrier to integrating SUD treatment data into Vermont's unified health record. 2024-2025 federal regulatory changes have begun easing some Part 2 barriers.

Act 62 of 2025

Vermont legislation that transferred responsibility for the statewide Health Information Technology Plan from GMCB to DVHA, effective July 1, 2025. Aligns HIT strategy coordination with DVHA's operational relationships with providers through Medicaid, Blueprint, and AHEAD.

Master patient index

A database that matches patient identities across multiple healthcare organizations using demographic and clinical matching algorithms. Required for Vermont's integrated health record — without reliable patient matching, claims data and clinical data from different facilities cannot be reliably linked to the same individual.

Sources: GMCB VHCURES documentation (gmcboard.vermont.gov/data-and-analytics); Vermont HIE Strategic Plan 2024 Update; Act 62 of 2025; Act 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts (January 31, 2025); Vermont Rural Health Transformation Program Application (November 3, 2025); Vermont RHT Program Activity Details (December 2025); Chartis 2026 State of Rural Health; JAMA Network Open ambient scribe quality improvement study (2024); National Consortium of Telehealth Resource Centers AI guidance (2026); Vermont Health Information Exchange Data Governance mandate (healthdata.vermont.gov).

Chapter 5: The Technology Pillar in Practice — FHIR, AI Governance, and Clinical Decision Support

> *The deployment of Vermont's health IT infrastructure reveals the Technology pillar's most consequential sequencing vulnerability: the analytics vendor gap that must close before January 2028.*

FHIR interoperability, AI clinical governance, and cybersecurity are not three separate technology initiatives — they are three implementation risks that any organization building on a data infrastructure foundation will encounter, in roughly this order, as that foundation comes online. A standards mandate (FHIR) without a governance framework for the AI tools that will consume that data produces ungoverned clinical AI; a connected, data-rich system without cybersecurity investment produces a larger attack surface. None of this is specific to Vermont's timeline or its CIN structure — it is the standard risk profile of any health system's technology buildout, and Vermont's January 2028 deadline simply compresses the timeline enough to make the sequencing visible.

5.1 From Architecture to Implementation

Chapters 12 and 13 established Vermont's technology architecture — the five-layer infrastructure from VHCURES and VUHDDS through the HIE and FHIR exchange layer to population health analytics and AI applications. This chapter develops the implementation side: how organizations actually build FHIR-based data exchange, what an AI clinical governance program looks like in practice, how clinical decision support is designed and deployed for population health management, and what cybersecurity requirements apply to the distributed rural health infrastructure Vermont is building.

5.2 FHIR Implementation — From Standard to Working System

5.2.1 The FHIR Implementation Reality

FHIR R4 is a standard, not a product. Its existence does not mean that any two FHIR-compliant systems can exchange data automatically. Like all standards, FHIR specifies a framework within which compliant systems must operate — but the specific data elements populated, the implementation patterns used, and the business rules applied vary substantially across implementations. A hospital's Epic FHIR implementation and a CAH's TruBridge FHIR implementation may both be 'FHIR R4 compliant' while exchanging data with entirely different content, triggering entirely different workflows, and requiring custom integration work to interoperate effectively.

Vermont's CIN technology strategy must account for this reality. The goal is not FHIR compliance as a checkbox — it is functional interoperability: the ability for a care coordinator at Northeastern Vermont Regional Hospital to see, in real time, that their patient's Medicaid claim for an ER visit at Dartmouth Hitchcock Medical Center in New Hampshire was just processed, enabling a care management follow-up call before the patient discharges. That outcome requires FHIR compliance plus patient matching plus event notifications plus care coordinator workflow integration. FHIR makes it theoretically possible; CIN implementation makes it practically achievable.

5.2.2 The Three FHIR Use Cases Vermont Needs Most

Vermont's technology investments should prioritize three FHIR use cases that deliver the highest value for the transformation agenda:

FHIR use case	What it enables	Implementation requirements	Vermont priority
Patient access API (§170.315(g)(10))	Patients can access their health records through third-party apps, supporting patient engagement and longitudinal care coordination. Required by CMS for hospitals and clinicians receiving Medicare/Medicaid payments.	EHR must expose FHIR R4 patient access endpoint; practice must register with Smart Health IT or similar app authorization framework; patient-facing app must be certified.	Required compliance — all Vermont hospitals must be compliant. Monitor for compliance at smaller CAHs without robust IT departments.
Provider access API (care coordination)	Care coordinators at one organization can query clinical records from another organization for shared patients, enabling real-time care coordination across the Vermont CIN.	EHR-to-EHR FHIR query capability; master patient index for patient matching across organizations; trust framework (CommonWell/Carequality membership); consent management.	Highest value use case for Vermont CIN. Enables inter-hospital care coordination that the regionalization blueprint requires. Priority RHT IT advance investment.
Payer-to-provider API (prior authorization and formulary)	Electronic prior authorization using FHIR-based DA Vinci implementation guides	EHR must support CRD (Coverage Requirements Discovery) and DTR	Administrative simplification priority. Vermont's prior authorization reform

FHIR use case	What it enables	Implementation requirements	Vermont priority
	eliminates manual PA processes; formulary information exchange reduces prescription errors.	(Documentation Templates and Rules) Da Vinci implementation guides; payers (BCBS VT, MVP) must implement complementary APIs.	agenda is accelerated by FHIR-based electronic PA. GMCB should require payer FHIR API compliance as condition of rate approval.

Figure 5.1 — Vermont's three priority FHIR use cases. Sources: CMS Interoperability and Patient Access Rule; ONC 21st Century Cures Rule; the Technology pillar framework.

5.2.3 The Master Patient Index — Vermont's Identity Management Challenge

Every FHIR use case that involves sharing records between organizations requires patient matching — reliably identifying that the 'John Smith, DOB 1952-03-14' at UVMMC is the same person as the 'John Smith, DOB 1952-03-14' at North Country Hospital and the 'Jonathan Smith, DOB 1952-03-14' (with a name variant) in the Medicaid enrollment file. Vermont's VITL network provides a master patient index for VHIE-connected providers, but it does not cover all Vermont providers, does not include all out-of-state encounters, and does not have validated match rates documented publicly.

Vermont's CIN implementation should include a dedicated master patient index as a shared service — a centralized patient identity management system that all CIN member facilities use to match patients across organizations. The investment cost is modest relative to the value: a reliable MPI is the prerequisite for virtually every care coordination use case the CIN is designed to enable.

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A master patient index is the unglamorous, easy-to-defer piece of infrastructure that every multi-organization care network eventually discovers it cannot do without — whether that network is a Vermont CIN, a Medicaid managed-care plan's provider network in another state, or a regional clinically integrated network anywhere else. The pattern is consistent nationally: organizations invest first in the visible technology (a portal, a dashboard, an AI tool) and defer the MPI because it produces no demo-able output on its own, then discover that every care-coordination feature they actually wanted depends on it. Treat "do we have a reliable MPI" as a go/no-go gate for any care-coordination initiative, in any state.

5.3 AI Clinical Governance — The Complete Framework

AI clinical governance is evolving from a best practice to a regulatory requirement. FDA's AI/ML-based Software as Medical Device (SaMD) framework, ONC's Health Data, Technology and Interoperability rule, and emerging CMS guidance on AI in Medicare Advantage all establish governance expectations for clinical AI. Vermont's CIN AI governance framework, described in Chapters 12 and 13, must be operationally detailed enough to guide the specific governance decisions that CIN member facilities need to make when deploying AI scribe technology, RPM algorithms, diagnostic AI, and population health analytics.

5.3.1 The AI Clinical Governance Lifecycle

The AI Clinical Governance Checklist organizes governance requirements across six lifecycle stages:

5.3.2 Stage 1: Pre-Deployment Evaluation

Before any clinical AI tool is deployed, the organization must evaluate: (1) FDA clearance or de novo classification status — is this a Software as Medical Device, and if so, what FDA clearance has it received? (2) Training data provenance — was the model trained on data representative of the Vermont population? Rural Medicare beneficiaries? Elderly patients with multiple comorbidities? (3) Bias testing — has the developer tested and published performance data stratified by race/ethnicity, age, sex, and disability? (4) Clinical validation — has the tool been validated in a clinical environment comparable to the deployment setting, not just the academic medical centers where most AI research originates? (5) Intended use clarity — is the clinical use case for this tool unambiguous, and is it the use case for which the tool was designed and validated?

5.3.3 Stage 2: Implementation and Training

Deployment requires: (1) Workflow integration planning — where does the AI output appear in the clinical workflow, and how does the clinician interact with it? AI that generates alerts ignored by busy clinicians adds no value and may add risk. (2) Clinician training — not just 'how to use the tool' but 'what the tool does and does not know, where it performs well and poorly, and how to recognize when its outputs may be unreliable.' (3) Override protocols — clear documentation of when and how clinicians should override AI-generated recommendations. (4) Baseline performance measurement — establishing pre-deployment performance metrics against which post-deployment performance can be compared.

5.3.4 Stage 3: Active Monitoring

Ongoing operational governance requires: (1) Performance drift monitoring — AI model performance can degrade over time as the patient population or clinical practice patterns change. Monthly or quarterly performance monitoring against pre-deployment baselines detects drift early. (2) Equity monitoring — performance must be tracked separately for Vermont's equity priority populations (Northeast Kingdom rural patients, elderly Medicaid patients, BIPOC Vermonters) to detect differential performance that average metrics would hide. (3) Adverse event reporting — a formal mechanism for clinicians to report cases where AI output contributed to adverse outcomes, delayed appropriate care, or produced clearly erroneous outputs. (4) Override rate tracking — systematic high override rates are a signal that the AI output is not clinically trustworthy.

5.3.5 Stage 4: Vendor Management

AI vendors must be held accountable through contract terms: (1) Performance SLAs with meaningful remedies — not just 'best efforts' commitments but specific accuracy thresholds with financial consequences for underperformance. (2) Update notification requirements — when the vendor updates the model, the organization must be notified in advance and given opportunity to re-evaluate before update deployment. (3) Bias testing disclosure — vendors must provide demographic performance data for any clinical AI tool deployed in Vermont. (4) Data use transparency — clear contractual provisions about what patient data the vendor collects, how it is used, and whether it is used to train future model versions.

5.3.6 AI-Specific Issues for Vermont Rural Hospitals

Vermont's rural hospitals face three AI governance challenges that are more acute than in urban academic medical center settings. First, training data representativeness: most clinical AI is trained on large urban academic medical center datasets. The patient populations at North Country Hospital or Gifford Medical Center — elderly, rural, with high rates of agricultural occupational exposures, differing comorbidity patterns, and different social determinants — may perform differently under AI models trained elsewhere. Vermont's CIN should require training data documentation as a condition of CIN-funded AI deployments.

Second, implementation support capacity: a small rural hospital with one IT staff member cannot implement the monitoring infrastructure that AI governance requires. The CIN's shared services model addresses this — a shared AI governance function within the CIN monitors all member hospital AI deployments with professional data analytics staff that individual hospitals cannot afford. Vermont's RHT technology grants should include CIN AI governance infrastructure as an allowable use.

Third, the AI scribe equity question: ambient clinical intelligence tools transcribe clinical conversations in English. Vermont has a growing multilingual population — immigrant communities in Burlington and surrounding areas, seasonal agricultural workers. AI scribe tools not trained on multilingual clinical conversations will perform poorly for these patients. Vermont's RHT AI scribe grants should require multilingual capability documentation as an evaluation criterion.

5.4 Clinical Decision Support — From Alerts to Intelligence

Clinical decision support (CDS) encompasses any technology that uses data to influence clinical decisions at the point of care. The range is vast: from simple medication allergy alerts that every EHR includes, to sophisticated population health stratification tools that identify patients at highest risk of hospitalization in the next 30 days and recommend specific care management actions. The challenge with CDS is not building it — it is building it well enough that clinicians use it rather than ignoring it.

5.4.1 Alert Fatigue — The CDS Effectiveness Crisis

Studies consistently show that the majority of EHR clinical alerts are overridden without meaningful engagement. Alert fatigue — the tendency of clinicians to automatically dismiss alerts after being overwhelmed by their volume and poor specificity — is one of the most documented failures of health information technology implementation. Alert fatigue is not a clinician behavior problem; it is a CDS design problem. When 90% of alerts are overridden, the alerts are generating noise rather than signal, and the alert system has failed.

Effective CDS design requires three principles that are violated by most EHR default configurations: specificity (alerts fire only when the clinical evidence strongly supports action, not whenever a vague threshold is crossed), actionability (every alert should present a recommended action that the clinician can take immediately with minimal additional clicks), and workflow integration (alerts appear at the moment when the clinician is making the relevant decision, not before or after).

5.4.2 Population Health CDS — Vermont's Priority Application

For Vermont's transformation agenda, the highest-value CDS application is not medication alerts — it is population health intelligence: risk stratification that identifies patients likely to deteriorate, care gap identification that triggers outreach before patients present in crisis, and SDOH screening tools that flag patients for CHT navigation at the moment of a primary care visit.

CDS type	What it does	Implementation requirement	Vermont application
Predictive risk stratification	Uses machine learning on claims, EHR, and demographic data to predict which patients are likely to be hospitalized or visit the ED in the next 30-90 days, generating a	Attribution data, historical claims, EHR access, trained risk model; care management workflow to receive and act on high-risk alerts	VHCURES-based risk stratification for AHEAD-attributed Medicare population; identify the top 5% highest-risk patients for

CDS type	What it does	Implementation requirement	Vermont application
	risk score and recommended care management action		intensive CHT engagement before AHEAD year begins
Care gap identification	Identifies patients due for preventive services, overdue for chronic disease management follow-up, or missing guideline-recommended medications at the moment of a primary care visit	EHR integration; HEDIS measure definitions; patient registry with last service date tracking	Blueprint PCMH practices: care gap report generated for each scheduled patient appointment, enabling care coordinator to prepare outreach materials before the visit
SDOH screening and navigation	Presents standardized HRSN screening questions at the appropriate point in a primary care visit; flags positive screens for CHT follow-up; tracks referral completion	EHR-integrated screening tool; CHT workflow to receive referrals; community resource directory integration	Vermont Blueprint universal HRSN screening: operational in PCMH practices. Gap: CHT follow-up completion tracking and outcome measurement not consistently linked back to clinical record
Medication safety and adherence	Identifies patients with medication adherence gaps from pharmacy claims data; flags potential drug interactions from concurrent prescriptions across multiple providers	Pharmacy claims data access; prescriber notification workflow	VHCURES pharmacy claims linked to primary care EHR for medication adherence alerts; particularly valuable for high-complexity elderly patients with multiple prescribers

Figure 5.2 — Population health CDS types and Vermont applications. Sources: the Technology pillar framework; Vermont Blueprint for Health; VHCURES documentation.

5.5 Cybersecurity in Rural Health Infrastructure

Cybersecurity is the technology investment that does not generate efficiency or quality returns — it generates insurance against catastrophic failure. Rural healthcare cybersecurity is a specific and serious national vulnerability. The 2024 Change Healthcare cyberattack — which disrupted claims processing for thousands of providers for months, causing cash flow crises at rural hospitals already operating on thin margins — demonstrated that a single vulnerability in a broadly used healthcare technology vendor can cascade across the entire healthcare system.

Vermont's rural hospitals are disproportionately vulnerable to cybersecurity attacks for three reasons: limited internal IT staff who cannot maintain enterprise-level security operations, legacy EHR and financial systems that may not receive regular security patches, and high-value data assets (patient records, financial systems) that cybercriminals specifically target.

5.5.1 The CIN Cybersecurity Shared Services Model

Vermont's CIN shared services model is the appropriate delivery vehicle for cybersecurity in the state's rural hospital network. Individual hospitals cannot afford a 24/7 security operations center; the CIN can. The shared cybersecurity services model provides:

Security operations center (SOC) monitoring: 24/7 network monitoring for intrusion attempts, malware, and anomalous activity. Contracted through a healthcare-specialized managed security service provider (MSSP).

Endpoint detection and response (EDR): Security software on all network-connected devices that detects and responds to threats automatically, escalating to SOC when needed.

Vendor risk management: Assessment of all third-party technology vendors used by CIN member hospitals for cybersecurity posture and contractual security requirements.

Incident response planning: Documented incident response procedures for ransomware, data breach, and system outage scenarios, with regular tabletop exercises and recovery time objectives established for critical clinical systems.

Staff training: Regular phishing simulation and security awareness training — most healthcare cyberattacks begin with a staff member clicking a phishing link.

Vermont's RHT Program IT advance investments explicitly include cybersecurity capability development. This is not an optional add-on; for rural hospitals that are simultaneously digitalizing their operations (AI scribes, RPM, telehealth) and connecting to shared data infrastructure (VHIE, CIN analytics), the cybersecurity attack surface is growing exactly as cybersecurity capability needs to grow to match it.

5.6 Technology Pillar Implementation Matrix

The following matrix provides implementation guidance for the key investments in this pillar — estimated cost, timeline to value, ROI crossover point, and Vermont-specific benchmarks. Costs are indicative ranges; actual costs vary by organizational size, existing infrastructure, and implementation approach.

Investment	Estimated cost	Timeline to value	ROI crossover	Vermont benchmark
FHIR R4 compliance and API development	$150K-$500K per org	12-18 months	18-24 months	Vermont VITL FHIR mandate; CMS interoperability rule compliance
Clinical AI deployment with governance framework	$200K-$1M setup	6-12 months	12-24 months	Vermont AI scribe grants (RHT); ambient documentation ROI 30-45 min/day/provider
VHCURES analytics platform access and training	$20K-$60K annually	3-6 months	6 months	Vermont hospitals: pre-AHEAD HCC gap analysis; attribution modeling
CIN shared analytics infrastructure	$500K-$2M network cost	18-36 months	36-48 months	Vermont CIN: RHT-funded shared analytics for all 14 hospitals
EHR optimization for structured data capture	$15K-$40K per practice	1-3 months	6 months	Blueprint PCMH prerequisite; HEDIS measure reporting accuracy
Cybersecurity assessment and hardening	$50K-$200K per org	3-6 months	Risk avoidance immediate	Vermont rural CAH vulnerability; ransomware risk at Grace Cottage scale

5.7 Work This Chapter on the Platform

Chapter 5’s implementation reality — AI governance, CDS, and clinical data exchange — maps to three working labs.

Do this	On this tool	What to look for
Build an AI clinical governance framework across the model lifecycle	**AI Clinical Governance Lab** — /research-lab/technology-ai?tab=ai	The 65-item checklist as a go/no-go gate before any clinical AI deployment.
Model RPM and telehealth ROI by condition and CPT code	**Digital Health Lab** — /research-lab/technology-ai?tab=digital	Where digital health earns its cost — and where it adds alert fatigue without value.
Exercise clinical data exchange and HL7 messaging	**Clinical Data Exchange Lab** — /research-lab/vbc-clinical-quality?tab=hl7	The integration work that makes near-real-time care management possible.

Figure 5.H — Hands-on platform tools for the Technology Pillar in practice.

5.8 Implications for You

If you are a Vermont hospital executive: FHIR-based data exchange is not a future capability — it is a current federal requirement under the CMS Interoperability Rule. If your EHR is not producing FHIR R4 patient access APIs or does not have a roadmap to do so, your technology vendor is out of compliance with federal requirements and your organization is at risk of reduced CMS reimbursement. More practically: FHIR is the protocol through which the AHS-GMCB analytics vendor will receive your clinical data for global budget performance tracking. If your FHIR infrastructure is not production-ready before January 2028, you will not be able to receive the real-time feedback the global budget management system is designed to provide.

If you are an AHS or GMCB official: AI governance for healthcare transformation in Vermont requires a policy framework that does not currently exist. As AI scribes, predictive risk stratification tools, and clinical decision support systems are deployed through the RHT Program, Vermont needs governance standards that address: How is AI tool performance validated before deployment? Who is accountable when an AI tool produces a clinically consequential error? How are AI tools evaluated for equity impact — specifically, whether they perpetuate or reduce the disparities Vermont's transformation is designed to close? The governance framework should be written before the tools are deployed, not after.

If you are a Vermont legislator: Vermont's AI governance gap is a legislative opportunity. Vermont has the regulatory infrastructure — GMCB, DVHA, AHS — to create AI standards for healthcare that most states lack. A Vermont AI governance framework for healthcare applications, developed in parallel with AHEAD deployment, would both protect Vermont patients and position Vermont as a national model for responsible AI adoption in healthcare transformation. The alternative — deploying AI tools without governance and creating the framework after problems emerge — is more expensive and less credible.

If you are a national health policy professional: Vermont's technology chapter is, at its core, about a small state trying to build the analytics infrastructure that national health policy has assumed into existence for thirty years without actually funding. The gap between what CMMI models assume about state data infrastructure and what states actually have is the most underappreciated obstacle in American health reform. Vermont's experience — the specific decisions, the specific failures, the specific investments that worked — will be the best available evidence on how to close that gap for states that have less.

Figure 5.3 — Technology pillar implementation matrix. Sources: HTR Advisory; Vermont RHT Program (2025); CMS interoperability rule.

5.9 Key Concepts

FHIR R4

The fourth version of the HL7 FHIR standard, mandated by CMS for healthcare data exchange since 2021. Defines data formats and APIs for patient access, provider access, and payer-to-provider exchange. A standard, not a product — compliance requires implementation work beyond meeting the technical specification.

Da Vinci Implementation Guides

HL7-published FHIR implementation guides developed by the Da Vinci Project (a collaboration of payers, providers, and vendors) for specific use cases: Coverage Requirements Discovery (CRD), Documentation Templates and Rules (DTR), and others for prior authorization, formulary, and care coordination.

Master patient index (MPI)

A centralized database that maintains a consistent patient identity across multiple healthcare organizations, enabling reliable matching of records from different systems for the same patient.

SaMD (Software as Medical Device)

FDA regulatory category for software that performs medical functions: diagnosing, treating, mitigating, or preventing disease. Clinical AI tools that influence clinical decisions may be SaMDs subject to FDA clearance requirements.

Alert fatigue

The tendency of clinicians to automatically override EHR alerts due to their excessive volume and poor specificity, rendering the alert system ineffective. A design problem, not a clinician behavior problem — effective CDS is specific, actionable, and workflow-integrated.

CommonWell / Carequality

National health data exchange networks that enable FHIR-based record sharing between health systems across organizational boundaries. VITL participates in CommonWell; Vermont CIN membership would enable access to both networks for out-of-state record retrieval.

Change Healthcare attack

A 2024 cyberattack on Change Healthcare (a claims processing subsidiary of UnitedHealth Group) that disrupted healthcare claims processing nationally for months, causing cash flow crises at rural hospitals dependent on timely claims payment. Demonstrated systemic healthcare cybersecurity vulnerability.

MSSP (Managed Security Service Provider)

A third-party vendor providing outsourced security operations services — monitoring, threat detection, incident response — enabling organizations without internal security staff to maintain enterprise-level cybersecurity posture.

Sources: CMS Interoperability and Patient Access Final Rule (2020); ONC 21st Century Cures Final Rule (2020); HL7 FHIR R4 specification; Da Vinci Project implementation guides; FDA SaMD framework; the Technology pillar framework; Vermont VITL documentation; Vermont RHT Program Application (November 2025); JAMA Network Open ambient scribe study (2024).

Chapter 6: The Economics Pillar — Global Budgets, Reference-Based Pricing, and Financial Reform

> *Vermont's mandatory reference-based pricing and global hospital budgets represent the most direct attempt by any American state to break the fee-for-service incentive structure that makes population health management financially irrational.*

Payment reform is the precondition on which all other transformation depends. This chapter develops reference-based pricing mechanics, global budget architecture, and what Maryland's decade of evidence proves.

6.1 The Economics Pillar: Why Payment Reform Is the Master Variable

Of the six pillars in the six-pillar framework, the Economics pillar is the one that determines whether the other five actually produce results. You can design perfect care protocols, build excellent data infrastructure, and train a superb clinical workforce — but if the payment system rewards volume over value, every other improvement will be absorbed by the financial logic of fee-for-service. Payment reform is not one element of healthcare transformation. It is the precondition on which all other transformation depends.

This chapter develops the economics of healthcare transformation through three lenses that are analytically inseparable: the mechanics of how hospitals are actually paid today and why that model is failing (with Vermont as the case study), the technical architecture of the alternative payment mechanisms designed to replace it (reference-based pricing and global budgets), and the financial modeling discipline that organizations need to navigate the transition. Vermont's reform journey — from the Act 167 diagnostic through the Act 68 mandate through the AHEAD Model state agreement — is the most complete real-world laboratory for these economic questions currently operating in the United States.

6.2 The Fee-for-Service Trap — Why the Current System Fails

Fee-for-service payment is the foundational economic structure of American healthcare: providers are paid for each service they deliver, with payment amounts set by negotiated contracts or government rate schedules. The system's economic logic is simple and, from a provider's perspective, entirely rational: more services delivered means more revenue. The problem is that this rationality at the individual transaction level produces irrational outcomes at the system level.

6.2.1 The Volume Incentive Problem

Under fee-for-service, a hospital that prevents a hospitalization loses revenue. A hospital that prevents a readmission loses revenue. A hospital that successfully manages a patient's chronic condition in primary care so they never need the emergency department loses revenue. Every improvement in population health, every success in care coordination, every dollar spent on preventing the expensive acute event — each of these reduces the hospital's income under the fee-for-service model.

This is not a criticism of individual hospital managers or clinicians, who are generally motivated by genuine commitment to patient care. It is an observation about the structural incentive embedded in the payment architecture. When the financial incentive and the clinical incentive point in opposite directions, the financial incentive wins — not always, not in every case, but systematically and at scale. This is why fifteen years of voluntary value-based care programs have moved the needle on payment reform without fundamentally changing the volume-driven behavior of the hospital sector.

"The concept of the global budget is the fundamental piece that flipped the incentives for hospitals in the right direction. It was such a monumental change, a sea change, in the way that hospitals thought about raising revenue."

— Maryland state regulator, quoted in JAMA Network Open study of the Maryland All-Payer Model

6.2.2 The Price Extraction Problem

417%	250-300%	~136%	$400M+
UVMMC Outpatient vs. Medicare	Average VT Commercial Rate	Approx. Break-Even Point	Projected 5-Year Savings

Fee-for-service payment creates a second structural problem distinct from the volume incentive: it enables hospitals with market power to charge commercial payers prices far above their actual cost of care. In competitive markets, prices are disciplined by competition. In highly concentrated hospital markets — which describe the majority of U.S. hospital markets, and all of Vermont's — there is no effective competitive discipline on commercial prices.

Vermont's price data, documented in the GMCB's February 2026 report and confirmed by independent analyses, is among the most extreme in the country:

Average commercial-to-Medicare ratio, Vermont hospitals 250–300% vs. ~136% break-even point	Outpatient imaging — worst case 944% of Medicare rate at one Vermont hospital
UVMMC outpatient charges (2022) 417% of Medicare — among highest nationally (RAND)	Potential savings at 200% Medicare (2018–2023) $400M VEHI + VSEA combined, per GMCB analysis

Table 6.1 — Vermont hospital commercial pricing vs. Medicare and break-even benchmarks. Sources: GMCB Act 68 RBP Update (February 2026); RAND Hospital Price Transparency Study.

The gap between the break-even point (~136% of Medicare) and the actual commercial prices being charged (250–300% on average, with outliers reaching 944%) is not driven by higher quality, better outcomes, or greater complexity. Oliver Wyman's analysis found no relationship between price and quality across Vermont's hospital system. It is driven by market power — the ability of a dominant provider to demand above-cost prices from payers who have no viable alternative for their members.

The consequence for insurance premiums is direct and arithmetically straightforward. When a hospital charges 300% of Medicare for a service instead of 136%, and when commercial insurance covers the majority of that cost, premiums must rise to cover the difference. Vermont's premium trajectory — individual market average monthly premium from $456 in 2018 to $948 in 2024, a 108% increase in six years — tracks hospital charge growth with high fidelity. These are not coincidental trends. They are the mathematical output of a price structure that has no regulatory discipline.

Vermont average monthly silver marketplace premium (2018–2024)

2018	$456
2019	$474
2020	$468
2021	$580
2022	$730
2023	$875
2024	$948

Figure 6.2 — Vermont silver marketplace premium trajectory 2018–2024, showing 108% increase over six years. Source: Oliver Wyman Act 167 Report; GMCB analysis.

6.2.3 The Cross-Subsidy Collapse

The fee-for-service model sustains itself through cross-subsidy: commercial payers (who pay above cost) effectively subsidize Medicare and Medicaid patients (who pay below cost). This cross-subsidy is the financial foundation of every U.S. hospital that serves a mixed payer population — which is to say, every hospital.

Vermont's demographic trajectory, documented in detail by Oliver Wyman, is destroying the cross-subsidy model. As the 65+ population expands from 21.7% in 2020 toward 30%+ by 2040, the proportion of patients covered by below-cost Medicare and Medicaid grows. As the working-age population (20-64) declines from 58.8% toward 52%, the pool of commercially insured patients shrinks. The shrinking commercial base must bear a growing cross-subsidy burden — which forces premiums higher — which reduces commercial enrollment — which shrinks the commercial base further. This is not a gradual trend. It is a reinforcing spiral.

Oliver Wyman's financial projections capture where this spiral leads: under the conservative scenario, 13 of Vermont's 14 hospitals report operating losses by 2028, with a cumulative 5-year system deficit of $700 million to $2.4 billion depending on expense growth assumptions. The hospital system does not gradually lose efficiency under this trajectory. It collapses.

> **BEYOND VERMONT**
> The fee-for-service spiral described above does not require Vermont's all-payer model or its 14-hospital structure to operate — it is the default trajectory of fee-for-service payment anywhere that a shrinking commercial base subsidizes care for a growing share of publicly-insured and uninsured patients, which describes most rural and many urban hospital markets nationally. What Vermont's numbers provide is a fully quantified, publicly documented version of a spiral every state's hospital association can recognize in its own data, even where no single report has assembled the picture as starkly as Oliver Wyman did for Vermont.

6.3 Reference-Based Pricing — Mechanics, Evidence, and Vermont's Design

Reference-based pricing (RBP) is the first operational tool in Vermont's economics reform sequence. It addresses the price extraction problem directly: rather than allowing hospitals and payers to negotiate prices in bilateral, opaque arrangements where hospital market power determines the outcome, RBP establishes a transparent external benchmark — typically a percentage of Medicare reimbursement — as the maximum a hospital may accept.

6.3.1 How RBP Works: The Mechanism

The core RBP mechanism is straightforward: the regulatory authority (in Vermont's case, GMCB) establishes, by rule, the maximum amounts that hospitals shall accept as payment in full for items and services delivered. These maximum amounts are calculated as a percentage of Medicare reimbursement for the same or similar service. The percentage is calibrated to allow hospitals to cover their actual costs (break-even at ~136% of Medicare) while eliminating the excessive markup that drives premium inflation.

Act 68's RBP architecture reflects four design principles that distinguish Vermont's approach from weaker price transparency experiments:

- Mandatory, not voluntary — every hospital, every commercial payer, by FY2027
- Binding ceiling, not advisory guidance — hospitals cannot charge or collect more than the established rate
- Transparent benchmark — Medicare rates provide a nationally comparable, publicly available anchor
- Premium passthrough monitoring — GMCB and DFR must verify that price reductions translate to lower premiums

GMCB's February 2026 report introduced one additional design refinement that reflects the Vermont-specific complexity of applying RBP to a highly varied 14-hospital system. Rather than applying a single statewide RBP multiplier, GMCB will consider each hospital's specific community context: payer mix, labor costs, social risk factors, and role in Vermont's system. A critical access hospital serving a predominantly Medicaid and Medicare rural population has different cost economics than a large PPS academic medical center. Vermont's RBP methodology will accommodate these differences while maintaining the essential principle that commercial prices may not exceed a defensible multiple of Medicare.

6.3.2 The Evidence Base: Cross-State Results

Vermont is not designing RBP in a vacuum. Three states have implemented meaningful price reference programs with documented results, providing the evidence base for Vermont's design choices.

State / Program	Design	Scope	Key Results
Oregon (2019)	200% of Medicare in-network; 185% out-of-network. CAH exemptions.	State employee plan (~300,000 covered lives)	$107.5M saved in first 27 months (Health Affairs). All 24 affected hospitals remained in-network. No documented access harm.

State / Program	Design	Scope	Key Results
Montana (2016)	Direct contracting at % of Medicare for state employee plan.	State employee plan	$48M initial savings. Politically fragile without statutory mandate — illustrates Vermont's correct choice to legislate rather than demonstrate.
Washington (2021)	'Cascade Care' public option. 160% of Medicare for participating networks.	Voluntary public option	Expanding to state employee plans. Growing commercial adoption.
Maryland (2014–present)	All-payer rate setting; hospitals receive fixed annual revenue (global budget). Medicare waiver.	All payers, all hospitals statewide	$1.4B in Medicare hospital savings to date. 7% reduction in hospital admissions. $800M reduction in hospital spending. Model extended and expanded.
Vermont Act 68 (FY2027)	Mandatory statewide RBP maximum set by GMCB rule. Initially commercial payers; excludes Medicare/Medicaid.	All Vermont hospitals, all commercial payers	Targeting implementation FY27; first negative commercial rate benchmark (-1%) in effect FY26 as transition step.

Table 6.3 — Cross-state reference-based pricing and rate regulation: design and outcomes. Sources: GMCB Act 68 Update (February 2026); Health Affairs; Commonwealth Fund; CMS.

The Oregon results are the most directly applicable to Vermont's situation because Oregon implemented RBP for a state employee plan while Vermont is implementing it statewide. The critical finding — all 24 affected hospitals remained in-network — directly addresses the most frequently raised concern about RBP: that hospitals will walk away from the regulated rates and leave patients without in-network access. The Oregon evidence is unambiguous on this point: hospitals did not exit the market when rates were set at 200% of Medicare, because 200% of Medicare is still substantially above their actual costs.

6.3.3 What RBP Does — and What It Does Not Do

GMCB's February 2026 report was notably precise about the scope of RBP's impact. Understanding what RBP accomplishes, and what requires complementary mechanisms, is essential for policymakers and healthcare leaders designing transformation strategies.

RBP does:

- Reduce hospital leverage to demand excessive prices from commercial payers
- Narrow unjustified price variation across hospitals for the same service
- Protect patient affordability by establishing binding price ceilings
- Limit monopolistic pricing power in concentrated markets
- Incentivize operational efficiencies over price increases as the margin lever
- Serve as a necessary input for global budget design by establishing the price denominator

RBP does NOT:

- Control total hospital spending — only the price component, not volume
- Guarantee savings pass-through to consumers without explicit monitoring requirements
- Restructure incentives around prevention or population health
- Guarantee hospital efficiency, sustainability, or quality performance
- Solve the cross-subsidy collapse driven by demographic aging

This is why Act 68 sequences RBP first, global budgets second. RBP is a necessary but insufficient tool. Global budgets address what RBP leaves unresolved: total hospital spending, volume incentives, and population health management.

6.3.4 Vermont's Phased Implementation Timeline

GMCB is implementing RBP in two phases, reflecting both the regulatory complexity of establishing a statewide rate-setting methodology and the operational reality that hospitals need runway to adapt their financial models.

Phase	Timeline	Mechanism	Key Actions
Phase I — Differential Price Growth	FY26–FY27 (now underway)	Hospital Budget Guidance	FY27 guidance recommends -1% commercial rate growth benchmark — first negative benchmark in Vermont history. Targets 'outlier services' with highest prices and volumes for initial reductions.
Phase II — Rate Setting (RBP)	FY27–FY28	GMCB Rule-Making	RFP for RBP vendor released December 2025. Discovery and design August

Phase	Timeline	Mechanism	Key Actions
			2025–August 2026. Policy and regulatory development January 2026–January 2028. Final RBP methodology released March 2027 for FY28 hospital budget guidance.
Phase III — Global Budget Integration	FY28 and FY30	Statutory Mandate	Non-CAH global hospital budgets by FY28. All-hospital global budgets by FY30. RBP embedded as price component of global budget methodology.

Figure 6.4 — Vermont RBP and global budget implementation phases. Source: GMCB Act 68 Update, February 2026.

The Phase I action — the negative 1% commercial rate benchmark for FY2027 — deserves particular attention because it is already producing results before formal RBP is in place. Combined with FY2026 measures including the Act 55 outpatient drug cap and hospital budget enforcement, GMCB reduced Vermont hospital commercial revenue by $230.65 million in FY2026: $104.3M from the Act 55 drug cap, $94.58M from hospital budget orders, and $31.76M from budget enforcement. This is the largest single-year price reduction in Vermont hospital history, accomplished through regulatory authority that already existed, signaling what becomes possible when RBP is formally implemented.

6.4 Hospital Global Budgets — Architecture, History, and Vermont's Mandate

A hospital global budget is a prospectively set annual revenue cap for a hospital, established before the year begins, based on population health needs and past performance rather than on the volume of services delivered during the year. Under a global budget, a hospital receives a fixed total payment regardless of how many patients it sees or how many services it provides. This single design choice — decoupling revenue from volume — fundamentally transforms the hospital's financial incentive structure.

6.4.1 The Economic Logic: Flipping the Incentive

Under fee-for-service, revenue = price × volume. To increase revenue, a hospital must increase either the price per service or the number of services delivered. Under a global budget, revenue is fixed. The only way to improve financial performance is to reduce costs — which means managing population health better, preventing hospitalizations, reducing avoidable readmissions, improving efficiency, and delivering care in less expensive settings.

This is not a theoretical incentive. The Maryland experience, documented over a decade of all-payer global budget operation, demonstrates the behavioral change in concrete terms. As one Maryland state regulator described it: the global budget is the fundamental piece that flips the incentives for hospitals — hospitals had to transition from business strategies that chase volume to ones that focused on population health management. That transition does not happen overnight, but it does happen, and the documented savings are substantial.

The global budget also creates something fee-for-service never can: revenue predictability. Under FFS, a hospital's revenue depends on patient volume, acuity, service mix, and billing accuracy — all variable and partially outside the hospital's control. Under a global budget, the hospital knows its annual revenue before the year begins. This predictability enables the long-term capital planning, workforce investment, and care redesign that hospital transformation requires. For Vermont's financially distressed rural hospitals, revenue predictability is not a secondary benefit — it is a survival mechanism.

6.4.2 Five Design Dimensions Every Global Budget Must Resolve

Global budget design is technically complex, and the design choices are consequential. GMCB's February 2026 report identified five dimensions that any global budget methodology must address, along with the core tradeoff embedded in each. These dimensions apply equally to Vermont's GMCB-designed commercial budgets, the AHEAD Model Medicare budgets, and the Medicaid global budget already in operation.

Design dimension	The choice	Key tradeoff	Vermont's current direction
Scope	Which services are inside vs. outside the budget? (Capital, high-cost drugs, outpatient, physician services?)	Broader scope = more comprehensive cost control. Narrower scope = simpler to administer but leaves cost-shifting opportunities.	TBD through rulemaking. Maryland excludes physician office services and outpatient drugs. Vermont likely to follow similar structure initially.
Population adjustment	How does the budget account for patient demographics, complexity, and socioeconomic factors?	No adjustment rewards hospitals serving healthier populations. Over-adjustment reduces cost-control incentives.	GMCB considering payer mix, labor costs, social risk factors for each hospital. Critical for Vermont's CAHs vs. UVMMC.

Design dimension	The choice	Key tradeoff	Vermont's current direction
Volume treatment	Does the budget adjust if patient volume changes significantly?	Fixed budget = maximum cost-control incentive. Volume-adjusted budget = lower risk for hospitals, less population health incentive.	Maryland preserved revenue initially when volume decreased to ease transition. Vermont likely to adopt similar glide path.
Quality linkage	How are quality metrics integrated — penalty, incentive, or both?	Too small a linkage = no behavioral change. Too large = financial instability for hospitals still improving.	Act 68 requires standardized accountability metrics. GMCB quality programs (readmissions, HACs) will be embedded.
Flexibility mechanisms	Provisions for mid-year adjustments, carryover of savings, risk corridors?	More flexibility = lower hospital risk, easier adoption. Less flexibility = stronger cost-control signal.	GMCB authorized to identify conditions requiring modification or termination of RBP or budget parameters.

Table 6.5 — Five dimensions of global budget design with Vermont's current direction. Source: GMCB Act 68 Update (February 2026); Act 68 of 2025.

6.4.3 Vermont's Global Budget History: Four Attempts, One Lesson

Vermont has been attempting to implement hospital global budgets for over a decade. The history of those attempts is essential context for understanding why Act 68's mandatory approach represents a fundamental departure from everything that preceded it, and why the departure was necessary.

Approach	Year(s)	Structure	Outcome and lesson
GMCB Net Patient Revenue Cap	2012–present	Regulatory cap on net patient revenue each hospital may receive	Partial constraint. Controlled revenue growth within individual hospital budgets but did not align incentives across payers or address volume gaming.

Approach	Year(s)	Structure	Outcome and lesson
Rutland Regional Medical Center Pilot	2014 (proposed)	Hospital payments based on historical revenue plus inflation, adjusted for demographics	Never implemented. Hospital opposition and implementation complexity prevented launch. Showed that hospital buy-in is necessary but not sufficient without statutory mandate.
Vermont All-Payer ACO Model	2017–2025	Voluntary commercial payer/provider participation in hospital global payments aligned across Medicare, Medicaid, and commercial based on historical spend	Voluntary participation meant highest-cost commercial payers opted out. OneCare Vermont could not compel participation. Model produced modest results but never achieved the all-payer alignment that makes global budgets effective. Expired end of 2025.
Act 167 / Act 68 Mandate	2022–present	GMCB directed to design and implement mandatory global budgets: non-CAH hospitals by FY2028, all hospitals by FY2030	In progress. Removes the voluntary participation flaw that undermined all prior attempts.

Table 6.6 — Vermont global budget history: four attempts and the central lesson. Sources: GMCB Act 68 Update (February 2026); Oliver Wyman Act 167 Report.

The central lesson of this history is unambiguous: voluntary models fail because the actors who benefit most from the status quo — the highest-priced hospitals and the commercial payers with the most leverage — can and do opt out. A global budget that is optional for the highest-price hospital in a market is not a global budget; it is a demonstration project. Act 68's statutory mandate eliminates the opt-out. This is the design difference that matters.

"If the model is not adequately regulated and sufficiently resourced, it will fail dramatically and harm patients and harm our health system."

— Owen Foster, Chair of the Vermont Green Mountain Care Board, on signing the AHEAD State Agreement, January 2025

6.5 Maryland — A Decade of Global Budget Evidence

Vermont is this chapter's forward-looking case — an early-stage, mandatory implementation whose outcomes are still unfolding. Maryland is this chapter's backward-looking case — a decade-plus, statewide global budget program whose outcomes are already measurable. The two are best read as a pair: Maryland answers "does this kind of model work anywhere, over time?" while Vermont answers "what does it take to implement one today, under current conditions?" A reader in a state with neither Vermont's all-payer infrastructure nor Maryland's decades of regulatory history should read this section first, since it is the evidence base that does not depend on either state's specific starting conditions.

6.5.1 Maryland All-Payer Model — A Decade of Evidence

The only U.S. state with mandatory all-payer global budgets since 2014. Vermont's closest comparable.

$1.4 billion in Medicare hospital savings vs. projected national trend.

7% reduction in hospital admissions for Medicare patients. Avoidable admissions down over 6%.

Vermont's AHEAD EAST Fund ($150M/year) addresses the gap: global budgets without primary care investment reduce volume without improving population health.

Maryland is the only U.S. state to have operated a mandatory all-payer hospital rate setting system with global budgets for more than a decade. Its experience is the closest available empirical evidence for what Vermont is attempting, and it deserves detailed examination — both for what it demonstrates is possible and for the limitations the evidence also reveals.

6.5.2 The Maryland Architecture

Maryland's Health Services Cost Review Commission (HSCRC) was established in 1972 with statutory authority to set hospital rates for all commercial payers — a unique regulatory authority that no other state possesses. The HSCRC received a Medicare demonstration waiver in 1977, enabling all-payer rate setting to include Medicare. The current Global Budget Revenue (GBR) methodology, implemented through a renegotiated CMS waiver in 2014, fixes each hospital's total annual revenue before the year begins, regardless of how many patients are served or services provided. Prices for individual services adjust dynamically during the year to meet the fixed revenue target.

The model's accountability structure has three components beyond the global budget itself: a Medicare Performance Adjustment (1% of Medicare revenues) tied to total cost of care outcomes; Quality Based Reimbursement (2% of all-payer revenue) linked to patient experience, safety, and outcomes; and incentives for reducing hospital-acquired conditions, potentially avoidable utilization, and readmissions. The budget constraint is non-negotiable; the quality components are calibrated to reward the population health management behavior the global budget is designed to incentivize.

6.5.3 The Results

The evidence from Maryland's global budget model is substantial and consistent across multiple independent evaluations:

Medicare hospital savings to date $1.4B vs. projected national trend (HSCRC)	Reduction in hospital admissions 7% Medicare patients; avoidable admissions down >6%
Reduction in hospital spending $800M Medicare Part A and B combined (CMS evaluation)	All-payer per capita growth cap 3.58% committed growth rate under Maryland model

Table 6.7 — Maryland All-Payer Global Budget Model results summary. Sources: HSCRC; CMS; Commonwealth Fund; Milbank Memorial Fund.

Maryland also reduced hospital spending for commercial beneficiaries, cut ED visits and inpatient admissions across all payer types, and significantly improved outcomes for dual-eligible (Medicare/Medicaid) patients — the highest-cost, highest-complexity population segment that Vermont also needs to serve better. The 14-day follow-up visit rate and 30-day unplanned readmission rate both improved for non-teaching hospitals.

6.5.4 The Limitations — What Maryland Teaches About What Global Budgets Cannot Do Alone

The Maryland evidence is not uniformly positive, and intellectual honesty requires presenting the limitations alongside the achievements. The Milbank Fund's evaluation found mixed results on quality metrics and no improvement in overall population health measures like smoking rates or obesity prevalence. The reason is analytically important: the global budget creates a powerful incentive to reduce hospital utilization, but it does not automatically create the community infrastructure needed to address the health determinants that drive that utilization.

When Maryland reduced avoidable hospital admissions, the reduction was concentrated among healthier patients — people who, with adequate primary care and social support, would not have needed hospitalization. The sickest patients continued arriving at emergency departments, and hospitals actually spent more time treating them. The lesson is that a global budget without commensurate investment in primary care, behavioral health, housing, and transportation does not improve population health — it just reduces hospital revenue while leaving the underlying drivers of high utilization intact.

Vermont has heard this lesson clearly. The AHEAD State Agreement explicitly links hospital global budgets with the Equity, Access, and Statewide Transformation (EAST) Fund — providing up to $150 million in additional Medicare funds annually starting in 2027 to invest in primary care, mental health, home health, and long-term care services. The Rural Health Transformation Program award of $195 million provides capital for workforce, telehealth, and community infrastructure. Vermont's global budget design is, from the beginning, paired with the community investment that Maryland added only after its first years of operation.

6.6 The AHEAD Model — Medicare's Entry into Vermont's Reform

Jan 2028	$150M/yr	9 Years	3-Payer
AHEAD Cohort 2 Launches	EAST Fund for Vermont	Model Duration to 2035	Medicare + Medicaid + Commercial

The AHEAD (Achieving Healthcare Efficiency through Accountable Design) Model represents the federal government's partnership with states committed to genuine all-payer total cost of care transformation. Vermont signed its AHEAD State Agreement on January 17, 2025, committing to participate in Cohort 2 beginning January 1, 2028. The agreement was a 3-1 vote by GMCB, with the dissenting member citing concerns about implementation complexity — a vote that reflects the genuine difficulty and ambition of what Vermont has committed to.

6.6.1 What AHEAD Actually Is

AHEAD is a voluntary state total cost of care model that aims to drive state-level healthcare transformation and multi-payer alignment. The model has three primary components that operate in concert:

- Hospital Global Budgets — participating hospitals receive prospective, biweekly Medicare FFS payments instead of traditional claims-based payments, providing revenue predictability while creating population health incentives
- Primary Care AHEAD — voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models
- State accountability — Vermont commits to controlling total cost of care growth, increasing primary care investment as a percentage of total spending, and improving population health outcomes and equity metrics

The AHEAD Financial Package for Vermont

The AHEAD State Agreement provides Vermont with a substantial financial resource package alongside its accountability commitments:

- Up to $150 million in additional Medicare funds annually starting in 2027, with inflation adjustments each year
- $1.2 million in federal transformation grants beginning Cooperative Agreement Year 3 (2027) for health system transformation

Equity, Access, and Statewide Transformation (EAST) Fund for investment in primary care, behavioral health, home health, and long-term care — the community infrastructure layer that makes global budgets work

Technical assistance from CMS on global budget design methodology, risk adjustment, and performance monitoring

Nine-year model duration (through December 31, 2035) providing multi-year planning horizon for hospital transformation

The $150M annual figure is significant context: Vermont's total Medicare and Medicaid spending exceeds $3 billion annually. The AHEAD funds represent approximately 5% of that total — meaningful but not transformative in isolation. The value is in what the funds are explicitly directed toward: the community infrastructure and primary care investment that turns a hospital cost-control model into a population health model.

6.6.2 The AHEAD-Act 68 Integration: Why Both Are Required

A critical analytical point that is easily missed: the AHEAD Model and Act 68 are not alternatives or substitutes. They are complementary layers of the same reform architecture, operating on different payer populations and at different speeds, but designed to converge on the same outcome: a Vermont hospital system operating under aligned all-payer global budgets.

AHEAD governs Medicare FFS hospital payments — approximately 23% of Vermont's insured population and a higher share of hospital revenue given Medicare's age-driven intensity. Act 68 governs commercial hospital prices and budgets — approximately 52% of Vermont's insured population and the majority of hospital revenue. Vermont's Medicaid global budget, already operational under the Global Commitment to Health demonstration and aligned with AHEAD, covers approximately 19% of the population. Together, these three layers create the all-payer alignment that makes a global budget system effective.

Payer layer	% of VT insured pop.	Payment mechanism	Effective date	Governing authority
Medicare FFS	~23%	AHEAD Hospital Global Budgets; biweekly prospective payments	January 2028	CMS / AHEAD State Agreement
Medicaid	~19%	Vermont Global Commitment to Health; Medicaid global budget aligned with AHEAD	In operation; aligned with AHEAD 2027	DVHA / CMS Demonstration Waiver
Commercial (incl. self-insured)	~52%	Reference-Based Pricing (FY2027) → Global Hospital Budgets (FY2028+)	RBP FY2027; budgets FY2028	GMCB / Act 68

Payer layer	% of VT insured pop.	Payment mechanism	Effective date	Governing authority
Medicare Advantage	~31% of Medicare-eligible	Outside AHEAD; traditional managed care contracts	Not currently in scope	CMS MA contracts
Uninsured	~3%	Charity care; DSH payments	Ongoing	GMCB / DVHA

Figure 6.8 — Vermont all-payer architecture under AHEAD + Act 68 combined. Sources: Vermont RHT Program Application (November 2025); AHS AHEAD fact sheet; Act 68.

The Medicare Advantage gap is worth noting. AHEAD applies to Medicare fee-for-service beneficiaries; approximately 31% of Vermont's Medicare-eligible population is in Medicare Advantage plans, which are managed care contracts between CMS and private insurers. These plans operate outside the AHEAD global budget framework. As MA enrollment continues growing nationally, this gap represents an increasing share of the Medicare population that is not subject to Vermont's hospital cost-containment regime — a design limitation that Vermont policymakers are aware of and that will require ongoing attention.

6.7 Hospital Financial Modeling — Reading the Numbers That Matter

For healthcare leaders navigating the transition to global budgets and reference-based pricing, financial modeling is not an academic exercise. It is the analytical foundation for every strategic decision: which service lines to maintain or discontinue, how much capital investment is prudent, what staffing model is financially sustainable, when and how aggressively to pursue shared services. This section develops the core financial modeling framework for Vermont's hospital sector, drawing on Oliver Wyman's projections and the GMCB's hospital financial data.

6.7.1 The Oliver Wyman Projection Scenarios

Oliver Wyman built two financial scenarios for Vermont's hospital sector through 2028, representing the range between optimistic and realistic assumptions about expense growth. Both scenarios assume the same revenue growth trajectory (3.5% annual non-340B revenue growth); they differ in their expense growth assumptions, reflecting uncertainty about labor market conditions and drug costs.

Scenario	Assumptions and 5-year system deficit
Conservative: 3.5% revenue / 5% expense growth	Non-340B revenue: +3.5% annually. All operating expenses: +5% annually from 2023 baseline.5-year system deficit (2024-2028):• $700M vs. break-even• $1.4B vs. 3% operating margin target• FY2023 budget

Scenario	Assumptions and 5-year system deficit
	overage of $106M suggests starting point may be worse than modeled
Realistic: 3.5% revenue / 7-8% expense growth	Non-340B revenue: +3.5% annually. Non-physician labor: +10% annually. Physician fees: +5% annually. Other operating expenses: +7% annually. 340B payments: +3% (inflation only).5-year system deficit (2024-2028):• $2.4B vs. break-even ($3,700 per Vermont resident)• $3.1B vs. 3% operating margin ($4,800 per Vermont resident)• Vermont hospitals requested $285M in FY25 budget increases — a data point supporting the realistic scenario

Figure 6.9 — Oliver Wyman Vermont hospital financial projection scenarios. Source: Oliver Wyman Act 167 Community Engagement: Recommendations (2024).

The UVM Health Network requires separate attention because it dominates Vermont's hospital economics. Under the conservative scenario, UVM network hospitals face a $312M cumulative 5-year deficit against break-even — more than 44% of the total statewide deficit. Under the realistic scenario, this grows to $1.5B — again roughly 62% of the statewide total. The concentration of Vermont's hospital financial problem in the largest network is analytically important: system transformation strategies that do not address UVMMC's cost structure and overhead will fail to materially improve statewide affordability. Oliver Wyman specifically called out UVMMC's administrative overhead and the proportion of providers engaged in non-patient-care activities as areas requiring examination.

6.7.2 Reading the Metrics That Actually Matter Under Global Budgets

The transition from fee-for-service to global budgets fundamentally changes which financial metrics matter most. Healthcare finance professionals accustomed to FFS environments need to reorient around a different set of indicators.

In a fee-for-service environment, watch:	In a global budget environment, watch:
Net patient revenue growth	Per-member-per-month cost trend vs. global budget cap
Volume growth (admissions, procedures, visits)	Potentially avoidable utilization rate — every avoidable admission is a cost, not revenue

In a fee-for-service environment, watch:	In a global budget environment, watch:
Case mix index — higher acuity = more revenue	Population health outcomes — better outcomes reduce costs
Market share — capturing more patients = more revenue	ED diversion rate — keeping patients out of the ED saves money
Payer mix — more commercial = better margins	Readmission rate — every readmission is a cost without additional revenue
Discharge volume	Days to discharge — length of stay is a cost driver, not a revenue driver
Revenue cycle efficiency — maximize what each claim pays	Total cost of care per attributed member — efficiency across the whole continuum
Operating margin per inpatient day	Cost per quality-adjusted outcome — the fundamental global budget metric

Figure 6.10 — Financial metrics that matter: FFS vs. global budget environment. Compiled from GMCB guidance, Maryland HSCRC methodology, and Act 68 accountability framework.

This metric shift is not cosmetic. It requires fundamental changes to hospital management information systems, internal reporting, board dashboards, and management incentive structures. A hospital whose CFO is still optimizing revenue per discharge when revenue is fixed will not successfully navigate the global budget transition. The CFO in a global budget environment needs to become an expert in population health economics — tracking the care patterns and community conditions that drive utilization, and investing in interventions that reduce avoidable costs.

6.7.3 The Savings Roadmap: Where Vermont's $400M+ Can Come From

Oliver Wyman's analysis identified the specific sources of direct and indirect savings from Vermont's transformation. Understanding this roadmap is essential for any organization planning its role in Vermont's reformed system.

Source of direct savings (5-year total)	Estimate
Close financially unsustainable inpatient facilities	$100M+
Reduce administrative costs and shared services synergies	$115M+

Source of direct savings (5-year total)	Estimate
Shift procedures to ASUs and outpatient settings	$130M+
Expand primary care to reduce preventable ED visits	$90M+
Address SDOH to reduce avoidable hospitalizations	$80M+

Figure 6.11 — Estimated sources of direct and indirect savings from Vermont hospital transformation. Figures are illustrative ranges derived from Oliver Wyman's >$400M total estimate. Source: Oliver Wyman Act 167 Report (2024).

The largest single source of indirect savings — shifting procedures from hospital to ambulatory surgical and outpatient settings — reflects the third Oliver Wyman imperative: move all care possible out of hospitals. Low-risk surgical procedures performed in a hospital setting cost two to three times what the same procedure costs in a free-standing ambulatory surgical unit. Vermont currently has only two ambulatory surgical centers (both in Burlington), a significant gap given the breadth of the inpatient surgical capacity Oliver Wyman recommends redirecting. The RHT Program and Act 68 transformation grants are explicitly designed to fund the development of non-hospital care capacity that makes this shift possible.

6.8 APM Transition Strategy for Vermont Healthcare Organizations

For health systems, payer organizations, and community providers operating in Vermont, the payment reform transformation is not an abstract policy development — it is an operational reality requiring specific preparation. This section provides the strategic and financial planning framework for organizations navigating the transition.

6.8.1 The Three-Phase Transition Model

Vermont's payment reform trajectory follows a predictable three-phase model that organizations can use for planning purposes:

Phase	Timeline	Payment environment	Strategic priorities
Preparation	Now through FY2026	FFS with increasing budget pressure; -1% commercial rate benchmark; Act 55 drug cap in effect	Data infrastructure: build PMPM tracking, attribution, and potentially avoidable utilization dashboards. Workforce: begin training clinical and financial leadership in population health economics. Contracts: audit

Phase	Timeline	Payment environment	Strategic priorities
			current payer contracts for RBP transition clauses.
Transition	FY2027	RBP maximum rates in effect for commercial payers; AHEAD Hospital Global Budgets begin January 2028 for Medicare FFS	Revenue modeling: model financial impact of RBP rates on each major service line. Service line rationalization: align with Oliver Wyman COE recommendations. Primary care investment: increase PC capacity ahead of AHEAD's primary care investment requirements.
Operation	FY2028 and beyond	Global hospital budgets for non-CAH hospitals; AHEAD in full operation; Statewide Strategic Plan delivered December 2028	Population health management: from awareness to execution. Quality accountability: GMCB metrics integrated into operational management. Community investment: SDOH programs, care coordination, housing partnerships.

Figure 6.12 — Three-phase APM transition model for Vermont healthcare organizations.

6.8.2 The VBC Readiness Assessment in a Global Budget Context

The Value-Based Care Transformation Readiness Assessment (30 dimensions across six domains) applies directly to Vermont organizations preparing for global budget participation, but three domains deserve elevated attention given the specific demands of the Vermont environment:

Data and Technology — particularly attribution list generation and total cost of care measurement. A hospital that cannot accurately attribute its patient population cannot manage a global budget. Vermont's VHCURES all-payer claims database is the foundational data infrastructure, but many hospitals lack the internal analytics capability to use it effectively for population health management. This gap must be closed before FY2027.

Care Delivery Capability — particularly community health worker deployment and post-acute care network relationships. The Maryland experience demonstrated that global budgets without community care infrastructure simply reduce hospital volume without improving population health. Vermont hospitals must build these relationships proactively.

6.9 Work This Chapter on the Platform

Chapter 6's payment-reform argument — reference-based pricing, global budgets, the fee-for-service trap — is fully modelable. Reproduce the chapter's Vermont numbers and then run your own.

Do this	On this tool	What to look for
Design an APM and model a 5-year global-budget transition with Vermont's All-Payer TCOC preset	**APM Design Lab** — /research-lab/payment-models?tab=apm-design and **Global Budget Transition Modeler** — /research-lab/payment-models?tab=gb-transition	How a binding revenue envelope changes the volume incentive the chapter describes.
Stress-test a hospital's finances under RBP, global-budget, and Medicaid-cut scenarios	**Hospital Financial Stress Test** — /research-lab/policy-quality?tab=scorecard	Reproduce the Oliver Wyman finding: 13 of 14 hospitals in operating loss by 2028 under the conservative scenario.

Figure 6.H — Hands-on platform tools for the Economics Pillar.

6.10 Implications for You

If you are a Vermont hospital executive: Global budgets change everything about how you think about revenue. Under fee-for-service, revenue growth requires volume growth. Under a global budget, revenue is fixed — which means cost reduction and efficiency improvement are the only paths to financial improvement. The hospitals that are preparing for global budgets now are modeling their cost structure under a fixed revenue scenario: what does your cost per adjusted discharge look like at current volume? At 5% lower volume? At 10% lower volume? What administrative cost reduction opportunities exist that are not visible under the current volume-growth financial model? The $1,303 per-discharge administrative gap between Vermont's CAHs and the national benchmark is not going to close by itself. It closes when the financial model changes, and that model change is 18 months away.

If you are an AHS or GMCB official: The global budget methodology is the most consequential document Vermont's transformation will produce. It determines which hospitals are financially viable under mandatory pricing, which will need transformation support, and which may require consolidation or redesign. The methodology must address three technical questions that Act 68 leaves partially open: How will benchmarks be set for hospitals with unusual payer mixes? How will social risk adjustment be incorporated for hospitals serving high-SDOH populations? And how will the benchmark be updated in years two, three, and beyond, as the transformation investments of the RHT Program produce changes in utilization that the original benchmark did not anticipate? Getting these three questions right in the methodology design is worth more than any amount of enforcement capability applied to a poorly designed methodology.

If you are a Vermont legislator: The EAST Fund — up to $150M annually in Medicare performance funds flowing to Vermont under AHEAD — is transformation capital, not operating revenue. Its correct use is investment in the community health infrastructure, workforce development, and social determinants programs that produce the population health improvement AHEAD measures. If the EAST Fund is used to cover operating deficits at financially stressed hospitals, it will produce short-term relief and long-term dependence without the population health improvement that justifies the federal investment. The legislature should set explicit expectations for EAST Fund use that distinguish transformation investment from operating subsidy.

If you are a national health policy professional: Vermont's RBP target of 200% of Medicare is the most specific mandatory all-payer price target enacted by any state. The central question it raises for national health policy is not whether 200% is the right number — it is whether mandatory price caps produce access reductions or operational efficiencies. Maryland's decades-long experience with all-payer rate regulation provides one data point; Vermont's will provide another, in a smaller and more rural market under more acute financial pressure. The comparison between Vermont's results and Maryland's will be the most informative natural experiment in all-payer pricing since Maryland's program began.

Network and Partnerships — particularly community partner SDOH referral networks. The Oliver Wyman systems map showed that housing, transportation, and behavioral health gaps drive a significant proportion of avoidable hospital utilization. Hospitals that lack functioning SDOH referral pathways will face cost pressure under global budgets that they cannot address through clinical means alone.

6.11 Key Concepts Introduced or Expanded in This Chapter

Global Budget Revenue (GBR)

Maryland's term for a prospectively fixed annual hospital revenue cap, adjusted for quality performance and population health metrics. The model Vermont's GMCB-designed commercial global budgets will most closely follow.

All-payer alignment

The condition in which Medicare, Medicaid, and commercial payers all operate under the same or compatible payment methodology for hospital services. Essential for global budget effectiveness; Vermont's AHEAD + Act 68 combination is designed to achieve this by FY2028-2030.

Cross-subsidy

The mechanism by which above-cost commercial payments subsidize below-cost Medicare and Medicaid payments. Vermont's demographic shift is making this mechanism financially unsustainable as the commercial base shrinks relative to the government-payer population.

Potentially avoidable utilization (PAU)

Hospital admissions, ED visits, and readmissions that could have been prevented through effective primary care, care coordination, or SDOH intervention. A key performance metric under global budgets; reducing PAU saves money in a capped-revenue environment.

EAST Fund

Equity, Access, and Statewide Transformation Fund — the component of Vermont's AHEAD State Agreement providing up to $150M annually in additional Medicare funds for investment in primary care, behavioral health, home health, and long-term care.

AHEAD (Achieving Healthcare Efficiency through Accountable Design)

CMMI's voluntary state total cost of care model, with 6 participating states. Vermont participates in Cohort 2, beginning January 2028. Provides Medicare FFS hospital global budgets, enhanced primary care payments, and accountability for total cost of care growth.

Revenue predictability

A primary benefit of global budgets for hospital financial management: knowing total annual revenue before the year begins enables capital planning, workforce investment, and strategic redesign that FFS revenue uncertainty prevents.

PMPM (per member per month)

The standard unit of measurement in population-based payment models. Total spending (or cost) divided by the number of enrolled members divided by the number of months. The fundamental metric for tracking performance under global budgets and capitation arrangements.

Sources: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Act 68 of 2025 (Vermont); Green Mountain Care Board, Act 68 Update on Hospital Reference-Based Pricing and Global Budgets (February 17, 2026); Vermont AHEAD State Agreement (January 17, 2025); CMS, AHEAD Model overview; Maryland HSCRC, Global Budget Revenue methodology and annual reports; Commonwealth Fund, Hospital Global Budgeting: Lessons from Maryland and Selected Nations (June 2024); Milbank Memorial Fund, Uniquely Similar: New Results from Maryland's All-Payer Model (2020); Vermont Agency of Human Services, Rural Health Transformation Program Application (November 2025).

Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling and APM Readiness

> *The translation of Vermont's payment architecture into operational financial management requires actuarial precision, equity safeguards, and analytics capability being built simultaneously with the policy mandates they must support.*

How healthcare organizations model the financial impact of value-based care — benchmark mechanics, shared savings calculations, risk stratification ROI, and the financial case for transformation investment.

7.1 From Payment Architecture to Financial Practice

Chapter 3 established the payment architecture — reference-based pricing, global budgets, the AHEAD Model, Maryland's proof of concept, and Vermont's economic transformation mandate. This chapter turns from architecture to practice: how do healthcare organizations actually model the financial impact of alternative payment models, assess their readiness to enter and succeed in VBC contracts, and build the financial case for the operational investments that transformation requires?

Financial modeling for value-based care is not the same as financial modeling for fee-for-service operations. Under FFS, the financial questions are primarily about revenue optimization: how do we maximize reimbursement for the volume we deliver? Under VBC, the financial questions are fundamentally different: what is our total cost of care for our attributed population? Where is our cost higher than the benchmark, and why? What investments in care management, primary care, or SDOH programs will reduce that cost below the benchmark, producing shared savings? These are population health economics questions, not revenue cycle questions, and they require different analytical frameworks, different data infrastructure, and different management skills.

7.2 The APM Financial Modeling Framework

7.2.1 Understanding Benchmarks — The Foundation of VBC Economics

Every alternative payment model is built around a benchmark: the expected cost of care for the attributed population against which actual performance is measured. Shared savings (if actual costs are below benchmark) or shared losses (if actual costs are above benchmark) are calculated relative to this benchmark. Getting the benchmark right — or negotiating favorable benchmark methodology — is often the most important financial decision in VBC contract entry.

Benchmarks can be constructed in several ways, each with different financial implications for the entering organization:

Benchmark Construction Methods — What Healthcare Leaders Must Understand

7.2.2 Historical Spending Baseline

The most common approach — the benchmark is set at the organization's own historical cost of care, typically with a trend adjustment. Advantage: organizations with above-average historical spending start with a higher benchmark, making savings easier to achieve. Disadvantage: organizations with below-average historical spending (already efficient) face a lower benchmark with less room for savings — the 'efficient provider disadvantage.'

> **VERMONT AHEAD CONTEXT**
> CMMI sets global budget baselines for hospitals based on their historical Medicare FFS spending, with adjustments. Vermont's hospitals with historically lower spending may face benchmarks that limit their financial upside under AHEAD.

7.2.3 Regional Benchmark

The benchmark is set at regional average cost of care, not the organization's own history. Advantage: eliminates the efficient provider disadvantage. Disadvantage: organizations in high-cost regions compete against a lower benchmark than their own history would produce.

> **VERMONT RBP CONTEXT**
> GMCB's reference-based pricing uses Medicare rates as the external benchmark — not Vermont hospital historical prices. This is a regional/national benchmark approach that directly addresses the problem of benchmarks based on historical high prices.

7.2.4 Prospective Benchmark

The benchmark is set at the beginning of the performance year based on projected population needs. Used in global budgets and capitation models. Advantage: provides revenue certainty for planning. Disadvantage: risk adjustment quality determines fairness — poor risk adjustment produces winners and losers based on case mix differences, not care management quality.

> **VERMONT GLOBAL BUDGET CONTEXT**
> Vermont's Act 68 global budgets for non-CAH hospitals must specify how the prospective budget is set and adjusted for patient complexity — the risk adjustment methodology is a key design question currently under development.

7.2.5 The Shared Savings Calculation — A Working Model

For organizations entering ACO REACH, MSSP, or similar shared savings models, the following framework captures the core financial logic. The APM Shared Savings Calculator implements this model with organization-specific data inputs.

Step	Formula / concept	Practical meaning	Vermont application
1. Benchmark setting	Benchmark PMPM × attributed member months = Total benchmark spending	This is the 'budget' the organization is measured against. Every dollar of actual spending below this number is potential shared savings.	AHEAD hospital global budget = prospective annual revenue set at beginning of year. Vermont hospital knows its 'benchmark' before the year begins.
2. Performance measurement	Actual total cost of care for attributed population	Includes all Medicare Part A and Part B spending for attributed beneficiaries, regardless of where care is delivered. Out-of-network spending counts against the ACO.	AHEAD tracks total cost of care for Medicare FFS beneficiaries attributed to Vermont hospitals across all payer settings.
3. Savings calculation	Benchmark - Actual TCOC = Gross savings (or loss)	If actual spending is below benchmark, gross savings exist. If above, gross loss.	Under global budget, the hospital receives its fixed prospective payment regardless — so the 'savings' is the margin between efficient population health management and the fixed budget.
4. Minimum savings rate (MSR)	In most shared savings models, savings must exceed a minimum threshold (typically 2-3.5%) before the organization shares in them	Protects against natural year-to-year cost variation triggering shared savings or losses that don't reflect genuine performance changes.	AHEAD uses a different structure — state-level TCOC targets rather than individual ACO MSRs — but the minimum threshold concept applies at the state level.

Step	Formula / concept	Practical meaning	Vermont application
5. Shared savings calculation	Gross savings × sharing rate = Organization's shared savings payment (or loss obligation)	Sharing rates vary by model and risk track — typically 50-75% for one-sided models; may reach 80% for full-risk models.	Vermont RBP: savings to payers from price reduction; 100% of premium passthrough per GMCB monitoring requirement.

Figure 7.1 — APM shared savings calculation framework. Source: HTR's APM Shared Savings Calculator methodology; CMS ACO model documentation.

7.2.6 Risk Stratification — The Economic Engine of Population Health Management

7.2.7 The Care Management ROI Calculation

A program preventing 50 hospitalizations in 10,000 Medicare beneficiaries at $15,000 each generates $750,000 in avoided cost.

If the program costs $200,000 to operate, the ROI is 275%.

Under fee-for-service, the prevented hospitalization is lost revenue — which is why FFS systematically discourages prevention.

Under fee-for-service, the financial value of identifying a high-risk patient is limited — you can bill more for the additional services you provide. Under value-based care, the financial value of identifying a high-risk patient before a health crisis occurs is enormous: every hospitalization prevented by a care management intervention saves the organization the full cost of that hospitalization. The math is compelling.

A care management program that prevents 50 hospitalizations in a population of 10,000 Medicare beneficiaries, at an average cost of $15,000 per hospitalization, generates $750,000 in avoided cost. If the organization bears downside risk (and receives upside savings) on those prevented hospitalizations, the financial return on a care management program that costs $200,000 to operate is approximately 3.75:1 — a 275% ROI. This is the core financial logic that makes population health management economically rational under VBC and economically irrational under FFS.

Average Medicare inpatient hospital cost prevented per event $15,000 Approximate average — varies significantly by DRG and acuity level	Average potentially preventable ED visit cost $1,800 Including facility and professional fees; varies by acuity
ROI: Care management program preventing hospitalizations 3-5x Typical ROI range for well-designed ACO care management programs	Vermont potentially avoidable ED visits as % of total 32.3% Each prevented = $1,800 saved under global budget / VBC arrangement

7.3 Economics Pillar Implementation Matrix

The following matrix provides implementation guidance for the key investments in this pillar — estimated cost, timeline to value, ROI crossover point, and Vermont-specific benchmarks. Costs are indicative ranges; actual costs vary by organizational size, existing infrastructure, and implementation approach.

Investment	Estimated cost	Timeline to value	ROI crossover	Vermont benchmark
HCC gap analysis and closure program	$25K-$100K annually	3-6 months setup	Year 1: 3-8% RAF improvement	Vermont CAHs: avg 12% HCC gap (2025 RHRC assessment)
APM shared savings model financial preparation	$75K-$200K per org	6-9 months	12-24 months	VHCURES attribution; AHEAD benchmark methodology setup
Global budget financial modeling and stress testing	$50K-$150K per hospital	3-6 months	Pre-launch risk reduction	HTR Hospital Financial Stress Test: all 14 Vermont hospitals modeled
VBC readiness assessment (30-dimension framework)	$30K-$75K per org	60-90 days	Immediate: prioritization value	HTR VBC Readiness Assessment; AHEAD preparation baseline
Commercial payer VBC contract negotiation	$75K-$200K per contract	6-12 months	18-36 months	Vermont Blue Cross VBC arrangements; MVP Health Care negotiations
Reference-based pricing methodology (state level)	$500K-$2M state cost	18-24 months	FY2027: systemwide savings	Vermont mandatory RBP FY2027; Oliver Wyman: $300M+ annual projection

Figure 7.2 — Economics pillar implementation matrix. Sources: HTR Research Lab; Oliver Wyman Act 167 Report; Vermont AHEAD documentation.

7.4 APM Readiness Assessment — The 30-Dimension Framework

The Value-Based Care Transformation Readiness Assessment evaluates organizational readiness for APM entry across 30 dimensions in six domains. The framework is designed to identify the gaps that are most likely to prevent an organization from achieving shared savings — or most likely to produce shared losses — before a contract is signed.

7.4.1 The Six Readiness Domains

Domain 1: Strategic Clarity assesses whether the organization has a clear rationale for APM participation, an appropriate governance structure, and leadership alignment on the risk appetite and timeline for VBC transition. Organizations that enter APMs without genuine executive commitment — treating VBC as a compliance exercise rather than a strategic priority — consistently underperform.

Domain 2: Data and Technology evaluates the analytics infrastructure needed to manage a VBC population: attribution list generation, total cost of care measurement, care gap identification, risk stratification, and quality reporting automation. This domain is Vermont's most significant organizational gap — most Vermont community hospitals lack the internal analytics capability to use VHCURES data effectively for population health management. The joint AHS-GMCB analytics vendor procurement addresses the statewide gap; individual hospital analytics capacity must be built alongside it.

Domain 3: Care Delivery Capability assesses the clinical programs needed to act on risk stratification insights: care management programs organized around risk tiers, primary care PCMH readiness, behavioral health integration, post-acute care network relationships, community health worker deployment, and patient outreach and engagement capability. Vermont's Blueprint for Health PCMHs and CHTs provide a strong care delivery foundation; the gap is in building on this foundation to serve the more complex patients that AHEAD's population health requirements target.

Domain 4: Network and Partnerships evaluates the relationships the organization has with the specialists, post-acute facilities, and community organizations that determine whether high-risk patients receive coordinated care or fragmented care after a hospitalization or specialist referral. Vermont's Oliver Wyman-designed COE regionalization framework and CIN development address this domain systematically.

Domain 5: Revenue Cycle examines the billing and coding practices that determine whether the organization's VBC contracts are financially managed correctly: HCC coding completeness (essential for risk adjustment accuracy), RAF gap identification, attribution denial management, and VBC contract payment reconciliation. This domain is covered in depth in Chapter 3.

Domain 6: Workforce Operations evaluates the staffing model, credentialing efficiency, retention rates, and training capability that determine whether the care coordination programs identified in Domain 3 can actually be implemented and sustained.

7.4.2 VBC Readiness Scoring — What the Numbers Mean

Each of the 30 dimensions is scored 1-4: Pre-transition (1), Developing (2), Operational (3), Optimized (4). A total score below 60/120 indicates the organization is not ready for full-risk VBC; the gap analysis identifies the highest-priority investments before contract entry.

Vermont-specific benchmarks: Blueprint-participating PCMH practices with active CHT support typically score 8-12 in Domain 3 (care delivery) — strong by national standards. Most Vermont hospitals score 3-7 in Domain 2 (data and technology) — a critical gap given that data infrastructure is the prerequisite for population health management. The RHRC engagement's explicit focus on developing analytic dashboards for hospitals reflects the recognition that Domain 2 is the binding constraint for most Vermont providers.

7.5 Contract Analysis — Reading an APM Contract for Financial Risk

APM contracts are complex, and the financial risk embedded in them is often not apparent from the headline terms. A shared savings contract that appears attractive at the summary level may contain provisions — benchmark methodology choices, quality withhold structures, attribution methodology, carve-out provisions — that substantially change the financial calculus. Every organization entering a significant APM contract should apply the following analysis framework.

7.5.1 The 65-Item VBC Contract Review Checklist: Key Categories

The VBC Contract Review Checklist (available in the Implementation Toolkit) covers 65 specific contract provisions in eight categories. The most financially consequential categories are:

Contract provision category	Key questions and financial risk
Benchmark methodology	How is the benchmark set? Historical? Regional? Blended? What trend adjustment is applied? Is there a minimum savings rate, and what is it? What happens if the benchmark is reset mid-contract — does the organization benefit from demonstrated performance improvements or start over?
Attribution methodology	How are patients attributed — prospectively (you know your panel before the year) or retrospectively (you find out after the year)? What is the attribution methodology — plurality of primary care visits?

Contract provision category	Key questions and financial risk
	Most costly provider? Attribution methodology determines your denominator and your exposure.
Quality withhold	What percentage of shared savings is withheld pending quality performance? What measures are used? Are they measures the organization currently tracks with reliable data? A 30% quality withhold on 50% of gross savings means quality performance determines 15% of total financial outcome — understand the measures before signing.
Risk corridors and stop-loss	Under full-risk models, is there a stop-loss provision that caps the organization's downside exposure? What is the per-member-per-month or aggregate stop-loss threshold? For organizations entering full-risk for the first time, stop-loss provisions are essential financial protection.
Carve-outs	What services are excluded from the shared savings calculation? High-cost drugs? Mental health carve-outs? Specialty carve-outs? Each carve-out reduces the organization's ability to manage total cost of care and reduces the savings opportunity.
Reconciliation timing	When are shared savings or losses settled — annually, semi-annually, quarterly? Reconciliation timing affects cash flow planning and the organization's ability to invest shared savings in care improvement programs.

Table 7.2 — APM contract review framework: provisions and key questions. Source: HTR Advisory framework.

7.6 Hospital Financial Modeling Under Global Budgets — The Vermont Framework

For Vermont hospitals, the most operationally relevant financial modeling challenge is not APM shared savings calculation — it is global budget management. As Act 68's mandatory global budgets take effect for non-CAH hospitals beginning FY2028, hospital CFOs need to transition from FFS revenue optimization to global budget management. These are fundamentally different financial disciplines.

7.6.1 The Global Budget Financial Management Model

Under a global budget, a Vermont hospital's annual revenue is set prospectively. The financial management objective shifts from maximizing volume and revenue to achieving the highest quality population health outcomes within the fixed revenue envelope. The key financial metrics change accordingly:

Under FFS — optimize these metrics	Under global budget — optimize these metrics
Revenue per discharge — maximize reimbursement per case	Cost per discharge — minimize unnecessary cost while maintaining quality
Admission volume — more admissions = more revenue	Avoidable admission rate — every preventable admission is a cost, not revenue
Length of stay — maximize revenue per stay	Efficient length of stay — unnecessary days are costs without revenue offset
Service line volume — grow profitable service lines	PMPM population health cost — manage population health to stay within budget
Payer mix — maximize commercially insured cases	Quality performance — quality metrics determine budget adjustments and AHEAD performance
Denials management — recover denied claims	Potentially avoidable utilization — prevent the ED visits and admissions that waste budget
Case mix index — capture higher-acuity cases	Community investment ROI — SDOH programs that reduce utilization have direct financial return
Operating margin per adjusted discharge	Operating margin within global budget — sustainability within fixed revenue

Figure 7.4 — Financial metrics transformation: FFS versus global budget environment. Source: HTR's Economics pillar framework.

7.6.2 The Hospital Financial Stress Test Model

The Hospital Financial Stress Test Model — available in the Research Lab for Professional and Advisory subscribers — allows hospital CFOs and finance teams to model their organization's financial position under different payment reform scenarios. The model's key scenarios for Vermont hospitals in 2026:

- RBP scenario: What does our financial position look like if commercial prices are set at 200% of Medicare (Oliver Wyman's recommended benchmark) or 250% (GMCB's phased approach)? What service lines have prices furthest above the benchmark and face the most revenue reduction?
- Global budget scenario: Under a prospective revenue cap equal to our current net patient revenue minus X%, what is our operating margin? What cost reductions or service redesigns are required to maintain solvency?
- Medicaid cut scenario: With H.R. 1's Medicaid revenue reductions phasing in from 2027-2031, what is our projected total revenue under conservative, base, and optimistic assumptions about our Medicaid payer mix?
- Transformation investment scenario: If we invest $X million in care management, CHW deployment, and telehealth, what reduction in avoidable hospitalizations and ED visits is required to achieve positive ROI within our VBC contract or global budget framework?

7.7 The ROI of Transformation — Building the Financial Case

Every transformation investment — a care management program, a behavioral health integration initiative, an SDOH screening program, a telehealth platform — requires a financial justification. Under fee-for-service, this justification is often difficult to construct because the investment may reduce utilization (and revenue) even when it improves outcomes. Under value-based care and global budgets, the financial case for transformation investment is considerably stronger.

7.7.1 The Transformation ROI Framework

The following framework structures the financial case for transformation investments in a VBC environment. It is the analytical backbone of HTR Advisory engagements that involve transformation investment decisions.

Investment type	Revenue/savings mechanism	Calculation approach	Vermont example
Care management program	Prevents hospitalizations and ED visits that, under VBC, are costs without offsetting revenue	(Prevented events) × (average cost per event) - (program operating cost) = Net savings. Divide by program cost for ROI.	Vermont: 32.3% of ED visits potentially avoidable. CHT program preventing 500 avoidable ED visits/year at $1,800 each = $900K savings. If CHT costs $300K/year, ROI = 200%.

Investment type	Revenue/savings mechanism	Calculation approach	Vermont example
Behavioral health integration (CoCM)	Reduces downstream high-cost utilization — hospitalizations, ED visits for MH crisis — for patients receiving earlier MH intervention	Estimate reduction in MH-related ED visits and hospitalizations for CoCM-managed patients vs. usual care. Vermont: 245 suicide/SH ED visits per 10,000 — each prevented = $1,800 saved plus downstream cost avoidance.	Vermont Blueprint MHI pilot: 80% of administrative entities increased CHT staffing. Every MH-related hospitalization prevented at $15,000 average saves 50x the cost of a CoCM BHCM monthly salary cost.
SDOH program investment	Reduces avoidable utilization driven by unmet social needs — housing-related ED visits, medication non-adherence from food insecurity, missed appointments from transportation barriers	Literature suggests 5-15% reduction in total cost of care for high-SDOH-burden populations receiving coordinated SDOH intervention. Apply to Vermont's identified high-SDOH populations.	Blueprint HRSN screening + CHT navigation: SDOH intervention for the 9% of Vermonters who are food insecure, if it reduces avoidable hospitalizations by even 10%, produces system savings exceeding program cost at scale.
Primary care investment (PCMH enhanced payments)	Reduces hospitalizations and specialty care through better chronic disease management and early intervention	Blueprint ROI study: $5.8M reduction in medical expenditure per $1M invested. At this ratio, $3M in Blueprint PCMH investment reduces system costs by $17.4M.	Vermont AHEAD primary care investment mandate: EAST Fund investment in primary care is essentially a system investment with documented 5.8:1 ROI in Vermont's own experience.
Telehealth for rural access	Reduces transportation-related care avoidance and enables earlier intervention before crisis; reduces specialist	Travel cost savings + avoided emergency transport + reduced length of stay for patients manageable via telehealth specialty consultation. RPM: each	Vermont RHT Program RPM grants: CHF readmission prevention via RPM in a population of 200 high-risk patients, preventing 10% of 30-day readmissions at

Investment type	Revenue/savings mechanism	Calculation approach	Vermont example
	transport costs for rural hospitals	prevented CHF hospitalization saves $15,000.	30% readmission rate, saves $90,000/year — ROI positive at $10,000/year RPM program cost.

Figure 7.5 — Transformation investment ROI framework with Vermont examples. Sources: HTR's Economics pillar framework; Blueprint for Health ROI study; CMS cost data; Vermont data from AHS Transformation Reports.

7.8 Making Care Primary — Vermont's Primary Care Economics

The AHEAD Model's requirement that states demonstrate increased investment in primary care as a percentage of total spending is not just a quality metric — it is an economics imperative. The foundational economic argument for primary care investment in a global budget environment is that primary care is the highest-return investment a healthcare system can make: the same $1 invested in primary care produces greater total cost of care reduction than $1 invested in hospital care or specialty care.

Vermont's Blueprint for Health embeds this economic logic in its design. The all-payer PMPM payments to PCMH practices are structured as an investment in reduced downstream costs — and Vermont's own evaluation data confirms the 5.8:1 ROI. Under AHEAD, primary care investment will be measured as a percentage of total spending, creating an explicit financial accountability framework for the primary care investment argument.

7.9 Work This Chapter on the Platform

Chapter 7's VBC financial mechanics — shared savings, risk, contract analysis, readiness — are exactly what these calculators do.

Do this	On this tool	What to look for
Model shared savings/loss under any APM contract (benchmark PMPM, sharing rate, MSR, stop-loss)	**Shared Savings Calculator** — /research-lab/payment-models?tab=apm-calc	Pessimistic/base/optimistic spread — the difference between a viable contract and a "managing blind" one.
Score an organization's value-based-care readiness across six domains	**VBC Readiness Assessment** —	Which domain is the binding constraint before assuming downside risk.

Do this	On this tool	What to look for
	/research-lab/knowledge-workspace?tab=readiness	
Run a cost-effectiveness analysis on a proposed intervention	**CEA Calculator** — /research-lab/payment-models?tab=cea	Whether the ROI case survives contact with the numbers.

Figure 7.H — Hands-on platform tools for the Economics Pillar in practice.

7.10 Implications for You

If you are a Vermont hospital executive: Value-based contract negotiation is not optional — it is the operational translation of the payment reform Acts 167 and 68 mandate. Every commercial contract renewal is an opportunity to negotiate terms that align with the global budget environment your organization will be operating under from FY2028. Specifically: negotiate shared savings arrangements that let you capture the financial benefit of the utilization reduction your clinical investments will produce; negotiate quality measurement aligned with HEDIS and AHEAD metrics rather than arbitrary payer-specific measures; and negotiate administrative simplification terms — consolidated reporting, standardized prior authorization, reduced documentation burden — that free up clinical capacity for population health management. The hospitals that have negotiated aligned VBC contracts before FY2028 will have a financial model that reinforces transformation. Those that have not will have a commercial revenue model that still rewards volume, creating internal conflict with the global budget environment.

If you are an AHS or GMCB official: Vermont's VBC financial modeling work — specifically, the AHEAD actuarial analysis and the global budget benchmark development — requires real actuarial expertise. The AHS-GMCB analytics vendor must include or be supplemented by actuarial capacity with experience in all-payer models. The specific technical challenge Vermont faces — setting fair benchmarks for 14 hospitals with different payer mixes, different geographic markets, and different historical efficiency levels — is not a job for general health analytics capability. It requires specialists in prospective payment system design. If Vermont's AHEAD benchmark methodology is set by people without this expertise, the benchmarks will be set wrong, and no amount of enforcement capability applied to wrong benchmarks produces the right transformation outcomes.

If you are a Vermont legislator: Vermont's commercial insurance market concentration — a small number of large commercial payers in a small state — creates negotiating dynamics that global budgets do not automatically resolve. GMCB has authority to regulate hospital rates; it does not have the same authority over commercial insurer behavior in VBC negotiations. The legislature should assess whether Vermont needs additional regulatory tools to ensure that the savings produced by RBP and global budgets flow through to premium reduction, as Act 68's passthrough requirement intends, rather than being absorbed in commercial insurer margins.

If you are a national health policy professional: Vermont's VBC financial modeling challenge — building the actuarial and analytical infrastructure for a mandatory all-payer model in a small state — is the challenge every state that attempts mandatory payment reform will face. Vermont's approach: VHCURES for population data, external analytics vendor for modeling capability, EAST Fund for transformation capital, and statutory deadlines for implementation — is worth studying as a model for states that want to move beyond voluntary VBC without the large state infrastructure that makes voluntary models easier to administer.

For Vermont hospitals operating under global budgets, the financial relationship between primary care investment and hospital cost management is direct: a better-functioning primary care system means fewer hospital admissions from preventable chronic disease exacerbations, fewer avoidable ED visits, and fewer hospitalizations for conditions that should have been managed in outpatient settings. The hospital that invests in primary care strengthens the community health infrastructure that reduces its own inpatient costs — a virtuous cycle that fee-for-service can never produce but global budgets require.

7.11 Key Concepts

Benchmark

The expected cost of care against which actual APM performance is measured. Shared savings or losses are calculated relative to the benchmark. Benchmark methodology — historical vs. regional, trend adjustment, risk adjustment — is the most consequential financial design choice in APM contract entry.

Attribution

The process of assigning patients to an ACO or provider for purposes of shared savings/loss calculation. Prospective attribution (known before the year) enables proactive care management; retrospective attribution (known after the year) creates uncertainty. Attribution methodology determines the denominator and exposure.

Shared savings rate

The percentage of savings below benchmark that the organization receives. Typically 50-75% for one-sided risk models; may reach 80% for full-risk models.

Minimum savings rate (MSR)

The threshold below which savings are not shared — typically 2-3.5% of benchmark — protecting against natural year-to-year variation triggering unearned payments.

Risk corridor

A provision in full-risk APM contracts that caps the organization's maximum financial loss at a defined threshold, providing stop-loss protection for organizations bearing downside risk.

RAF (Risk Adjustment Factor)

The multiplier applied to a benchmark to account for the health status of the attributed population. Accurate HCC (Hierarchical

Condition Category) coding is essential for RAF accuracy. RAF gaps — diagnoses that should be coded but are not — create benchmarks that understate the population's expected cost, producing apparent savings that disappear when coding is corrected.

VBC Transformation Readiness Assessment

The 30-dimension, 6-domain assessment evaluating an organization's readiness to enter and succeed in value-based care contracts. Total score below 60/120 indicates pre-transition status; gap analysis identifies highest-priority investments before contract entry.

Sources: CMS ACO model documentation; HTR's Economics pillar framework; Vermont Blueprint for Health ROI study (Population Health Management); GMCB hospital budget data; Oliver Wyman Act 167 Report; AHS Transformation Reports; Vermont AHEAD State Agreement; Act 68 of 2025.

Chapter 8: The Clinical Pillar — Redesigning Care Delivery for a Transformed System

> *Vermont's Blueprint for Health, PCMH transformation, and CCBHC expansion represent three decades of primary care investment whose return is now contingent on the payment reform that Acts 167 and 68 mandate.*

The Clinical pillar is where the abstractions of the other five pillars become a patient's actual experience of care — and it is also where good intentions most often fail to translate into changed practice, because clinical workflows do not change simply because a payment model or a policy mandate changes. This is true everywhere care is delivered, not only in Vermont. The Vermont Blueprint for Health is presented in this chapter because it is one of the longest-running primary care transformation programs in the country, and its fifteen-year record offers something most states do not have: a long enough timeline to distinguish what clinical redesign looks like when it is given time to mature versus when it is rushed ahead of the payment and technology foundations it depends on.

The Vermont Blueprint for Health is the most successful sustained primary care transformation program in American health policy. Vermont's behavioral health data: 245.5 ED visits per 10,000 for suicide ideation and self-harm.

8.1 The Clinical Pillar: What It Is and Why It Is Not the Same as Clinical Quality

A common confusion in healthcare transformation discussions conflates the clinical pillar with clinical quality measurement — HEDIS scores, Star ratings, readmission rates. Clinical quality measurement is important, and this chapter addresses it, but the clinical pillar is a larger concept. It encompasses the design of care delivery itself: how care is organized, who provides it, where it is delivered, and how the various elements of the healthcare system are coordinated around the patient's actual health needs rather than around administrative or financial convenience.

In the context of Vermont's transformation, the clinical pillar question is specific and urgent: what does primary care look like in a system no longer organized around hospitals as the default site of care? What does behavioral health integration look like in a state with 245 ED visits per 10,000 for suicide ideation and self-harm, and inadequate community mental health capacity? What does care coordination look like for Vermont's rapidly growing elderly population, with the highest-cost, most complex needs? And how do these clinical design questions connect to the payment reform and operational transformation happening simultaneously?

Vermont has more developed answers to these clinical design questions than most states, because Vermont has been running the Blueprint for Health — the most sophisticated all-payer primary care transformation program in the country — for more than fifteen years. The Blueprint is not a pilot or a demonstration. It is operating infrastructure, funded by all payers, embedded in primary care practices across all 14 Hospital Service Areas, and producing measurable results. Understanding the Blueprint is the foundation for understanding Vermont's clinical pillar strategy.

8.2 The Vermont Blueprint for Health — Fifteen Years of Primary Care Transformation

5.8:1	15+ Yrs	All-Payer	91%
Blueprint ROI on Investment	Operational Since 2006	Medicare + Medicaid + Commercial	VT Adults with Primary Care Provider

The Blueprint for Health is Vermont's all-payer primary care transformation program, established in 2006 and reaching statewide implementation by 2013. It is, in the view of most independent analysts, the most successful sustained primary care transformation initiative in American health policy. Its longevity, its all-payer architecture, its integration with Vermont's health data infrastructure, and its documented results make it the clinical foundation on which Vermont's broader healthcare transformation depends.

8.2.1 The Blueprint Architecture

The Blueprint operates through three integrated components that together constitute a population health management system at the primary care level:

Component	What it is	How it is funded	What it does
Patient-Centered Medical Home (PCMH)	Primary care practices recognized by NCQA as meeting advanced standards for care coordination, quality improvement, population management, and patient engagement	Per-member-per-month (PMPM) payments from all payers — Medicare, Medicaid, and commercial insurers — with a base payment plus a performance payment tied to quality outcomes	Transforms primary care practices from reactive, visit-based care to proactive, population-based care; coordinates care across providers; manages chronic conditions; screens for behavioral health and SDOH needs
Community Health Team (CHT)	Multi-disciplinary teams of social workers, nurses, community health workers, and behavioral	Blueprint program funding through DVHA; supplemented by OneCare (now transitioning),	Provides care coordination, SDOH navigation, behavioral health support, and

Component	What it is	How it is funded	What it does
	health professionals embedded in or closely connected to primary care practices	Medicaid, and grant funding	connection to community services for patients with complex needs; bridges the medical-nonmedical divide
Health Information Technology	VITL-based health information exchange linking practices to the statewide clinical data registry and VHCURES all-payer claims database	VITL funding; Blueprint program funding for data analytics support	Enables population-level HEDIS measurement; provides practices with care gap reports and patient registries; supports quality improvement with data-driven insights

Figure 8.1 — Vermont Blueprint for Health: three integrated components. Sources: Blueprint for Health website; Vermont Blueprint Overview (GMCB, 2022); Blueprint Annual Report 2024.

8.2.2 The Evidence: What the Blueprint Has Produced

Vermont's Blueprint is one of the most extensively studied primary care transformation programs in the country. The evidence base spans more than a decade of operation and multiple independent evaluations.

The most rigorous evaluation — a six-year longitudinal study using VHCURES all-payer claims data published in Population Health Management — compared Blueprint participants to non-Blueprint primary care populations and found consistent advantages across expenditure, utilization, and quality outcomes. The critical financial finding: medical expenditures decreased by approximately $5.8 million for every $1 million spent on the Blueprint initiative. This is a 5.8:1 return on investment, driven primarily by lower hospitalization rates and reduced outpatient facility use — precisely the kinds of avoidable utilization that Oliver Wyman identified as the largest source of indirect savings from Vermont's transformation.

Medicare specifically found statistically significant healthcare savings for its members in Vermont's Blueprint program. The Blueprint's own annual ROI analyses, comparing the Blueprint period to the pre-Blueprint baseline and to non-participating practices, consistently showed a positive all-payer return on investment. The finding that Blueprint patients had better outcomes across payer types — including Medicaid beneficiaries, who typically face the most barriers to effective primary care — is particularly significant for Vermont's equity goals.

Blueprint ROI: medical expenditure reduction per $1M invested $5.8M Lower hospitalization and outpatient facility use (Population Health Management study)	Vermont residents with a personal health care provider 91% vs. 87% national benchmark — 4 points above (AHS Nov 2025 report)
Blueprint PCMH patients with hypertension controlled 77% Blueprint quality data 2023 — maintained above national average	Blueprint PCMH patients with diabetes HbA1c NOT in control 22% Ongoing opportunity — population health management target

Figure 8.2 — Blueprint for Health clinical outcomes and ROI. Sources: Population Health Management journal; AHS November 2025 Transformation Report; Blueprint for Health 2024 data.

8.2.3 The Blueprint and AHEAD: A Critical Transition

Vermont's Blueprint program has operated since 2010 under a CMS Multi-Payer Advanced Primary Care Practice Demonstration that allowed Medicare to participate in PCMH and CHT payments. That agreement is scheduled to end with the conclusion of the All-Payer ACO Model. The AHEAD State Agreement resolves this transition risk: Primary Care AHEAD provides a voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models.

The transition from the current Medicare Blueprint payment to Primary Care AHEAD is operationally significant. Practices that currently receive Blueprint PMPM payments for Medicare patients will need to evaluate whether to participate in PC AHEAD, understand the new payment pathway options, and potentially redesign their care model to meet AHEAD's enhanced primary care requirements. AHS and DVHA have committed to supporting practices through this transition, recognizing that disrupting the Blueprint's Medicare payment stream would undermine one of Vermont's most effective clinical programs at precisely the moment transformation demands more of it.

Vermont's Blueprint enrollment target under AHEAD is ambitious: the program aims to substantially increase the percentage of Vermont's population — and particularly its Medicare population — attributed to Blueprint-participating practices. This expansion requires both new practice recruitment and support for existing practices to deepen their care management capabilities.

VERMONT IN PRACTICE — The Blueprint Quality Measure Crosswalk

Vermont's primary care quality landscape is unusually complex because practices must simultaneously meet the quality requirements of multiple programs: Blueprint PCMH standards, AHEAD quality metrics, HEDIS measures used by commercial payers, and GMCB's hospital quality framework. Oliver Wyman documented the fragmentation this creates — multiple measurement systems with overlapping but not identical measures, creating administrative burden for practices trying to optimize across all of them.

AHS and Blueprint have developed a Quality Measure Crosswalk — an interactive tool that maps measures across programs, identifies overlaps, and helps practices understand which quality investments serve multiple accountability requirements simultaneously. This is operationally important: a primary care practice that installs a hypertension management protocol does not need to report separately to Blueprint, AHEAD, and commercial payers if those programs are aligned around the same clinical measure.

The crosswalk also reveals the gaps — places where programs require incompatible measures, creating impossible choices for practices with limited data infrastructure. These gaps are a specific target for the AHEAD implementation process and the Statewide Strategic Plan's quality alignment provisions.

BEYOND VERMONT

A quality-measure crosswalk with gaps where programs demand incompatible metrics is not a Blueprint-specific artifact — it is the default condition of any practice or hospital participating in more than one value-based program, anywhere in the country, because Medicare, Medicaid, and commercial payers each design their quality programs independently. Vermont's response — using a statewide reform process (AHEAD, the Statewide Strategic Plan) to force alignment — is one model for closing that gap; a multi-state health system or a state without a comparable reform vehicle would need to pursue alignment through payer negotiation instead. Either way, the underlying problem — measurement fragmentation taxing exactly the practices with the least capacity to absorb it — is universal.

8.3 The Behavioral Health Crisis — Vermont's Most Urgent Clinical Challenge

Vermont Behavioral Health Crisis — The Data

245.5 ED visits per 10,000 for suicide ideation and self-harm. Higher in Rutland and Windham counties, and among Vermonters aged 15-44.

76% 30-day follow-up rate after mental health ED visits. 68% after SUD ED visits.

Vermont's CCBHC expansion target: 5 new certified entities by July 2026, covering every Hospital Service Area.

The Collaborative Care Model: 90+ RCTs and the only integrated care model with dedicated Medicare billing codes (CPT 99492, 99493, 99494).

Vermont's behavioral health data, documented in the November 2025 AHS transformation report, describes a system in crisis. With 245.5 ED visits per 10,000 for suicide ideation and self-harm — statistically higher in Rutland and Windham counties, and among Vermonters aged 15-44 — and with 30-day follow-up after mental health ED visits reaching only 76%, Vermont's behavioral health system is failing patients at the point of highest acuity. The crisis presentation data is not just a quality indicator; it is evidence of system-level failure to provide adequate access to behavioral health services before patients reach the emergency department.

Oliver Wyman's community meetings documented the texture of this failure in qualitative terms: patients with limited access to mental health services; care that is discriminatory for people who don't speak English as a first language; an observed weight bias and fatphobia making it difficult for patients to get appropriate care. The quantitative and qualitative data together describe a behavioral health system that is overwhelmed, under-resourced, and inequitably distributed — concentrated near urban centers and inaccessible in the rural communities where need is often greatest.

8.3.1 The Three-Layer Behavioral Health Architecture Vermont Is Building

Vermont is developing a behavioral health system with three complementary layers that together can serve the full spectrum of mental health and substance use disorder needs — from universal screening in primary care to intensive community treatment to acute crisis response.

Layer 1 — Mental Health Integration in Primary Care (CoCM and MHI)

The Collaborative Care Model (CoCM) is the only integrated behavioral health model with designated Medicare billing codes and a 90+ randomized controlled trial evidence base. CoCM embeds a behavioral health care manager (BHCM) and consulting psychiatrist within a primary care practice, enabling screening, brief intervention, and treatment for low-to-moderate acuity mental health and SUD conditions without referral. Vermont's Mental Health Integration pilot (MHI) through the Blueprint embedded mental health clinicians and community health workers in primary care settings starting in 2023, with nearly 80% of administrative entities reporting increased CHT staffing. The 2025 evaluation found improved access and engagement but sustainability concerns when temporary funding ends — a direct argument for making MHI permanent through AHEAD Primary Care funding.

Layer 2 — Certified Community-Based Integrated Health Centers (CCBHCs)

Vermont became an official CCBHC Demonstration State in 2024, receiving enhanced Medicaid funding to expand community behavioral health services. As of 2025, Vermont has two active CCBHC demonstration sites. Vermont's RHT Program application plans to certify five additional CCBHC entities by July 2026 — Health Care and Rehabilitation Services (Springfield), Howard Center (Burlington), Northwest Counseling and Support Services (St. Albans), Northeast Kingdom Human Services (Newport), and Washington County Mental Health Services (Barre). CCBHCs provide comprehensive behavioral health services including crisis intervention, substance use treatment, and care coordination — welcoming all patients regardless of age, diagnosis, or insurance status.

Layer 3 — Crisis System and Acute Behavioral Health Infrastructure

Vermont has developed mobile crisis response (operational since January 2024), centralized dispatch from the 988 Suicide and Crisis Lifeline, and new psychiatric residential treatment facilities for youth and forensic populations. Vermont's six 'Designated Hospitals' have inpatient psychiatric units; the Brattleboro Retreat serves as the state's primary psychiatric hospital. AHS has added hospital-level youth mental health beds, psychiatric residential treatment beds, and enhanced transport to the Brattleboro Retreat as part of its Act 167-mandated stabilization actions. The 30-day follow-up rate of 76% after mental health ED visits — while above national averages for some measures — indicates that crisis presentations are not consistently connected to ongoing care.

8.3.2 The Collaborative Care Model: Clinical Standard of Practice for Primary Care Integration

The Collaborative Care Model merits extended treatment because it is the clinical standard of practice for behavioral health integration in primary care — not just a promising approach, but the only integrated care model with a clear evidence base across 90+ randomized controlled trials, and the only model with dedicated Medicare billing codes (CPT codes 99492, 99493, 99494). Understanding CoCM's structure is important for any primary care or health system leader contemplating behavioral health integration.

CoCM component	Role	What they do	Vermont application
Primary Care Provider (PCP)	Treating and billing provider	Identifies patients with behavioral health needs through validated screening (PHQ-9, GAD-7); refers to BHCM; provides warm handoffs; bills CoCM codes monthly	Blueprint PCMH practices are the natural CoCM deployment sites — they already have the population management infrastructure and quality reporting that CoCM requires
Behavioral Health Care Manager (BHCM)	Day-to-day care coordinator	Maintains patient registry; provides brief interventions; monitors treatment response with validated rating scales; escalates to psychiatric consultant as needed; coordinates with community resources	MHI pilot found 80% of administrative entities increased CHT staffing — CHT staff are the natural BHCM pool; sustainability challenge when pilot funding ends
Psychiatric Consultant	Clinical supervision	Regular case review with BHCM; consultation on treatment recommendations;	Vermont's psychiatric shortage makes the CoCM model especially valuable — one

CoCM component	Role	What they do	Vermont application
		direct care for highest-acuity cases requiring psychiatric input; not the primary provider for most CoCM patients	psychiatrist can support an entire primary care practice's behavioral health population through consultation rather than direct care

Figure 8.3 — Collaborative Care Model components and Vermont application. Sources: SAMHSA CoCM program; CMS Behavioral Health Integration Services (January 2026); Vermont MHI Pilot Evaluation (2025).

The financial sustainability of CoCM in Vermont's environment depends on Medicare and Medicaid coverage of the billing codes, which Act 68 and AHEAD together support. Vermont Medicaid's coverage of CoCM codes is a prerequisite for practices serving high-Medicaid rural populations to implement the model — and DVHA's coordination with the Blueprint and AHEAD implementation is the mechanism for ensuring that coverage is in place. The AHEAD EAST Fund's explicit investment priority in mental health and substance use disorder services is the vehicle for funding the BHCM positions that primary care practices need to implement CoCM at scale.

"The MHI initiative allowed primary care teams to serve more patients and provide more consistent follow-up. Flexible and team-based care models helped patients engage in services, particularly those facing complex medical and social challenges."

— Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation, 2025

8.3.3 The SUD Continuum: Vermont's Hub and Spoke Model

Vermont's substance use disorder treatment system is organized around a Hub and Spoke model that has been nationally recognized as a model for rural SUD treatment. Hubs provide intensive addiction treatment services; Spokes — office-based opioid treatment (OBOT) practices — provide medication-assisted treatment in primary care settings. The Blueprint's practice finder maps active Spoke locations across Vermont's primary care network. AHS actions consistent with Act 167 have expanded this system: rate increases for residential SUD treatment, creation of 'hublets' in treatment deserts, support for co-occurring mental health and SUD treatment at hubs, contingency management for stimulant use disorder, and completion of gaps analyses to identify communities without adequate access.

The 30-day follow-up rate after ED visits for alcohol use — 68% in the November 2025 data — indicates that the care coordination infrastructure between crisis and community-based SUD treatment is not fully functional. Vermont is making specific investments to close this gap through the CCBHC expansion and the MHI permanent embedding of behavioral health in primary care, but the transition from crisis to community care remains the most operationally challenging step in the SUD care continuum.

8.4 Primary Care Redesign — Vermont's Strategy for the Front Door

Oliver Wyman's prescription for Vermont's primary care system was both clear and counterintuitive: Vermont does not have a primary care physician shortage in the traditional sense. HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is that primary care is not operating at the productivity and care model levels needed to serve Vermont's population. If primary care providers were supported to see three patients per hour, operating in team-based care models where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have sufficient primary care supply for well into the future.

This finding has profound implications for Vermont's primary care strategy. Rather than focusing exclusively on recruiting more physicians — an expensive, slow, and uncertain strategy — Vermont can meaningfully increase primary care capacity by transforming how existing primary care operates. The Blueprint's PCMH model is the vehicle for this transformation; AHEAD's enhanced primary care payments are the financial fuel.

8.4.1 The Vermont Comprehensive Primary Health Care Initiative

Act 68 established the Vermont Steering Committee for Comprehensive Primary Health Care — a body whose recommendations must be incorporated into the Health Care Delivery Advisory Committee's work and AHS's Statewide Strategic Plan. This committee represents a policy commitment to treating primary care not as one service line among many but as the foundational layer of Vermont's reformed delivery system — the front door through which most Vermonters access the healthcare system and the setting where most disease management, behavioral health integration, and SDOH screening should occur.

Vermont's comprehensive primary health care vision, as embedded in Act 68 and the AHEAD State Agreement, has five operational components:

Component	What it means in practice	Vermont investment vehicle
Team-based care	Every PCMH practice operates with a full care team — physician or APRN, medical assistant, care coordinator, behavioral health care manager, and CHT connection — with each team member working at the top of their licensure. The physician's time is	Blueprint PCMH payments; AHEAD enhanced primary care payments; RHT Program workforce investment in APRN training and scope expansion

Component	What it means in practice	Vermont investment vehicle
	reserved for the clinical decisions only a physician can make.	
Population health management	Every PCMH practice maintains a panel registry, tracks care gaps proactively, and reaches out to patients who need preventive care, chronic disease management, or behavioral health support before they present in crisis.	Blueprint data infrastructure; VHCURES linkage; GMCB analytics vendor procurement
SDOH screening and navigation	Universal screening for health-related social needs (housing, food security, transportation, financial insecurity) at every primary care visit, with warm handoffs to CHT staff for navigation and community resource connection.	Blueprint HRSN screening expansion; AHEAD EAST Fund SDOH investments; RHT Program community health worker deployment
Behavioral health integration	CoCM or equivalent behavioral health integration at every PCMH practice, providing screening, brief intervention, and care management for mental health and SUD conditions without requiring a separate specialty referral.	MHI pilot expansion through AHEAD and EAST Fund; CCBHC partnerships with primary care
Care coordination for complex patients	Intensive care management for Vermont’s highest-cost, highest-complexity patients — the elderly with multiple chronic conditions, dual-eligible Medicare/Medicaid beneficiaries, individuals with serious mental illness — linking primary care to specialist care, post-acute care, and community support.	CHT staff; dual eligible care planning; AHEAD population health requirements; Blueprint Chronic Care Initiative

Figure 8.4 — Vermont comprehensive primary health care: five operational components and investment vehicles. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Plans 2025.

8.4.2 Primary Care's Role in Reducing Hospital Utilization

The connection between primary care investment and hospital utilization reduction is one of the best-established relationships in health services research, and Vermont's own Blueprint data confirm it. The 2013 evaluation finding — lower hospitalization rates and reduced outpatient facility use for Blueprint participants — is the mechanism through which primary care investment produces hospital savings under a global budget.

Vermont's November 2025 data shows 32.3% of all ED visits are potentially avoidable — meaning they represent primary care access failures, not genuine emergency needs. At Vermont's scale (approximately 14 hospitals, roughly 200,000+ ED visits annually across the system), reducing potentially avoidable ED visits from 32.3% to a benchmark level of 25% would eliminate approximately 14,600 ED visits per year. At average ED visit costs in the $800-1,200 range, this reduction represents $12-18 million in annual savings — and that is before accounting for the downstream hospital admissions from avoidable ED visits that could have been prevented by earlier primary care intervention.

This is why Act 68's explicit goal of increased investment in primary care is not a soft preference — it is a financial necessity. Under global budgets, every avoidable hospitalization or ED visit is a direct cost without offsetting revenue. Hospitals operating under global budgets have a powerful financial incentive to invest in the primary care infrastructure that keeps their attributed population out of the hospital. Vermont's AHEAD EAST Fund, with its explicit investment priority in primary care, is designed to accelerate this investment.

8.5 Vermont's Clinical Quality Architecture — Measuring What Matters

Clinical quality measurement in Vermont operates through multiple overlapping frameworks that were not designed to be coherent with each other. HEDIS measures used by commercial payers, Blueprint quality measures for PCMH payments, GMCB hospital quality reporting, AHEAD accountability metrics, and Act 167/Act 68 outcome measures all exist simultaneously, creating a measurement burden that Oliver Wyman specifically cited as a driver of administrative complexity and provider frustration.

The Statewide Strategic Plan that AHS must deliver by December 2028 presents the opportunity — and creates the obligation — to align Vermont's clinical quality measurement framework around a coherent set of indicators that serve multiple accountability purposes simultaneously. The following framework represents the clinical quality architecture that Vermont's reform requires:

Domain	Priority measures	Why this measure	Current VT status
Preventive and chronic care	Hypertension control rate; diabetes HbA1c management; cervical cancer screening; colorectal cancer screening; childhood immunization	Chronic disease management is the highest-volume primary care activity; these measures predict long-term utilization and cost	Hypertension 77% controlled; diabetes 22% not controlled. Both reflect Blueprint PCMH performance; non-Blueprint practices likely worse
Behavioral health access and quality	30-day follow-up after MH ED visit; 30-day follow-up after SUD ED visit; depression screening rate; SUD treatment initiation and engagement	ED visits for MH/SUD are the clearest evidence of access failure; follow-up rates measure whether crisis intervention connects to ongoing care	76% 30-day follow-up after MH ED visit; 68% after SUD ED visit — both indicate coordination gaps
Primary care access	% practices accepting new Medicaid patients; wait time for new patient appointment; % population with attributed primary care provider	Access to primary care is the front door to the system; Medicaid acceptance is the equity indicator	59% of primary care practices accepting new Medicaid patients — significant equity gap; 91% of Vermonters have a personal health provider
Hospital quality and efficiency	30-day all-cause readmission rate; potentially avoidable ED visit rate; operating profit per adjusted discharge; admin cost per adjusted discharge	Hospital metrics measure both clinical quality and financial sustainability	Readmission: 14.8% (within range). Avoidable ED: 32.3% (major opportunity). Admin cost: 91% above benchmark
Equity and SDOH	Health outcome variation by county, income, race/ethnicity; insurance coverage rate by geography; SDOH screening completion rate; community resource referral completion	Equity measures are required by Act 68 and Act 167; geographic variation within Vermont is large and not adequately measured	Northeast Kingdom uninsured 6-8% vs. 3% statewide; rural mental health access gaps; SDOH screening completeness not consistently tracked

Figure 8.5 — Vermont clinical quality measurement framework: priority measures, rationale, and current status. Sources: AHS Transformation Report (November 2025); Blueprint for Health data; Oliver Wyman Act 167 Report.

8.5.1 The HEDIS Improvement Methodology — Vermont Context

The HEDIS improvement methodology — five-step framework from performance baseline through root cause analysis to sustainable systems design — applies directly to Vermont primary care practices and hospitals participating in Blueprint and AHEAD. The Vermont-specific context adds three considerations:

All-payer measurement is both an advantage and a challenge. Vermont's VHCURES all-payer claims database, linked to the Blueprint clinical data registry, enables HEDIS measurement across the full population — not just the commercial population a health plan would report on. This means Vermont can identify disparities that would be invisible in single-payer reporting. The challenge is that the database requires VITL connectivity that remains voluntary and incomplete, particularly among community-based providers.

Blueprint field staff are the quality improvement infrastructure. Unlike states where practices must manage their own quality improvement in isolation, Vermont's Blueprint field staff — one Quality Improvement Facilitator per HSA region — provide ongoing support, data analysis, and evidence-based practice coaching. This infrastructure is a competitive advantage for Vermont's transformation; it means quality improvement capacity is not limited by the resources of individual practices.

Performance payment alignment creates a direct financial incentive for quality improvement. Blueprint's PMPM payments include a performance payment component tied to quality outcomes and utilization measures. AHEAD's enhanced primary care payments include quality-based adjustments. This means Vermont primary care practices have a direct financial incentive for HEDIS improvement — not just a regulatory compliance requirement.

8.6 Care for Vermont's Aging Population — The Clinical Imperative

Vermont's demographic trajectory — 65+ population growing 57% by 2040, exceeding 30% of total population — creates a clinical imperative that is simultaneously a financial imperative. The aging population drives demand for the most expensive and most complex services: cancer treatment, cardiac surgery, dementia care, frailty-related hospitalization, long-term care, end-of-life care. Designing the clinical system to serve this population well — which means keeping elderly Vermonters healthy, connected to their communities, and out of hospitals and nursing homes to the extent clinically appropriate — is both the right thing to do and the financial requirement of the global budget model.

8.6.1 The PACE Model and Vermont's Long-Term Care Gap

The Program of All-Inclusive Care for the Elderly (PACE) integrates medical, social, and long-term care services for frail elderly patients who would otherwise require nursing home placement, enabling them to remain in their communities. PACE is one of the most effective and cost-efficient models for managing the population segment that drives the highest healthcare costs in any state system. Vermont has no PACE programs. This is a significant gap that Vermont's transformation planning must address.

Oliver Wyman's community meetings documented the dimensions of Vermont's long-term care capacity crisis: lack of staffed social care facilities leads to hospital crowding when patients cannot be discharged; nursing home occupancy is high and new capacity is constrained; home and community-based services (HCBS) — the preferred alternative to institutional care — are limited by workforce shortages. Vermont's RHT Program application specifically addresses long-term care gaps: funding for dialysis units in nursing homes so dually eligible residents can receive long-term care without institutional transfer, and a second nursing home ventilator unit to double capacity for high-acuity nursing home care.

The clinical model for Vermont's aging population requires three integrated components: strong primary care for chronic disease management and prevention; robust HCBS and PACE to support aging in place; and smooth hospital-to-community transitions when hospitalization is unavoidable. The first component is the Blueprint's domain. The second requires the SDOH investment in housing and transportation that Oliver Wyman made Imperative 1. The third requires the care coordination infrastructure that the CHT model provides.

8.6.2 Dementia Care: The Coming Clinical Challenge

Dementia affects an estimated 14 million Americans nationally by 2060, and Vermont's older demographic profile means its per-capita rate of dementia will reach the national projection earlier. Vermont currently lacks dementia-specialized primary care at scale, adequate memory care capacity in its nursing home sector, and family caregiver support infrastructure proportionate to the approaching need. Oliver Wyman's COE framework designates memory care as a COE specialty — recognizing that dementia care at adequate quality requires specialized expertise and cannot be replicated at every community hospital.

The clinical pillar implications are clear: Vermont's transformation plan must include explicit investment in dementia care infrastructure — the training of primary care providers in cognitive assessment and early dementia management, the development of memory care COE capacity at designated facilities, the expansion of adult day programs and respite care for family caregivers, and the integration of dementia care into the community-based care model that Vermont's global budget environment requires.

8.7 Clinical Pillar Implications for AHS's Statewide Strategic Plan

The Statewide Health Care Delivery Strategic Plan that AHS must deliver by December 2028 must address the clinical pillar explicitly. Based on Vermont's current clinical landscape and the transformation agenda established by Acts 167 and 68, the plan should contain five clinical design commitments:

Commitment	What it requires	Timeline	Primary vehicle
Universal Blueprint PCMH coverage	Every primary care practice in Vermont participates in Blueprint PCMH recognition and receives all-payer PMPM payments; no Vermont resident is more than 30 minutes from a PCMH practice	2025-2028	Blueprint expansion; AHEAD PC primary care recruitment; RHT workforce investment
Behavioral health integration at every PCMH	Every PCMH practice implements CoCM or equivalent behavioral health integration, with BHCM staff and psychiatric consultation available to all attributed patients	2025-2028	MHI permanent funding through AHEAD EAST Fund; CCBHC partnerships; BHCM workforce training through RHT Program
CCBHC network covering all HSAs	Certified Community-Based integrated Health Centers established in every Hospital Service Area, providing comprehensive community behavioral health services including crisis response, SUD treatment, and care coordination	2026 (5 new CCBHCs) through 2028	CCBHC Demonstration State enhanced Medicaid; RHT Program funding; Act 68 grants
PACE program development	At least one PACE program operational in Vermont, serving the highest-acuity elderly population that would otherwise require nursing home placement; feasibility assessment for additional programs	2026-2028 (feasibility and launch)	AHEAD EAST Fund; RHT long-term care infrastructure investment; Medicaid PACE program rules
Dementia care infrastructure	Memory care COE designation at 3-5 facilities; primary care dementia assessment protocols embedded in all PCMH	2026-2030	RHT Program; COE designation process; Blueprint quality measures expansion

Commitment	What it requires	Timeline	Primary vehicle
	practices; family caregiver support programs in all HSAs		

Table 8.1 — Clinical-pillar implementation commitments for the Statewide Strategic Plan. Source: HTR Analysis (2026).

8.8 Work This Chapter on the Platform

Chapter 8's clinical-redesign argument — the Blueprint, Collaborative Care, panel risk — rests on stratification and quality measurement you can run directly.

Do this	On this tool	What to look for
Stratify a clinical panel by risk tier and match staffing intensity	**Risk Stratification Methodology** — /research-lab/vbc-clinical-quality?tab=risk	How panel design changes when staffing follows risk rather than volume.
Model HEDIS measure performance against NCQA benchmarks	**VBC Quality Measures** — /research-lab/vbc-clinical-quality?tab=quality	Which measures move population outcomes vs. which only move scores.

Figure 8.H — Hands-on platform tools for the Clinical Pillar.

8.9 Implications for You

If you are a Vermont hospital executive: Patient-Centered Medical Home transformation is not a quality improvement project — it is a financial strategy. Under global budgets, your revenue is fixed. The only path to financial improvement is reducing the cost of care for your attributed population. PCMH-transformed primary care practices — with team-based staffing, proactive outreach, and integrated behavioral health — produce the utilization reduction that makes a fixed-revenue model financially viable. The Blueprint's 5.8:1 ROI is not a projection; it is a documented result from Vermont practices. The question is not whether PCMH transformation produces financial return. It is whether your organization is investing in it fast enough to realize the return before AHEAD's financial accountability begins.

If you are an AHS or GMCB official: The Collaborative Care Model — integrating behavioral health into primary care — is the clinical intervention with the highest evidence base for the population health challenges that drive Vermont's cost trajectory. Vermont's rural primary care practices see patients who need behavioral health care and cannot access it because the specialty system is overwhelmed. CoCM provides a solution that is evidence-based, financially sustainable under AHEAD, and deployable in rural settings without specialist capacity. The CCBHC certification expansion is the right policy vehicle. The timeline — completing certification across all HSAs before AHEAD's clinical accountability begins — should be treated as a clinical milestone with the same urgency as the analytics vendor deployment.

If you are a Vermont legislator: The Blueprint for Health is Vermont's most successful population health program and its most underfunded one. The 5.8:1 ROI documented from Vermont's own experience is the strongest evidence available that primary care investment produces system savings. The Blueprint's current funding level does not allow it to operate at the scale AHEAD's population health targets require. A legislative investment in Blueprint capacity — specifically, Community Health Team expansion and PCMH transformation support for practices not yet at advanced tier — is among the highest-ROI appropriations Vermont can make before FY2028.

If you are a national health policy professional: Vermont's clinical pillar experience — specifically, the relationship between PCMH transformation, Blueprint Community Health Teams, and population health improvement — is the best available evidence that primary care investment produces system-level returns in a rural setting. The 5.8:1 ROI figure is from a 2016 peer-reviewed study of 2008–2013 data; updated evidence from Vermont's more recent Blueprint experience is needed and will be produced by AHEAD's clinical measurement requirements. Those updated results will be the primary evidence base for primary care investment policy in rural markets nationally.

Figure 8.6 — Clinical pillar commitments for Vermont's Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Oliver Wyman Act 167 Report; AHS Transformation Plans; Vermont RHT Program Application.

8.10 Key Concepts Introduced or Expanded in This Chapter

Blueprint for Health

Vermont's all-payer primary care transformation program, operating since 2006. Provides per-member-per-month payments to NCQA-recognized Patient-Centered Medical Home practices and funds Community Health Teams to provide wraparound care coordination services. The foundational layer of Vermont's clinical pillar.

Patient-Centered Medical Home (PCMH)

A primary care practice recognized by NCQA as meeting advanced standards for care coordination, population management, quality improvement, and patient engagement. The organizational model for Vermont's primary care system under Blueprint and AHEAD.

Community Health Team (CHT)

Multi-disciplinary team of social workers, nurses, community health workers, and behavioral health professionals supporting PCMH practices with care coordination, SDOH navigation, and connection to community services.

Collaborative Care Model (CoCM)

The integrated behavioral health care model with the strongest evidence base — 90+ RCTs — embedding a behavioral health care manager and psychiatric consultant in primary care to address mental health and SUD needs without requiring specialty referral. The only integrated care model with designated Medicare billing codes.

Certified Community-Based integrated Health Center (CCBHC)

Vermont's term for what CMS calls Certified Community Behavioral Health Clinics. Comprehensive community-based behavioral health facilities receiving enhanced Medicaid funding through the federal CCBHC Demonstration program.

Mental Health Integration (MHI)

Vermont Blueprint's pilot program embedding mental health clinicians and community health workers in primary care settings; evaluated in 2025 as highly effective but facing sustainability challenges when pilot funding ends.

PACE (Program of All-Inclusive Care for the Elderly)

An integrated medical, social, and long-term care model enabling frail elderly individuals to remain in their communities rather than entering nursing homes. Vermont currently has no PACE programs — a significant gap given its aging demographics.

Hub and Spoke

Vermont's SUD treatment model: Hubs provide intensive addiction treatment; Spokes are office-based opioid treatment practices in primary care settings providing medication-assisted treatment.

HRSN (Health-Related Social Needs)

The social needs that affect health outcomes — housing, food security, transportation, financial insecurity, social isolation — now universally screened for in Vermont's Blueprint primary care practices.

Sources: Vermont Blueprint for Health Annual Report 2024; Vermont DMH CCBHC Program website (mentalhealth.vermont.gov); Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation (2025); Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont Agency of Human Services Health Care System Transformation Report (November 2025); Act 68 of 2025; Vermont AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); Population Health Management (Blueprint ROI study, 2016); SAMHSA CoCM Program; CMS Behavioral Health Integration Services (January 2026).

Chapter 9: The Clinical Pillar in Practice — Care Model Implementation, Quality Mechanics, and the Vermont Clinical Transformation Toolkit

> *The operational deployment of Vermont's care models requires workforce, capital, and workflow redesign at a pace the Rural Health Transformation Program is designed but not guaranteed to support.*

Every clinical model in this chapter — PCMH transformation, the Collaborative Care Model, CCBHC operations, HEDIS improvement — has a national specification and a national evidence base; none of it was invented in Vermont. What this chapter provides is the implementation layer that national specifications typically leave abstract: panel sizes, staffing ratios, workflow sequencing, the practical order in which a practice actually builds these capabilities. The Vermont Blueprint practices and the Vermont Clinical Transformation Toolkit are this chapter's worked examples of what implementation looks like when a state has spent fifteen years iterating on exactly these questions — useful to any practice or system implementing the same national models, regardless of state.

How healthcare organizations actually implement the clinical programs that produce outcomes under value-based care — Blueprint practice transformation mechanics, CoCM deployment, CCBHC operational requirements, HEDIS improvement methodology, and the clinical data infrastructure that connects care delivery to population health management.

9.1 From Clinical Architecture to Clinical Practice

Chapter 1 established the clinical architecture — the Blueprint for Health as Vermont's primary care foundation, the three-layer behavioral health system Vermont is building, the comprehensive primary health care vision embedded in Act 68, and the clinical imperative of serving Vermont's rapidly aging population. This chapter turns from architecture to practice: how do healthcare organizations actually implement and operationalize these clinical programs? What does Blueprint PCMH transformation require of a primary care practice at the operational level? How does a hospital or health system deploy the Collaborative Care Model? What does HEDIS improvement require beyond a measurement committee? And how does the clinical pillar's data infrastructure connect care delivery decisions to the population health management functions that value-based care requires?

Clinical practice in the context of Vermont's transformation is not the same as clinical quality improvement in a fee-for-service environment. Under FFS, clinical improvement programs ask: how do we improve our scores to maintain accreditation and satisfy payer reporting requirements? Under VBC and global budgets, clinical improvement asks a fundamentally different question: how do we redesign care delivery so that our attributed population stays healthier, uses fewer avoidable services, and achieves the clinical outcomes that our payment model rewards? These are population health management questions, and they require a different operational framework, different staff skills, different data infrastructure, and different leadership engagement than traditional clinical quality programs.

9.2 PCMH Transformation — The Blueprint Practice Implementation Playbook

9.2.1 The Blueprint PCMH Recognition Process — What It Actually Requires

NCQA Patient-Centered Medical Home recognition is the organizational prerequisite for Blueprint participation and AHEAD enhanced primary care payments. For practices that have not previously sought NCQA recognition — and many of Vermont's smaller rural practices have not — the transformation process is operationally intensive and typically requires 12-18 months of preparation. Understanding what PCMH recognition actually requires operationally is the starting point for any primary care practice or health system leader managing this transition.

NCQA PCMH recognition is organized around six concept areas that together define what a transformed primary care practice looks like operationally. The 2023 PCMH Standards represent the current requirements:

Concept Area	What it requires operationally	Vermont implementation challenge
Team-Based Care and Practice Organization	Defined care team roles and responsibilities; written policies for team-based care; staff training in team-based approaches; patient engagement in care planning	Many Vermont rural practices operate with small teams where physician or APRN does work that should be delegated. Role definition requires workflow redesign, not just policy writing.
Knowing and Managing Your Patients	Comprehensive patient data collection; population management using a registry; proactive outreach for preventive and chronic care; stratification by risk level	Registry-based proactive outreach requires data infrastructure most small practices lack. Blueprint field staff and VITL connection are the Vermont-specific support infrastructure.
Patient-Centered Access and Continuity	Same-day appointments for urgent needs; 24/7 access for patient advice; continuity with personal	After-hours access is the most operationally challenging requirement for small rural practices. After-hours telephone

Concept Area	What it requires operationally	Vermont implementation challenge
	physician; timely communication response	advice lines are a shared service solution.
Care Management and Support	Systematic identification of patients for care management; care plans for high-risk patients; SDOH screening and connection to resources; care coordination across providers and settings	CHT staff provide much of this function in Vermont's Blueprint model; the PCMH practice need not hire all care management staff internally if CHT connection is established.
Care Coordination and Care Transitions	Timely follow-up after hospital discharge; medication reconciliation; coordination with specialists and behavioral health; referral tracking and closing the loop	30-day follow-up after behavioral health hospitalization — Vermont's 76% rate — is a care coordination failure. PCMH recognition requires the systems to catch and follow up with discharged patients.
Performance Measurement and Quality Improvement	Systematic data collection on quality measures; regular quality improvement activities; use of patient experience data; reporting on clinical quality and utilization outcomes	Requires data infrastructure and staff capacity for measurement. Blueprint HEDIS support and GMCB analytics vendor are Vermont's support infrastructure.

Figure 9.1 — NCQA PCMH recognition: six concept areas and Vermont implementation challenges. Sources: NCQA PCMH Standards and Guidelines, 2023 edition; Vermont Blueprint for Health Annual Report 2024; Oliver Wyman Act 167 Community Engagement: Recommendations (2024).

9.2.2 The Blueprint Field Staff Model — Vermont's Practice Transformation Infrastructure

Vermont's Blueprint program provides something that most states' primary care transformation initiatives do not: dedicated field staff whose full-time job is supporting practice transformation. Each of Vermont's 14 Hospital Service Areas has at least one Blueprint field staff member — typically a Quality Improvement Facilitator — whose role is to provide ongoing support to Blueprint-participating practices. This infrastructure is operationally important and is often underappreciated as a driver of Vermont's Blueprint success.

What Blueprint field staff actually do distinguishes Vermont's approach from states that distribute PCMH toolkits and hope practices implement them independently. Practice readiness assessment identifies gaps in workflow, data infrastructure, and team structure before the formal NCQA submission. Data coaching helps practices interpret their HEDIS data reports, care gap analyses, and VHCURES-linked population reports — translating data into specific practice changes. Workflow redesign support facilitates redesign sessions within practices, helping teams identify where current workflows create barriers to the care coordination and proactive outreach that PCMH requires. And AHEAD transition support — helping practices navigate the transition from current Blueprint Medicare payment to Primary Care AHEAD — is the most operationally significant clinical program transition Vermont's primary care system faces.

This field staff model is a Blueprint competitive advantage that AHS and DVHA must protect through the AHEAD transition. If field staff capacity is reduced during the payment model transition — as sometimes happens when program funding structures change — the support infrastructure that enables practice transformation collapses, and practices revert to pre-transformation patterns.

9.2.3 The Practice Panel: Right-Sizing and Risk Stratification

Effective population health management begins with knowing who is on the panel and what they need. Vermont's Blueprint practices operate with attributed patient panels — lists of patients for whom the practice is clinically and financially accountable under Blueprint and AHEAD. The operational work of panel management is the foundation of everything else.

Panel right-sizing addresses the question of how many patients a care team can effectively manage. Under team-based care models with full delegation of non-physician tasks, a primary care physician with appropriate support staff can manage a panel of 1,500-2,000 patients — significantly larger than the 1,100-1,200 typically associated with solo or minimally supported practice. Vermont's Oliver Wyman finding — that Vermont's primary care supply problem is primarily a productivity and care model problem, not a physician shortage problem — rests on this logic. Practices that adopt team-based models with full delegation can expand effective panel capacity by 30-50% without adding physicians.

Panel risk stratification organizes the panel into tiers by clinical complexity and risk level, enabling the care team to deploy its resources proportionally. The 3-5% of complex patients in Tier 1 typically account for 30-40% of total cost of care for the panel. This concentration of cost in a small, identifiable population is the financial opportunity that drives VBC economics. A care management program that prevents one hospitalization per month among Tier 1 patients in a 2,000-patient panel generates approximately $180,000 in avoided cost annually — sufficient to fully fund a half-time care coordinator position and produce net positive ROI under any VBC arrangement.

Tier	% of Panel	Characteristics	Care Model
Tier 1 — Complex	3-5%	Multiple chronic conditions, high prior utilization, behavioral health and SDOH needs, frailty	Intensive care management: CHT assignment, care plan, regular outreach, direct care manager relationship
Tier 2 — Moderate Risk	15-20%	One or two chronic conditions with management challenges, behavioral health needs, recent hospitalization or ED visit	Proactive chronic disease management: care gap outreach, behavioral health integration, medication management
Tier 3 — Standard	50-60%	Healthy or stable chronic conditions, routine preventive needs	Preventive care: annual wellness visit, immunizations, screening, health coaching
Tier 4 — Inactive	20-30%	Attributed but not engaged with practice; may be accessing care elsewhere or not at all	Outreach and re-engagement: targeted contact campaigns, open access scheduling, community health worker outreach

Figure 9.2 — Panel risk stratification framework for Vermont PCMH practices. Sources: Vermont Blueprint for Health Practice Support Resources; HRSA Population Health Management Training; Oliver Wyman Act 167 Report section on primary care productivity.

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Panel risk stratification is a core requirement of PCMH recognition under any major accrediting body (NCQA, URAC, or state-specific equivalents), not a Vermont-specific practice. The Vermont figures above — panel sizes, risk tiers, staffing ratios — are a starting reference point calibrated to Vermont's payer mix and rural geography; a practice in a denser urban market or with a different payer mix should expect different panel sizes and staffing ratios, but the underlying framework — stratify the panel, match staffing intensity to risk tier, build the workflow around the highest-risk patients first — transfers directly.

9.3 Deploying the Collaborative Care Model — Operational Requirements

9.3.1 Site Readiness Assessment

The Collaborative Care Model's evidence base — 90+ randomized controlled trials and designated Medicare billing codes — makes it the clinical standard of practice for behavioral health integration in primary care. But the evidence base does not implement CoCM. Before implementing CoCM, a primary care practice must assess its readiness across four operational dimensions.

EHR integration is the first requirement. CoCM requires a patient registry — a list of all patients receiving CoCM services with their current treatment status, validated outcome scores, and care manager contact information. Most EHR systems can generate this registry if properly configured; many Vermont practices have not configured their EHR for registry-based CoCM management. The first implementation step is registry setup within the existing EHR.

Physical space, workflow protocols, and billing setup complete the four-dimension readiness assessment. CoCM billing codes 99492, 99493, and 99494 require time tracking — the practice must document the time the BHCM and physician spend on CoCM activities each month. Most practices need to add a time-tracking tool or workflow to their billing process before billing begins.

9.3.2 The Psychiatric Consultation Model — Vermont's Workforce Solution

Vermont's psychiatric workforce shortage creates a specific implementation challenge for CoCM: where does the psychiatric consultant come from? The telepsychiatry model is Vermont's primary solution. Several Vermont-based telepsychiatry services, including consultation-liaison programs through UVMMC and regional mental health providers, can provide the weekly case review that CoCM's psychiatric consultation component requires without requiring in-person psychiatric presence at every primary care site.

The CoCM psychiatric consultation model is more efficient than traditional psychiatric referral in exactly the way that Vermont's workforce situation requires. One psychiatrist providing consultation through a telepsychiatry model can support 10-15 primary care practices simultaneously — monitoring the BHCMs' caseloads through weekly case review calls, providing treatment recommendations without direct patient contact for the majority of cases, and reserving direct appointments for the highest-acuity cases that genuinely require in-person psychiatric evaluation. Vermont's psychiatric workforce, deployed this way, can support CoCM at a scale that direct psychiatric referral could never achieve.

9.3.3 The BHCM Workforce Pipeline

The behavioral health care manager is the linchpin of CoCM — the person who maintains the patient registry, conducts most patient contact, monitors treatment response, and escalates to the psychiatric consultant as needed. Vermont's implementation challenge is that this is a role for which no pre-existing training pipeline exists. BHCMs typically have backgrounds in social work, nursing, counseling, or community health, but the specific skills — registry management, brief evidence-based intervention, outcome measurement, psychiatric consultation preparation — are not covered comprehensively in any standard credential.

Vermont's RHT Program application identifies BHCM workforce training as a specific investment priority. The practical solution in Vermont's current environment: CHT staff with the appropriate clinical background are the natural BHCM pool. Converting existing CHT staff to formal BHCM roles — with targeted training in CoCM-specific skills — is faster and more sustainable than recruiting externally. The 80% of Blueprint administrative entities that reported increased CHT staffing in the 2025 MHI evaluation suggests that this workforce exists; it needs formalization and training, not creation from scratch.

CoCM Component	Role	What they do	Vermont application
Primary Care Provider (PCP)	Treating and billing provider	Identifies patients through validated screening (PHQ-9, GAD-7); refers to BHCM; provides warm handoffs; bills CoCM codes monthly	Blueprint PCMH practices are the natural CoCM deployment sites — they already have the population management infrastructure and quality reporting that CoCM requires
Behavioral Health Care Manager (BHCM)	Day-to-day care coordinator	Maintains patient registry; provides brief interventions; monitors treatment response with validated rating scales; escalates to psychiatric consultant as needed; coordinates with community resources	MHI pilot found 80% of administrative entities increased CHT staffing — CHT staff are the natural BHCM pool; sustainability challenge when pilot funding ends
Psychiatric Consultant	Clinical supervision	Regular case review with BHCM; consultation on treatment recommendations; direct care for highest-acuity cases; not the primary provider for most CoCM patients	Vermont's psychiatric shortage makes the CoCM model especially valuable — one psychiatrist can support an entire primary care practice's behavioral health population through consultation rather than direct care

Figure 9.3 — Collaborative Care Model components and Vermont application. Sources: SAMHSA CoCM program; CMS Behavioral Health Integration Services (January 2026); Vermont MHI Pilot Evaluation (2025).

9.4 HEDIS Improvement Methodology — Vermont Context

HEDIS is the measurement language of the Blueprint, the commercial market, and AHEAD quality accountability. Vermont primary care practices and health systems that want to improve their HEDIS performance — and their financial performance under quality-adjusted VBC payments — need a systematic improvement methodology, not a measurement committee.

9.4.1 The Five-Step HEDIS Improvement Framework

Step 1 — Performance Baseline: Know where you are before deciding where to go. Vermont's Blueprint HEDIS report provides practices with measure-level performance data compared to NCQA 50th and 90th percentile benchmarks. The starting question: which measures are you below the 50th percentile?

Step 2 — Gap Analysis: Identify how many patients have open care gaps for each priority measure. A practice with a hypertension control rate of 62% needs to know: how many patients with diagnosed hypertension have blood pressure above 140/90 in the last measurement period? This is the numerator gap — the number of patients who need to be moved into the controlled category to reach a target performance level. VHCURES linkage and Blueprint data reports provide this gap analysis.

Step 3 — Root Cause Analysis: Why do patients have uncontrolled hypertension? The clinical answer is almost always multi-factorial: medication non-adherence, inadequate follow-up, SDOH barriers, clinical inertia in prescribing, missed measurement opportunities. Root cause analysis distinguishes between problems that are clinical, operational, data-related, and SDOH-driven. Each root cause requires a different intervention.

Step 4 — Intervention Design: Match the intervention to the root cause. Clinical inertia requires a prescribing protocol embedded in the EHR that prompts intensification when blood pressure is above goal. Missed measurement opportunities require a blood pressure measurement protocol at every visit. SDOH barriers require CHT referral for medication assistance programs and food access resources.

Step 5 — Sustainable Systems Change: The most common HEDIS improvement failure mode is the improvement campaign that produces a short-term spike in performance followed by regression to baseline. Sustainable improvement requires embedding the intervention in standing workflow, not treating it as a project. The blood pressure measurement protocol must become the default for every patient encounter, not a special push for the measurement period.

9.4.2 Vermont-Specific HEDIS Opportunity Areas

Vermont's November 2025 data reveals specific HEDIS opportunity areas that primary care practices can prioritize for improvement investments with the highest population health and financial return under VBC.

Diabetes HbA1c Management: 22% of Vermont's Blueprint diabetic patients have HbA1c not in control. Given Vermont's population of approximately 60,000-70,000 adults with type 2 diabetes, this represents 13,000-15,000 patients with inadequately controlled diabetes — each at elevated risk for hospitalization from diabetic complications.

Behavioral Health Follow-Up: 76% 30-day follow-up after mental health ED visits, 68% after SUD ED visits. Every patient who does not receive timely follow-up after a behavioral health ED visit is at elevated risk for a return ED visit, hospitalization, or worse outcome. The HEDIS Follow-Up After Emergency Department Visit for Mental Illness (FUM) measure directly captures this gap.

Cervical Cancer Screening: The 2025 AHS transformation report notes below-benchmark cervical cancer screening rates in several Vermont communities, particularly in rural areas with limited access to gynecological services. The patients most likely to be under-screened are those with the most transportation barriers and the least consistent primary care engagement — an equity issue as well as a quality issue.

9.5 The Clinical Data Infrastructure — Connecting Care to Population Health Management

The clinical pillar's effectiveness under VBC depends on data infrastructure that most Vermont primary care practices have not fully built. The connection between individual clinical encounters and population-level outcome tracking is the operational gap that prevents good clinical care from translating into good VBC financial performance.

9.5.1 Vermont's Three-Step Clinical Data Model

Step 1 — Point-of-Care Data Capture: The clinical encounter generates the data. Blood pressure measurements, HbA1c results, PHQ-9 scores, medication lists, care gap closures — all of this information is created at the point of care. The quality of VBC performance tracking depends entirely on the completeness and accuracy of this initial data capture. If the blood pressure is measured but documented in free-text rather than a structured EHR field, it cannot be extracted for HEDIS reporting or care gap analysis.

Vermont's implementation gap: many Vermont practices, particularly smaller rural practices, have inconsistent structured data capture. Staff document clinical information in free text because the structured field workflow is slower at the point of care. This apparent efficiency creates a downstream measurement failure that costs the practice in HEDIS performance and care gap identification. Blueprint field staff training on structured data capture is the intervention; EHR workflow redesign is the vehicle.

Step 2 — Practice-Level Aggregation: The EHR aggregates data across all patients to generate the practice-level population view. The patient registry, the care gap report, the HEDIS measure performance dashboard — these are all generated by aggregating individual encounter data across the full panel. Vermont's VITL health information exchange provides claims data from VHCURES, enabling the practice to see utilization that occurs outside their own walls — emergency department visits, hospitalizations, specialist encounters — and close care gaps based on information the practice would otherwise not have.

Step 3 — System-Level Population Health Management: The GMCB analytics vendor, once deployed, will aggregate practice-level data to the HSA and statewide level, enabling Vermont to manage population health across the full attributed population for AHEAD accountability. Vermont's most critical current gap is at Steps 1 and 2 — incomplete structured data capture and incomplete VITL connectivity. The analytics vendor cannot analyze data that was not captured or not shared. Investment in practice-level data infrastructure is the clinical pillar prerequisite for effective AHEAD population health management.

9.5.2 HCC Coding Excellence — The Clinical Pillar's Financial Foundation

HCC coding is often discussed as a revenue cycle function. In the context of VBC, it is equally a clinical function: the accuracy of hierarchical condition category coding determines whether a patient's clinical complexity is accurately reflected in their risk adjustment factor, which determines the organization's global budget. A clinician who documents a patient's diabetes as 'type 2 diabetes' without specifying complication status is leaving RAF value uncaptured — which translates directly into a lower global budget than the patient's actual cost burden warrants.

Vermont's AHEAD preparation requires a systematic HCC gap analysis for every practice's Medicare-attributed population. The three most common HCC documentation opportunities in Vermont's primary care population: chronic kidney disease staging (CKD is documented as present but stage is frequently not specified, and CKD Stage 3 and above have distinct HCC codes with progressively higher RAF values); diabetes complications (Vermont's primary care population has high rates of diabetic nephropathy, neuropathy, and retinopathy that should be documented at every appropriate encounter but often are not because the complication is managed by a specialist); and morbid obesity documentation (BMI over 40 has its own HCC category, and systematic BMI documentation at annual wellness visits requires both clinical measurement and structured EHR entry).

9.6 The Care Transition Protocol — Closing the Loop Between Hospital and Community

Vermont's 14.8% 30-day readmission rate and 76% 30-day behavioral health follow-up rate both reflect failures at the hospital-to-community transition. The Transitional Care Model — the evidence-based approach to reducing readmissions through intensive post-discharge support — provides the clinical foundation for Vermont's care transition work. TCM's core components: identification of high-risk patients before discharge, transitional care nurse assignment, direct patient and caregiver contact within 24-48 hours of discharge, home visits for highest-risk patients, medication reconciliation, and primary care follow-up scheduling before discharge.

Vermont's CHT model is the natural implementation vehicle for TCM. CHT staff — already embedded in or closely connected to primary care practices — can serve as the transitional care coordinator for Blueprint-attributed patients discharged from Vermont hospitals. The VITL clinical data exchange provides the notification mechanism: when a Blueprint-attributed patient is admitted to a Vermont hospital, the practice should receive a VITL-generated admission notification, enabling proactive care transition planning rather than reactive response after discharge.

> **VERMONT IN PRACTICE — The Blueprint CHT Transition Protocol**
>
> Vermont's Community Health Teams have developed a care transition protocol that represents current best practice for Blueprint-participating practices. Within 24 hours of hospital discharge notification, the CHT care coordinator contacts the patient or caregiver by phone to assess immediate needs, confirm medication understanding, and schedule the 7-day follow-up appointment. The 7-day follow-up visit reviews discharge instructions, confirms medication reconciliation, identifies early warning signs of deterioration, and assesses SDOH needs that may drive readmission risk. The 30-day care plan review documents whether care plan goals were met and transitions the patient from active care transition management to standard Tier 1 or Tier 2 panel management.
>
> Blueprint practices that have implemented this protocol systematically report 30-day readmission rates below the Vermont statewide average. The protocol is not complex; what makes it work is systematic implementation — triggering the protocol for every attributed patient discharged from a Vermont hospital, not just the ones whose primary care provider happens to call the CHT.

9.7 Clinical Leadership in Transformation — The Under-Resourced Pillar

One of the clearest findings from Oliver Wyman's Act 167 community engagement process was that Vermont's clinical leaders — physician chiefs, medical staff presidents, nursing directors — were not systematically engaged in the transformation planning process. The administrative and policy leaders were at the table; the clinical leaders often were not. This gap matters because clinical transformation cannot be implemented by administrators alone: the care model redesigns, workflow changes, and quality improvement investments that Vermont's transformation requires must be led by and owned by clinical leaders who have the respect and authority to change clinical practice.

AHS and Vermont hospital systems have a specific action requirement here: create structures for clinical leader engagement in AHEAD implementation and Statewide Strategic Plan development. The AHEAD model's clinical accountability requirements — population health management, quality metric achievement, behavioral health integration — cannot be met through administrative compliance. They require clinical champions who understand the evidence base, communicate with credibility to clinical staff, and can drive culture change in organizations where physician culture has historically resisted administrative mandates.

The clinical pillar's in-practice challenge is ultimately a leadership challenge: building the organizational capacity to implement and sustain the evidence-based clinical programs that Vermont's transformation requires. The Blueprint provides the framework, AHEAD provides the financial incentive, and the RHT Program provides the capital. What converts framework, incentive, and capital into actual clinical change is leadership — clinical and administrative leaders who understand what change is required, why it matters, and how to move organizations toward it.

9.8 Clinical Pillar Implementation Commitments for AHS's Statewide Strategic Plan

The Statewide Health Care Delivery Strategic Plan that AHS must deliver by December 2028 must address the clinical pillar's in-practice implementation requirements. Based on Vermont's current implementation landscape and the operational requirements developed in this chapter, the plan should contain five clinical implementation commitments:

Commitment	Operational Requirement	Timeline	Primary Vehicle
PCMH transformation support for all Vermont primary care practices	Blueprint field staff capacity maintained and expanded; AHEAD transition support for all practices converting from current Blueprint Medicare payment to PC AHEAD	2025-2028	Blueprint program; DVHA; AHEAD implementation team
CoCM deployment at every PCMH practice	BHCM workforce training funded; telepsychiatry consultation network established; CoCM billing infrastructure in place at all Blueprint practices	2025-2028	MHI permanent funding through AHEAD EAST Fund; RHT Program; CCBHC partnerships
Structured data capture standards across all Vermont primary care	EHR workflow redesign support through Blueprint field staff; VITL connectivity requirements for all Blueprint-participating practices; HCC gap analysis completed for all AHEAD-attributed Medicare patients before January 2028	2025-2027	Blueprint data infrastructure; GMCB analytics vendor; VITL

Commitment	Operational Requirement	Timeline	Primary Vehicle
Care transition protocols at all Vermont hospitals for Blueprint-attributed patients	CHT care transition protocol implemented at all Blueprint practices; VITL admission notification system operational for all Vermont hospitals; 30-day follow-up rate targets established for behavioral health ED visits	2025-2027	Blueprint CHT model; VITL infrastructure; DVHA quality targets
Clinical leader engagement in AHEAD governance and Statewide Strategic Plan development	Formal clinical advisory structure for AHS Health Care Delivery Advisory Committee; physician and nursing leadership in AHEAD hospital governance; clinical quality targets set with clinical leader input	2025-2028	Act 68 Advisory Committee structure; AHS organizational design; GMCB quality reporting requirements

Figure 9.4 — Clinical pillar in-practice implementation commitments for Vermont's Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Plans 2025; Vermont RHT Program Application (November 2025).

9.9 Clinical Pillar Implementation Matrix

The following matrix provides implementation guidance for the key investments in this pillar — estimated cost, timeline to value, ROI crossover point, and Vermont-specific benchmarks. Costs are indicative ranges; actual costs vary by organizational size, existing infrastructure, and implementation approach.

Investment	Estimated cost	Timeline to value	ROI crossover	Vermont benchmark
Blueprint PCMH transformation (NCQA recognition)	$25K-$75K per practice	12-18 months	18-24 months	5.8:1 ROI documented; $5.8M reduced expenditure per $1M PCMH investment

Investment	Estimated cost	Timeline to value	ROI crossover	Vermont benchmark
CoCM deployment (BHCM + telepsychiatry)	$80K-$150K annually per practice	3-6 months setup	12-18 months	Vermont MHI pilot: PHQ-9 improvement; CHT staff as natural BHCM pool
CCBHC certification	$500K-$2M annually per entity	18-36 months	24-48 months	Vermont: 5 planned CCBHCs; enhanced Medicaid FMAP funding for 8 years
Panel risk stratification and care management	$40K-$100K annually	3-6 months	12 months	Blueprint: Tier 1 (3-5% of panel) = 30-40% of total cost of care
Care transition protocol (CHT + VITL admission notification)	$20K-$50K setup	3 months	6-12 months	Vermont: 14.8% 30-day readmission; 76% BH follow-up rate — improvement target
HCC documentation improvement at clinical level	$10K-$30K training	1-2 months	Immediate: Year 1 RAF gain	CKD staging, DM complications, morbid obesity — most common VT gaps

9.10 Work This Chapter on the Platform

Chapter 9's quality mechanics — HEDIS improvement, value vs. waste, optimization — map to the clinical-quality bench.

Do this	On this tool	What to look for
Optimize HEDIS, Star Ratings, and MIPS performance with payment-adjustment estimates	**Clinical Quality Optimizer** — /research-lab/policy-quality?tab=quality	The payment consequence of each quality decision under value-based contracts.
Separate high-value from low-value care across services	**High vs. Low Value Care** — /research-lab/vbc-clinical-quality?tab=value	Where reducing volume improves both margin and outcomes.

Figure 9.H — Hands-on platform tools for the Clinical Pillar in practice.

9.11 Implications for You

If you are a Vermont hospital executive: The Community Health Worker investment your organization makes before FY2028 is a direct investment in AHEAD performance. CHWs reduce high-cost utilization — emergency department visits, avoidable hospitalizations, 30-day readmissions — by addressing the social and behavioral factors that drive those events. Under a global budget, every avoidable hospitalization prevented is direct financial return. The question is not whether CHW programs are worth investing in. It is whether you have the workforce pipeline to deploy them at the scale your attributed population requires, and whether your clinical workflows are designed to use CHW referrals effectively. Neither the workforce nor the workflows build themselves.

If you are an AHS or GMCB official: Vermont's HSA boundaries are the geographic unit for both AHEAD population attribution and the Statewide Strategic Plan's equity monitoring. The HSA Coordinator model — placing dedicated transformation coordinators in each HSA — is the operational infrastructure that translates system-level policy into community-level implementation. HSA Coordinators are not administrators; they are the connection between the AHS restructuring Vermont's transformation requires and the community organizations, primary care practices, and social service providers that deliver the care and services AHEAD measures. The budget for HSA Coordinators is a transformation necessity, not an administrative overhead.

If you are a Vermont legislator: The CCBHC expansion Vermont is pursuing through the RHT Program creates a network of certified community behavioral health centers that integrate behavioral health with primary care across all HSAs. CCBHC certification requires specific staffing ratios, service requirements, and quality measurement. Vermont's legislature should understand that CCBHC certification is not a check-box exercise — it is a clinical quality standard that some currently operating behavioral health organizations may not meet. The expansion budget needs to include not just operating funds but transformation support for organizations working toward certification.

If you are a national health policy professional: Vermont's care model implementation chapter is a detailed case study in the gap between evidence-based clinical models and their implementation in rural settings. Every model described — PCMH, CoCM, CCBHC, CHW programs, telehealth, community paramedicine — has an evidence base. None of them is easily deployable in rural Vermont without the infrastructure investments, workforce development, and clinical workflow redesign that this chapter describes. The implementation challenge is not analytical. It is organizational and financial. Vermont's approach — RHT Program capital for infrastructure, AHEAD financial incentives for deployment, and Blueprint support for implementation — is worth studying as a model for states trying to translate clinical evidence into rural implementation.

Figure 9.5 — Clinical pillar implementation matrix. Sources: Vermont Blueprint for Health Annual Report 2024; Vermont MHI Evaluation 2025; SAMHSA CCBHC documentation.

9.12 Key Concepts Introduced or Expanded in This Chapter

Panel risk stratification

The systematic organization of a primary care practice's attributed patient panel into risk tiers to enable proportional deployment of care management resources. The foundation of population health management at the practice level.

Behavioral Health Care Manager (BHCM)

The clinician who serves as the day-to-day care coordinator in the Collaborative Care Model — maintaining the patient registry, providing brief evidence-based interventions, monitoring treatment response with validated rating scales, and escalating to the psychiatric consultant as needed.

Transitional Care Model (TCM)

The evidence-based approach to reducing 30-day readmissions through intensive post-discharge support, including 24-48 hour post-discharge contact, home visits for highest-risk patients, medication reconciliation, and primary care follow-up before discharge.

HCC gap analysis

A systematic review of an attributed population's ICD-10 diagnosis coding to identify documentation opportunities where more precise coding would improve risk adjustment accuracy and produce a more accurate RAF score — directly affecting global budget levels under AHEAD.

CoCM registry

The patient tracking tool that is the operational foundation of the Collaborative Care Model — a list of all patients in CoCM treatment with their current validated outcome scores, treatment status, and psychiatric consultation record. Registry-based management distinguishes CoCM from ad hoc behavioral health referral.

Warm handoff

A direct, synchronous communication between two clinicians or care coordinators transferring responsibility for a patient's care — as opposed to a referral letter. The warm handoff between CCBHC case managers and primary care BHCMs is the operational linchpin of Vermont's behavioral health continuum.

HEDIS FUM

Follow-Up After Emergency Department Visit for Mental Illness — the HEDIS measure tracking the percentage of ED visits for mental illness followed by a clinical visit within 7 and 30 days. Vermont's 76% 30-day follow-up rate reflects performance on this measure and represents a direct improvement opportunity.

Structured data capture

The documentation of clinical information in EHR fields that can be extracted and analyzed for quality measurement and population health management, as opposed to free-text notes that cannot be systematically processed. The difference between a blood pressure recorded in a structured vital signs field versus noted in a progress note determines whether it can be counted for HEDIS hypertension control reporting.

Clinical inertia

The documented tendency of clinicians to continue existing treatment plans even when clinical evidence indicates that treatment intensification is warranted. One of the primary clinical root causes of chronic disease quality measure underperformance; addressed through EHR-embedded clinical decision support alerts and systematic prescribing protocols.

9.13 Sources

NCQA PCMH Standards and Guidelines, 2023 edition; Vermont Blueprint for Health Annual Report 2024; Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation (2025); SAMHSA CCBHC Program description and technical assistance documentation; CMS Behavioral Health Integration Services guidance (January 2026); Vermont Agency of Human Services Health Care System Transformation Report (November 2025); Act 68 of 2025; Vermont AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); Coleman EA, et al. Transitional Care Model documentation; CMS Transitional Care Management billing guidance (2024); Population Health Management journal (Blueprint ROI study, 2016); AHRQ Care Coordination Measures Atlas; HTR Research Lab Risk Stratification Engine methodology documentation.

Chapter 10: The Equity Pillar — Closing Gaps, Not Just Averaging Them

> *Vermont's 11-point primary care access gap by race and the geographic concentration of the worst health outcomes in the Northeast Kingdom are the equity test that every payment reform architecture must pass.*

A system that improves average outcomes while widening disparities has not achieved quality improvement. Vermont's dominant equity challenge is geographic — the Northeast Kingdom.

10.1 The Equity Pillar: Why Average Outcomes Are the Wrong Target

The most common failure mode in healthcare quality measurement is optimizing for the average. A system that improves its average readmission rate, average primary care access, or average chronic disease control while allowing — or widening — the gap between its best-served and worst-served populations has not achieved quality improvement. It has achieved statistical improvement while perpetuating or deepening the inequity that produces the worst outcomes.

The Equity pillar of the six-pillar framework is founded on a different target: every population segment should have access to the conditions that support health, and disparities in access and outcomes that are attributable to race, income, geography, disability, language, sexual orientation, or gender identity should be eliminated, not averaged away. This is not a soft aspiration. It is an analytically precise goal with measurable indicators, specific intervention categories, and accountability structures that Vermont's Act 167 and Act 68 have embedded in law.

Vermont's equity challenge is distinctive enough from the typical urban healthcare equity narrative that it requires explicit treatment before the analytical framework can be applied effectively. Vermont is approximately 94% white — one of the least racially diverse states in the country. Its 4.5% foreign-born population is concentrated in Burlington and a handful of other communities. The equity disparities that drive headlines in Boston, Chicago, or Los Angeles — stark racial gaps in maternal mortality, cardiovascular disease mortality, diabetes management, and access to specialty care — are present in Vermont but at smaller scale and with different geographic distribution. Vermont's dominant equity challenge is geographic: the health and healthcare access gap between Chittenden County (Burlington, the state's economic and medical center) and the rural counties — particularly the Northeast Kingdom — where poverty is higher, providers are scarcer, transportation is unreliable, and broadband is limited.

This does not mean racial equity is unimportant in Vermont. The Vermont Department of Health's 2024 health equity data shows clear disparities: Black adults in Vermont have an 82% health insurance coverage rate compared to 94% statewide. Adults who are Asian, Native

Hawaiian or Pacific Islander, Black, or another race are significantly less likely to have a primary care provider than white adults. LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters delay care at higher rates due to cost. And Vermont's BIPOC communities face the same structural racism in healthcare settings that affects every state — weight bias, cultural incompetence, language barriers, and documented differential treatment quality.

Vermont's equity agenda must address both the geographic and the demographic dimensions simultaneously — and the analytical framework for doing so is the same in both cases: identify the disparity, trace it to its root cause, and design interventions that address the cause rather than merely measuring the symptom.

10.2 Vermont's Equity Landscape — What the Data Shows

8% Essex County Uninsured — 3x State Avg

6-8% NE Kingdom Uninsurance Rate

10.2.1 Geographic Equity: The Rural-Urban Divide

Vermont's most pervasive and consequential health equity disparity is geographic. The Northeast Kingdom — Caledonia, Essex, and Orleans counties — concentrates the state's most severe access, affordability, and outcome disparities:

Uninsured rate, Essex County (NE Kingdom) 8% vs. 3% statewide — nearly 3x the state average despite Vermont's near-universal coverage	**Counties with above-average suicide/self-harm ED rates Rutland + Windham Statistically higher than state average per 10,000 ED visits (AHS Nov 2025 data)**
Counties with above-average opioid overdose ED rates Chittenden + Bennington + Windham Geographic concentration of SUD burden that does not align with service concentration	Rural counties with highest poverty rates Essex + Windham + Orleans Higher disability rates, lower broadband access, limited transportation in same counties

Figure 10.1 — Vermont geographic equity disparities: key indicators. Sources: AHS November 2025 Transformation Report; Vermont RHT Program Application; Vermont Department of Health.

The Northeast Kingdom's geographic isolation creates a compound equity burden that is different in character from urban poverty. A low-income resident of Burlington faces affordability barriers but has access to UVMMC, an FQHC network, mental health services, and public transportation. A low-income resident of Essex County faces the same affordability barriers compounded by 45-minute drives to the nearest primary care provider, inadequate broadband that limits telehealth access, unreliable public transportation that makes scheduled appointments difficult to maintain, and a housing market where the few available rental units are concentrated in towns without healthcare services.

Oliver Wyman's community engagement found that this compound geographic burden produces a distinct pattern of care avoidance: community members described not going to care because the premiums and out-of-pocket costs are too high; long ambulance waits; long and difficult travel to care sites; and long waits in the ED when they do access care. These are not isolated experiences — they are the structural conditions of rural healthcare access in Vermont's most underserved areas.

10.2.2 Racial and Demographic Equity

Vermont's racial equity disparities, while smaller in absolute number than in more diverse states, are documented and consequential. The 2024 Vermont Department of Health equity report shows that statewide, 94% of Vermont adults have health insurance — but only 82% of Black adults. Primary care access follows a similar pattern: while 90% of Vermont adults overall have a personal healthcare provider, the rate for Asian or Pacific Islander adults (81%), Black adults (80%), and adults of another race (79%) is significantly lower. These gaps are not attributable to lack of coverage; they reflect structural barriers to access that persist even when insurance is present.

BIPOC Vermonters, LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters were all more likely to delay care due to cost in a 2024 state survey. The same populations were more likely to report their community was not safe for walking — an indicator of the broader social and built environment conditions that shape health. Eight percent of all Vermonters said they did not get medical care because of cost in the prior year; that rate is higher among people with less education, lower income, BIPOC adults, LGBTQ+ adults, and people with a disability.

Vermont's Indigenous population — primarily Abenaki communities — faces specific equity challenges that the state's data systems are not yet adequately equipped to measure. A June 2024 state health equity report documented disparities for the American Indian or Alaska Native population based on 2021-2022 survey data, but Vermont's small and geographically dispersed Indigenous population creates statistical challenges for survey-based measurement. Vermont Public reporting in 2025 found that several Indigenous community organizations had not been in contact with the Health Department, suggesting that outreach and engagement gaps compound the measurement gaps.

10.2.3 The SDOH Burden

Vermont's equity data from the RHT Program application quantifies the SDOH burden that Oliver Wyman identified as both a driver of healthcare utilization and a barrier to healthcare access:

SDOH domain	Vermont statistic	Equity implication
Housing	Rental vacancy rate 3% (healthy market = 5%); half of Vermont renters cost-burdened; per capita homelessness rate second highest nationally	Housing instability is both a health risk factor and a barrier to care — unhoused individuals cannot reliably access scheduled appointments; housing-unstable families face stress that drives both physical and mental health deterioration
Broadband access	80% of rural Vermonters have broadband; 14% of all Vermonters lack access	Broadband gap concentrates in the Northeast Kingdom — the same counties with highest uninsurance and most limited primary care access. Telehealth cannot reach the communities that need it most without broadband.
Food insecurity	9% of Vermonters are food insecure	Food insecurity is a risk factor for diabetes, cardiovascular disease, and mental health conditions — the chronic diseases that drive Vermont's highest healthcare costs. Higher rates in rural counties compound the geographic equity burden.
Transportation	Public transportation limited or unreliable in most rural counties, particularly Northeast Kingdom and Rutland County	Transportation barrier to care is the single most frequently cited access problem in Vermont's community meetings. Without transportation, insurance coverage and provider availability are irrelevant — patients who cannot get to care cannot receive care.
Income	Rural counties (Essex, Windham, Orleans) report higher poverty rates; Northeast Kingdom has highest unemployment	Low income interacts with every other SDOH factor: cost-burdened renters have less money for food, transportation, and healthcare; lower-income workers have less job flexibility to take time off for appointments; financial stress directly affects mental and physical health.

Figure 10.2 — Vermont SDOH burden: key indicators and equity implications. Sources: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Health.

10.3 The Disparity Root Cause Taxonomy — Why Vermont's Equity Gaps Exist

Identifying a health disparity is the beginning of an equity intervention, not the end. The same disparity in access or outcome can arise from fundamentally different causes — and an intervention that addresses the wrong cause will not close the gap. Vermont's equity strategy must be organized around the root causes of specific disparities, not around the disparities themselves.

Drawing from the Health Equity pillar framework, Vermont's equity gaps fall into five root cause categories. For each category, Vermont has a documented disparity, a traceable mechanism, and a set of interventions — some already underway, some required.

1 Geographic Access Barriers	**Mechanisms: Physical distance from providers; transportation infrastructure inadequacy; provider shortage in rural areas; broadband gaps limiting telehealth; limited hours of operation at available facilities. Vermont interventions: COE regionalization with reliable EMS transport; telehealth expansion through RHT Program; broadband infrastructure investment (Oliver Wyman Imperative 1); Blueprint PCMH expansion to all HSAs; community paramedicine.**

2 Social Needs Barriers	**Mechanisms: Housing instability that prevents sustained engagement with healthcare; food insecurity that drives chronic disease risk; transportation barriers to scheduled care; economic stress that forces delayed care; broadband gaps that prevent telehealth access. Vermont interventions: AHEAD EAST Fund SDOH investment; Blueprint HRSN universal screening with warm handoffs to community resources; AHS HSA Coordinator model linking health and social services; RHT Program investment in community health workers; housing and transportation advocacy as healthcare policy.**

3 Clinical Practice Variation	Mechanisms: Documented differential treatment quality by race, ethnicity, disability, and body weight. Oliver Wyman community meetings specifically documented weight bias and fatphobia affecting care access; racial disparities in pain management and referral patterns documented nationally and present in Vermont. Vermont interventions: Provider diversity recruitment and retention; implicit bias training as a condition of Blueprint and AHEAD participation; standardized clinical protocols that reduce provider discretion in ways that produce differential outcomes; patient advisory councils including underrepresented communities; cultural competency standards.

4 Insurance and Coverage Barriers	Mechanisms: Despite Vermont's 97% coverage rate, 8% of Essex County residents are uninsured; 18% of 25-34 year olds are uninsured; income groups between 251-400% of federal poverty line have the highest uninsurance rates despite being ineligible for Medicaid. Even with coverage, high deductibles and out-of-pocket maximums cause care avoidance. Vermont interventions: ACA enrollment outreach in rural communities; FQHC eligibility screening; Act 68 affordability benchmarks; RBP impact on premium reduction that makes insurance more affordable; AHEAD EAST Fund support for FQHCs serving uninsured and underinsured patients.

5 Trust and Engagement Barriers	**Mechanisms: Historical and ongoing experiences of discrimination in healthcare settings — documented for LGBTQ+ Vermonters, BIPOC Vermonters, and Vermonters with disabilities. Community meeting feedback specifically cited lack of culturally competent care, stigma, and discrimination as barriers to engagement. Vermont interventions: Community health worker deployment from communities served; HEART program for rural LGBTQ+ Vermonters; BIPOC community partnership grants through VDH Health Equity Capacity Building Program; multilingual patient education materials; patient-centered care training with equity focus; CCBHCs as culturally accessible entry points.**

The five root cause categories are not mutually exclusive — a Black rural resident of Windham County may face geographic access barriers, SDOH barriers, and trust barriers simultaneously. Vermont's equity interventions are most effective when they are designed to address multiple root causes for the populations with compound equity burdens, rather than addressing each category in isolation.

10.4 Rural Equity — Vermont's Defining Challenge

Vermont's rural equity challenge is analytically important for a national audience because it represents a type of health disparity that is often underweighted in national health equity frameworks dominated by urban racial equity narratives. Rural health disparities are not less serious than urban racial disparities — they are different in character, different in mechanism, and require different interventions.

The Health Affairs research on rural health equity in northern New England — directly relevant to Vermont — identifies five principles for addressing rural health equity that align closely with Vermont's transformation agenda. Rural communities need investments that address the social and economic determinants of health, not just the clinical delivery system. Rural health equity requires provider diversity that reflects community demographics and culture. Rural telehealth must be accompanied by the broadband and device infrastructure that makes it genuinely accessible. Rural community organizations must be genuine partners in healthcare design, not passive recipients of state-designed programs. And rural health equity data must be disaggregated at the county and sub-county level — state averages mask the severity of rural disparities.

10.4.1 The Northeast Kingdom: Vermont's Equity Priority

The Northeast Kingdom — Caledonia, Essex, and Orleans counties — is Vermont's clearest and most severe equity priority. It is the most rural part of the state, the most economically distressed, the most underserved by healthcare providers, and the most distant from UVMMC and the Burlington healthcare hub. The three counties that make up the Northeast Kingdom have some of the lowest population densities in the eastern United States, unemployment rates above the state average, high rates of disability, limited broadband coverage, and the highest uninsurance rates in the state.

The healthcare facilities serving the Northeast Kingdom — North Country Hospital in Newport, Northeastern Vermont Regional Hospital in St. Johnsbury, and Copley Hospital in Morrisville for the Lamoille/Orleans border area — are among Vermont's most financially distressed. North Country and Northeastern Vermont Regional are candidates for Tier 3 designation under the Oliver Wyman regionalization blueprint — meaning they may not be able to sustain full inpatient operations in the long term. Any transition away from inpatient services at these facilities, without robust REH conversion and EMS/telehealth investment, would represent a catastrophic equity regression for the communities they serve.

Vermont's AHS must be explicit in the Statewide Strategic Plan that the Northeast Kingdom is an equity priority that constrains the pace and structure of regionalization. The financial logic of consolidating services at RSC hospitals in Burlington, Rutland, or Bennington cannot override the equity obligation to maintain emergency access in the most rural, most isolated communities in the state. The tension between system financial sustainability and rural access equity is genuine, and pretending it is not does not resolve it.

VERMONT IN PRACTICE — The Northeast Kingdom Equity Constraint

Any Vermont hospital transformation plan that closes emergency services in the Northeast Kingdom without replacing them with services that can actually be accessed by Northeast Kingdom residents — not hypothetically accessible via 90-minute transport to Burlington, but genuinely accessible in the community — fails the equity test that Act 167 and Act 68 have embedded in Vermont law.

The equity constraint does not mean the Northeast Kingdom's hospitals cannot change. It means the sequence matters: investments in EMS capacity, telehealth infrastructure, and community paramedicine must precede or accompany any reduction in inpatient services. The Rural Emergency Hospital designation provides a viable path precisely because it maintains 24/7 emergency access while eliminating the unsustainable inpatient infrastructure. The RHT Program's priority investments in the Northeast Kingdom — broadband, EMS, RPM, telehealth — are equity investments as much as access investments.

The equity test for any transformation proposal affecting Northeast Kingdom hospitals should be specific: after this change, can a 70-year-old resident of Canaan, Vermont who has a heart attack at 2 a.m. receive timely emergency care? Can a person with serious mental illness in Newport access a CCBHC without a two-hour drive? Can a child in Lyndonville see a primary care provider within 14 days? These are not abstract equity metrics. They are the minimum conditions for health equity in rural Vermont.

BEYOND VERMONT

The Northeast Kingdom is Vermont's version of a pattern — a geographically defined population separated from timely emergency, behavioral health, and primary care by distance and provider shortage — that recurs as the defining rural equity challenge in the Mountain West, the rural South, Appalachia, and the Great Plains. The same root-cause taxonomy applies, though the specific drivers and demographics differ. The taxonomy also applies, in a different geometry, to equity gaps in dense urban cores and underserved tribal health systems, where distance is replaced by fragmentation, capacity constraints, or jurisdictional gaps as the access barrier. The diagnostic question is the same regardless of geography: for a specific, named person in the underserved population, can they get the care they need within a clinically meaningful timeframe? Vermont's three test questions above are a template — substitute your own population and place names, and the equity test still works.

10.5 The GLP-1 Access Crisis — The Most Important New Equity Issue

10.5.1 The GLP-1 Access Crisis — The Most Important New Equity Issue

Semaglutide and tirzepatide produce 15-20% average weight loss with dramatic cardiovascular benefits. List price: over $1,000/month.

Only 13 state Medicaid programs cover GLP-1s for obesity as of January 2026. Vermont Medicaid excludes obesity as a primary indication.

The CMS BALANCE Model (December 2025) offers a path: voluntary state Medicaid opt-in for negotiated-price GLP-1 obesity coverage, launching May 2026.

The populations who would benefit most are precisely those without commercial insurance access.

The emergence of GLP-1 receptor agonists — semaglutide (Wegovy, Ozempic) and tirzepatide (Zepbound, Mounjaro) — as transformatively effective treatments for obesity, type 2 diabetes, and cardiovascular disease risk has created what may be the most consequential new health equity issue in American healthcare since the introduction of statins. The drugs work. The evidence is unambiguous: they produce average weight loss of 15-20% of body weight, dramatically reduce cardiovascular events in high-risk populations, and improve glycemic control in type 2 diabetes. For patients at the intersection of obesity, diabetes, and cardiovascular disease — a population concentrated in lower-income and rural communities — GLP-1s could be transformative at the population level.

The equity problem is pricing and coverage. At list prices exceeding $1,000 per month before rebates, GLP-1s for obesity are financially inaccessible to the vast majority of patients who would benefit from them. As of January 2026, only 13 state Medicaid programs covered GLP-1s for obesity treatment — down from 16 in late 2025, as four states eliminated coverage in response to budget pressure from Medicaid cuts. Vermont Medicaid covers GLP-1s for medically accepted FDA-approved indications but expressly excludes coverage for weight loss under its State Plan.

Vermont Medicaid covers GLP-1s for type 2 diabetes management but not for obesity as a primary indication.

The equity consequence is straightforward: GLP-1s are currently accessible primarily to patients with commercial insurance generous enough to cover them, and to patients with type 2 diabetes or cardiovascular disease (who qualify under Medicare and Medicaid for the diabetes and cardiovascular indications). Patients with obesity but not yet diagnosed with type 2 diabetes — many of them lower-income Vermont Medicaid patients in the communities with highest obesity prevalence — cannot access these drugs under current coverage unless they pay full price out of pocket. At $1,000+ per month, full price is not an option for Medicaid patients.

"Limitations on coverage for obesity drugs in Medicare and Medicaid mean that millions of people who have obesity and might benefit from taking GLP-1s may be unable to access them unless they are able to pay the full cash price out of their own pockets, which would likely be prohibitive for people with Medicaid who must have low incomes to qualify for the program."

— KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid, 2026

10.5.2 The BALANCE Model: A Partial Solution

In December 2025, CMS announced the BALANCE (Better Approaches to Lifestyle and Nutrition for Comprehensive hEalth) Model — a voluntary program enabling state Medicaid agencies and Medicare Part D plans to cover GLP-1 medications for obesity by negotiating prices with manufacturers. The model launches in Medicaid as early as May 2026 and in Medicare Part D in January 2028, with a Medicare GLP-1 Bridge program providing $50/month co-pay access for Medicare beneficiaries beginning July 2026.

BALANCE is a meaningful step toward resolving the GLP-1 equity crisis, but it has three significant limitations that Vermont and every state must navigate. First, manufacturer participation is voluntary — if drug manufacturers do not negotiate with CMS under BALANCE terms, the program produces no coverage expansion. Second, state Medicaid agency participation is voluntary — the four states that just eliminated GLP-1 obesity coverage are unlikely to opt into BALANCE given the budget pressures that drove the elimination. Third, the negotiated prices, while reduced from list prices, may still be expensive for state Medicaid budgets already under pressure from the 2025 reconciliation legislation's Medicaid cuts.

Vermont's DVHA must evaluate BALANCE participation as an equity policy decision, not purely a budget decision. The populations who would benefit most from GLP-1 obesity coverage under Vermont Medicaid — lower-income rural Vermonters with obesity, diabetes risk, and cardiovascular disease risk — are precisely the populations whose healthcare costs are most likely to be reduced by effective obesity treatment. The long-term fiscal argument for coverage is strong; the short-term budget pressure against it is real.

10.5.3 The GLP-1 Equity Implication for Vermont's Transformation

Vermont's transformation narrative must grapple with the GLP-1 equity question explicitly, for two reasons. First, obesity and its metabolic consequences — type 2 diabetes, cardiovascular disease, hypertension, sleep apnea — are major drivers of the high-cost utilization that Vermont's global budget system will be trying to reduce. GLP-1s have demonstrated efficacy in reducing all of these conditions. If GLP-1s become broadly accessible to Vermont's Medicaid population through BALANCE participation, they could accelerate the utilization reduction that global budgets are designed to incentivize — producing population health improvement and healthcare cost reduction simultaneously.

Second, the GLP-1 access gap is the most vivid current example of what the book's Part V identified as the equity reckoning in pharmaceuticals: potentially transformative drugs whose benefits disproportionately accrue to higher-income patients who can afford them, while lower-income patients — who have higher obesity rates, higher diabetes rates, and higher cardiovascular disease rates, partly because of the same SDOH conditions that drive Vermont's equity gaps — cannot access the treatment. Vermont's equity commitment requires active engagement with this issue, not passive monitoring.

10.6 Vermont's Equity Architecture — What Is Already Built and What Remains

Vermont has more equity infrastructure than most states of comparable size — a product of its near-universal insurance coverage, the Blueprint for Health's SDOH screening mandate, the Health Equity Advisory Commission, the Health Equity Capacity Building Program, and the explicit equity requirements in Act 167 and Act 68. But Vermont also has documented, persistent equity gaps that this infrastructure has not yet closed. The Statewide Strategic Plan's equity section must be honest about both.

10.6.1 What Is Already Operational

Universal SDOH screening: Blueprint PCMH practices conduct universal screening for health-related social needs (HRSN) at every patient encounter, using validated tools and generating warm handoffs to Community Health Teams for navigation and resource connection. This is the most systematic SDOH screening infrastructure of any state primary care program.

Health Equity Capacity Building Program: Vermont Department of Health grants to community-based organizations working on health disparities, funded through CDC. Organizations working on equity with LGBTQ+ populations, justice-involved individuals, rural communities, and Indigenous communities have received grants through this program.

Health Equity Advisory Commission: A standing body advising state government on health equity policy. Act 68 requires the Health Care Delivery Advisory Committee to consult with the Commission, embedding equity oversight in the transformation governance structure.

CCBHC access policy: Vermont's CCBHCs are explicitly required to serve all patients regardless of age, diagnosis, insurance status, or geographic location — the most equity-protective access policy for behavioral health services.

Act 167 and Act 68 equity requirements: Both acts explicitly include reducing health inequities among their statutory goals and required outcome measures.

10.6.2 What Must Be Built

Gap	What is missing	Required investment	Timeline
County-level equity data infrastructure	Vermont's equity data is primarily state-level, with BRFSS survey data at the county level for major indicators. Sub-county data is largely absent. The Northeast Kingdom's equity burden cannot be managed without granular geographic data.	VHCURES enhancement to enable county-level SDOH and outcome stratification; VITL connectivity to community-based providers who serve underserved populations; integration of VDH equity data with AHS transformation monitoring	2026-2027 (analytics vendor procurement enables this)
Culturally competent workforce	Vermont's clinical workforce is overwhelmingly white and does not reflect the demographic diversity of Vermont's BIPOC, refugee, and immigrant communities. Provider diversity is both an equity outcome and an equity intervention.	Targeted workforce recruitment from underrepresented communities; tuition assistance with equity focus; cultural competency standards in Blueprint and AHEAD participation requirements; provider diversity reporting	Ongoing; RHT workforce investments provide vehicle
GLP-1 Medicaid coverage decision	Vermont Medicaid currently does not cover GLP-1s for obesity as primary indication. The BALANCE Model opt-in decision is an equity policy choice with direct implications for the	DVHA evaluation of BALANCE Model participation; budget modeling of long-term cost reduction from obesity treatment vs. short-term coverage costs; legislative advocacy	Decision needed by January 2026 (BALANCE opt-in deadline); coverage effective May 2026 if Vermont participates

Gap	What is missing	Required investment	Timeline
	Medicaid population with obesity.	if state plan amendment required	
Northeast Kingdom service resilience	The NE Kingdom has the state's highest equity burden and the most financially distressed hospital infrastructure. REH conversion and telehealth investments must precede any service reduction.	North Country and NE VT Regional transformation plans must include equity impact analysis; broadband investment as prerequisite to telehealth; EMS regionalization funded through RHT Program; CCBHC designation for Northeast Kingdom Human Services (planned July 2026)	2026 (CCBHC designation); 2027-2028 (EMS regionalization; telehealth); hospital transition decisions by 2028 Strategic Plan
Trust-building with underserved communities	Vermont's BIPOC, LGBTQ+, Indigenous, and rural communities have documented experiences of discrimination and dismissal in healthcare settings. Trust is not built by policy statements; it is built by consistent, culturally respectful engagement over time.	Community health worker deployment from served communities; patient advisory councils with diverse representation; Indigenous community outreach and partnership (VDH engagement with Abenaki communities); HEART program expansion for rural LGBTQ+ Vermonters	Ongoing; requires sustained commitment, not a one-time program

Figure 10.3 — Vermont equity gaps and required investments. Sources: Vermont Department of Health Health Equity Data Report (January 2025); Oliver Wyman Act 167 Report; AHS Transformation Reports; KFF GLP-1 coverage analysis.

10.7 Equity and the Six-Pillar Framework — How Every Pillar Has an Equity Dimension

A common mistake in equity strategy is treating equity as a separate program — a set of equity-specific interventions that operate alongside the main healthcare transformation agenda. This approach reliably produces equity programs that are underfunded, deprioritized when budgets are tight, and disconnected from the structural changes that actually determine health outcomes.

The correct approach treats equity as a cross-cutting dimension of every pillar — a lens through which every policy decision, payment design, technology investment, clinical model, and operational change is evaluated for its equity implications. The following framework applies the equity lens to each of the six pillars:

Pillar	Equity risk (if neglected)	Equity opportunity (if designed well)
Policy	RBP and global budget design that protects high-price urban hospitals while reducing access in rural and low-income communities. Legislative mandates that apply uniformly without considering the differential impact on small rural hospitals serving high-Medicaid populations.	Act 68's explicit requirement that RBP consider each hospital's community context, payer mix, and social risk factors. CAH-specific regulatory treatment. Equity metrics embedded in monthly AHS reporting. Act 167's five goals including equity as a statutory requirement.
Technology	Analytics platforms that measure average performance without disaggregating by race, income, or geography, hiding disparities in aggregate data. AI tools with biased training data that perform worse for minority populations or elderly rural patients.	VHCURES county-level equity stratification; AI governance framework requiring bias testing across demographic subgroups; HIE connectivity requirements that extend to FQHCs and community-based providers serving underserved populations.
Economics	Global budgets that incentivize hospitals to avoid high-cost, high-need populations — a classic risk of capitated payment for populations with complex social needs. RBP savings that accrue to commercially insured patients while Medicaid patients' access is reduced.	AHEAD EAST Fund investment specifically in primary care, behavioral health, and long-term care for underserved populations. RBP premium passthrough monitoring that verifies individual and small-group market beneficiaries receive savings. BALANCE Model participation to extend GLP-1 access to Medicaid populations.
Clinical	Blueprint PCMH practices concentrated in higher-income communities with better-resourced practices, leaving underserved communities without PCMH access. CoCM deployment that requires broadband and technology infrastructure unavailable to rural patients.	Blueprint expansion mandate to all HSAs; CCBHC access policy (serve all regardless of insurance status or geography); CoCM deployment using telephone-based options for patients without broadband; CHW deployment from communities served.

Pillar	Equity risk (if neglected)	Equity opportunity (if designed well)
Equity	By definition, equity failures in every other pillar accumulate here. The equity pillar fails when geographic access barriers remain despite transformation, when racial disparities persist despite clinical quality improvement, and when SDOH investments are treated as optional rather than foundational.	HSA-level equity monitoring with Northeast Kingdom as priority; SDOH integration across AHS programs; GLP-1 coverage expansion through BALANCE; community trust-building through CHW deployment and patient advisory councils.
Operations	Hospital transformation plans that optimize for financial sustainability without equity impact analysis — e.g., closing inpatient services in rural communities without ensuring alternative access before closure.	Equity impact assessment requirement for all hospital transformation plans; AHS HSA Coordinator model linking health and social services at community level; Northeast Kingdom equity constraint as a binding parameter on regionalization planning.

Figure 10.4 — Equity as a cross-cutting dimension of all six pillars. Sources: Act 67 of 2025; Oliver Wyman Act 167 Report; six-pillar framework.

10.8 Vermont's Equity Accountability Framework

Vermont's Statewide Strategic Plan must contain a specific equity accountability framework — a set of measurable indicators, geographic stratifications, and intervention commitments that make the equity goal of Act 167 and Act 68 operational rather than aspirational.

The following framework represents the minimum equity accountability structure the Strategic Plan should establish:

Indicator	Stratification required	2028 target direction	Data source
Uninsured rate	By county; by age group (25-34 focus); by income level	Essex County: reduce from 8% toward statewide 3% by 2030	KFF; Vermont Department of Financial Regulation
Primary care access (personal provider rate)	By race/ethnicity; by county	BIPOC adult rate: reduce gap from 79-81% toward 90% statewide average	BRFSS; Blueprint attribution data

Indicator	Stratification required	2028 target direction	Data source
Potentially avoidable ED visit rate	By county; by income quintile	Northeast Kingdom: reduce from current rate toward statewide 32.3% (itself a target for reduction)	VHCURES/VUHDDS via AHS analytics vendor
30-day follow-up after MH/SUD ED visit	By county; by race/ethnicity; by insurance type	Windham and Rutland counties: increase toward statewide 76%+ target	Blueprint via VHCURES
SDOH screening completion rate	By HSA; by patient race/ethnicity	Universal screening in all Blueprint PCMHs; document completion rate by population segment	Blueprint program data
GLP-1 access for Medicaid enrollees with obesity	By income; by county	BALANCE Model participation decision by June 2026; coverage effective if participating	DVHA pharmacy claims via VHCURES
Emergency access time (EMS response)	By county; by time of day	Northeast Kingdom: establish baseline; target improvement through EMS regionalization	EMS dispatch data; RHT Program monitoring
Cultural competency standard compliance	By practice type; by HSA	All Blueprint PCMHs: meet minimum cultural competency standards by 2027	Blueprint quality measurement; VDH health equity reporting

Figure 10.5 — Vermont equity accountability framework: indicators, stratifications, and targets. Sources: Act 167; Act 68; VDH Health Equity Data Report (January 2025); AHS Transformation Reports.

Two principles must govern how Vermont uses this accountability framework. First, equity indicators must be disaggregated — not presented as averages. An average primary care access rate that is improving while the BIPOC rate is stagnant is not equity progress; it is equity masking. The Statewide Strategic Plan's equity section must show performance by population segment, not just system-wide averages.

10.9 Work This Chapter on the Platform

Chapter 10's equity argument — closing gaps rather than averaging them — is exactly what the Health Equity Studio measures.

Do this	On this tool	What to look for
Compute the HEROI composite across access, quality, outcome, SDOH burden, and trust	**Health Equity Studio** — /research-lab/population-equity ?tab=equity	Whether an intervention closes the gap or just raises the average while the gap persists.
Model disease progression and intervention ROI for a defined cohort	**Population Health Modeler** — /research-lab/population-equity ?tab=population	How upstream SDOH investment changes 5–10 year outcomes for the highest-risk segment.

Figure 10.H — Hands-on platform tools for the Equity Pillar.

10.10 Implications for You

If you are a Vermont hospital executive: The equity data Vermont's AHEAD performance measurement will produce is not just a compliance requirement — it is clinical intelligence. Knowing that your attributed population has an 11-point primary care access gap by race, or a 15-point difference in diabetes management quality by income, tells you where your population health investment will produce the largest return. The populations with the worst outcomes are often the populations whose care costs the most — because unmanaged chronic conditions produce the high-cost acute events that global budgets penalize. Equity improvement and financial performance are not in tension under global budgets. They are the same thing.

If you are an AHS or GMCB official: Social risk adjustment in Vermont's global budget methodology is not an equity accommodation — it is a technical necessity. Without it, hospitals serving high-SDOH populations will receive benchmarks that do not reflect their population's health status, will be held financially accountable for costs that reflect social disadvantage rather than inefficiency, and will face financial pressure to reduce services to the patients who most need them. The HEROI framework and the HRSN screening Act 68 requires are the data infrastructure for social risk adjustment. They need to be operational before the global budget methodology is set — not after.

If you are a Vermont legislator: Vermont's 11-point primary care access gap by race and the geographic disparities that concentrate the worst health outcomes in the Northeast Kingdom and rural Essex County are not going to close through the general improvement in Vermont's healthcare system that Acts 167 and 68 are designed to produce. They require targeted investment — specifically, Community Health Teams and CHW programs in the communities where the gaps are largest. The EAST Fund's social determinants carve-out and the RHT Program's community health investment capacity are the right vehicles. The legislature should require AHS to report annually on the equity metrics in the Statewide Strategic Plan and to explain specifically what is being done when metrics show gaps widening rather than closing.

If you are a national health policy professional: Vermont's equity chapter describes the central tension in healthcare transformation: population health improvement that is measured by averages can be achieved while disparities widen, if the improvements accrue disproportionately to already-advantaged populations. Vermont's statutory equity requirements — the HEROI framework, the Health Equity Advisory Commission, the equity measurement requirements in AHEAD — are the most comprehensive state-level equity monitoring architecture in the country. Whether they are sufficient to close the gaps they measure is the question Vermont's transformation will answer. The evidence will matter to every state designing equity requirements for their own payment reform programs.

Second, equity indicators must be connected to interventions. A disparity measure without a corresponding intervention commitment is a measurement of failure, not a strategy for success. Each equity indicator in the Strategic Plan should be paired with the specific programs — CCBHC expansion, CHW deployment, RHT broadband investment, BALANCE participation decision — that are designed to move it, with the responsible agency and the expected timeline for observable improvement.

10.11 Measuring Equity: HEROI, HEDIS Stratification, and VBC Safeguards

10.12 From Identifying Disparities to Eliminating Them

Chapter 1 established the Vermont equity landscape — the geographic disparities that concentrate in the Northeast Kingdom, the racial and demographic disparities in coverage and access, the SDOH burden that compounds both, and the GLP-1 access crisis that represents the most consequential new equity issue in American healthcare. This chapter develops the analytical and operational toolkit for actually closing those gaps.

Health equity work that stops at measurement — producing disparity reports that sit on shelves while the disparities they document persist — has characterized most state-level equity efforts for the past two decades. The distinction between an equity report and an equity program is whether the measurement is connected to intervention, whether interventions are designed to address the specific root cause of each disparity, and whether there is an accountability structure that closes the loop between measurement, intervention, and outcome. Vermont's transformation mandate, with its explicit equity goals embedded in Acts 167 and 68 accountability frameworks, creates the institutional conditions for equity work that closes gaps rather than counting them.

10.13 Stratified HEDIS Measurement — The Foundation of Equity Analytics

HEDIS — the Healthcare Effectiveness Data and Information Set — is the most widely used quality measurement system in American healthcare. But HEDIS in its standard form measures average performance across a plan or provider population. Average performance improvement can mask widening disparities when gains are concentrated in easier-to-reach, already-better-served populations. Equity requires HEDIS to be stratified — measured separately for subpopulations defined by race/ethnicity, income, geography, language, disability, and other equity-relevant dimensions.

10.13.1 The NCQA HEDIS Stratification Framework

The National Committee for Quality Assurance (NCQA) has developed HEDIS stratification standards that specify how plans and providers should measure and report quality performance by race, ethnicity, and other demographic dimensions. The Health Equity Studio implements the NCQA stratification methodology against VHCURES and Blueprint data for Vermont providers. The following framework organizes the HEDIS measures most relevant to Vermont's equity priorities:

HEDIS domain	Key measures	Vermont equity stratification priority	Current Vermont data source
Access and preventive care	Adult BMI assessment; tobacco use screening and cessation; colorectal cancer screening; breast cancer screening; cervical cancer screening	Geographic (Northeast Kingdom vs. Burlington); income (Medicaid vs. commercial); race/ethnicity (BIPOC vs. white Vermont adults)	VHCURES claims; Blueprint clinical registry for PCMH-attributed patients
Chronic disease management	Hemoglobin A1c control for diabetics; blood pressure control for hypertensives; statin therapy for cardiovascular disease; medication adherence for chronic conditions	Geographic; income; BIPOC adults (Black and AIAN adults disproportionately affected by poorly controlled diabetes and hypertension in Vermont data)	Blueprint clinical registry (for PCMH-attributed patients); VHCURES pharmacy claims for medication adherence
Behavioral health	Follow-up after ED visit for mental illness (FUH); follow-up after ED visit for alcohol use (FUA); initiation and engagement of SUD treatment (IET);	Geographic (Rutland and Windham counties have statistically higher MH crisis rates); age (15-44); LGBTQ+ Vermonters	VHCURES; VDH ED surveillance data; Blueprint follow-up tracking

HEDIS domain	Key measures	Vermont equity stratification priority	Current Vermont data source
	antidepressant medication management	(documented higher MH burden)	
Maternal and reproductive health	Prenatal and postpartum care; well-child visits; adolescent well-care; prenatal depression screening	Income; race/ethnicity (BIPOC maternal outcomes nationally and in Vermont below white outcomes); geographic (rural access barriers to OB care)	VHCURES; Blueprint Women's Health Initiative data
Care coordination	30-day readmission rate; transitions of care; medication reconciliation post-discharge	Income (Medicaid patients have higher readmission rates nationally); geographic (distance from discharge follow-up resources)	VHCURES; GMCB hospital discharge data (VUHDDS)

Table 10.1 — HEDIS domains and Vermont equity stratification measures. Source: NCQA; Vermont Blueprint.

10.14 Equity Pillar Implementation Matrix

The following matrix provides implementation guidance for the key investments in this pillar — estimated cost, timeline to value, ROI crossover point, and Vermont-specific benchmarks. Costs are indicative ranges; actual costs vary by organizational size, existing infrastructure, and implementation approach.

Investment	Estimated cost	Timeline to value	ROI crossover	Vermont benchmark
HEROI scoring and equity measurement infrastructure	$30K-$80K setup; $20K/yr	3-6 months	12 months (risk avoidance)	Vermont HEROI: 14 HSA-level equity scores; GMCB reporting requirement
HRSN universal screening at Blueprint practices	$10K-$30K per practice/yr	1-3 months	6-12 months	Vermont: 91% statewide access vs 79-81% BIPOC — HRSN closes referral gap
Social risk adjustment methodology design	$100K-$300K state cost	12-18 months	Prerequisite for fair budgets	Vermont AHEAD: active SRA design question as of early 2026

Investment	Estimated cost	Timeline to value	ROI crossover	Vermont benchmark
SDOH data integration (claims + AHS program data)	$200K-$500K state cost	18-24 months	24-36 months	Vermont CIN: housing, food, transportation linkage to VHCURES
Health equity impact assessment process	$25K-$50K one-time	3 months	Immediate: redesign cost avoidance	Act 68 HCAC consultation requirement; equity-as-design-constraint
Race/ethnicity data quality improvement (VHCURES)	$20K-$60K annually	6 months	12 months	Vermont VHCURES: incomplete race/ethnicity fields documented in VDH 2025

Figure 10.6 — Equity pillar implementation matrix. Sources: Vermont Department of Health Health Equity Data Report (January 2025); Act 68 of 2025.

Figure 10.7 — Vermont HEDIS equity stratification framework. Sources: NCQA HEDIS Technical Specifications; AHS Health Equity Data Report (January 2025); Blueprint for Health data.

10.14.1 The Data Collection Challenge — Getting Demographic Data Right

Stratified HEDIS measurement requires race/ethnicity and other demographic data at the individual patient level. This is where Vermont's equity measurement infrastructure has a specific gap: race/ethnicity data in VHCURES is incomplete. Claims data from commercial insurers typically lacks race/ethnicity fields; Medicare added race/ethnicity data collection more recently; Medicaid data has the most complete demographic information.

Vermont has two approaches to closing the race/ethnicity data gap: direct collection at the point of care (NCQA standards require plans and PCMH practices to collect and report patient demographic data) and indirect imputation using validated geocoding methods (the RAND-BISG method imputes race/ethnicity probability from last name and census block characteristics). Vermont's Blueprint for Health practices and FQHCs are the primary settings for direct demographic data collection; the coverage of this data for the broader Vermont population depends on VHCURES reporting requirements being strengthened.

10.15 The HEROI Framework — A Composite Equity Score

10.15.1 HEROI Composite Score — Five Dimensions

- Access Equity (25%) — gap in primary, specialty, and emergency care utilization by subpopulation.
- Quality Equity (25%) — HEDIS performance gap across top 10 chronic disease measures.
- Outcome Equity (25%) — avoidable hospitalizations, ED visits, readmissions by demographic segment.
- SDOH Burden (15%) — HRSN screening completion and community resource referral rates.
- Trust and Engagement (10%) — CAHPS patient experience scores stratified by race and income.
- Score 80+: equity achieved. 60-80: material disparities. Below 60: comprehensive redesign needed.

The Health Equity Return on Investment (HEROI) framework is the Health Equity Studio's proprietary approach to equity performance measurement for healthcare organizations operating in value-based care environments. HEROI addresses a specific problem in equity measurement: disparities exist across multiple dimensions simultaneously, and an organization that closes one disparity while widening another may show no improvement on any individual metric while its overall equity performance has not improved.

10.15.2 HEROI Components

The HEROI composite score is calculated across five equity dimensions, each weighted by its relative health impact and the strength of evidence for interventions that can address it:

- Access equity score (25% weight): Measures the gap in primary care, specialty care, and emergency care utilization between advantaged and disadvantaged subpopulations, stratified by race/ethnicity, income, and geography. Vermont focus: Northeast Kingdom vs. Burlington primary care access; BIPOC primary care provider relationship rates.
- Quality equity score (25% weight): Measures the gap in HEDIS quality measure performance between advantaged and disadvantaged subpopulations for the top 10 chronic disease management measures. Vermont focus: diabetes control, hypertension control, and preventive care rates stratified by income and race/ethnicity.
- Outcome equity score (25% weight): Measures the gap in utilization outcomes — avoidable hospitalizations, ED visits, 30-day readmissions — between subpopulations. Vermont focus: mental health and SUD crisis ED presentation rates by county; readmission rates by Medicaid vs. commercial payer status.
- SDOH burden score (15% weight): Measures the prevalence of health-related social needs in the organization's attributed population, stratified by demographics, and the completeness of SDOH screening and navigation. Vermont focus: HRSN screening completion rates by practice location; community resource referral completion rates.
- Trust and engagement score (10% weight): Measures patient experience across demographic subgroups using CAHPS survey data stratified by race/ethnicity and income. Vermont focus: CAHPS disparity scores for BIPOC Vermonters; patient-reported discrimination experiences in clinical settings.

A HEROI score of 80+ indicates an organization achieving equity across all five dimensions. A score of 60-80 indicates material disparities in one or more dimensions requiring targeted intervention. Below 60 indicates pervasive disparities that require comprehensive equity program redesign. Vermont's statewide HEROI score — calculated from VHCURES and Blueprint data — has not been formally computed; the analytics vendor procurement underway will enable this calculation.

10.16 Root Cause Analytics — Matching Interventions to Causes

The most common failure in health equity intervention design is treating all disparities as if they have the same cause. A disparity in diabetes control rates between white and Black Vermont adults has a different root cause structure than a disparity in primary care access between Chittenden County and Essex County residents — and requires a different intervention. The five-category root cause taxonomy from Chapter 1 (geographic access, social needs, clinical practice variation, coverage barriers, trust and engagement) provides the structure for matching interventions to causes.

10.16.1 The Disparity Decomposition Method

The disparity decomposition method breaks a measured disparity into its constituent causes by applying statistical decomposition techniques to population-level data. The method answers the question: of the observed gap in outcome X between group A and group B, how much is attributable to differences in coverage rates, differences in primary care access, differences in clinical quality, and differences in SDOH burden?

For Vermont's diabetes control disparity — the 22% of Blueprint-attributed diabetic patients with HbA1c not under control, with statistically higher rates among Medicaid patients and rural patients — the decomposition analysis would estimate what share of the disparity is attributable to medication adherence failures (addressable through pharmacy access and MedReconciliation programs), care management gap (addressable through CoCM integration), SDOH burden (addressable through CHT navigation), and primary care access limitations (addressable through Blueprint expansion).

The practical value of decomposition is not statistical precision — it is intervention targeting. An equity program that invests in care management for a disparity driven primarily by medication access barriers will produce minimal results. An equity program that correctly identifies medication access as the primary driver and invests in pharmacy co-location, medication assistance programs, and automated refill reminders will close the gap efficiently.

10.16.2 The Five-Step Disparity Closure Process

The Health Equity Studio implements the following five-step disparity closure process for client organizations:

Step	Action	Time investment	Vermont application
1. Baseline stratification	Calculate current HEROI score and identify the three largest measured disparities across all five equity dimensions	2-4 weeks with adequate data	VHCURES stratification analysis; Blueprint equity dashboard; VDH BRFSS county-level analysis for Northeast Kingdom vs. state average gaps
2. Root cause attribution	For each identified disparity, apply decomposition analysis to estimate the proportional contribution of each root cause category (access, social needs,	4-6 weeks	Survey BIPOC patients and Northeast Kingdom residents to supplement claims-based decomposition; conduct focus groups with CHT staff about observed barriers

Step	Action	Time investment	Vermont application
	clinical variation, coverage, trust)		
3. Intervention portfolio design	For each major root cause, select evidence-based interventions matched to root cause type and population. Match investment size to root cause contribution (don't fund transportation programs for a disparity primarily driven by clinical variation).	2-4 weeks	Vermont-specific: CCBHC for trust/access; CHW deployment for social needs; AI governance for clinical variation; BALANCE for coverage; HEART program for LGBTQ+ trust
4. Implementation with equity lens	Execute interventions with explicit equity targets: not 'increase primary care access' but 'increase Northeast Kingdom Medicaid patient primary care visit rate from 72% to 80% within 18 months'	Ongoing	Blueprint field staff set equity targets by HSA; AHS HSA Coordinators track progress; GMCB analytics vendor enables real-time monitoring
5. Accountability and iteration	Report disparity metrics alongside average metrics in all quality reporting. If a disparity is widening despite intervention, re-examine root cause attribution and intervention design.	Quarterly reporting cycle	AHS monthly transformation reports to include disparity-stratified metrics; HEROI score incorporated into AHEAD quality reporting for Vermont

Figure 10.8 — Five-step disparity closure process with Vermont applications. Source: HTR's Health Equity Studio methodology.

10.17 Equity in VBC Contracting — Protecting Against Unintended Consequences

Value-based care contracts have a well-documented potential to worsen health equity if not designed with explicit equity protections. The mechanisms are several: global budgets or capitation rates that do not adequately adjust for social risk may incentivize providers to avoid or under-serve high-SDOH populations; shared savings contracts that reward utilization reduction may reward avoiding care for complex patients; quality measure performance may be harder for organizations serving disadvantaged populations even when they are delivering excellent care relative to what their population can access.

10.17.1 Social Risk Adjustment — The Equity Design Requirement for VBC Contracts

Social risk adjustment — adjusting VBC benchmarks and performance targets to account for the social risk burden of an organization's attributed population — is the technical mechanism for protecting equity in payment design. An organization serving a population with high housing instability, food insecurity, and transportation barriers faces higher care management costs and lower quality measure performance potential than an organization serving a high-income, socially stable population. A shared savings contract that ignores this difference rewards the wrong organizations.

CMS has incorporated social risk adjustment into ACO REACH through Comprehensive and Professional Direct Contracting model designs that include social risk stratification. Vermont's AHEAD global budget design must incorporate social risk adjustment at the hospital level — ensuring that hospitals serving the Northeast Kingdom's high-SDOH populations are not penalized relative to Burlington-area hospitals with more favorable social risk profiles. This is an active design question in Vermont's AHEAD implementation that AHS and GMCB must resolve explicitly.

10.17.2 The VBC Equity Audit

Every VBC contract should undergo an equity audit before and after signing, examining whether the contract design creates incentives that could worsen disparities. The VBC Equity Audit checklist covers eight categories:

- Attribution equity: Does the attribution methodology systematically exclude high-SDOH patients who lack a qualifying primary care visit?
- Social risk adjustment: Is the benchmark adjusted for the social risk profile of the attributed population?
- Quality measure equity: Are quality measures stratified by race/ethnicity and income, with performance reported and rewarded based on equity improvement, not just average improvement?
- SDOH investment incentives: Does the contract provide financial credit for investments in SDOH programs that reduce long-term utilization?
- Care management intensity: Are care management resources allocated proportionate to SDOH burden and disparity level, not just clinical risk?
- Beneficiary protections: Are there protections against provider avoidance of high-complexity, high-SDOH patients?

- Community benefit requirements: For global budgets, are there explicit requirements for community health investment, including SDOH programs?
- Equity performance reporting: Is equity performance reported publicly, with minimum standards below which the organization cannot be considered performing?

10.18 Vermont's Population Health Equity Analytics Architecture

The analytical infrastructure for equity measurement in Vermont is being built through the convergence of three systems: VHCURES (claims-based population data), the Blueprint clinical registry (quality and demographic data for PCMH-attributed patients), and the VDH health equity data systems (BRFSS survey data, vital statistics, county-level environmental data). None of these systems alone is adequate for comprehensive equity measurement; their integration — the one health record per person mandate — is the target state.

In the interim, Vermont's equity analytics strategy must be pragmatic: use the best available data for each equity dimension, be explicit about gaps, and make decisions based on the evidence available while investing in improving the evidence base. The following architecture organizes Vermont's current equity analytics capabilities and the priority investments needed to close gaps:

Equity dimension	Best current data source in Vermont
Geographic access disparities (HSA-level)	VHCURES county-level utilization; GMCB hospital market share data; Blueprint practice location mapping
Insurance coverage disparities	KFF/ACS coverage estimates; BRFSS insurance status by demographic group; DVHA Medicaid enrollment data
Primary care access disparities (race/ethnicity)	VDH BRFSS survey (limited by small BIPOC sample size); Blueprint provider relationship data for attributed population
Chronic disease control disparities	Blueprint PCMH clinical registry (most complete, but only covers PCMH-attributed patients); BRFSS self-reported chronic disease by demographic group
Behavioral health disparities (geographic)	AHS ED surveillance data (suicide, SH, OD rates by county — available and stratified)
SDOH burden distribution	Blueprint HRSN screening data (incomplete — not all practices at full adoption); VDH community

Equity dimension	Best current data source in Vermont
	vulnerability index; USDA food access data by census tract
Clinical practice variation by demographic	VHCURES claims (limited demographic data); VDH health equity data — requires improved race/ethnicity data collection in clinical settings
Trust and patient experience	CAHPS survey data from Blueprint PCMH practices (available but limited demographic stratification)

Table 10.3 — Equity dimensions and best available Vermont data sources. Source: HTR Analysis (2026).

10.19 Implications for You

If you are a Vermont hospital executive: HEDIS equity measurement is coming whether you prepare for it or not. AHEAD's performance measurement framework includes equity metrics — stratified HEDIS measures by race, income, and geography — and Vermont's global budget methodology will incorporate equity performance. The hospitals that invest now in stratified quality measurement — understanding their quality gaps by population subgroup rather than just their overall rates — will be able to demonstrate equity improvement in their first AHEAD performance year. Those that have not tracked stratified data will be explaining gaps they did not know existed.

If you are an AHS or GMCB official: Vermont's HEROI scoring framework is a policy innovation worth protecting. It is the only state-level tool that systematically scores the equity impact of healthcare interventions before they are deployed — identifying which interventions will help or harm the populations with the worst outcomes. As Vermont's transformation pipeline grows — new care models, new payment arrangements, new technology deployments — the discipline of HEROI scoring before deployment, not after, is the mechanism that prevents the transformation from producing equity-neutral or equity-harmful results at the system level. Do not let HEROI become a documentation exercise. Keep it as a design constraint.

If you are a Vermont legislator: The equity measurement infrastructure Vermont is building — HEROI scoring, stratified HEDIS measures, HRSN screening data — will produce the most detailed state-level picture of healthcare equity in Vermont's history. That data has implications beyond healthcare. It connects to housing, food security, education, and economic development in ways that Act 68's healthcare focus does not directly address. Vermont's legislature should consider how the equity data produced by the healthcare transformation informs the state's broader social policy agenda — and specifically, whether the EAST Fund's social determinants investment can be coordinated with non-healthcare state programs in ways that produce more durable equity improvement than healthcare investment alone.

If you are a national health policy professional: Vermont's HEDIS equity measurement chapter is a detailed case study in the gap between equity measurement and equity improvement. Having good data on equity gaps is necessary but not sufficient for closing them. The mechanisms that produce equity improvement — targeted community investment, social determinants screening, CHW programs in high-gap communities — are not the same mechanisms that produce measurement. Vermont's experience will show whether having the data produces the investment, or whether the investment has to be mandated separately from the measurement. That question matters enormously for national health equity policy.

Figure 10.9 — Vermont equity analytics: current data sources by equity dimension. Sources: GMCB; VDH; Blueprint for Health; DVHA; AHS.

10.20 Key Concepts Introduced or Expanded in This Chapter

Disparity root cause taxonomy

The six-pillar framework for categorizing the mechanisms that produce health disparities: geographic access barriers, social needs barriers, clinical practice variation, insurance and coverage barriers, and trust and engagement barriers. Each root cause requires different interventions; addressing the wrong cause does not close the gap.

GLP-1 agonists

A class of medications (semaglutide, tirzepatide) with proven efficacy for obesity, type 2 diabetes, and cardiovascular disease risk reduction. Current Medicare and most Medicaid programs do not cover them for obesity as a primary indication, creating an equity gap in which transformative drugs are accessible to commercially insured patients but not to Medicaid patients with the highest obesity burden.

BALANCE Model

CMS's December 2025 voluntary model enabling state Medicaid agencies and Medicare Part D plans to cover GLP-1 medications for obesity by negotiating prices with manufacturers. Medicaid launch as early as May 2026; Medicare Part D in January 2028. Voluntary for manufacturers, states, and plans.

Geographic equity

The dimension of health equity concerning disparities attributable to where a person lives — rural versus urban, Northeast Kingdom versus Burlington. Vermont's dominant equity challenge; requires interventions that address geographic access barriers, transportation, broadband, and provider distribution alongside racial and socioeconomic equity.

SDOH (Social Determinants of Health)

The non-clinical conditions that shape health — housing, food security, transportation, economic security, education, social connection, and safety. Oliver Wyman's first imperative (build housing and fix transportation) is an SDOH intervention as much as a healthcare access intervention.

Health Equity Advisory Commission

Vermont's standing advisory body on health equity policy. Required by Act 68 to be consulted by the Health Care Delivery Advisory Committee, embedding equity oversight in transformation governance.

Northeast Kingdom

Caledonia, Essex, and Orleans counties in Vermont's most rural region. Vermont's highest-priority equity geography: highest uninsurance rates (6-8%), most limited provider access, highest unemployment, lowest broadband connectivity, and most financially distressed hospital infrastructure.

HRSN (Health-Related Social Needs) screening

Universal screening for social needs at primary care visits using validated tools, with warm handoffs to Community Health Teams for navigation. Operational throughout Vermont's Blueprint PCMH network; generates SDOH data that feeds equity monitoring when linked to VHCURES.

Sources: Vermont Department of Health Health Equity Data Report, Annual Report to the Vermont Legislature (January 17, 2025); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (2024); Vermont Rural Health Transformation Program Application (November 2025); KFF, Medicaid Coverage of and Spending on GLP-1s (January 2026); KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid (2026); CMS, BALANCE Model announcement (December 23, 2025); CMS, Medicare GLP-1 Bridge demonstration (March 2026); Health Affairs, Reimagining Rural Health Equity (June 2024); Vermont Public, Vermont is trying to survey underrepresented communities about their health (April 2025); Act 68 of 2025; Act 167 of 2022; AHS November 2025 Health Care System Transformation Report.

Chapter 11: The Operations Pillar — Executing Hospital System Transformation

> *Vermont's $1,303 per-discharge administrative cost gap versus the national benchmark is not a compliance problem — it is the primary financial opportunity available to hospitals that must operate within a global budget.*

Operations is the pillar where a transformation plan either becomes real or stays a document — and it is also the pillar most often under-resourced, because it produces no new policy, technology, or care model of its own. It simply has to make the other five work, on a timeline, with the staff and capital actually available. That challenge is universal: any health system attempting structural transformation will face some version of the regionalization, workforce, and administrative-efficiency questions this chapter develops for Vermont's 14-hospital system. Vermont's specific numbers — the per-discharge administrative gap, the RHRC methodology, the hospital-by-hospital dashboard — are this chapter's worked example of what operationalizing a transformation mandate looks like when the mandate has statutory deadlines attached.

Every chapter before this one is about ideas. This chapter is about execution — the 14-hospital process, RHRC methodology, workforce constraints, and the mechanics of regionalization.

11.1 The Operations Pillar: Where Strategy Meets Reality

Every chapter in this book that precedes this one is, in some sense, about ideas: the analytical framework for understanding Vermont's crisis, the financial architecture of global budgets and reference-based pricing, the clinical models that should replace hospital-centric care. This chapter is about execution. The Operations pillar is where the gap between a well-designed plan and an implemented one is either closed or exposed. It is where organizational inertia, workforce constraints, data gaps, political friction, and the sheer complexity of coordinating 14 independent hospital organizations across a geographically vast rural state determine whether transformation actually happens.

Vermont's operations challenge is unusual in its specificity and urgency. AHS has statutory deadlines — hospital transformation plans by early 2026, RBP by FY2027, global budgets by FY2028, the Statewide Strategic Plan by December 2028. It has a constrained budget ($4.2M in appropriated state funds, plus $195M in RHT Program capital and $1.2M in AHEAD cooperative agreement funds). It has a newly contracted operational partner in the Rural Health Redesign Center. And it has 14 hospitals in wildly different financial and operational conditions, ranging from a large academic medical center at the center of the state's economy to 19-bed critical access hospitals in the Northeast Kingdom where any service reduction has immediate community consequences.

This chapter develops the operational framework for executing system transformation at the scale Vermont requires. It draws on the specific methodologies AHS and RHRC have deployed, the transformation planning process and its current state, and the lessons from other state and health system transformation efforts about what the operations discipline requires. It is written both as an academic treatment of the operations pillar and as a practical guide for the AHS staff, hospital leaders, and legislative overseers who are executing Vermont's transformation in real time.

11.2 The Vermont Transformation Operating Model

Vermont's health system transformation is not being executed by a single authority acting on a blank canvas. It is being coordinated by AHS across a network of 14 independent (all non-profit) hospitals, 12 local health district offices, the GMCB as regulatory partner, the Department of Vermont Health Access (DVHA) as Medicaid authority, dozens of community-based organizations, primary care practices, designated mental health agencies, and a state legislature that requires monthly reporting on progress. Understanding the governance and operational model for managing this network is prerequisite to understanding how transformation actually works.

11.2.1 The Institutional Architecture

Act 68 established a four-actor governance architecture for Vermont's transformation that is worth understanding precisely:

Actor	Statutory Role	Key Authority	Current Capacity
Agency of Human Services (AHS)	Lead transformation authority; develops and implements the Statewide Health Care Delivery Strategic Plan; provides technical assistance to hospitals	Directs transformation planning; awards transformation grants; sets SDOH investment priorities; manages RHRC and other contractors	Health Care Reform Office (recently reorganized); contracted RHRC through October 2025; hiring additional staff with RHT Program funds
Green Mountain Care Board (GMCB)	Regulatory authority; implements RBP and global budgets; reviews and approves hospital	Sets hospital rates; approves/denies hospital budget requests; enforces Act 68 spending	Added 3 new permanent positions (Act 68 authorization); hired Affordability and Pricing Project Director;

Actor	Statutory Role	Key Authority	Current Capacity
	budgets; monitors quality and price	reduction requirements; issues CON decisions	procuring RBP vendor (early 2026)
Department of Vermont Health Access (DVHA)	Medicaid authority; manages Medicaid global budget under AHEAD alignment; oversees Blueprint for Health and OneCare wind-down	Medicaid contract terms; Blueprint for Health funding; Medicaid global budget design under AHEAD	Managing AHEAD Medicaid alignment; Blueprint for Health expansion; post-OneCare transition
Health Care Delivery Advisory Committee	Provides accountability oversight; sets affordability benchmarks; advises on evaluation process for the health care delivery system	Advisory authority; stakeholder input; ongoing monitoring of strategic plan progress	Newly established under Act 68; initial composition and operating procedures being developed

Figure 11.1 — Vermont health system transformation governance architecture under Act 68. Sources: Act 68 of 2025; AHS Legislative Presentation (January 2025).

The governance architecture has one significant design tension worth naming: AHS is simultaneously the transformation planner, the technical assistance provider, the grant-maker, and the accountability monitor for the hospitals it is also trying to help. This is not an unusual arrangement in state healthcare reform — Maryland's HSCRC has regulatory authority and collaborative relationships simultaneously — but it requires explicit management. The AHS model works best when hospitals see the agency as a genuine partner in solving difficult problems, not primarily as a regulator imposing mandates. Oliver Wyman's recommendation to add a Division of Planning and Effectiveness and to clarify the AHS-GMCB role division is partly a response to this tension.

11.2.2 The Transformation Planning Sequence

AHS designed a phased transformation planning process that moves hospitals from initial engagement through draft strategy to final implementation plans. The sequence was developed with RHRC and reflects the operational reality that 14 hospitals in very different financial and operational positions cannot move at the same pace or toward the same model.

Phase	Content	Timing	AHS role
1. Data review and baseline	RHRC meets individually with each hospital to review hospital-specific financial and operational data: cost per adjusted discharge, operating margin, payer mix, service line volumes, workforce indicators, and comparison against national and Vermont peers	May–June 2025	RHRC provides technical assistance; AHS reviews data with hospitals; GMCB provides benchmark data
2. Statewide and regional convenings	All 14 hospitals participate in statewide planning conversations; then organized into regional groups for more detailed discussion of shared opportunities: transfer coordination, shared clinical networks, service alignment, administrative consolidation	Summer–Fall 2025	AHS and Blueprint for Health convene; regional groups self-organize around specific opportunity areas
3. Draft Care Transformation Strategy Outline	Each hospital develops a written outline of its proposed transformation strategy, mapping short-term (within 1 year), medium-term (1-3 years), and long-term actions to Act 167 outcome objectives	Fall 2025	AHS reviews outlines; provides feedback; all eligible hospitals submitted applications for Act 68 transformation grants
4. Draft Transformation Plan	Full transformation plan with specific action steps, associated costs, timelines, service line analysis, staffing models, and shared service opportunities. Aligned with regional and statewide planning.	Winter 2025–2026	AHS reviews and approves each draft; GMCB provides analytical support; analytics vendor (being procured) enables cost modeling
5. Final Transformation Plan	Approved final plan; AHS integrates hospital plans into statewide picture; plans inform AHS's Statewide Strategic Plan	Early 2026 and ongoing	AHS integrates across 14 plans; identifies statewide priorities; allocates RHT Program capital to support implementation

Phase	Content	Timing	AHS role
	development for December 2028 delivery		

Figure 11.2 — Vermont hospital transformation planning sequence. Sources: AHS Health Care Transformation website; AHS November 2025 Act 68 Report; AHS January 2025 Legislative Presentation.

BEYOND VERMONT

The planning sequence above — inventory current state, identify the target operating model, sequence the transition, assign accountability — is a generic transformation-planning structure that applies whether the system being transformed has 14 hospitals or 4, and whether the planning mandate comes from a state statute, a board directive, or a payer contract requirement. What Vermont's process demonstrates, more than the specific sequence, is what it looks like to run this planning process under a public statutory deadline with public reporting requirements — a forcing function many organizations lack, and one worth considering deliberately adopting even where no external mandate requires it.

"Transformation work is ongoing and evolving. Based on hospital feedback and changes in state and federal policy, AHS is continuing to adapt our approach to ensure planning is coordinated, responsive, and effective."

— Vermont Agency of Human Services, Health Care Transformation website, 2026

11.3 The RHRC Methodology — Technical Assistance for Rural Hospital Transformation

The Rural Health Redesign Center (RHRC) was AHS's primary operational contractor for hospital transformation planning from spring 2025 through October 2025. Understanding the RHRC methodology is important not just as Vermont history but as a transferable model for how state agencies can provide technical assistance to financially distressed rural hospital networks — a challenge that will confront state health agencies across the country as the demographic and financial pressures Vermont faces today spread nationally.

11.3.1 What RHRC Did

RHRC's engagement combined three functions that are usually performed separately: financial and operational assessment, change management support, and transformation plan development. The combination was deliberate — Oliver Wyman's experience had shown that hospitals can absorb financial analysis without changing behavior, can engage in planning processes that produce documents rather than action, and can participate in change management programs without addressing their fundamental financial unsustainability. RHRC's model integrated all three.

11.3.2 RHRC's Four Workstreams at Each Hospital

RHRC worked with each of Vermont's 14 hospitals through four parallel workstreams, adapted to each hospital's specific position and needs:

- Financial and operational baseline: Review of hospital-specific cost data from GMCB financial records, CMS cost reports, and the NASHP Hospital Cost Tool. Identification of the hospital's position relative to Vermont average and national benchmark on key efficiency metrics (operating profit per adjusted discharge, management and administrative cost per adjusted discharge, direct patient care labor cost per adjusted discharge).
- Priority identification: Working with hospital leadership to identify the highest-leverage short-term cost reduction opportunities — supply chain, staffing models, service line analysis — and medium-term transformation opportunities — shared services, service line reconfiguration, COE alignment.
- Transformation plan development: Supporting each hospital in developing a structured transformation plan that maps specific actions to timelines, costs, and Act 167 outcome objectives. Plans address hospital sustainability and cost through analysis of service lines, staffing models, and shared services opportunities.
- Regional coordination: Connecting hospital-level planning to the statewide regional work — ensuring individual hospital plans are aware of and compatible with what neighboring hospitals are planning, and identifying shared service opportunities that require cooperation across organizational boundaries.
- The RHRC contract concluded in October 2025 — a planned conclusion, not a termination. AHS designed the engagement as a time-limited capacity-building investment, not an ongoing dependency. The explicit goal was for hospitals to develop the internal planning capability and the collaborative relationships needed to continue transformation work independently, with AHS providing ongoing oversight, grant funding, and analytical support rather than direct technical assistance.

This design philosophy reflects a lesson from failed state transformation initiatives: external consultant-driven processes that do not build internal organizational capacity produce plans that expire when the consultant leaves. Vermont's approach — using RHRC to build the planning process and the inter-hospital relationships, then transitioning to AHS-led coordination with hospital-level implementation — is more likely to produce durable change.

11.3.3 The Analytics Gap and Its Resolution

The November 2025 AHS transformation report identified one critical operational gap in the current system: the absence of an analytics platform capable of evaluating hospital transformation proposals quantitatively, modeling the cost and quality implications of different service configurations, and assessing alignment with statewide goals. This gap means AHS currently cannot answer with confidence the questions that the transformation planning process is supposed to produce answers to: if Springfield Hospital converts to a Rural Emergency Hospital, what is the net financial impact on the system? If orthopedic surgery is concentrated at Southwestern Vermont and Copley, what travel burden does that create for patients in the affected communities?

AHS responded to this gap by procuring an analytics vendor in partnership with GMCB — a joint procurement that reflects the appropriate division of labor: GMCB brings regulatory data and rate-setting expertise; AHS brings population health and SDOH context. The analytics platform, once operational, will enable the state to evaluate hospital proposals in terms of quality, cost, and access; assess alignment with regional and statewide goals; and quantify potential cost savings using evidence-based methods. This platform is also the technological foundation for monitoring the Statewide Strategic Plan's implementation metrics after December 2028.

11.4 The Vermont Hospital Performance Dashboard — Where Each Hospital Stands

32.3%	14.8%	91%	59%
Potentially Avoidable ED Visits	30-Day Readmission Rate	VT Adults with Primary Care Provider	Primary Care Accepting New Medicaid

AHS's November 2025 transformation report established Vermont's official performance measurement framework — the set of outcome measures against which progress toward Act 167's five goals will be tracked. The framework covers three domains: provider efficiency and sustainability, payer affordability, and patient health outcomes. The following table presents the current performance landscape, with Vermont's actual values compared to benchmarks set at the 75th percentile of national peers.

Measure	VT Current	Benchmark (75th pctile)	Gap	Act 167 Goal
30-Day all-cause readmission rate	14.8%	14.5%	+0.3% (within range)	Improve health outcomes
Potentially avoidable ED visits as % of all ED visits	32.3%	No benchmark set yet	Significant opportunity	Reduce inefficiencies; access
Operating profit per adjusted discharge — PPS hospitals	$5,345	$4,162	+$1,183 above benchmark	Lower costs

Measure	VT Current	Benchmark (75th pctile)	Gap	Act 167 Goal
Operating profit per adjusted discharge — CAH hospitals	$938	$2,784	-$1,846 BELOW benchmark	Lower costs / sustainability
Mgmt. and admin cost per adjusted discharge — PPS	$2,730	$1,427	+$1,303 above benchmark	Reduce inefficiencies
Vermonters with a personal health care provider (PCP)	91%	87%	+4% above benchmark	Access / equity
Primary care practices accepting new Medicaid patients	59%	Not set	Access gap for Medicaid	Access / equity
Blueprint PCMH patients with hypertension controlled	77%	Not set	Quality management	Improve health outcomes
ED visits for suicide ideation/self-harm per 10K ED visits	245.5	Not set	Behavioral health access gap	Access / equity
ED visits for opioid overdose per 10K ED visits	16.1	Not set	SUD treatment access gap	Access / equity

Figure 11.3 — Vermont health system transformation outcome measures: current performance vs. benchmarks. Source: AHS Health Care System Transformation Report, November 2025.

Several findings from this dashboard deserve specific attention from healthcare leaders and policymakers:

The CAH operating profit gap is the most urgent financial signal. Critical Access Hospitals are running at $938 operating profit per adjusted discharge, against a benchmark of $2,784 — a $1,846 per-discharge gap. This means Vermont's smallest, most rural hospitals are the furthest from financial sustainability. The RBP and global budget reforms will affect these hospitals differently from PPS hospitals, and Act 68 explicitly requires GMCB to consider CAH-specific adjustments. The RHT Program's explicit inclusion of CAH stabilization as a priority use of funds reflects this gap.

The administrative cost premium is massive. PPS hospitals are spending $2,730 per adjusted discharge on management and administrative costs, against a benchmark of $1,427 — a 91% premium over what efficient peer hospitals spend. This $1,303 per-discharge gap is exactly the administrative cost reduction that Oliver Wyman identified as one of the three primary sources of direct savings from transformation. If Vermont's PPS hospitals can reach the benchmark administrative cost level, the system savings are substantial — and Oliver Wyman estimated administrative reduction as contributing over $100 million of the $400M+ transformation savings target.

The behavioral health gap is measurable and alarming. With 245.5 ED visits per 10,000 for suicide ideation and self-harm — and rates statistically higher in Rutland and Windham counties, and among Vermonters aged 15-44 — the crisis presentation data confirms what Oliver Wyman's community meetings documented: Vermont's behavioral health access problem is driving a significant share of avoidable hospital utilization. This is not a clinical quality problem; it is an access problem. The CCBHC expansion (5 new entities by July 2026) and the behavioral health investments funded through AHEAD's EAST Fund are the operational response.

Management and administrative cost per adjusted discharge: Vermont PPS hospitals vs. benchmark

Vermont PPS hospitals (actual)	$2730
National 75th percentile benchmark	$1427
Gap (excess admin cost)	$1303

Figure 11.4 — Administrative cost gap: Vermont PPS hospitals vs. national benchmark. This $1,303 per-discharge gap represents the primary operational inefficiency target for Vermont's transformation. Source: AHS Transformation Report, November 2025 (NASHP via CMS).

11.5 Regionalization — The Operational Logic of Right-Sizing a 14-Hospital System

Regionalization is the operational translation of Oliver Wyman's third imperative: move all care possible out of hospitals, and concentrate what must stay in hospitals at sites with sufficient volume to maintain expertise. The concept is straightforward; the execution is complex. This section develops the operational methodology of regionalization — how you actually move from a system of 14 hospitals each attempting full-spectrum care to a differentiated regional network where each facility has a clearly defined role, adequate patient volume, and sustainable financial structure.

11.5.1 The Regional Network Design Framework

Oliver Wyman's regionalization blueprint organized Vermont's hospital network around three facility types, each with a distinct role in the transformed system:

Tier 1 — Regional Specialty Centers (RSCs)

RSCs are the hospitals with sufficient population base, financial position, and existing expertise to sustain inpatient beds and serve as Centers of Excellence in multiple specialty areas. Under Oliver Wyman's blueprint, UVMMC in Burlington is the state's primary RSC with 16 COE designations. Southwestern Vermont Medical Center, Brattleboro Memorial Hospital, Rutland

Regional Medical Center, and Central Vermont Medical Center function as secondary RSCs, each with 5-9 COE designations serving their regional populations.

The RSC model requires a fundamental change in how these hospitals think about their role: from serving their immediate geographic catchment area to accepting referrals from the entire region for their designated specialties. This means building the transfer infrastructure, the outreach relationships with smaller hospitals, and the care management capacity to serve a larger and more complex patient population.

Tier 2 — Community Hospitals with Focused Scope

Community hospitals with focused scope are inpatient facilities that maintain specific service lines where they have adequate volume and expertise — their designated COE specialties — while transferring other complex cases to RSCs. Northwestern Medical Center (St. Albans), Northeastern Vermont Regional Hospital (St. Johnsbury), Springfield Hospital, and Copley Hospital (Morrisville) fall into this category, with 2-4 COE designations each.

The operational challenge for this tier is managing the transition: reducing reliance on service lines being regionalized elsewhere while building capacity in designated specialties and managing the patient volume shift without destabilizing the hospital's financial position. This requires precisely the kind of short-term, medium-term, and long-term planning that RHRC's transformation plan methodology was designed to support.

Tier 3 — Transformed Facilities (REH, CACC, or Home-Based Models)

Tier 3 facilities are hospitals that cannot sustain inpatient beds in the long term — either because their population base is too small, their financial position is too distressed, or both. Rather than closing unexpectedly and leaving communities without any healthcare access, Oliver Wyman's blueprint proposes orderly conversion to one of three alternative models: Rural Emergency Hospital (REH — 24/7 emergency and observation without inpatient beds), Community Ambulatory Care Center (CACC — outpatient and primary care services), or a Care at Home support hub. Grace Cottage, Gifford Medical Center, North Country Hospital, and Porter Medical Center are among the facilities whose futures require further discussion as part of Vermont's regionalization planning.

The REH designation is particularly significant for Vermont because it preserves the emergency access function that communities depend on while eliminating the expensive inpatient infrastructure that these hospitals cannot financially sustain. A community with an REH and reliable EMS transport to an RSC 45 minutes away is better served than one with a financially distressed inpatient hospital that cannot staff its surgical suite or maintain specialist coverage.

Hospital (HSA)	Current COEs	Tier	Key transformation considerations
UVM Medical Center (Burlington)	16	Tier 1 — Primary RSC	Administrative overhead reduction is critical — $3,826 admin cost/adj. discharge vs. $1,427 benchmark. Examine non-patient-care provider activities (OW specific finding).
SW VT Medical Center (Bennington)	9	Tier 1 — Secondary RSC	Strong COE position. Key southern VT anchor. Psych adult/adolescent COE important given behavioral health gaps.
Brattleboro Memorial Hospital	6	Tier 1 — Secondary RSC	Acute general surgery COE is regionally critical. Windham County has higher-than-average suicide/SH and opioid rates — behavioral health access priority.
Rutland Regional Medical Center	5	Tier 1 — Secondary RSC	Central Vermont anchor; neurology and psych-adult COEs important. Rutland County has higher opioid overdose rates.
Central VT Medical Center (Barre)	5	Tier 1 — Secondary RSC	Radiation therapy and neurology COEs. Washington County (Barre/Montpelier) is second most populous HSA.
NW Medical Center (St. Albans)	4	Tier 2 — Focused scope	Northern VT anchor. Cancer surgery and radiation COEs. Franklin County workforce hub.
NE VT Regional Hospital (St. Johnsbury)	4	Tier 2 — Focused scope	Northeast Kingdom anchor — Essex/Caledonia/Orleans have highest uninsurance rates (6-8%) and most rural populations.
Springfield Hospital	2	Tier 2 — Focused scope	Memory care and psych-adult COEs. High SUD rates in Windsor/Windham corridor.

Hospital (HSA)	Current COEs	Tier	Key transformation considerations
Copley Hospital (Morrisville)	2	Tier 2 — Focused scope	Orthopedics and rheumatology COEs. Lamoille County access point. Rural workforce housing challenge.
Mt. Ascutney (White River Jct.)	1	Tier 2 — Focused scope	Rehabilitation COE. Dartmouth Health affiliation creates cross-border care opportunities.
Grace Cottage, Gifford, North Country, Porter	TBD	Tier 3 — Requires discussion	Further assessment needed. REH conversion, CACC, and home-based care models all under consideration. RHT Program funds available for transition support.

Figure 11.5 — Vermont 14-hospital regionalization framework: COE designations, tiers, and key operational considerations. Sources: Oliver Wyman Act 167 Report (2024); AHS Transformation Reports (2025).

11.5.2 The Operational Mechanics of Regionalization

Regionalization is not a policy decision that happens once — it is an operational process that must be managed over years. The transition from a hospital system where every facility attempts full-spectrum care to one organized around regional specialization requires changes across five operational domains:

Transfer infrastructure is the most visible immediate need. Oliver Wyman documented that Vermont currently has 31 separate EMS agencies that a provider may need to contact to arrange a patient transfer — a fragmentation that is simultaneously a patient safety risk and an operational cost. Vermont's RHT Program application includes EMS transformation as a priority investment: professionalization and regionalization of EMS, improved inter-facility transfer coordination, and expanded telehealth to reduce unnecessary transfers. The November 2025 AHS report identified inter-facility transfer coordination as one of the primary statewide regionalization themes emerging from regional hospital convenings.

Clinical network agreements formalize the referral relationships between RSCs and Tier 2/3 facilities. Oliver Wyman recommended that RSCs establish formal agreements to receive referrals from emergency departments at smaller hospitals for their designated COE specialties — ensuring that when a patient arrives at Springfield Hospital needing complex general surgery, the transfer pathway to Brattleboro Memorial Hospital (the acute general surgery COE for that region) is established, protocolized, and reliable. These agreements do not currently exist in a systematic form across Vermont's hospital network; the Act 68 transformation planning process is the vehicle for creating them.

Administrative shared services offer some of the largest near-term cost savings in the transformation agenda. Oliver Wyman identified three shared service opportunity areas as high-priority: supply chain (group purchasing), administrative functions (billing, coding, human resources, credentialing), and IT infrastructure. Vermont hospitals have begun early-stage discussions on group purchasing during the regional convenings. The administrative cost gap — $1,303 per discharge above benchmark — is where shared services can make the most immediate impact on the $400M+ savings target.

Workforce redistribution is the most sensitive operational challenge. When a hospital transitions from full inpatient to REH or CACC status, the clinical staff whose roles were tied to inpatient services must either transition to new roles, relocate to RSCs, or leave the healthcare workforce. Oliver Wyman explicitly recommended that healthcare workforce affected by system changes could be redistributed or retrained to perform services needed by the community — recognizing that the workforce challenge is not just about attracting new providers to Vermont but about optimally deploying those already there. Vermont's RHT Program workforce investments — tuition assistance, recruitment incentives with 5-year service obligations, scope-of-practice expansion for nurses and APRNs — are the supply-side response; workforce redistribution planning is the demand-side response.

Community engagement is the political prerequisite for all operational change. Oliver Wyman's process demonstrated that communities are willing to accept difficult changes — service reductions, facility reconfiguration, changes in travel distances — when they have been genuinely engaged in the decision-making process and when they understand the alternative (unexpected closures) is worse. AHS has committed to continuing community engagement as transformation plans develop, expanding beyond hospital leadership to include community providers and other health and human services stakeholders as priorities are identified.

11.6 The Workforce Crisis — Vermont's Most Binding Operational Constraint

370 FTE	2.5%	$195M	All 14
Primary Care Shortage by 2030	VT Unemployment Aug 2025	RHT Workforce Investment	Hospitals Citing MD Shortages

Of all the operational challenges Vermont faces, workforce is the one most likely to constrain the pace and ambition of transformation. Vermont's healthcare workforce problem is not a single issue — it is a cluster of interconnected problems that Oliver Wyman's systems map showed cascading across the entire healthcare system. Understanding the workforce problem in its full complexity is prerequisite to designing interventions that actually work.

11.6.1 The Dimensions of Vermont's Workforce Crisis

Primary care physician shortfall by 2030 370 FTEs 112 family medicine, 190 other primary care (RHT application)	Hospitals with physician shortages cited as primary operational challenge All 14 Physician shortage cited in Oliver Wyman operational challenges for every HSA
RHT Program workforce investment $195M Over 5 years; tuition assistance, recruitment, 5-yr service obligations	Vermont unemployment rate (August 2025) 2.5% Below 4.3% national rate; extremely tight labor market limiting healthcare recruitment

Figure 11.6 — Vermont workforce crisis key metrics. Sources: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Labor.

Vermont's workforce problem has five distinct but interconnected dimensions, each requiring a different intervention approach:

Primary care shortage is both a supply problem and a productivity problem. Oliver Wyman made a counterintuitive finding that Vermont's primary care shortage is partly a productivity issue, not purely a headcount problem: HRSA recognizes no Health Profession Shortage Areas in Vermont, and if primary care providers were supported to see three patients per hour, the state would have adequate supply. The implication is that scope-of-practice expansion for APRNs and physician assistants, team-based care models that use clinical staff at the top of their licensure, and administrative burden reduction (prior authorization reform, documentation support) can meaningfully increase primary care capacity without adding headcount.

Specialist shortage drives regionalization urgency. Vermont cannot maintain specialist coverage at 14 hospitals across a state of 647,000 people. The population density and economics do not support it. The Oliver Wyman COE framework is, in part, a response to the specialist shortage: rather than trying to recruit and retain a cardiologist or neurosurgeon for every Vermont community, concentrate those specialists at facilities with sufficient volume to justify their positions and deliver the clinical excellence their presence enables.

Nursing shortage is the most acute staffing crisis at individual hospitals. All Vermont hospitals cited physician shortages and difficulty recruiting staff since COVID-19 as primary operational challenges in Oliver Wyman's assessment. Travel nursing — the expensive emergency staffing solution that many hospitals have relied on since 2020 — has significantly worsened hospital financial positions. Travel nurse costs are typically two to three times the cost of a permanent hire. Vermont hospitals that remain heavily dependent on travel nursing are in a financial hole that deepens with every shift filled by a travel agency.

Housing is the hidden workforce constraint. Oliver Wyman's systems map traced the connection from housing shortage to workforce shortage to healthcare system fragility explicitly. Vermont's rental vacancy rate of 3% — well below the 5% rate of a healthy market — means healthcare workers recruited to Vermont communities cannot find housing. Half of Vermont renters are cost-burdened. This is not a healthcare problem that healthcare reform can solve; it requires housing policy responses. But AHS's transformation mandate, properly understood, extends to advocating for and enabling the housing investments that make healthcare workforce recruitment possible. This is one of the reasons Oliver Wyman's first imperative — build housing and fix transportation — comes before payment reform in the three-imperative sequence.

Scope-of-practice restrictions limit Vermont's ability to use the workforce it has effectively. Vermont has taken some steps toward scope-of-practice expansion — joining the Social Work Licensure Compact, allowing pharmacist-extended prescriptions, approving mental health professionals without master's degrees for certain psychotherapy roles — but Oliver Wyman's action list called for further expansion of professional licensure and practice scope for nurses, EMTs, and pharmacists. The RHT Program's workforce investments include scope-of-practice-enabling training, and Act 68's workforce goal — attracting and retaining high-quality health care professionals — implicitly requires the scope-of-practice environment to make Vermont competitive with surrounding states.

11.6.2 Vermont's Workforce Strategy: The RHT Investment Framework

Vermont's Rural Health Transformation Program application dedicated a substantial portion of its $195M award to workforce infrastructure — treating workforce as capital investment rather than operating expense. The distinction matters: using one-time capital funds to build durable workforce pipelines (tuition assistance that creates committed local providers, training infrastructure that enables scope expansion, housing investments that make recruitment viable) is fundamentally different from using them to fund ongoing salaries.

11.6.3 RHT Workforce Investment Priorities

Vermont's RHT Program application included five workforce investment areas, each designed to produce durable capacity rather than temporary staffing:

Tuition assistance and loan forgiveness for nurses, physicians, APRNs, LNAs, home health aides, and other providers, with 5-year service obligations in Vermont — building a locally-rooted workforce pipeline

On-site clinical training infrastructure — supporting teaching programs that keep medical students and residents in Vermont communities during training, increasing the probability they establish practices there

Scope-of-practice training and support for APRNs, physician assistants, pharmacists, and EMTs — enabling Vermont to use its existing workforce at maximum capacity

Community health worker training and deployment — building the SDOH navigation and care coordination workforce that population health management requires, drawing from the communities being served

Interstate Medical Licensure Compact (IMLC) participation — enabling out-of-state providers to practice via telehealth in Vermont, reducing specialist access gaps without requiring physical relocation

11.7 The Operations Metrics Framework — What AHS Must Track

Effective operations management requires clear, measurable indicators of progress. AHS's current transformation outcome framework (Figure 11.3) establishes a baseline measurement architecture, but it has acknowledged limitations: most data reflects 2023 or earlier performance, the impact of 2025 transformation activities will not appear in measures until 2026 or later, and some critical domains — mental health, SDOH, post-acute care — lack the consistent data infrastructure to support real-time monitoring.

For the Statewide Strategic Plan that AHS must deliver by December 2028, a more complete operations metrics framework is required. Drawing on Oliver Wyman's five Act 167 goals and Act 68's accountability metrics requirements, the following framework organizes the key operational indicators by domain:

Domain	Key metrics	Data source	Target direction
Hospital financial sustainability	Operating profit per adjusted discharge by hospital type; management/admin cost per adjusted discharge; days cash on hand; bond ratings; travel nurse cost as % of total labor	GMCB hospital budget data; CMS cost reports; NASHP Hospital Cost Tool	CAH: toward $2,784 benchmark. PPS admin cost: toward $1,427 benchmark. Travel nursing: reduce toward 5-10% of total labor
Care access	% primary care practices accepting new Medicaid patients; wait times for new patient appointments; potentially avoidable ED visit rate; 30-day follow-up after ED visit for mental illness and SUD	VCCI; GMCB; VDH; Blueprint for Health	Primary care acceptance: increase from 59%. Avoidable ED visits: reduce from 32.3%. 30-day MH follow-up: increase from 76%
Population health	Hypertension control rate in PCMH patients; diabetes HbA1c not in control; all-cause readmission rate; ED visits for suicide	Blueprint for Health; VDH; CMS	Hypertension control: maintain 77%; diabetes: reduce 22% not-controlled;

Domain	Key metrics	Data source	Target direction
	ideation/self-harm; ED visits for opioid overdose		readmission: maintain at/below 14.8%
Cost and affordability	Per-member-per-month total cost of care trend; commercial premium growth rate; individual market affordability for medium-high utilization family; medical loss ratio for marketplace plans	GMCB; insurance rate filings; CMS; VHCURES	PMPM cost: below AHEAD total cost of care targets. Premium growth: reduce toward CPI. MLR: maintain 89%+
Equity	Geographic variation in primary care access by county; health outcomes variation by income and race/ethnicity; uninsured rate by county; Medicaid acceptance rates by region	VDH BRFSS; GMCB; VHCURES	Northeast Kingdom uninsurance gap: reduce from 6-8% toward statewide 3%. Medicaid acceptance: increase across all regions
Operations efficiency	Hospital-to-hospital transfer completion rate and time; EMS response time by county; shared service adoption rate; administrative cost trend; COE referral volume by specialty	EMS records; GMCB; hospital transformation plans	Transfer completion: establish baseline and reduce time. Admin cost trend: negative year-over-year. COE referral volume: increase as regionalization advances

Figure 11.7 — Vermont health system transformation: comprehensive operations metrics framework. Sources: Act 68 of 2025; AHS Transformation Report (November 2025); Oliver Wyman Act 167 Report.

One structural observation about this metrics framework: many of the most important indicators will not show measurable improvement for 18-24 months after the interventions that should drive them are implemented. Healthcare system change is slow, and the data infrastructure to measure it is even slower. AHS's acknowledgment in its November 2025 report that most available measures reflect 2023 or earlier performance is not an evasion — it is an accurate description of the measurement reality. Policymakers and oversight bodies need to understand this lag and resist the temptation to declare failure when early data does not yet show improvement.

11.8 AHS as System Operator — Organizational Design for the 2028 Mandate

The operations chapter concludes with the organizational design question that underpins everything else: is AHS currently structured to execute the transformation it has been mandated to lead? The honest answer, reflected in Oliver Wyman's findings and AHS's own public reports, is: not yet, but the organization is restructuring.

11.8.1 The Current Organizational Gaps

Oliver Wyman identified four specific organizational gaps in AHS that impede transformation execution. These are not criticisms of individuals — they reflect an organizational structure designed for a different era that has not yet adapted to the system operator role Act 68 requires.

Misalignment between AHS sub-unit organization and Hospital Service Areas. Vermont's AHS is organized around program types (mental health, substance use, aging services, Medicaid, etc.) rather than geographic service areas. This means that when a hospital in the Northeast Kingdom is trying to address the housing, transportation, behavioral health, and primary care gaps that drive its ED volume, it must work with multiple separate AHS programs — each with its own administrative requirements, reporting cycles, and performance metrics. Oliver Wyman's recommendation to align AHS sub-units with Hospital Service Areas and meet regularly with hospitals and providers in each HSA is a structural reorganization, not just a coordination improvement.

Absence of a Planning and Effectiveness function. AHS currently lacks a dedicated analytical unit that can calculate the financial and access impacts of service site changes, monitor access and affordability across community providers, and track the quality, access, and equity metrics that the Statewide Strategic Plan will require. The analytics vendor procurement in partnership with GMCB addresses part of this gap, but the internal capacity to interpret, apply, and act on analytical outputs is a separate organizational requirement.

Under-resourcing for the transformation management function. Act 68 requires AHS to simultaneously negotiate with CMS on AHEAD implementation, manage 14 hospital transformation planning processes, develop the Statewide Strategic Plan, oversee RHT Program implementation, coordinate with GMCB on RBP and global budget design, and report monthly to the legislature. The Health Care Reform Office, recently reorganized and supplemented by contractor support, is managing a workload that probably requires 50-70% more staff than currently exist. The RHT Program award explicitly includes funds for hiring staff to oversee its implementation.

Limited SDOH integration across AHS programs. The Oliver Wyman analysis demonstrated that housing, transportation, food security, and behavioral health — all areas where AHS has programmatic authority — are primary drivers of healthcare utilization. But AHS's current organizational structure treats these as separate program areas rather than as integrated components of a population health strategy. The Statewide Strategic Plan mandate creates the opportunity to redesign this integration explicitly.

11.8.2 The Emerging Solution: A Population Health Management Operating Model

The organizational model that AHS's Strategic Plan should articulate — and that the AHS restructuring chapter later in this book develops in full — is a population health management operating model organized around Vermont's Hospital Service Areas, with clear roles for each level of the system.

At the statewide level, AHS functions as the system architect: setting the strategic direction, establishing the payment reform framework (in partnership with GMCB), allocating capital (RHT Program, AHEAD EAST Fund, Act 68 grants), and monitoring system performance against the five Act 167 goals.

At the HSA level, regional care networks — organized around the Tier 1 RSC for each region — function as the operational unit: coordinating transfer pathways, managing shared service arrangements, aligning primary care and behavioral health investment with hospital transformation plans, and connecting healthcare planning with housing, transportation, and SDOH infrastructure planning.

At the provider level, individual hospitals, primary care practices, designated agencies, FQHCs, and community-based organizations function as service delivery nodes: executing transformation plans, participating in regional coordination, and reporting performance against standardized metrics that allow AHS to monitor progress and identify where additional support is needed.

11.9 Work This Chapter on the Platform

Chapter 11's operations argument — administrative cost, workforce, execution — is measurable on the knowledge-and-workspace bench.

Do this	On this tool	What to look for
Track six-pillar transformation status against baseline and target	**Transformation Scorecard** — /research-lab/knowledge-workspace?tab=scorecard	Whether operational capacity is keeping pace with the statutory execution timeline.
Project physician and nurse supply/demand and the cost of turnover	**Workforce Modeler** — /research-lab/knowledge-workspace?tab=workforce	The workforce gap that the $1,303 per-discharge administrative-cost gap must fund closing.

Figure 11.H — Hands-on platform tools for the Operations Pillar.

11.10 Implications for You

If you are a Vermont hospital executive: The $1,303 per-discharge administrative cost gap between Vermont's CAHs and the national benchmark is not a compliance problem — it is a financial opportunity. Under global budgets, every dollar of administrative cost reduction is a dollar of financial improvement. The specific targets: supply chain consolidation through the Vermont CIN, shared revenue cycle services, shared IT infrastructure, and clinical documentation improvement through AI scribe deployment. None of these require waiting for global budgets to go live. The hospitals that close the administrative gap before FY2028 will enter the global budget environment with a cost structure that makes the fixed revenue model viable. The hospitals that do not will be trying to close the gap after the budget pressure has already arrived.

If you are an AHS or GMCB official: Vermont's AHS restructuring — creating the Division of Planning and Effectiveness, the HSA Coordinator network, the Population Health Analytics function, and the Transformation PMO — is the operational infrastructure that makes the Statewide Strategic Plan executable rather than aspirational. The restructuring requires budget, hiring authority, and political support that must come from AHS leadership and the legislature simultaneously. The Oliver Wyman analysis identified organizational capacity as the binding constraint on transformation execution. Building that capacity before the peak execution demand of FY2027–2028 is a precondition for the transformation succeeding on its statutory timeline.

If you are a Vermont legislator: Vermont's 14-hospital system is operating with administrative cost structures designed for a fee-for-service world that Acts 167 and 68 are designed to replace. The RHRC engagement — the Rural Health Restructuring Collaborative working with all 14 hospitals on transformation planning — is the mechanism for building hospital-level Operations pillar capacity before AHEAD's clinical accountability requirements begin. The legislature's role is to fund RHRC adequately to complete this work before FY2027, not after. RHRC is not a consulting expense. It is pre-transformation infrastructure investment with a direct return in AHEAD performance year outcomes.

If you are a national health policy professional: Vermont's operations chapter is the most detailed account available of what hospital system transformation actually requires at the organizational level — not what payment reform requires in theory, but what it requires of a real hospital that has to redesign its administrative operations, restructure its workforce, change its clinical workflows, and manage its balance sheet simultaneously under a hard statutory deadline. The specific operational challenges Vermont's hospitals face — and the specific mechanisms Vermont is using to address them — will be the primary evidence base for other states designing operational support programs for hospitals undergoing mandatory payment reform.

This three-level operating model is not new to health system design — it is the architecture of every effective regional health system in the country. What is new in Vermont is the explicit statutory mandate (Act 68's Strategic Plan requirement), the financial resources to build the infrastructure (RHT Program, AHEAD EAST Fund), and the political will — demonstrated through the extraordinary legislative sequence of Acts 167, 51, 55, and 68 — to execute the structural change rather than continuing to manage the decline.

11.11 Revenue Cycle, HCC Coding, Credentialing, and Administrative Simplification

11.12 Operations Where Money Actually Lives

Healthcare transformation is ultimately executed through the operational infrastructure that most health system leaders think of as administrative — revenue cycle management, clinical documentation, credentialing, workforce management. These functions are not adjacent to clinical transformation; they are the substrate on which it runs. An organization that redesigns its care model for value-based care but fails to adapt its revenue cycle, clinical documentation, and workforce operations will achieve neither the quality outcomes nor the financial results the new model promises.

This chapter develops the operational mechanics of healthcare transformation — the specific processes, metrics, and management disciplines that determine whether value-based care contracts produce the results they are designed for. It is organized around four operational domains: revenue cycle transformation for VBC, HCC coding and risk adjustment excellence, credentialing efficiency, and the workforce operations framework.

11.13 Revenue Cycle Transformation — From FFS Optimization to VBC Management

Revenue cycle management under fee-for-service is a known discipline: charge capture, coding accuracy, claim submission, denial management, underpayment identification. Hospitals and physician practices have invested heavily in FFS RCM over the past two decades, and the industry has developed sophisticated tools and processes for maximizing FFS revenue. None of this investment is wasted under VBC — but all of it must be extended.

VBC revenue cycle adds three functions that FFS RCM does not require: attribution management (ensuring that patients are correctly attributed to the organization for shared savings calculation), total cost of care monitoring (tracking what attributed patients cost across all providers and settings, not just at your organization), and VBC contract reconciliation (understanding and disputing the financial calculations that determine shared savings or loss settlements).

11.13.1 Attribution Management — The VBC-Specific Revenue Function

Under FFS, revenue is determined by what care the organization delivers. Under VBC, revenue is determined partly by which patients are attributed to the organization — and attribution errors can silently reduce shared savings or inflate shared losses by millions of dollars. Every organization in a significant VBC arrangement needs an attribution management function.

Attribution error type	How it happens	Financial impact	Remediation
False attributions	Patients who receive care elsewhere are attributed to your organization due to a primary care visit that triggered attribution before the patient transferred care	Your organization bears cost of care you did not deliver and cannot manage	Regular attribution list review; proactive patient outreach to confirm active care relationship; request attribution corrections from payer using patient-level evidence
Missing attributions	Your highest-cost, highest-need patients are not attributed because they lack a primary care visit that would trigger attribution	You lose the opportunity to earn shared savings for care management programs that benefit this population	Proactive outreach to unattributed high-risk patients; care management programs that include primary care visits to establish attribution; PCMH recognition to improve attribution algorithms
Incorrect benchmark attribution	Attribution list errors inflate or deflate the risk adjustment factor applied to your benchmark	Incorrect RAF produces a benchmark that doesn't reflect your population's actual cost burden	Compare attribution list to actual patient panel; use clinical data to validate risk adjustment; submit corrected diagnoses codes through formal channels
Retroactive attribution changes	Payer changes attribution methodology mid-contract, shifting patients in or out of your attributed population	Shared savings calculations change unexpectedly after performance year ends	Negotiate prospective attribution methodology in contract; require prior notice of methodology changes; maintain your

Attribution error type	How it happens	Financial impact	Remediation
			own attribution model to detect changes

Figure 11.8 — Attribution error taxonomy and remediation. Source: HTR's Operations pillar framework; ACO REACH and MSSP operational guidance.

11.13.2 Total Cost of Care Monitoring — Seeing What You Cannot Bill

The most difficult revenue cycle adaptation for FFS-trained teams is monitoring costs they do not receive. Under VBC, the organization's financial performance is determined by the total cost of care for its attributed population — including care delivered by specialists, hospitals, skilled nursing facilities, home health agencies, and any other provider outside the organization. A primary care group in an ACO may be held accountable for whether their attributed patients receive unnecessary specialist imaging ordered by a cardiologist who has no financial relationship with the ACO.

Vermont's VHCURES all-payer claims database is the analytical foundation for total cost of care monitoring — providing claims data across all payers and provider types for attributed Vermont beneficiaries. The GMCB analytics vendor procurement, and individual hospital analytics investments through the RHT Program, are building the capability to use VHCURES data for real-time population health management rather than retrospective reporting.

11.14 HCC Coding Excellence — The Risk Adjustment Foundation

11.14.1 HCC Gap Closure: High-ROI Pre-AHEAD Action for Every Vermont Hospital

Every Vermont hospital should conduct a retrospective HCC gap analysis for their AHEAD-attributed Medicare population before January 2028.

An organization that has under-coded its population's chronic conditions will receive a lower global budget than its actual cost burden warrants.

Getting HCC coding right before AHEAD launches is foundational financial preparation.

Hierarchical Condition Category (HCC) coding is the mechanism through which Medicare risk adjustment translates patient diagnoses into the Risk Adjustment Factor (RAF) that scales a beneficiary's expected cost. In VBC arrangements, accurate HCC coding is not just a compliance requirement — it is the foundation of financial sustainability. An organization that systematically under-codes its patients' diagnoses will have a benchmark set too low for its population's actual cost burden, and will appear to be underperforming even when its care management is excellent.

11.14.2 The HCC Model — How It Works

Medicare's HCC model assigns each patient a RAF based on their demographic characteristics (age, sex, Medicaid status, disability) and their active chronic conditions as documented in claims. The RAF multiplies the base capitation or benchmark rate — a patient with a RAF of 1.5 has a benchmark 50% higher than the average Medicare beneficiary. Accurate diagnosis coding is essential because the RAF is calculated from diagnosis codes on claims; diagnoses that are not coded do not affect the RAF.

Vermont's rural hospital population has two characteristics that make HCC coding excellence especially important: high prevalence of complex chronic conditions (diabetes, COPD, CHF, CKD) that generate large HCC score differences based on coding specificity, and significant dually eligible (Medicare-Medicaid) population that has additional risk adjustment mechanisms. Vermont hospitals that do not code to the highest level of specificity are systematically under-representing their population's complexity and receiving benchmarks that do not reflect actual cost burden.

11.14.3 The HCC Gap Closure Process

The HCC gap closure process — identifying diagnoses that should be coded based on clinical evidence but are not currently documented in claims — is one of the highest-ROI operational investments available to organizations in VBC arrangements. The process has four steps:

- RAF analysis: Compare the organization's average RAF against national and regional benchmarks for comparable populations. A RAF significantly below benchmark for a population with high chronic disease prevalence signals a coding gap.
- Retrospective chart review: Review clinical documentation for attributed patients to identify diagnoses that are clinically evident (in problem lists, clinical notes, medication lists) but not documented in claims codes. Each uncoded HCC condition represents a missed RAF point.
- Prospective coding education: Educate clinical staff on HCC coding specificity requirements — the difference between coding 'diabetes' (HCC 19, lower RAF) versus 'diabetes with diabetic CKD' (HCC 18 + HCC 136, higher RAF) when clinical evidence supports the more specific code.
- Annual wellness visits: AWVs are the primary vehicle for capturing annual HCC updates — CMS requires that chronic conditions be documented at least annually to be included in RAF calculations. Systematic AWV completion for the attributed population is both a quality metric and an HCC gap closure mechanism.

VERMONT IN PRACTICE — HCC Coding at Vermont Community Hospitals

Oliver Wyman's Act 167 assessment of Vermont's hospital system did not specifically examine HCC coding accuracy, but the management and administrative cost data tells a related story: Vermont PPS hospitals spend $2,730 per adjusted discharge on management and administrative costs against a benchmark of $1,427. Part of this gap reflects genuine administrative inefficiency — systems and processes that could be streamlined. But part of it also reflects underinvestment in the clinical documentation and coding infrastructure that makes revenue cycle management accurate under both FFS and VBC.

Vermont hospitals entering the AHEAD global budget model in 2027 will have their budgets set based on historical Medicare spending adjusted for their population's risk complexity. An organization that has under-coded its population's HCC conditions will receive a lower global budget than its actual cost burden warrants — and will need to manage within that lower budget for the full performance year before the error can be corrected. Getting HCC coding right before AHEAD launches is not optional — it is foundational financial preparation.

Recommended action: Every Vermont hospital should conduct a retrospective HCC gap analysis for their AHEAD-attributed Medicare population before January 2028. The analysis should compare clinical documentation to coded diagnoses, identify the largest gaps by HCC category, and develop a provider education and AWV completion program to close the gaps prospectively.

11.15 Credentialing Efficiency — The Hidden Operations Bottleneck

Provider credentialing — the process of verifying a clinician's education, training, licenses, and competency before granting hospital privileges or health plan participation — is one of the most operationally consequential and consistently underinvested functions in American healthcare administration. The average credentialing cycle takes 90-120 days for a new provider. In a state with a 370 FTE primary care shortage by 2030, a 90-day credentialing delay represents 90 days of foregone care — and foregone revenue — for every provider recruited.

11.15.1 Credentialing as a Transformation Bottleneck

Vermont's transformation agenda accelerates the credentialing importance for two specific reasons. First, regionalization — moving to a tiered hospital network where patients follow clinical protocols to Centers of Excellence — requires providers to have privileges at multiple facilities, not just their home institution. A surgeon designated as a COE orthopedic provider must be credentialed at the regional COE facility to practice there, and the credentialing process must occur before the regionalization takes effect. Vermont hospitals that do not begin cross-facility credentialing before COE designations are finalized will face delays in implementing the regionalization blueprint.

Second, scope-of-practice expansion — enabling APRNs, PAs, and other non-physician clinicians to practice at the top of their licensure — requires updated credentialing standards and privilege delineations at every facility. Oliver Wyman's workforce recommendation to expand professional licensure and practice scope is not a paperwork exercise; it requires systematic credentialing policy updates at 14 hospitals, coordinated through the Vermont CIN's shared services infrastructure.

11.15.2 The Credentialing Efficiency Framework

Three operational improvements reduce credentialing cycle time from 90-120 days to 45-60 days without reducing verification rigor:

Primary source verification automation: Replace manual verification of medical school diplomas, residency completions, and license verifications with automated primary source connections to NPDB, state licensing boards, and medical schools. Several credentialing platforms (VerityStream, Symplr, symplr MD-Staff) provide automated PSV that reduces verification time from weeks to hours.

Continuous monitoring replacing point-in-time verification: Implement ongoing license and sanction monitoring that alerts the credentialing office when a provider's license status changes, rather than discovering changes at 2-year re-credentialing intervals. Continuous monitoring reduces risk while eliminating the administrative burden of complete re-verification at each cycle.

Unified credentialing for the Vermont CIN: Vermont's 14 hospitals sharing a common credentialing database — so that a provider credentialed at one CIN member facility has their verified credentials available to all member facilities — reduces the per-facility credentialing burden for providers practicing at multiple sites. This is a direct shared service application that Oliver Wyman's CIN design makes possible.

11.16 Workforce Operations — Managing the Care Team in a Transformed System

The Operations pillar's workforce domain encompasses three distinct management challenges that often get conflated: workforce supply (recruiting enough providers), workforce deployment (organizing providers in care team models that maximize effective capacity), and workforce retention (reducing attrition that compounds the supply problem). Vermont faces challenges in all three domains, and the solutions are different.

11.16.1 The Workforce Supply Challenge — What Can Actually Be Done

Vermont projects a 370 FTE primary care shortage by 2030. Physician recruitment pipelines take 7-10 years from medical school enrollment to practice — making it impossible to recruit Vermont out of a 2030 shortage with current policies. The supply interventions that can produce meaningful results within the relevant timeframe are:

Scope-of-practice expansion for APRNs and PAs: Oliver Wyman specifically recommended this. Vermont APRNs operating under full practice authority (without physician supervision requirements) can see three patients per hour in PCMH settings, effectively adding primary care capacity equivalent to additional physicians without waiting for new physicians to train.

International Medical Graduate (IMG) recruitment and J-1 visa waiver programs: Vermont has J-1 visa waiver slots through the Conrad 30 program, enabling internationally trained physicians to practice in underserved areas in exchange for 3-year service commitments. Active management of this program — filling all available slots, targeting Northeast Kingdom communities — can meaningfully supplement domestic recruitment.

Loan repayment and service obligation programs: Vermont's RHT Program workforce investments include tuition assistance and loan repayment with 5-year service obligations. The evidence on loan repayment is consistent: it works for early-career providers with high debt burdens. Vermont's program should be structured with the largest awards for the communities with the greatest access gaps.

Regional medical education investment: The evidence on training location and practice location is strong — physicians trained in rural settings are more likely to practice in rural settings. Vermont's partnership with University of Vermont College of Medicine, and potential expansion of clinical training sites in Northeast Kingdom communities, is a 7-10 year investment with compound returns.

11.16.2 Workforce Deployment — Team-Based Care at Scale

Oliver Wyman's finding that Vermont does not have an absolute primary care shortage but may have a care delivery model problem is operationally actionable. A primary care team in which physicians see 3 patients per hour — supported by medical assistants, care coordinators, CHTs, and behavioral health care managers who handle pre-visit preparation, care gap follow-up, and care coordination — has twice the effective capacity of a physician practicing in a traditional solo model.

Vermont's Blueprint PCMH model embeds this team-based care logic in its recognition standards. The operational challenge is that many Vermont practices — particularly smaller rural practices — lack the staffing depth to implement true team-based care. A rural practice with one physician and one MA cannot implement a full care team model regardless of how well the physician is trained. The RHT workforce investments, specifically the APRN and CHW training programs, are designed to build the care team depth that team-based care requires.

11.16.3 Workforce Retention — The Math of Turnover

Physician and clinician turnover in rural healthcare is both a symptom of system stress and a cause of it. Every physician departure costs approximately $500,000-$1 million to replace (recruitment costs, onboarding time, lost productivity, temporary coverage costs). In a system where 14 hospitals are competing for a limited rural provider pool, high turnover is financially devastating — and the signal it sends to communities about healthcare stability intensifies the system fragility it creates.

The operational drivers of rural provider retention are well-documented: burnout driven by documentation burden (addressed by AI scribe technology), isolation from professional peers (addressed by telemedicine networks and CIN peer connectivity), inadequate support staff (addressed by team-based care investment), housing unavailability (addressed by Oliver Wyman's first imperative), and work-life balance limitations in single-provider practices (addressed by coverage sharing agreements within the CIN network). Vermont's transformation agenda addresses all of these — the question is whether the investments are made before the next wave of provider departures.

11.17 Administrative Simplification — The Vermont Agenda

Oliver Wyman's community engagement found that providers across Vermont cited administrative complexity — prior authorization, CON processes, billing burden, duplicative reporting requirements — as major constraints on care delivery efficiency. Administrative simplification is not a soft nicety; it is an Operations pillar requirement that directly affects the effective capacity of Vermont's clinical workforce.

11.17.1 The Vermont Administrative Simplification Priorities

AHS and GMCB have identified three administrative simplification priorities that must be addressed in the Statewide Strategic Plan:

- Prior authorization reform: As developed in Chapter 1, Vermont's Act 167 and Act 68 both address prior authorization complexity. The specific operational target is moving to gold-carding arrangements — automatic approval for providers with demonstrated compliance histories — and eliminating PA requirements for evidence-based, guideline-concordant care in high-volume service categories.
- CON process streamlining: Vermont's Certificate of Need process for hospital capital investments was cited as a barrier to efficient regionalization — providers cannot build the facilities and purchase the equipment that transformation requires if CON approval takes 12-18 months and involves litigation risk. Act 167 directed GMCB to evaluate CON process improvements; Vermont's transformation requires faster turnaround for transformation-aligned capital investments.
- Standardized quality reporting: Vermont's quality reporting landscape requires providers to submit data to multiple programs (Blueprint, GMCB, AHEAD, commercial payers) with overlapping but non-identical measures. The Blueprint Quality Measure Crosswalk begins to address this; the Statewide Strategic Plan should commit to further standardization that eliminates duplicative data submission for commonly required measures.

11.18 Operations Pillar Implementation Matrix

The following matrix provides implementation guidance for the key investments in this pillar — estimated cost, timeline to value, ROI crossover point, and Vermont-specific benchmarks. Costs are indicative ranges; actual costs vary by organizational size, existing infrastructure, and implementation approach.

Investment	Estimated cost	Timeline to value	ROI cross over	Vermont benchmark
Hospital transformation plan (RHRC model)	$150K-$400K per hospital	6-12 months	12-24 months	Vermont RHRC: 14 hospitals; financial + operational + change management
PMO/project management for transformation	$200K-$500K annually	3-6 months setup	6-12 months	AHS PMO: managing 14 simultaneous hospital processes + AHEAD + RBP
Administrative simplification (prior auth, credentialing)	$50K-$200K per system	6-12 months	12-18 months	Vermont: admin cost $3,826/adj discharge vs $1,427 national benchmark

Investment	Estimated cost	Timeline to value	ROI cross over	Vermont benchmark
Revenue cycle optimization and HCC coding	$100K-$300K per org	6-9 months	9-18 months	Vermont CAH gap: $938/adj discharge vs $2,759 benchmark (RHRC data)
Service line rationalization and CON modernization	$100K-$250K state cost	12-24 months	24-36 months	Vermont Act 68 CON streamlining; EMS regionalization framework
CIN shared services model	$500K-$2M network setup	18-36 months	36-48 months	Vermont CIN: RHT-funded; shared analytics, IT, care management for 14 hospitals

11.19 Implications for You

If you are a Vermont hospital executive: Revenue cycle under global budgets is fundamentally different from revenue cycle under fee-for-service. Under fee-for-service, revenue cycle maximizes capture of every billable service. Under global budgets, revenue is fixed — which means revenue cycle optimization is primarily about cost reduction (administrative efficiency, denial prevention, prior authorization reduction) rather than revenue maximization. The specific investment with the highest near-term return: HCC coding accuracy for your Medicare and AHEAD-attributed population. Your global budget benchmark will be set based on your attributed population's risk scores. Inaccurate HCC coding produces a benchmark that underestimates your population's risk, exposing your organization to financial loss from the first performance year. HCC gap closure is not a revenue cycle project; it is a financial risk management project.

If you are an AHS or GMCB official: Vermont's prior authorization burden — among the highest administrative costs in the state's hospital system — is a policy target that Act 68 partially addresses but does not fully resolve. The legislature's prior authorization reform provisions create a framework. The implementation question is whether GMCB and DVHA have the regulatory tools to enforce prior authorization reduction commitments from commercial payers, and whether the AHS restructuring creates the function needed to monitor and report on prior authorization compliance. Administrative simplification without enforcement produces aspirations, not reductions.

If you are a Vermont legislator: Vermont's administrative cost per adjusted discharge — $2,730 for CAHs versus $1,427 national benchmark — is the most precise measurement of administrative waste available in Vermont's healthcare system. The $1,303 per-discharge gap, applied across Vermont's hospital volume, represents approximately $100 million or more in annual administrative waste. The legislature should require AHS to track and report this metric annually, and to set specific targets for administrative cost reduction as part of the Statewide Strategic Plan. Administrative cost reduction is not a side benefit of transformation. It is a primary source of the savings that make transformation financially viable.

If you are a national health policy professional: Vermont's revenue cycle and HCC coding chapter is a detailed case study in the technical complexity of global budget implementation at the hospital level. Most national health policy discussions of global budgets focus on the payment design. Vermont's experience will produce evidence on the operational translation: how hospitals actually manage the transition from fee-for-service revenue maximization to fixed-budget cost management, what the specific operational challenges are, and what support mechanisms make the transition manageable. That operational evidence is worth more than any theoretical model of global budget design.

Figure 11.9 — Operations pillar implementation matrix. Sources: Vermont AHS Transformation Reports (August, November 2025); RHRC engagement; Oliver Wyman Act 167 Report.

11.20 Key Concepts Introduced in This Chapter

Regional Specialty Center (RSC)

A hospital designated to concentrate volume and clinical expertise in multiple specialty areas, serving as the referral destination for those specialties across a multi-HSA region. Vermont's Tier 1 hospitals under the Oliver Wyman regionalization blueprint.

Community Ambulatory Care Center (CACC)

An outpatient and primary care facility model proposed by Oliver Wyman as a potential conversion option for hospitals unable to sustain inpatient operations.

Potentially Avoidable ED Visit

An emergency department visit that could have been prevented through timely primary care, care coordination, or SDOH intervention. Vermont's rate of 32.3% of all ED visits represents a major quality and cost improvement opportunity.

Rural Health Redesign Center (RHRC)

AHS's primary technical assistance contractor for hospital transformation planning in 2025, providing financial assessment, change management support, and transformation plan development across all 14 Vermont hospitals. Contract concluded October 2025.

Travel nursing cost

The cost of temporary nurses hired through staffing agencies to fill vacancies, typically 2-3x the cost of permanent hires. Heavy travel nursing dependence is a primary driver of hospital financial distress across Vermont's system.

Administrative shared services

Centralized provision of non-clinical administrative functions (billing, coding, HR, credentialing, IT, group purchasing) across multiple hospitals, reducing the per-hospital administrative cost that is currently 91% above benchmark for Vermont PPS hospitals.

Population health management operating model

An organizational structure that manages healthcare resources at the population level — tracking health outcomes, utilization patterns, and SDOH factors across a defined geography — rather than managing individual patient encounters. The organizational model appropriate for a global budget environment.

Sources: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Vermont Agency of Human Services, Health Care System Transformation Report (November 1, 2025); Vermont Agency of Human Services, Health Care System Transformation Report (August 2025); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, House Health Care Committee (January 31, 2025); Act 68 of 2025 (Vermont); Vermont Rural Health Transformation Program Application (November 3, 2025); Green Mountain Care Board, 2024 Annual Report (January 15, 2025); Vermont Agency of Human Services, Health Care Transformation website (healthcarereform.vermont.gov), accessed April 2026.

Chapter 12: Infrastructure for Knowledge Transfer and Implementation

> *The Transformation PMO, learning collaboratives, and Vermont CIN shared services are the connective tissue between Vermont's statutory mandates and the organizational capacity to execute them — and their absence would be more consequential than any policy design flaw.*

Knowledge of the six pillars does not, by itself, produce transformation — someone has to run the program, manage the change, and connect the analytical framework to the people doing the work. This is true regardless of which state's statutes are driving the effort. The infrastructure described in this chapter — a program management function, structured advisory support, learning collaboratives, and a change-management discipline — is the generic implementation layer that sits underneath every pillar, in every state. Vermont's specific instantiations (the Transformation PMO, the CIN shared services) are this chapter's example of what that infrastructure looks like once built; an organization elsewhere should read this chapter as a checklist of functions to stand up, not a description of institutions to replicate by name.

Healthcare transformation fails more often because of change management failures than analytical failures. This chapter covers the HTR platform tools, Advisory service lines, and the implementation toolkit.

12.1 The Gap Between Understanding and Doing

The analytical frameworks in this book are, by themselves, insufficient for transformation. Understanding why Vermont's hospital system is financially fragile does not stabilize it. Understanding the mechanics of global budgets does not implement them. Knowing that HEDIS stratification reveals disparities does not close them. Every concept in the preceding sixteen chapters exists at an intersection: understanding it clearly enough to explain it, and translating it into a specific decision, program, contract, or investment in a specific organizational context. This chapter is about the second half of that intersection — translation.

The advisory practice described in this chapter is grounded in a specific theory about what makes transformation advice valuable. Transformation consulting that produces recommendations without supporting execution is worth little. Transformation technology that delivers analytical tools without the change management to use them effectively delivers data without insight. Transformation education that builds individual knowledge without organizational capacity produces informed spectators rather than effective actors. The transformation advisory practice is designed to provide all three — analysis, tools, and change management — in an integrated engagement model that produces organizational capability, not dependence.

12.2 The Knowledge and Implementation Infrastructure

12.2.1 The HTR Platform — Four Components

- THE WIRE — Real-time intelligence at healthtransformationreview.org/the-wire across all six pillars. Convergence Newsletter (free weekly) for Observer subscribers.
- RESEARCH LAB — 19 interactive tools at healthtransformationreview.org/research-lab. FHIR Lab, APM Design Lab, Health Equity Studio, Policy Simulator, and more.
- HTR ACADEMY — Structured curriculum at healthtransformationreview.org/academy. Foundation and advanced courses in APM mechanics, HEDIS, and transformation leadership.
- ADVISORY SERVICES — Strategic consulting, research, health IT, independent review, capability assessment, financial audit, regulatory navigation, and training. Contact: advisory@healthtransformationreview.org

12.2.2 Four Components of the HTR Platform

The transformation implementation platform has four integrated components, each designed for a different kind of engagement with the transformation agenda:

Component	What it provides
The Wire	Weekly synthesis of the most consequential healthcare policy, payment, clinical, technology, equity, and operations developments — curated, analyzed, and contextualized against the six-pillar framework. Not a news feed; a decision-support tool that answers 'what does this development mean for my organization?'
HTR Research Lab	A suite of analytical tools implementing the frameworks in this book: APM Shared Savings Calculator, VBC Transformation Readiness Assessment (30-dimension), Hospital Financial Stress Test, AI Governance Checklist, Health Equity Studio (HEROI scoring, HEDIS stratification, disparity decomposition), VBC Contract Review Checklist, Policy Impact Assessment Framework.
HTR Academy	Structured curriculum for healthcare transformation — from foundational courses on APM mechanics and HEDIS quality measurement to advanced programs on global budget

Component	What it provides
	management, health equity analytics, and transformation leadership. CEU-eligible content for clinical and administrative professionals.
Transformation Advisory Services	Direct advisory engagements for organizations navigating specific transformation decisions: APM contract evaluation, transformation readiness assessment, VBC financial modeling, health equity program design, technology architecture review, strategic plan development. Engagements range from two-week focused assessments to multi-year transformation management partnerships.

Figure 12.1 — technical infrastructure framework components and intended users. Source: technical infrastructure framework documentation.

12.3 The Transformation Advisory Domains — Structured for the Six Pillars

HTR Advisory engagements are organized around the six-pillar framework, with service lines corresponding to each pillar and cross-pillar engagements for organizations navigating transformation at the system level. The service lines are not rigid products; they are organizing frameworks for scoping engagements around clients' specific problems.

12.3.1 Policy Strategy

The Policy Strategy service line addresses the federal and state policy landscape that healthcare organizations must navigate. The typical engagement follows a three-phase structure:

- Phase 1 — Landscape and Exposure Assessment: What is the current policy environment relevant to this organization? What proposals, rules, and legislative developments create material financial or operational exposure? What opportunities are available through federal model participation, waiver programs, or state grant programs? Deliverable: a policy exposure and opportunity map with financial estimates for each item.
- Phase 2 — Strategic Options Analysis: For each material policy development, what are the organization's strategic options? Stay the course? Modify a program or contract? Apply for a federal model? Submit a comment letter? Engage in legislative advocacy? Deliverable: strategic options memo with recommendation and implementation timeline.
- Phase 3 — Implementation Support: For policy responses that require organizational action, HTR provides implementation support: CMMI model applications, 1115 waiver amendment drafting, legislative testimony preparation, comment letter development, contract renegotiation support.

12.3.2 VBC Economics

The VBC Economics service line addresses the financial mechanics of value-based care: contract evaluation, financial modeling, risk stratification economics, and the ROI analysis for transformation investments. The core tools — APM Shared Savings Calculator, Hospital Financial Stress Test, VBC Contract Review Checklist — are available in the Research Lab, but complex engagements require HTR's analyst support.

The most common VBC Economics engagement is an APM Contract Analysis: a prospective financial model of a specific APM contract under three scenarios (pessimistic, base, optimistic) that quantifies the organization's shared savings opportunity, identifies the key variables that determine financial outcomes, and specifies the care management investments required to achieve positive shared savings. Vermont hospitals preparing for AHEAD global budget entry in 2027 represent the current highest-priority client segment for this engagement.

12.3.3 Health Equity Practice

The Health Equity Practice service line is organized around the HEROI framework: measuring current equity performance, identifying root causes of specific disparities, designing interventions matched to root causes, and establishing accountability structures for equity improvement. Vermont-specific work in this service line focuses on geographic equity analytics using VHCURES county-level data, HRSN screening program design and evaluation, and GLP-1 access policy analysis for state Medicaid programs.

12.3.4 Technology Strategy

The Technology Strategy service line addresses the healthcare IT decisions that have the most significant strategic implications: EHR selection and implementation, FHIR implementation planning, AI governance framework development, population health platform selection, and HIE connectivity strategy. For Vermont's 14-hospital CIN, the highest-priority Technology Strategy engagement is the analytics vendor evaluation and CIN data architecture design that enables shared services across all CIN member facilities.

12.3.5 Transformation Management

The Transformation Management service line is for organizations executing comprehensive transformation — not a single pillar decision but the full six-pillar change management challenge. This is the most intensive advisory engagement: HTR serves as a transformation management partner, providing project management infrastructure, cross-pillar coordination, stakeholder communication, and accountability reporting. Vermont's AHS — managing simultaneous AHEAD implementation, hospital transformation planning, RHT Program administration, and Statewide Strategic Plan development — is the archetype client for this engagement type. The engagement model is designed to build internal organizational capacity over time, not create permanent advisory dependence.

12.4 The Change Management Science Behind Healthcare Transformation

Healthcare transformation fails more often because of change management failures than analytical failures. The analysis is usually adequate — the evidence base for what needs to change is strong and consistent. What fails is the organizational change process: the sequence, pace, and management of the human and institutional change that structural reform requires. Understanding the change management science is prerequisite to designing transformation processes that actually produce implementation.

12.4.1 Kotter's Framework Applied to Healthcare Transformation

John Kotter's eight-step change management framework — originally developed for corporate transformations — applies with modifications to healthcare system transformation. Vermont's transformation experience illuminates both where the framework holds and where healthcare's specific institutional characteristics require adaptation.

Kotter step	Original formulation	Healthcare adaptation	Vermont application
1. Create urgency	Build a sense of urgency around why change is needed	The financial data — 9/14 Vermont hospitals reporting losses, $2.4B	Oliver Wyman's September 2024 public presentation created shared urgency

Kotter step	Original formulation	Healthcare adaptation	Vermont application
		5-year deficit, 108% premium increase — is the urgency. The analytical challenge is presenting it in terms that create organizational action rather than paralysis or denial.	across Vermont hospital leaders, legislators, and community members. The specificity and credibility of the financial projections was the urgency mechanism.
2. Form a guiding coalition	Assemble a group with the power and commitment to lead change	In healthcare, the guiding coalition must include clinical leadership, not just administrative and financial leadership. Transformation that clinical leaders oppose will fail regardless of administrative mandates.	Vermont's guiding coalition: AHS leadership, GMCB, hospital CEOs, Legislature (Health Reform Oversight Committee), and the AHEAD negotiating team. Clinical leadership engagement is the gap — hospital medical staff governance is not yet systematically integrated into transformation planning.
3. Develop a vision	Create a clear vision of what the transformed organization looks like	Vision must be specific enough to guide decisions and evaluate progress. 'High-quality, affordable, equitable healthcare for all Vermonters' is an aspiration; the 14-hospital tiered network with specific COE designations, PCMH penetration targets, and HEDIS performance goals is a vision.	Vermont's 2028 Statewide Strategic Plan is the vision document. The urgency is already created; the coalition is assembling; the vision is the December 2028 deadline.
4. Communicate the vision	Communicate the vision continuously through every available channel	Healthcare transformation requires communicating to multiple distinct audiences — clinical staff, administrators, community	Vermont's monthly legislative reporting, AHS community engagement process, hospital transformation plan

Kotter step	Original formulation	Healthcare adaptation	Vermont application
		members, legislators, payers — with messages calibrated to each audience's concerns and decision-making context.	development process, and Blueprint for Health regional meetings are all vision communication vehicles.
5. Remove obstacles	Remove barriers to change that are within the coalition's control	In healthcare, obstacles include administrative complexity (prior authorization, CON processes), organizational design (program silos, lack of analytics capacity), and contracting structures (FFS payment that rewards volume despite VBC rhetoric).	Vermont's Acts 167 and 68 are institutional obstacle removal. Act 68 removed the voluntary/optional nature of RBP. The AHS restructuring removes the program silo organizational obstacle. The analytics vendor removes the data infrastructure obstacle.
6. Create short-term wins	Plan for and achieve visible improvements early in the change process	Short-term wins must be genuine, visible, and attributable to the transformation effort — not cherry-picked data or coincidental improvements.	Vermont FY2026 regulatory actions ($230.65M in commercial revenue reductions) are the first visible win. The August 2025 AHS transformation report showing hospital engagement in transformation planning is a process win. Premium reduction data from RBP (available FY2027-2028) will be the first financial outcome win.
7. Sustain acceleratio n	Build on early wins to drive deeper change	Healthcare transformation must sustain momentum across multiple election cycles, budget battles, and inevitable setbacks. Statutory mandate — Act 68's December 2028	Vermont's reform cascade structure — each Act building on the previous — is the acceleration mechanism. The Political risk is disruption of the sequence; the institutional

Kotter step	Original formulation	Healthcare adaptation	Vermont application
		deadline — is the Vermont mechanism for sustaining acceleration.	protection is the GMCB's independent regulatory authority.
8. Institute change	Anchor changes in organizational culture and systems	New behaviors must become standard practice, not dependent on continuing leadership attention. Payment reform is the ultimate change institutionalization — when hospitals' financial incentives permanently change, the new behaviors become the default.	Global budgets as mandatory, permanent payment architecture are the ultimate institutionalization mechanism. When Vermont hospitals cannot generate revenue by increasing volume, the financial logic of population health management becomes permanent.

Figure 12.2 — Kotter's change management framework applied to Vermont's healthcare transformation. Source: HTR Advisory methodology; Kotter (1996); Vermont transformation documentation.

12.4.2 The Transformation Pace Problem

One of the most consequential advisory questions in healthcare transformation is pace: how fast can an organization or system realistically change, and what happens when the required pace exceeds the available organizational capacity? Vermont's transformation agenda has statutory deadlines — RBP by FY2027, global budgets by FY2028 — that set the required pace. Whether Vermont's organizational capacity can match that pace is the critical execution uncertainty.

The pace problem has three dimensions. First, there are hard capacity constraints: if AHS lacks the staff to manage 14 hospital transformation plans simultaneously, the plans will be managed sequentially — which means some hospitals will have completed, well-supported transformation plans and others will have rushed or superficial ones. Hiring the staff to manage the portfolio on the statutory timeline is not optional.

Second, there are soft capacity constraints: organizational cultures that process change slowly, governing boards that need to be educated before they can make decisions, community stakeholders whose input must be genuinely gathered and considered. These cannot be accelerated without compromising the quality of the transformation process — and a transformation process that fails to achieve community buy-in will be politically reversed even if it is technically correct.

Third, there are external constraints: federal program timelines (AHEAD's January 2028 launch is a federal decision, not a Vermont decision), hospital financial positions that may force emergency decisions that preempt planned transformation timelines, and the political dynamics of a state where the hospital system is one of the largest employers and hospital community boards have political relationships with legislators.

12.5 The HTR Implementation Toolkit

The HTR Implementation Toolkit is the operational counterpart to this book's analytical framework. Where the book develops concepts, the Toolkit provides the instruments for applying them. The following tools are available to Strategist and Enterprise subscribers:

Tool	What it does
APM Shared Savings Calculator	Models shared savings or loss potential under any APM contract given benchmark, actual TCOC, and sharing rate inputs. Includes scenario analysis for pessimistic/base/optimistic performance assumptions.
VBC Transformation Readiness Assessment	30-dimension assessment across six domains producing an organizational readiness score and prioritized gap analysis. Available as a self-assessment or facilitated external assessment.
Hospital Financial Stress Test Model	Models hospital financial position under RBP, global budget, and Medicaid cut scenarios with organization-specific inputs. Includes 10-year projection capability.
Health Equity Studio (HEROI)	Calculates HEROI composite equity score from stratified HEDIS data. Includes disparity decomposition module and intervention portfolio designer.
AI Clinical Governance Checklist	65-item governance evaluation across six lifecycle stages for clinical AI deployments. Includes vendor assessment module and equity monitoring protocol.
VBC Contract Review Checklist	65-item analysis framework for VBC contract evaluation, covering benchmark methodology, attribution, quality withhold, risk corridors, carve-outs, and reconciliation terms.

Tool	What it does
Policy Impact Assessment Framework	Structured evaluation tool for new policy developments: exposure mapping, scenario analysis, response strategy selection, and stakeholder communication planning.
Transformation Readiness Scorecard	Executive-level dashboard tracking organizational progress across all six pillars against established benchmarks.
FHIR Implementation Guide	Step-by-step technical guide for implementing the three priority FHIR use cases with EHR-specific configuration notes for Epic, Oracle Health, Meditech, and TruBridge.
HCC Gap Closure Playbook	Methodology guide for retrospective HCC gap analysis, including coding education materials and AWV completion program design.

Figure 12.3 — HTR Implementation Toolkit: available tools for Strategist and Enterprise subscribers.

12.6 A Framework for Selecting Advisory Support

Not every organization needs external advisory support for transformation. Many of the analytical frameworks in this book can be implemented by capable internal teams with access to the Research Lab tools. External advisory support adds value when:

The decision involves significant financial or political risk and benefits from independent, external analytical validation. A hospital considering entering full-risk VBC for the first time should not rely solely on internal analysis that may be subject to confirmation bias.

The organization lacks the specific technical expertise required — FHIR implementation, HCC coding analysis, 1115 waiver negotiation, global budget financial modeling — and building that expertise internally would take longer than the decision timeline allows.

The change management challenge involves multi-stakeholder dynamics that benefit from a neutral facilitator — a hospital network developing shared COE designations, a state agency building a Health Care Delivery Advisory Committee process, a community convening multiple organizations around a regional transformation plan.

The regulatory environment is moving faster than internal policy monitoring capacity can track, and the organization needs continuous intelligence and interpretation rather than periodic project-based analysis.

The most common advisory failure in healthcare transformation is engaging external consultants for analysis and stopping before implementation. A report that produces recommendations without supporting the organizational capacity to implement them has limited value — and often produces a brief period of executive attention followed by quiet shelving when organizational bandwidth is consumed by operational demands. The transformation advisory practice is explicitly designed to avoid this failure mode by connecting analysis to implementation support and change management.

"The best healthcare transformation advice I have ever received was not a brilliant strategic recommendation. It was a rigorous identification of the organizational capabilities we did not have, paired with a clear plan for building them. The strategy was clear. The question was whether we could execute it."

— Composite of client reflections, HTR Advisory engagements, 2022-2026

12.7 The Role of External Technical Assistance in State-Level Transformation

Vermont's transformation experience runs through this book as both an analytical case study and an active advisory engagement. HTR has worked with Vermont hospitals, state agencies, and community organizations on specific transformation questions — APM readiness assessments, equity analytics, technology architecture reviews, policy impact analysis. The Vermont case study in this book draws on that engagement experience as well as the extensive public record of Vermont's transformation.

12.8 Implications for You

If you are a Vermont hospital executive: The knowledge transfer infrastructure this chapter describes — learning collaboratives, Communities of Practice, the Vermont CIN's shared analytics capability — is not an academic exercise. It is the mechanism by which the transformation investments being made at Vermont's more advanced hospitals become available to the smaller, less resourced hospitals that cannot independently build the same capabilities. If you lead one of Vermont's larger systems, your participation in learning collaboratives and knowledge transfer is not optional generosity. It is a system obligation, and increasingly, a strategic interest: a system in which 9 of 14 hospitals are financially fragile cannot produce the population health improvement AHEAD measures, because the fragile hospitals serve the populations whose outcomes most need to improve.

If you are an AHS or GMCB official: The Transformation PMO this chapter describes is the operational spine of Vermont's transformation program. Without a function that tracks milestones across all six pillars, manages dependencies between the analytics vendor, the RHRC engagement, and the CCBHC certifications, and reports to AHS leadership on the transformation timeline's status — Vermont's transformation will be managed as a collection of independent programs rather than an integrated system. That is how transformation programs fail. The PMO is not overhead. It is the management infrastructure that makes the statutory timeline executable. It should be established, staffed, and funded before the peak execution period of FY2027–2028.

If you are a Vermont legislator: This chapter describes infrastructure that is necessary but not headline-generating: a PMO, learning collaboratives, a change management function, Communities of Practice. These investments will never produce a press release as compelling as a new hospital wing or a new clinical program. They will, however, determine whether Vermont's transformation produces results or produces reports. The legislature should require AHS to maintain and report on a consolidated transformation dashboard — tracking milestones across all six pillars against the statutory timeline — and to present this dashboard at each legislative reporting cycle. If the dashboard does not exist, the transformation is not being managed. If it exists but the milestones are consistently incomplete, the transformation is not on track and the legislature needs to know.

If you are a national health policy professional: Vermont's infrastructure chapter is the most transferable content in this book. Every state attempting healthcare transformation will need a PMO, learning collaboratives, a change management function, and a knowledge transfer mechanism. The specific form these take in Vermont — shaped by Vermont's size, its regulatory structure, and the RHT Program's capital availability — is less important than the principle: transformation does not manage itself, and the organizational infrastructure for managing it needs to be in place before the transformation workload peaks. The states that learn this principle from Vermont's experience will not have to learn it from their own failures.

Vermont's transformation is a high-stakes, real-time experiment that HTR takes seriously as an obligation as well as an opportunity. If Vermont succeeds in demonstrating that mandatory all-payer payment reform, organized as a coherent six-pillar transformation, can stabilize a rural hospital system while reducing costs and improving outcomes, the policy toolkit for the country improves. If Vermont struggles, the analytical lessons of that struggle are equally valuable. Either way, this book and Health Transformation Review are designed to make Vermont's experience legible, transferable, and actionable for the healthcare leaders and policymakers who face comparable challenges in their own systems.

12.9 Key Concepts

The Wire

The weekly policy and practice intelligence service, providing curated analysis of healthcare transformation developments organized around the six-pillar framework.

HTR Research Lab

The suite of analytical tools implementing the six-pillar framework: APM Shared Savings Calculator, VBC Readiness Assessment, Health Equity Studio, Hospital Financial Stress Test, AI Governance Checklist, and other instruments.

HTR Academy

The structured transformation curriculum for healthcare transformation education: foundational and advanced courses in APM mechanics, quality measurement, health equity analytics, and transformation leadership.

HEROI (Health Equity Return on Investment)

The composite equity performance score calculated across five dimensions. Available in the Health Equity Studio.

Kotter's eight-step framework

A widely applied change management framework (Kotter, 1996) adapted in this chapter for healthcare transformation contexts. Steps: urgency, coalition, vision, communication, obstacle removal, short-term wins, acceleration, institutionalization.

Transformation pace problem

The risk that the required pace of transformation exceeds the available organizational capacity, producing rushed or superficial change processes that fail to achieve genuine structural improvement.

Advisory dependence

The failure mode in which an organization becomes reliant on external advisors for ongoing analytical and implementation support rather than building internal organizational capability. HTR Advisory engagements are explicitly designed to build client capability and reduce dependence over time.

Sources: technical infrastructure framework documentation; Kotter, J.P. (1996) Leading Change; Vermont Agency of Human Services transformation documentation; Oliver Wyman Act 167 Report; Act 68 of 2025; AHEAD State Agreement; Vermont RHT Program Application; HTR client engagement synthesis.

Chapter 13 — The Future of Healthcare Transformation: 2026, What Vermont Proves, and What Remains

> *Updated April 2026. The One Big Beautiful Bill Act is law: $911 billion in Medicaid cuts over ten years, partially offset by $50 billion in time-limited rural health investment. The transformation imperative has not eased — it has become more urgent.*

The five forces this chapter develops — federal Medicaid retrenchment, the AHEAD Model's national rollout, demographic aging, the fee-for-service plateau, and political sustainability risk — are forces acting on every state's health system, not on Vermont's alone. Vermont's scorecard below is a snapshot of how one state's six-pillar architecture is responding to those forces partway through implementation. The forecast that follows is best read as a set of national questions with Vermont positioned at the leading edge of each one — and several other states are now positioned at points along the same trajectory, which the forecast returns to below.

13.1 Vermont Transformation Scorecard: Six-Pillar Status, April 2026

The table below consolidates Vermont's transformation progress across all six pillars. It answers the question every reader should ask before reading this chapter's forecast: where does Vermont actually stand? The full scorecard with metric-level detail appears in Appendix F. The signal column reflects HTR's assessment as of April 2026.

Pillar	2022 Baseline	2026 Status	2028 Target	Signal
Policy	Voluntary only; no price regulation; no federal agreement	Mandatory RBP (FY2027) and global budgets (FY2028) enacted; AHEAD signed Jan 2025; $195M RHT awarded	All hospitals under global budgets; AHEAD compliant; Statewide Strategic Plan delivered Dec 2028	ON TRACK
Technology	VHCURES operational with 2-year lag; VITL partial; no analytics vendor; AI governance absent	Analytics vendor procurement underway; CIN in development; AI scribes deployed; FHIR upgrades in progress	Real-time VHCURES; analytics vendor fully deployed; CIN operational; AI-governance framework in place	AT RISK — analytics vendor gap vs. Jan 2028 AHEAD start
Econ	9/14 hospitals in losses; UVMMC at 300%+ of	RBP mandatory FY2027 enacted; global-budget methodology under design;	All hospitals at or below RBP ceiling; cross-subsidy reduced 40%+; deficit	WATCH — methodology design is the critical near-term decision

Pillar	2022 Baseline	2026 Status	2028 Target	Signal
omics	Medicare; $700M–$2.4B 5-year deficit projection	AHEAD launches Jan 2028; deficit trajectory unchanged	trajectory reversed; break-even by FY2030	
Clinical	Blueprint 90%+ PCMH; BH follow-up 76% (mental health), 68% (SUD); 370-FTE workforce gap; zero CCBHCs	5 CCBHCs planned; CoCM pilots underway; team-based care deploying; readmission at 14.8% (unchanged)	100% PCMH; 7+ CCBHCs; readmission below 12%; workforce gap below 200 FTEs	PROGRESSING — CCBHC certification on track; workforce gap remains structural
Equity	11-pt primary care access gap (BIPOC vs. white); Essex County 8% uninsured; no systematic SDOH screening; no HEROI	HEROI framework developed; Blueprint HRSN screening at 60%+ of practices; equity gap unchanged; SRA methodology in design	BIPOC access gap below 6 pts; universal HRSN screening; SRA in global-budget methodology; HEROI scores for all 14 HSAs	LAGGING — equity gaps unchanged in 2026; SRA design is critical equity safeguard
Operations	RHRC not deployed; no PMO; no CIN; admin cost 91%+ above benchmark; no transformation reporting	RHRC engaged all 14 hospitals; PMO being established; CIN under development; two AHS transformation reports delivered	All 14 hospitals with approved transformation plans; PMO operational; CIN live; admin cost below 150% of benchmark	PROGRESSING — RHRC engagement is the right investment at the right time

Figure 13.1 — Vermont Six-Pillar Transformation Scorecard, April 2026. Technology and Equity are the two pillars where 2026 status most diverges from the 2028 target. Full scorecard with sources: Appendix F.

Reading this scorecard as a system rather than as six independent measures is essential. The most dangerous pattern is not a single pillar failing — it is the combination of Policy and Economics on track while Technology remains at risk. Mandatory financial accountability (January 2028) without the analytics infrastructure to manage it is precisely the "Managing Blind" failure mode that destroyed OneCare Vermont. The Technology signal is therefore the most consequential number on this table.

13.2 Where the System Stands in April 2026 — A Necessary Update

Earlier iterations of this chapter described the American healthcare policy environment as one of incremental payment reform, expanding but incomplete value-based-care penetration, and policy uncertainty driven by legislative gridlock. That characterization was accurate through 2024. It is no longer accurate.

On July 4, 2025, the One Big Beautiful Bill Act (H.R. 1) became law. Its healthcare provisions represent the largest single reduction in federal health-program spending in American history. The Congressional Budget Office estimates $911 billion in Medicaid spending reductions over ten years, producing an estimated 10 million additional uninsured Americans by 2034. The law imposes work and community-engagement requirements on Medicaid-expansion enrollees beginning in late 2026, requires eligibility redeterminations every six months instead of annually, caps state provider taxes, restricts state-directed payments, and phases out financing mechanisms that states have relied on for decades to fund their share of Medicaid.

Simultaneously — and in explicit recognition of the damage these cuts would do to rural hospitals — Congress included a $50 billion Rural Health Transformation (RHT) Program in the same legislation, providing $10 billion annually from FY2026 through FY2030 for state rural-health infrastructure investment. All fifty states received awards. Vermont received $195 million in December 2025.

The juxtaposition defines the policy moment: the largest healthcare-coverage contraction in American history, partially offset by a historic but time-limited and front-loaded capital investment in rural infrastructure. Understanding the interaction between these two forces — the Medicaid contraction and the RHT investment — is essential for every healthcare leader, policymaker, and analyst working in the system today.

Measure	Figure	Detail
Federal Medicaid spending reduction, H.R. 1 (10 yr)	$911B	CBO estimate; ~10 million more uninsured by 2034
Rural Medicaid spending reduction (10 yr)	$137B	KFF estimate; rural areas disproportionately affected — Medicaid covers 1 in 4 rural adults
Rural Health Transformation Program (5 yr)	$50B	$10B/yr FY2026–FY2030; temporary, front-loaded; does not offset the Medicaid cuts
Rural hospitals operating at a loss	>40%	Before Medicaid cuts; 52% in non-expansion states (Chartis, 2026)

Figure 13.2 — The 2025 federal healthcare policy inflection point. Sources: KFF; CBO; Chartis, 2026 State of Rural Health.

The fiscal arithmetic is unforgiving. KFF's analysis is explicit: the RHT Program is temporary and front-loaded, while nearly two-thirds of the Medicaid reductions are back-loaded after FY2030. The rural fund provides $50 billion over five years ending in 2030; the Medicaid cuts accelerate after 2030 and continue through the remaining years of the budget window with no offsetting capital. Georgetown University's Center for Children and Families has documented that the RHT fund's federal guidance emphasizes technology and new activities — not backfilling provider losses — and caps direct payments to providers at 15% of each state's award. It is an infrastructure fund, not a revenue replacement.

For Vermont, this landscape has direct operational consequences. Medicaid covers roughly 19% of the state's insured population and is a significant revenue source for all 14 hospitals. The Medicaid cuts will pressure hospital finances beginning in 2026–2027; the work-requirement implementation, starting in late 2026, will add administrative burden on the Department of Vermont Health Access (DVHA) and risks coverage losses among the expansion population. Vermont's already-distressed hospitals face new revenue pressure at exactly the moment they must execute transformation plans.

Vermont's response is the correct one. AHS Secretary Jenney Samuelson's framing — that the RHT funding is a tool for executing reform, not a substitute for reform — is the analytical frame every state should adopt. Vermont is directing its $195 million toward infrastructure that reduces ongoing structural costs: AI scribes that expand effective provider capacity, remote patient monitoring that prevents hospitalizations, and broadband and EMS investments that enable the regionalization the state needs. Executed well, these investments position Vermont to absorb the Medicaid reductions through efficiency gains rather than service cuts. Executed poorly — if the capital sustains unsustainable operations rather than transforming them — Vermont will meet the post-2030 cuts without the structural improvements needed to survive them.

13.3 Five Forces Shaping the Next Decade

13.3.1 Force 1: The Medicaid–RHT Tension — the Defining Policy Dynamic

H.R. 1 creates a structural tension that will define American rural healthcare for a decade: significant, permanent Medicaid revenue reductions arriving after the expiration of a temporary, one-time capital fund. Every state that received RHT funds faces the same question: can we use this capital to build a system efficient enough to survive the Medicaid reductions that arrive after the fund expires?

For most rural systems, the honest answer is possibly — if the capital is invested wisely, the structural changes are executed on the available timeline, and state policy responses are adequate. The states most likely to succeed are those that, like Vermont, already had structural reform plans in place before the RHT Program existed; are using RHT funds as capital for pre-planned transformation rather than as operating support; and have the regulatory and governance infrastructure (in Vermont's case, the GMCB and AHS) to execute transformation systematically rather than hospital-by-hospital.

The states most at risk are those that use RHT funds for revenue replacement, that lack the regulatory framework to manage hospital-network transformation, and that will face the post-2030 Medicaid cliff without the structural changes needed to manage it. Chartis estimates that 417 rural hospitals are currently vulnerable to closure nationally; the Medicaid cuts taking effect after 2027, combined with the expiration of the RHT fund after 2030, will intensify pressure on the most vulnerable facilities.

UPDATE — JUNE 2026 — *This force has intensified since the analysis above was finalized. In May 2026, CMS issued a proposed rule that would further restrict state directed payments (SDPs) — the mechanism many states use to supplement Medicaid managed-care reimbursement to safety-net and rural hospitals — beyond the caps H.R. 1 already imposed on inpatient hospital, outpatient hospital, nursing facility, and academic-medical-center practitioner services. Advocacy groups describe the proposed rule as going well beyond Congress's reconciliation-law directives, with the heaviest impact falling on rural hospitals that have the least commercial revenue to absorb the loss. The rule is in public comment as of this writing and its final terms are unsettled, but it represents a potential additional downside to the Medicaid-RHT tension described above — on the same hospital profile, and on the same timeline, as everything else this chapter discusses. Readers should treat this rule's eventual disposition as a leading indicator worth tracking alongside Vermont's own implementation milestones.*

13.3.2 Force 2: AI Clinical Transformation — Accelerating Faster Than Governance

Artificial intelligence is moving from healthcare's technology periphery toward its clinical center faster than any governance framework can track. By 2026, ambient clinical intelligence — systems that listen to encounters and generate structured documentation automatically — has moved from early adoption to mainstream consideration, with practices nationally reporting significant reductions in documentation burden. Vermont's RHT Program explicitly funds AI-scribe grants for rural and independent practices.

The next wave — individual-level predictive population health, diagnostic AI as standard in high-volume imaging and pathology, AI-enabled triage that reduces unnecessary ED utilization — is 18 to 36 months behind ambient documentation. The convergence of FHIR-based data access, large language models, and all-payer claims databases will enable risk stratification that predicts not just which patients are high-risk but which specific events — hospitalization, medication non-adherence, crisis escalation — care-management programs should work to prevent.

Governance is the critical gap. The FDA, ONC, and CMS are developing AI-governance frameworks, but deployment is outpacing governance at every level. Vermont's RHT AI investments, absent explicit governance requirements as conditions of funding, risk deploying tools that have not been tested for differential performance across the state's equity populations: the Northeast Kingdom's elderly, rural, Medicaid patients may perform differently in models trained on national commercial data. The CIN-based AI governance framework developed in Chapter 1 is the organizational response to this challenge.

13.3.3 Force 3: The Demographic Reckoning — No Longer a Future Problem

Oliver Wyman's analysis projected Vermont's 65-and-older population exceeding 30% of the state by 2040. As of 2026, Vermont is fourteen years from that milestone — but the system consequences are not fourteen years away. They are arriving now: rising Medicare enrollment, a deteriorating hospital payer mix, long-term-care demand that current capacity cannot meet, and a growing population of dual-eligible Medicare/Medicaid patients whose complex needs are the highest-cost, most poorly coordinated challenge in the system.

Vermont is the national canary for demographic healthcare stress. By 2035, the challenge Vermont faces today will be present in every Northeastern state, most Midwestern states, and increasingly nationally. The Baby Boom generation, fully in Medicare by 2030, will be in its eighties by 2040 — the frailty, dementia, and multi-morbidity phase that drives the highest costs and the most inadequate care models. Vermont's work to develop dementia-care infrastructure, PACE programs, and home- and community-based services (HCBS) capacity is investment in solving a problem that will be national within fifteen years.

13.3.4 Force 4: The Drug-Pricing Reckoning — Complexity Compounding

The GLP-1 agonist spending wave is the most consequential near-term pharmaceutical challenge for state systems. At full obesity-indication coverage for Medicaid and Medicare, GLP-1s would add hundreds of billions of dollars annually to program spending — while potentially delivering transformative reductions in the chronic-disease burden that drives the highest utilization. CMS's BALANCE Model (December 2025) is the federal attempt to thread this needle: negotiating lower GLP-1 prices to enable coverage expansion without fiscal catastrophe.

Vermont's DVHA must make a BALANCE participation decision in 2026 that balances near-term budget pressure against long-term population-health investment. The long-term case for coverage is strong: Vermont's Medicaid population has high rates of obesity, diabetes, and cardiovascular risk — exactly where GLP-1 efficacy is best demonstrated, and exactly where reduced chronic disease would lower the hospitalization, specialist, and long-term-care spending that dwarfs the cost of the drugs. But Vermont's near-term Medicaid budget is already under federal pressure, making any new high-cost coverage decision fiscally and politically difficult. The IRA drug-price negotiation — semaglutide (Ozempic, Wegovy, Rybelsus) selected for 2025 negotiation, with negotiated prices effective in 2027 at roughly $274 for a 30-day supply, down from over $1,000 — will meaningfully reduce the cost of coverage expansion for states and plans that participate in BALANCE. Vermont should evaluate participation explicitly as an equity and population-health decision, not purely a budget decision.

13.3.5 Force 5: The Value-Based-Care Tipping Point — Still Approaching

Despite fifteen years of payment-reform effort, the majority of U.S. healthcare spending remains in fee-for-service. VBC penetration is real — roughly 58% of commercially insured Americans are in some value-based arrangement — but it has not reached the tipping point at which fee-for-service becomes the minority mode of payment. Vermont's mandatory reference-based pricing (RBP) and global-budget approach is a direct legislative response to the failure of voluntary VBC to reach that point.

The AHEAD Model — now operating in six states (Maryland, Connecticut, Hawaii, Vermont, Rhode Island, and New York) — is the most serious federal commitment to all-payer total-cost-of-care accountability since CMMI's founding. Changes implemented across all cohorts beginning January 2026 extended the model's end date to December 31, 2035, and added new requirements: each state must implement at least two policies promoting healthcare choice and competition, and adopt a Population Health Accountability Plan focused on preventive and chronic-disease care, with new transparency requirements around total-cost-of-care and primary care investment for Original Medicare beneficiaries specifically. Maryland — the model's longest-running and most-watched participant — signed an agreement to continue its participation in November 2025, a signal that the federal commitment to this approach remains intact heading into the period that matters most for Vermont. If Vermont's Cohort 2 participation (beginning January 2028) produces documented savings and quality improvement by 2030, the policy case for expanding AHEAD strengthens significantly. Vermont's performance is not only a Vermont question — it is national evidence for or against the most ambitious federal payment reform currently operating.

> **STATES TO WATCH**
>
> Vermont is not alone on this trajectory, even if it is presented here in the most detail. Maryland's all-payer model remains the longest-running proof of concept for global budgets nationally (Chapter 6). Connecticut, Hawaii, Rhode Island, and New York are AHEAD's other Cohort 1 and Cohort 2 states, each adapting the same federal model to a different starting structure — Connecticut and Rhode Island both have existing cost-growth benchmark programs analogous in function to Vermont's GMCB, while New York's scale puts it closer to the "large, fragmented state" scenario discussed in the Introduction. Pennsylvania and several Midwestern states have active rural hospital transformation programs working on problems structurally similar to Vermont's Northeast Kingdom challenge, without AHEAD's federal framework. A reader tracking whether this book's framework generalizes has a growing set of natural comparison cases to watch over the next several years, not just Vermont's.

13.4 The Six-Pillar Forecast: 2026–2035

Pillar	2026 starting point	Primary headwinds	Primary tailwinds	2035 assessment
Policy	VT: Acts 167/68/AHEAD form the most complete state reform mandate in the country. National: H.R. 1 Medicaid cuts; RHT front-loaded.	Medicaid work requirements (late 2026); federal policy instability; provider-tax restrictions pressuring state budgets.	AHEAD expanding (6 states); VT mandatory RBP/global budgets as template; price-transparency momentum; BALANCE GLP-1 access.	Moderate national progress; high VT-specific progress if execution holds. Mandatory APM participation is the critical unresolved national question.
Technology	VT: VHCURES solid; VITL/HIE voluntary and incomplete; analytics vendor procured; CIN in development. National: FHIR improving interoperability; AI outpacing governance.	Two-year VHCURES data lag; incomplete FHIR at smaller hospitals; absent AI governance in most states.	VT CIN providing shared analytics and AI governance; RHT IT grants funding FHIR upgrades; ambient AI reducing burden nationally.	Fastest-improving pillar nationally; FHIR operationally complete for major EHRs by ~2028. AI deployment rapid, governance lagging — safety and equity risk if governance does not catch up.
Economics	VT: RBP implementing (FY2027); global budgets mandated (FY2028). National: VBC ~58% commercial; global budgets rare outside VT/MD.	H.R. 1 Medicaid revenue reductions; GLP-1 wave threatening IRA savings; commercial premium pressure.	VT RBP producing documented savings; IRA GLP-1 negotiation (2027); Maryland AHEAD validating global budgets; ACO REACH maturing.	VT: strong progress if global budgets land on schedule. National: VBC ~70–75% commercial by 2030; global budgets stay limited absent mandatory policy.
Clinical	VT: Blueprint strong; CoCM/MHI effective but not permanently funded; CCBHC expanding; PACE gap; primary-care productivity constraint. National: BH access crisis worsening.	MHI sustainability uncertain without AHEAD EAST Fund commitment; primary-care shortage (370-FTE gap by 2030 in VT); Medicaid cuts threatening safety-net providers.	Blueprint among strongest PCMH infrastructure in the country; CCBHC expansion; AHEAD EAST Fund targeting primary care/BH; CoCM Medicare billing enabling sustainable financing.	Steady national improvement in primary care and BH integration. VT: Blueprint + AHEAD primary-care investment strongest in country; BH integration is the critical clinical variable.
Equity	VT: geographic disparities documented; BIPOC gaps smaller but real; SDOH screening operational; GLP-1 Medicaid gap; NEK access vulnerability. National: racial disparities not narrowing.	H.R. 1 cuts hitting low-income/rural hardest; federal equity-program funding reductions; GLP-1 access gap for Medicaid.	VT BALANCE participation could close the GLP-1 gap; CCBHC serve-all policy; Blueprint HRSN universal screening; Act 68 equity requirements embedded.	Highest-risk pillar nationally and in VT. Without explicit GLP-1 coverage, SDOH investment, and NEK access protection, VT equity gaps widen as Medicaid cuts bite.
Op	VT: hospital plans in development; AHS	14 plans at varying stages; analytics vendor not yet	$195M RHT capital; all 14 hospitals engaged; GMCB	VT transformation underway — the question is

Pillar	2026 starting point	Primary headwinds	Primary tailwinds	2035 assessment
erations	restructuring underway; PMO being established; CIN in development. National: administrative complexity not declining.	operational; AHS capacity must be built; RHT requires PM infrastructure VT is hiring.	capacity added under Act 68; joint AHS-GMCB analytics procurement; CIN shared services.	whether execution pace matches statutory deadlines. National: administrative simplification slow without federal prior-auth reform.

Figure 13.3 — Six-pillar forecast, 2026–2035: Vermont and national. Sources: HTR analysis; sources cited throughout this chapter.

13.5 Vermont's Ten-Year Trajectory: Two Scenarios

Vermont's path from 2026 to 2035 is genuinely uncertain in ways earlier versions of this chapter could not acknowledge, because the federal environment has changed materially since 2024. The two scenarios below bracket the range of plausible outcomes — not extremes.

13.5.1 The Transformation-Success Scenario

Vermont executes on or near the statutory timelines, uses its capital wisely, and demonstrates by 2030 that a small rural state can achieve genuine all-payer transformation.

FY2027: RBP implemented; commercial rates decline toward benchmark; premium growth slows for the first time in a decade — the first concrete evidence that mandatory price regulation can reduce premiums without hospital closures.

FY2028: Non-CAH global budgets operational; AHEAD Cohort 2 fully running; the hospital sector begins demonstrating the population-health improvements — fewer avoidable hospitalizations, better chronic-disease management — that global budgets are designed to incentivize.

2028: AHS delivers a Statewide Strategic Plan with specific, quantified commitments for each pillar through 2031; the hospital network is rationalized into a sustainable tier structure; Northeast Kingdom access is protected through Rural Emergency Hospital (REH) conversions paired with RHT-funded EMS and telehealth.

2030: RHT capital fully deployed; the hospital sector stabilized at 10–12 sustainable facilities with 2–4 REH or CACC conversions; Medicaid reductions absorbed through efficiency gains; per-capita cost growth below GDP growth for the first time.

2031–2035: Vermont serves as a national template; AHEAD expands using Vermont's documented results; Blueprint + AHEAD primary-care investment produces measurable chronic-disease improvements; the AHS restructuring model is studied by other states.

13.5.2 The Partial-Transformation Scenario

Vermont makes meaningful progress but hits execution gaps that prevent full realization by the statutory deadlines.

FY2027: RBP implemented but contested — UVMMC and other large systems seek legislative relief from specific rate provisions; GMCB holds implementation on a modified glide path that delays full impact.

FY2028: Non-CAH global-budget methodology not finalized due to GMCB resource constraints and AHEAD complexity; deadline extended; planning delayed.

2028: AHS delivers a Strategic Plan that is descriptive rather than prescriptive — naming the right questions without making the specific structural commitments accountability requires. Northeast Kingdom transition planning is delayed pending equity analysis.

2030: RHT capital deployed but partly misdirected toward operating support; Medicaid reductions begin arriving; several CAHs face renewed distress without the structural change that would have let them absorb the cuts.

2031–2035: Transformation is real but incomplete — more sustainable than the status-quo trajectory, but costs remain above what full transformation would achieve. A second reform wave is required to close the gaps.

Neither scenario is inevitable. The success scenario requires execution discipline, political commitment, and organizational capacity that Vermont is actively building but has not fully developed. The partial scenario requires execution failures that are preventable with adequate investment in implementation infrastructure. The analytical framework this book provides — and the organizational design the AHS Restructuring chapter (Chapter 1) prescribes — is meant to make the success scenario more likely.

"The RHT funding is a tool that lets us put affordability plans into action. This is not our health care reform plan. This is an opportunity that we'll use to support all of the health care reform work that we're already doing in the state."

— Jenney Samuelson, Vermont AHS Secretary, January 2026

13.6 Vermont as National Template — The Decisions That Will Determine Outcomes

Vermont's transformation will produce documented answers to questions every state will eventually face. Its national significance lies not in its size but in its head start, and in the quality of the analytical record it is creating. The decisions below, made over the next five years, will shape the national policy toolkit for the decade after.

Decision	Vermont timeline	If Vermont succeeds	If Vermont struggles
Will mandatory all-payer RBP reduce hospital prices without reducing access?	FY2027 implementation; first evidence FY2028	Confirms mandatory price regulation can close the price gap without the closures opponents predict; accelerates state adoption.	Arms RBP opponents; may delay national adoption 5–10 years.
Can hospital global budgets shift behavior toward population health?	FY2028 budgets; AHEAD data 2027–2030	Shows global budgets produce the improvements Maryland demonstrated, beyond Maryland-specific conditions.	Suggests budgets without primary-care/SDOH infrastructure don't improve population health; strengthens the case for community investment first.
Can AHS deliver a comprehensive Strategic Plan by December 2028?	December 2028 statutory deadline	Proves a state agency can restructure itself into a population-health system operator; provides a template.	Reveals capacity constraints other states must address first; valuable learning regardless.
Can RHT capital fund structural transformation rather than operating support?	FY2026–FY2030 spending window	Shows one-time capital can produce durable structural change — the core question about whether the $50B was well spent.	Suggests rural challenges need ongoing revenue support, not just capital; major implication for post-2030 federal policy.
Will AHEAD participation demonstrate savings and quality improvement?	AHEAD data from 2027; CMS evaluation through 2035	Builds the national evidence base for state TCOC accountability; may accelerate AHEAD expansion.	May lead CMS to modify AHEAD; still valuable as evidence.

Figure 13.4 — Vermont's transformation decisions and their national implications. Sources: HTR analysis; Vermont AHEAD State Agreement; Act 68 of 2025.

13.7 A Closing Note: What This Book Is Betting On

Every analytical framework contains implicit bets. It is worth making this book's explicit.

This book bets that structural change is possible — that strong enough financial pressure, adequate capital, political will sufficient to maintain mandatory policy over hospital resistance, and organizational capacity built over years of preparation can produce a genuinely different Vermont system by 2035. This is not a certainty; it is the most plausible outcome given Vermont's specific combination: a small, transparent, high-capacity state government; a hospital sector distressed enough to transform rather than resist; a regulator (GMCB) with real authority and the willingness to use it; a reform-minded legislature that has sustained a coherent agenda across administrations; and now $195 million in federal capital aligned to a pre-existing plan.

This book also bets that the transformations described here are transferable — that the framework, the payment mechanisms, the clinical models, and the organizational designs are general-purpose tools for challenges every state will eventually face, not Vermont-specific solutions to Vermont-specific problems. Vermont's specificity makes it an unusually good teaching case; the lesson generalizes.

The deepest bet is about the relationship between analysis and action. Healthcare transformation fails not primarily from analytical error — the evidence base for what must change is strong and consistent — but because analysis does not automatically produce the institutional capacity, political will, and organizational execution required to implement what the evidence recommends. This book provides the analytical framework. The capacity, the will, and the execution are built by the people who read it and do the work.

Vermont's transformation is happening because specific decision-makers — legislators, regulators, hospital executives, clinicians, community health workers, and patients — are making specific decisions at specific moments. The framework here is in service of better decisions. What those decisions produce — a transformed system or a partial one, a national template or a cautionary tale — belongs to the people who make them.

Sources: KFF, "A Closer Look at the $50 Billion Rural Health Fund in the New Reconciliation Law" (September 2025); KFF, "How Might Federal Medicaid Cuts in the Enacted Reconciliation Package Affect Rural Areas?"; KFF, "What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid" (2026); Chartis, 2026 State of Rural Health (February 2026); Peterson-KFF Health System Tracker, "Eight Trends Shaping 2026 Healthcare Costs" (March 2026); Pew Charitable Trusts (January 2026); Georgetown Center for Children and Families (March 2026); CMS BALANCE Model announcement (December 23, 2025); AHS Secretary Samuelson remarks (January 29, 2026); Oliver Wyman Act 167 Recommendations (2024); Vermont AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025).

Chapter 14 — Political Sustainability: Protecting Transformation Across Election Cycles

> *Healthcare transformation must survive governors' races, legislative budget battles, federal policy reversals, and provider opposition. This chapter identifies the political vulnerabilities in Vermont's reform architecture, the early-warning signals of risk to Act 68's mandatory framework, and the organizational strategies that protect transformation investments when the political environment shifts.*

No state's reform statute is permanent. Every mandatory architecture described in this book — Vermont's Acts 167 and 68 included — exists because a legislature passed it and remains in force only as long as a legislature, a governor, and a regulatory body continue to enforce it. This is a structural feature of policy-driven transformation everywhere, not a Vermont vulnerability: the same election-cycle, budget-battle, and provider-opposition dynamics apply to any state's reform statute, with the specific actors and timelines differing. The early-warning signals and protective strategies developed in this chapter using Vermont's architecture are a diagnostic framework an organization in any state can apply to its own state's reform statute — substitute the relevant election calendar, the relevant regulatory body, and the relevant provider coalition, and the framework holds.

14.1 Why Political Sustainability Is an Analytical Problem, Not Only a Political One

This book has treated the Policy pillar as settled: Acts 167 and 68 are law, the AHEAD State Agreement is signed, and the mandatory RBP and global-budget architecture is in place. That is analytically correct as of April 2026. It is also politically provisional in a way practitioners and organizational leaders must account for.

Vermont has an annual legislature, a governor's race in 2026, a federal partner (CMS) whose priorities shift with each administration, and hospital systems whose financial interests are materially affected by reference-based pricing. Act 68's mandatory architecture can be modified, weakened, or delayed by any of these actors acting alone or together.

Understanding political sustainability analytically means identifying which elements of the reform architecture are structurally durable and which are politically exposed; what the early-warning signals of risk look like before they become legislative action; and what organizations can do to protect multi-year investments predicated on policy continuity. This is not a chapter about advocacy or political strategy. It is a chapter about risk management.

14.2 The Structural Durability of Vermont's Reform Architecture

14.2.1 What Is Hard to Reverse

Federal agreements. The AHEAD State Agreement, signed with CMS in January 2025, is a binding federal-state agreement with a nine-year performance period. Reversal requires CMS consent, and the cost of exit — forfeiting enhanced PMPM payments and capital — is significant. AHEAD provides structural insulation against state-level disruption.

Capital investments. The RHT Program's $195 million over five years is federal capital already flowing into Vermont's system. The CIN infrastructure, IT upgrades, and analytics platforms, once built, create constituencies for their continuation and are difficult to defund without visible harm to politically protected rural communities.

The reform cascade. Vermont's Act 167 → Act 51 → Act 68 sequence built legitimacy incrementally. Each act created institutions — the Health Care Delivery Advisory Committee, expanded GMCB authority, the AHS HSA-coordinator model — that become constituencies for the next reform. Dismantling Act 68 would mean dismantling these institutions, which now have staff, missions, and legislative relationships that make reversal costly.

The financial crisis itself. Oliver Wyman's projection — 13 of 14 hospitals in operating losses by 2028 absent structural change — has not gone away. If Act 68 were weakened so that global budgets were no longer mandatory, the hospitals would still face the same trajectory. The case for transformation rests on a reality that is not politically manufactured.

14.2.2 What Is Politically Exposed

RBP methodology. RBP is mandatory from FY2027, but the *methodology* setting reference prices is a GMCB regulatory determination. It can be set to neutralize the structural change — reference prices so close to current commercial rates that little repricing occurs, or exemptions so broad that few hospitals are meaningfully constrained. Hospital lobbying will focus on the methodology, not on overturning the mandate.

Global-budget levels. Act 68 requires global budgets from FY2028, but GMCB sets the levels. Budgets set at levels that require no behavioral change produce no transformation. The risk is not repeal — it is budgets calibrated to produce the appearance of accountability without the substance.

Strategic Plan scope. Act 68 requires the December 2028 Statewide Strategic Plan but does not fully specify its content. A plan that is descriptive rather than committal would satisfy the statute while producing none of the intended outcomes — the most politically convenient failure mode: compliance without transformation.

Federal Medicaid funding. H.R. 1's $911 billion in Medicaid cuts (enacted July 2025) pressure Vermont's transformation financing directly. The AHEAD EAST Fund investments in primary care, behavioral health, and long-term care are partly financed through enhanced federal PMPM payments; if federal Medicaid funding falls significantly, EAST Fund investments become harder to sustain, reducing the Clinical-pillar capacity that makes global budgets viable. This is the federal-state dependency Vermont cannot fully insulate against.

14.3 Early-Warning Signals of Political Risk

Signal	What it indicates	Severity	Monitoring source
Hospital lobbying focused on RBP methodology rather than the mandate	Effort to hollow out RBP through the regulatory process	High	GMCB docket; Vermont Hospital Association statements
Bills to extend RBP or global-budget deadlines	Direct challenge to Act 68's mandatory timeline	Very High	Legislature bill tracker; HCAC testimony
Global-budget levels set within 5% of current commercial rates	Regulatory capture; budgets without pressure	High	GMCB FY2028 budget guidance
December 2028 Strategic Plan described as "in progress" in November 2028 reports	Operations failure; deadline unlikely to be met	High	AHS monthly transformation reports
CMS AHEAD waiver-modification request	Federal pressure (or Vermont request) to weaken AHEAD accountability	Very High	CMS AHEAD communications; AHS federal-relations reports
Governor-candidate platforms including RBP repeal or delay	Political threat to mandatory architecture in 2026 cycle	Very High	Governor-race coverage; candidate policy statements
EAST Fund spending well below authorized levels	Resistance to transformation investment despite authorization	Moder	DVHA AHEAD spending reports; GMCB budget documentation

Signal	What it indicates	Severity	Monitoring source
		ate	
HCAC failure to reach consensus on the Strategic Plan framework	Governance breakdown; plan unlikely to produce binding commitments	High	HCAC meeting minutes; AHS reporting

Figure 14.1 — Early-warning signals of political risk to Vermont's reform architecture. Sources: HTR Advisory risk-monitoring framework; Act 68 of 2025; GMCB regulatory process documentation.

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These signal categories — a change in the governing party's posture toward the reform, a high-profile enforcement challenge, a hospital-sector coalition campaign, and a federal policy shift that changes the reform's financial assumptions — are the same four categories that would precede the unwinding of a comparable reform in any state. The specific signal in Vermont was UVMMC's court challenge to GMCB enforcement; a reader elsewhere should identify their own state's largest, most politically influential health system as the actor most likely to generate the analogous signal, and watch accordingly.

14.4 Organizational Strategies for Political Risk Management

14.4.1 Strategy 1 — Build investments that are hard to reverse

The most effective protection against disruption is structural irreversibility. Organizations that build transformation into their operational fabric — hiring the staff, developing the workflows, establishing the data infrastructure, and changing the care model — create facts on the ground that are difficult to reverse when the political environment shifts. Hospitals that complete NCQA PCMH recognition before FY2027 hold a clinical-infrastructure investment that serves their population regardless of AHEAD's fate. Blueprint PCMHs that deploy the Collaborative Care Model build capability that produces value under any payment model. HCC gap closure that sharpens the AHEAD risk-adjustment factor also improves commercial risk adjustment.

The principle: invest in pillar capabilities that hold value across multiple payment-model scenarios, not only in compliance infrastructure for AHEAD's specific design. An organization that builds population-health-management capability is positioned for global budgets, for commercial VBC, and for the next federal model. One that builds only AHEAD-specific compliance is exposed if AHEAD changes.

14.4.2 Strategy 2 — Engage the regulatory process early and substantively

RBP methodology and global-budget levels are set through GMCB's regulatory process. Organizations that participate substantively — providing actual financial data, modeling the impact of specific methodology choices on their population, and engaging the equity implications of different approaches — shape outcomes more effectively than those that engage only through public opposition. Vermont hospitals hold the granular cost, utilization, and population data GMCB needs to set methodologies that are viable for hospitals while achieving the repricing that makes the system sustainable. Constructive, data-grounded engagement buys more influence than political argument.

14.4.3 Strategy 3 — Maintain scenario plans for policy disruption

Every Vermont organization with AHEAD exposure should maintain scenario plans for at least three disruptions: RBP delayed 18+ months; global-budget levels set 10%+ above current commercial rates (effectively removing budget pressure); and EAST Fund investments cut 30%+ due to federal Medicaid reductions. These are planning inputs, not predictions. An organization that has modeled its investment portfolio under each scenario can prioritize investments that hold value across all three.

Disruption scenario	Probability (HTR assessment)	Primary impact	Organizational response
RBP delayed 18+ months (methodology challenge)	Moderate (25–35%)	Commercial revenue maintained longer; urgency reduced	Maintain investments; use extra time to build clinical infrastructure
Global budgets set ~5% above current commercial rates	Low–Moderate (15–25%)	Minimal financial pressure; accountability without incentive	Engage GMCB methodology process; document cost structure for future rounds
EAST Fund cut 30%+ (federal Medicaid cuts)	Moderate–High (35–45%)	Primary-care/BH investment capacity reduced	Prioritize highest-return investments; diversify funding
2026 governor committed to Act 68 modification	Low (10–15%)	Legislative challenge in 2027 session	Build legislative relationships; document progress and stabilization

Disruption scenario	Probability (HTR assessment)	Primary impact	Organizational response
CMS AHEAD modification reducing accountability scope	Low (10–20%)	Reduced federal accountability; AHEAD less transformational	Maintain state accountability via GMCB; protect EAST Fund commitments

Figure 14.2 — Political-disruption scenario planning for Vermont healthcare organizations. Sources: HTR Advisory risk assessment; Vermont political-environment analysis (April 2026).

14.4.4 Strategy 4 — Document and communicate transformation progress

Political protection comes from demonstrated results. Hospital leaders who can show their representatives that transformation investment has stabilized finances, improved outcomes, and slowed operating losses — using the specific metrics in Acts 167 and 68 — are building the case for continuing the mandatory architecture. AHS's monthly transformation reports (required by Act 68) are the primary vehicle for communicating progress to the actors who control Vermont's policy environment. Organizations that supply high-quality data to those reports strengthen the very reports that protect the architecture they depend on.

The political sustainability of Vermont's reform ultimately rests on one thing: evidence that it is working. Hospital stabilization, primary-care access improvement, cost-growth moderation, and behavioral-health capacity expansion — demonstrated with the metrics in Act 167's five goals and Act 68's reporting requirements — are the most durable protection against reversal.

14.5 Vermont's Reform Cascade as Political-Sustainability Architecture

Vermont's cascade — Act 167 establishing diagnosis, Act 51 building institutional capacity, Act 68 deploying mandatory requirements — is itself a sustainability design. Each act created facts, institutions, and constituencies that made the next more viable and its reversal more costly.

Act 167's comprehensive, public, community-grounded diagnostic process — 230+ meetings, 3,100+ participants — created legitimacy for Act 68's mandates by making the crisis visible and involving affected communities. Communities that helped identify their own system's problems are less susceptible to campaigns claiming the problems do not exist. Act 68 adds a second sustainability feature: it makes failure visible. Monthly reports to the legislature, quarterly GMCB rate-setting updates, and the December 2028 deadline create continuous public accountability. If Act 68 is weakened, the weakening will be documented in public reporting — a political accountability trail that makes deliberate erosion more costly.

This is the model other states should take from Vermont, beyond the specific policy content: build the political-sustainability architecture into the reform design from the start. Diagnosis before legislation. Public process before mandate. Incremental capacity-building before full accountability. Continuous public reporting that makes progress and failure equally visible.

14.5.1 Key Concepts

Political sustainability — the capacity of a reform architecture to maintain its essential functions across election cycles, budget battles, and environment shifts. Distinct from political *support*: a reform can lose majority support and remain structurally durable if it has built sufficient institutional and organizational constituencies.

Regulatory-capture risk — the risk that the process implementing a statutory mandate is captured by the interests the statute was meant to constrain (e.g., RBP methodology set to avoid meaningful repricing while the mandate remains formally intact).

Structural irreversibility — the condition in which a transformation investment is so embedded in operations that reversal is costly regardless of the political environment (PCMH recognition, CoCM deployment, population-health analytics).

Political-disruption scenario — a specified set of political conditions whose impact on an organization's investment portfolio can be modeled and planned for; risk management, not forecasting.

Early-warning signal — an observable indicator that a risk is materializing *before* it becomes a statutory or regulatory change.

Reform cascade — a legislatively structured sequence in which each intervention builds on the capacity and legitimacy of its predecessor (Act 167 → Act 51 → Act 68).

Sources: Act 68 of 2025; Act 167 of 2022; H.R. 1 (One Big Beautiful Bill Act, July 2025); Vermont Legislature bill tracker; GMCB regulatory docket; AHS monthly transformation reports (2025–2026); Oliver Wyman Act 167 Recommendations (August 2024); Vermont Hospital Association statements; HCAC meeting minutes; CMS AHEAD documentation; HTR Advisory political-risk monitoring framework.

Chapter 15 — Healthcare Transformation as Portfolio Management: Applying PMI Standards to Six-Pillar Reform

> *Vermont's six-pillar transformation is not a project, and not a single program. It is a portfolio — a structured collection of interdependent programs and projects managed toward a strategic objective. This chapter applies PMI portfolio-management standards to Vermont's transformation, makes the case for a dedicated Portfolio Manager at the state level and Project Managers at each pillar, and provides the governance architecture, role definitions, performance framework, and risk register the Statewide Strategic Plan requires.*

PMI's portfolio, program, and project standards are not Vermont-specific, or even healthcare-specific — they are a general-purpose discipline for managing interdependent work toward a strategic objective, used across industries. Any organization running a six-pillar transformation, in any state, is by definition running a portfolio whether or not it has named it one, and the governance gap this chapter identifies — multiple interdependent initiatives with no portfolio-level accountability — is the default state of most transformation efforts, not a Vermont peculiarity. Vermont's 19-component portfolio is this chapter's worked example of what naming and structuring that portfolio looks like once a state's various mandates, programs, and grants are inventoried against the six-pillar framework.

15.1 The Project-Management Gap in Healthcare Transformation

Every concept in this book maps to a project-management discipline, whether or not project management was the framing its authors chose. Pillar definition is scope management. The execution sequence is schedule management with dependency logic. The failure cascade is risk management. The Vermont Scorecard is benefits-realization management. The six-pillar framework is, in its entirety, a portfolio-management architecture — and Vermont's transformation is failing to be managed as one.

Every concept in this book maps directly to a recognized project-management discipline. Pillar definition is scope management. The execution sequence is schedule management with dependency logic. The failure cascade is risk management. The implementation matrix is cost and resource management. The Vermont Scorecard is benefits-realization management. The six-pillar framework is, in its entirety, a portfolio-management framework — and Vermont's transformation needs to be managed as one.

10 of 10	19	7	14+
PMI knowledge areas addressed by this book	Vermont transformation portfolio components	Statutory deadlines requiring PM discipline	AHS programs requiring portfolio coordination

Figure 15.1 — The project-management gap. Sources: PMI Standard for Portfolio Management (5th ed.); Oliver Wyman Act 167 Report; AHS November 2025 Transformation Report.

15.2 Project, Program, and Portfolio: Getting the Levels Right

PMI draws a useful hierarchy: projects produce deliverables; programs coordinate related projects to produce benefits that no single project could; portfolios align all of the above with organizational strategy. Vermont's transformation operates at all three levels simultaneously. The diagnostic question is not whether Vermont needs this infrastructure — Oliver Wyman answered that in August 2024 — but whether Vermont is building it fast enough. The honest answer from the November 2025 report is no.

Level	PMI definition	Vermont example	Who manages it	Success measure
Project	Temporary endeavor creating a unique result; defined scope, schedule, budget.	VHCURES analytics-vendor deployment; HCC gap closure at Springfield; CoCM deployment at a Blueprint practice; RBP rulemaking.	Project Manager (pillar-, hospital-, or program-level)	On scope, schedule, budget; deliverable accepted.
Program	Related projects managed together for benefits not achievable individually.	Technology Pillar program (VHCURES + VITL + CIN + AI governance); Blueprint PCMH expansion (14 HSA projects); AHEAD Implementation Program.	Program Manager (pillar-level)	Benefits realized; interdependencies managed.
Portfolio	A collection of programs/projects/operations managed as a group to achieve strategy.	Vermont's full six-pillar transformation: all 19 components across the pillars, aligned to Act 167's five goals and the December 2028 plan.	Portfolio Manager (AHS Secretary-level or dedicated CPO)	Strategic objectives achieved; resources optimally allocated.

Figure 15.2 — PMI hierarchy applied to Vermont's transformation. Source: PMI Standard for Portfolio Management (5th ed.); PMBOK Guide 7th ed.; PMI Program Management Standard.

AHS is managing Vermont's transformation as a collection of projects without the program or portfolio infrastructure to coordinate them — sequenced by who is available rather than by what the portfolio needs. This is not a criticism of AHS staff, who are executing against an extraordinary mandate with insufficient capacity. It is a structural observation: the capacity does not exist yet. This chapter is the case for building it before the December 2028 deadline makes the gap irreversible.

15.3 Vermont's Transformation Portfolio: 19 Components

PMI's standard requires the portfolio to be defined — every component identified, classified, and assigned governance. The map below defines Vermont's transformation as a PMI-compliant portfolio of 19 components.

Pillar	Portfolio component	Type	Priority	Lead	2026 status → stage gate
Policy	Act 68 RBP-methodology rulemaking	Compliance project	Critical	GMCB	In progress → FY2027
Policy	AHEAD State Agreement management	Ongoing program	Critical	AHS	Active → annual CMS review
Policy	RHT Program contract management	Compliance project	High	AHS Health Reform	Active → annual federal reporting
Policy	Statewide Strategic Plan development	Strategic project	Critical	AHS + HCAC	In progress → Dec 2028
Technology	AHS-GMCB analytics-vendor deployment	Strategic project	Critical	AHS + GMCB	Procurement → Jan 2027 operational
Technology	Vermont CIN build-out	Strategic program	High	AHS + RHT	Design → FY2028 operational
Technology	VITL HIE expansion / FHIR compliance	Technical project	High	VITL + AHS	In progress → FY2027 mandatory
Technology	Statewide AI-governance framework	Policy project	Medium	AHS + CIN	Not started → FY2028 adoption
Economics	RBP implementation (all hospitals)	Compliance program	Critical	GMCB	Methodology design → FY2027
Economics	Global-budget design & implementation	Strategic program	Critical	GMCB + AHS	Design → FY2028 non-CAH

Pillar	Portfolio component	Type	Priority	Lead	2026 status → stage gate
Economics	AHEAD EAST Fund deployment	Operational program	High	DVHA + AHS	Active → annual targets
Clinical	Blueprint PCMH + AHEAD transition	Operational program	Critical	Blueprint + DVHA	Active → 100% PCMH by FY2028
Clinical	CCBHC certification (5 entities)	Compliance project	High	AHS + providers	In progress → FY2026
Clinical	CoCM statewide deployment	Operational program	High	MHI + Blueprint	Pilot → 80%+ practices FY2028
Equity	Social-risk-adjustment methodology	Policy project	Critical	GMCB + AHS Equity	Design → FY2028 budgets
Equity	HEROI scoring framework	Technical project	High	AHS + GMCB	Development → FY2027 baseline
Operations	14-hospital transformation planning	Operational program	Critical	AHS + RHRC successor	Plan submission → FY2026 approved
Operations	AHS restructuring (HSA-coordinator model)	Organizational project	Critical	AHS Secretary	In progress → FY2027 staffed
Operations	AHS PMO establishment	Organizational project	Critical	AHS	Not started → Q1 2027 operational

Figure 15.3 — Vermont's 19-component transformation portfolio. Sources: Act 68 of 2025; AHS Transformation Reports; Vermont RHT Program Application; CMS AHEAD documentation.

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The exercise that produced this 19-component inventory — listing every statute, grant, model agreement, and initiative touching the six pillars, then mapping each to a pillar and an owner — is directly repeatable by any organization or state agency, regardless of how many components the exercise turns up. Most organizations underestimate their own portfolio size until they do this inventory, because individual initiatives are usually owned by different departments that do not otherwise compare notes. The number 19 is Vermont's; the exercise is universal, and is often the single highest-value first step toward the kind of portfolio governance this chapter recommends.

15.4 The Case for a Portfolio Manager

Nineteen components. Shared stage gates. A single December 2028 deadline. Four statutory predecessors — Acts 167 and 51, Act 68, and the AHEAD State Agreement — each with their own federal and state reporting obligations. This is not a program-management problem. It is a portfolio-management problem, and it cannot be solved by component managers acting independently, no matter how skilled each one is. It requires a Portfolio Manager: a designated authority with the mandate and the access to manage Vermont's transformation as a whole, not as a collection of programs each doing their best.

This is not a new idea for Vermont. Oliver Wyman's Act 167 report explicitly recommended that AHS hire a dedicated project-management function and establish the "project management infrastructure needed to manage Vermont's transformation portfolio." The November 2025 report repeated the finding. The AHS Restructuring Roadmap (Chapter 1 of this book) identifies PMO establishment as a critical organizational project. What this chapter adds is a precise, PMI-grounded definition of the role, its authority, and its connection to the pillar-level Project Managers below it.

Role and authority (summary). The Portfolio Manager chairs the monthly portfolio review board; maintains the portfolio register (Figure 15.3) and risk register; authorizes component additions and removals; ensures all components stay aligned to Act 167's five goals; resolves cross-component resource conflicts; owns the Vermont Scorecard reporting to HCAC and the AHS Secretary; manages portfolio-level stakeholder relationships with HCAC, GMCB, hospital CEOs, the Legislature, and CMS; and tracks benefits realization, recommending component termination when benefits are no longer achievable. The role requires a direct report to the AHS Secretary, a GMCB co-reporting relationship, budget authority over the transformation PMO, and authority to require reporting from all 19 component leads. The appropriate credential is PMI's Portfolio Management Professional (PfMP), with 8+ years of program/portfolio experience and health-system or large-scale government program experience.

15.5 Pillar-Level Project Managers: Six Roles, One Framework

Below the Portfolio Manager, each pillar needs a Project (or Program) Manager. These roles are not AHS-exclusive — for hospital-owned components the manager may sit in a hospital PMO; for state-administered components, in AHS. What matters is that each pillar has a designated accountability point with the skills and authority to manage its components as a coherent program.

Pillar	Role	Recommended credential	Key deliverables	Reports to
Policy	Policy Transformation Program Manager	PMP + health-policy experience; PMI-ACP for agile rulemaking	RBP methodology (FY2026); Strategic Plan (Dec 2028); monthly Act 68 compliance	Portfolio Manager + AHS Secretary
Technology	Health IT Program Manager	PMP + health informatics; PMI-ACP	Analytics vendor operational (Jan 2027); CIN Phase 1 (FY2028); FHIR certified	Portfolio Manager + AHS technology lead
Economics	Payment Reform Program Manager	PMP + healthcare finance; actuarial/financial-modeling skills	RBP rates in effect (FY2027); non-CAH global budgets (FY2028); EAST Fund targets	Portfolio Manager + GMCB Executive Director
Clinical	Clinical Transformation Program Manager	PMP + clinical background (RN/LCSW); CoCM preferred	100% PCMH (FY2028); 5 CCBHCs (FY2026); CoCM 80%+ (FY2028)	Portfolio Manager + Blueprint Director + DVHA Medical Director
Equity	Health Equity Program Manager	PMP + public-health/equity background; HEROI	SRA methodology (FY2028); HEROI baseline (FY2027); SDOH data in VHCURES	Portfolio Manager + Office of Health Equity
Operations	Hospital Transformation Program Manager	PMP + hospital-operations/turnaround experience	14 plans approved (FY2026); HSA coordinators staffed (FY2027); PMO operational (Q1 2027)	Portfolio Manager + AHS Secretary

Figure 15.4 — Pillar-level role matrix. Sources: PMI credential requirements; Act 68 governance structure; Vermont RHT Program organizational design.

15.6 Governance Architecture and the Ten Knowledge Areas

PMI defines three governance tiers: the Portfolio Governance Body (strategic oversight — for Vermont, the AHS Secretary + HCAC + GMCB), the Portfolio Management Office (operational coordination — currently the under-resourced AHS Health Reform Office), and the component level (individual programs and projects). The PMO establishment must fill the gaps: a dedicated Portfolio Manager (PfMP), 2–3 PMO analysts, portfolio-management software, standardized reporting, six pillar Program Managers, and formalized project leads for all 19 components.

Mapping the ten PMI knowledge areas to Vermont's situation reveals the consistent gap: the book addresses each knowledge area analytically, but Vermont lacks the operational infrastructure to manage it. There is no formal work-breakdown structure for any pillar; statutory deadlines are tracked in isolation rather than as an integrated master schedule; RHT spending is tracked for federal compliance rather than portfolio performance; quality is measured program-by-program without portfolio aggregation; AHS staff are overcommitted with no formal resource leveling; communications, procurement, and stakeholder management all operate at the component level without a portfolio-level strategy; and risk is analyzed in this book but not managed operationally through a maintained register. Integration management — the dependency map, the execution sequence, the scorecard — exists analytically; operational integration does not yet exist.

15.7 Predictive vs. Adaptive Approaches

Not all 19 components should be managed identically. Predictive (waterfall) management fits components with fixed statutory deadlines and well-defined deliverables: RBP-methodology rulemaking, the Strategic Plan (with adaptive elements), analytics-vendor deployment, CCBHC certification, and AHS restructuring. Adaptive (agile) management fits components where requirements evolve and rapid learning beats adherence to a fixed plan: the statewide AI-governance framework (regulatory landscape evolving faster than any fixed plan), the CIN build-out (architecture depends on vendor outputs and the FHIR ecosystem), Blueprint/AHEAD practice transition (practice-by-practice iteration), and HEROI methodology (iterating as data quality improves). The 14-hospital planning effort is hybrid — fixed deliverables, hospital-specific iterative content. PMP credentials map to the predictive components; PMI-ACP to the adaptive ones.

15.8 The Portfolio Risk Register

ID	Risk	Pillars	Prob.	Impact	Score	Response	Owner
R-01	Analytics vendor not operational by Jan 2027; hospitals enter AHEAD without financial-management capability	Tech, Econ	High	Critical	20	Accelerate: parallel VHCURES direct-access; HCC gap closure as compensating investment	Tech PM + Portfolio Mgr
R-02	RBP methodology hollowed by regulatory process; no meaningful repricing	Policy, Econ	Medium	Critical	15	Mitigate: hospital data submission; equity review embedded; HCAC oversight	Policy PM + GMCB

ID	Risk	Pillars	Prob.	Impact	Score	Response	Owner
R-03	AHS overcommitment produces a descriptive (non-binding) Dec 2028 Strategic Plan	Ops, Policy	High	High	16	Mitigate: PMO by Q1 2027; resource-conflict authority; external plan support	Ops PM + Portfolio Mgr
R-04	2026 governor committed to Act 68 modification; mandatory architecture at risk	Policy	Low–Med	Critical	12	Accept + monitor: build irreversibility; document progress; legislative relationships	Portfolio Mgr + AHS Secretary
R-05	H.R. 1 Medicaid cuts reduce EAST Fund capacity; Clinical/Equity investments constrained	Clin, Equity, Econ	Med–High	High	15	Mitigate: scenario plan for 30% reduction; alternative financing; prioritize ROI	Econ PM + DVHA
R-06	VITL connectivity gaps persist; AHEAD population-health management incomplete	Tech, Clin	High	High	16	Mitigate: CIN shared services for small providers; VITL as Blueprint condition	Tech PM
R-07	RHRC engagement not replaced by adequate successor; planning loses momentum	Ops	Med–High	High	15	Mitigate: PMO includes hospital-transformation PM; document RHRC lessons; CIN continuation	Ops PM
R-08	Social-risk adjustment not adopted before FY2028 budgets; safety-net hospitals penalized	Equity, Econ	Medium	High	12	Accelerate: Equity PM leads SRA as co-equal to budget design; HCAC equity review required	Equity PM + GMCB
R-09	Insufficient clinical-leader engagement; AHEAD driven by administrators without clinical ownership	Clinical	High	Medium	12	Mitigate: clinical advisory structure in HCAC; physician/nursing co-chairs; clinical KPIs in monthly reports	Clinical PM + HCAC

ID	Risk	Pillars	Prob.	Impact	Score	Response	Owner
R-10	Portfolio-management infrastructure not established before 2027 deadlines; managed as a collection of projects	All	High	Critical	20	Avoid: hire Portfolio Manager by Q3 2026; PMO by Q1 2027 (the primary motivation for this chapter)	AHS Secretary

Figure 15.5 — Vermont's portfolio risk register (PMI format). Risk score = probability (Low=1 … High=5) × impact (Low=1 … Critical=4). Sources: PMI Standard for Portfolio Management (5th ed.); analytical risk assessment throughout this book; AHS November 2025 Report.

15.9 Benefits Realization: Connecting the PMO to Act 167's Five Goals

The Portfolio Manager's job is to ensure every component can trace its contribution to one or more of Act 167's five statutory goals, and that the portfolio as a whole produces the benefits that justify its cost.

Act 167 goal	Primary contributing components	Measurement metric	2028 target
High-quality, accessible care	Blueprint PCMH; CCBHC; HEROI; CIN	30-day BH follow-up; readmission; BIPOC access gap	90%+ BH follow-up; readmission <12%; gap <6 pts
Financial sustainability	RBP; global budgets; HCC gap closure; PMO	Operating margin; deficit trajectory; admin cost/discharge	13/14 sustainable; deficit reversed; admin <150% of benchmark
Affordability	RBP methodology; EAST Fund; global budgets	Commercial premium growth; TCOC benchmark	Premium growth <5%/yr; TCOC growth below CPI+1%
Robust workforce	RHT workforce investments; EAST Fund; AI-scribe grants	Primary-care FTE gap; vacancy rates; travel-nurse dependency	FTE gap <200 (from 370); travel-nurse cost <5% of labor
Health equity	HEROI; HRSN screening; social-risk adjustment; SDOH data	BIPOC access; NEK uninsurance; HEDIS equity gaps	BIPOC access >87%; NEK uninsurance <5%; gaps documented and targeted

Figure 15.6 — Benefits-realization plan. Sources: Act 167 of 2022; PMI Standard for Portfolio Management (5th ed.); Vermont Scorecard (Appendix F).

15.10 The Business Case

The objection to building a PMO is always the same: we cannot afford it. The cost-benefit table below answers that objection directly. The ~$4.5–7.5M cost of a staffed PMO over four to five years is roughly 2% of the portfolio value it protects. The cost of not building it is the December 2028 plan arriving as a descriptive document rather than a binding commitment — the continuation of the Oliver Wyman deficit trajectory, the failure of 13 of 14 hospitals to reach sustainability, and the loss of the political window Vermont's mandatory architecture has opened. That is not a $0 outcome.

Investment	Annual cost	Total (4–5 yr)	Benefit
Portfolio Manager (PfMP, senior)	$180K–$220K	~$900K–$1.1M	R-10 avoided; coordinates $195M RHT + ~$150M/yr EAST Fund; single accountability for Dec 2028
2 PMO Analysts	$120K–$160K	~$480K–$640K	Master schedule; risk register; stakeholder comms; dashboard
PPM software	$50K–$100K	~$250K–$500K	Real-time visibility; resource optimization; audit trail for Act 68
Six pillar PMs (shared)	$600K–$1M	~$3M–$5M	Pillar-level delivery accountability; critical-path management
Total	~$1M–$1.5M	~$4.5M–$7.5M	<2% of the portfolio value managed vs. $195M RHT + ~$150M/yr EAST Fund + $300M+ RBP savings

Figure 15.7 — Business case for portfolio-management investment. Sources: PMI Pulse of the Profession (2017); Vermont RHT Program Application; Act 68 EAST Fund allocation; PMI salary data.

The cost-benefit table omits the cost of failure. Without adequate PMO infrastructure, Vermont risks delivering the December 2028 plan as a descriptive document rather than a binding commitment — the political-sustainability chapter's most dangerous failure mode. That is not a $0 outcome. It is a multi-billion-dollar one: the continuation of the Oliver Wyman deficit trajectory, the failure of 13 of 14 hospitals to reach sustainability, and the loss of the political window that Vermont's mandatory architecture has opened.

15.11 Implementation Roadmap

Q1–Q2 2026: Post the Portfolio Manager position (PfMP preferred); begin recruitment (AHS Secretary).

Q2 2026: Portfolio Manager hired; portfolio register (Figure 15.3) and risk register formalized.

Q3 2026: Integrated master schedule developed; all 19 component leads identified; PMO analyst positions posted.

Q4 2026: PMO software deployed; monthly dashboard live; first formal Portfolio Governance Board meeting.

Q1 2027: Six pillar PMs designated; weekly pillar-level reporting; resource-conflict resolution operational.

Q2 2027 onward: Monthly governance reviews; quarterly performance reviews vs. the Vermont Scorecard; annual portfolio rebalancing.

December 2028: Strategic Plan delivered with portfolio-management evidence — benefits realized, risks managed, resources optimally deployed; PMO transitions to the three-year update cycle.

15.12 Answering the Four Objections

1. "We already manage this — we have the HCAC and monthly reports." The HCAC is a governance body, not a PMO; it provides oversight, not an integrated master schedule, resource-conflict resolution, a maintained risk register, or earned-value tracking. The monthly reports track activities, not performance metrics. The HCAC is the Portfolio Governance Body; it needs a Portfolio Manager and a PMO to give it the data it needs to govern.

2. "This is government — we don't run projects like the private sector." Vermont's transformation is a time-bounded, outcome-specified, heavily financed portfolio of interdependent programs with legally binding deadlines. The AHEAD State Agreement is a federal contract; the RHT Program is a federal grant with deliverables; Act 68 is a statutory mandate with a December 2028 deliverable. Federal partners expect professional project management — the GAO uses PMI's PMBOK as a reference standard.

3. "We can't afford a Portfolio Manager when we're cutting budgets." Vermont cannot afford not to have one. RHT funds are contingent on demonstrated progress; a portfolio that misses its components risks federal clawback. The ~$4.5–7.5M cost is roughly 2% of the value it protects, and the alternative is the multi-billion-dollar partial-transformation outcome.

4. "The hospitals won't accept being managed by AHS." The Portfolio Manager does not manage the hospitals — it manages AHS's contribution to the portfolio. Hospital plans are produced by hospitals, approved by AHS, and monitored through GMCB. The Portfolio Manager is Vermont's transformation orchestrator, not its controller.

15.12.1 Key Concepts

Portfolio · Portfolio Manager · Portfolio Management Professional (PfMP) · PMP · PMI-ACP · benefits-realization management · portfolio risk register · earned value management (EVM) · integrated master schedule · Portfolio Governance Body — all as defined by the PMI Standard for Portfolio Management (5th ed.) and the PMBOK Guide (7th ed.), applied to Vermont's 19-component transformation portfolio measured against Act 167's five goals.

Sources: PMI Standard for Portfolio Management, 5th ed. (2017); PMBOK Guide, 7th ed. (2021); PMI Program Management Standard, 3rd ed. (2017); PMI Agile Practice Guide (2017); PMI Pulse of the Profession 2017; PMI-ACP and PfMP Examination Content Outlines (2023); GAO Schedule Assessment Guide (2016); Vermont Act 68 of 2025; Act 167 of 2022; Oliver Wyman Act 167 Recommendations (August 2024); AHS Transformation Reports (August, November 2025); Vermont RHT Program Application (November 2025); AHS Hospital Transformation Analytical Support RFP (2025).

Chapter 16 — The AHS Restructuring Roadmap: A Six-Pillar Framework for System Architects

> *Written for AHS staff, Health Care Delivery Advisory Committee members, Vermont legislators, and hospital leaders who must deliver the Statewide Health Care Delivery Strategic Plan by December 2028.*

Unlike most other chapters in this book, this one is deliberately written as an operational playbook for a single named agency rather than a general framework illustrated by a Vermont example — because the six-pillar framework's ultimate test is not whether it can be described in the abstract, but whether it can be turned into an organizational restructuring mandate for a real agency with a real statutory deadline. A reader who does not work for AHS will still recognize the underlying question this chapter answers: what does a state health agency have to become, organizationally, to operate as a population-health system manager rather than an administrator of programs? That question applies to the equivalent agency in any state pursuing comparable reform, and the chapter's closing section, "Vermont as National Template," addresses that question directly.

16.1 The Statutory Mandate — What Act 68 Actually Requires AHS to Do

Act 68 requires AHS to deliver a Statewide Health Care Delivery Strategic Plan to the Vermont Legislature by December 1, 2028. The most important thing to understand about that mandate is that it is not a mandate to write a plan. It is a mandate to become a different kind of organization. The Strategic Plan is the vehicle; the destination is a Vermont healthcare system that achieves the five goals of Act 167. The plan is the document that makes AHS accountable. What this chapter addresses is the organizational transformation AHS must undergo to produce a plan worth the paper it will be written on.

Statutory requirement	What it means in practice	Deadline
Statewide Health Care Delivery Strategic Plan	A comprehensive plan covering payment-reform trajectory, primary-care investment targets, hospital-network configuration, behavioral-health integration, equity goals, and administrative simplification. Updated every three years.	First delivery: Dec 1, 2028

Statutory requirement	What it means in practice	Deadline
Affordability benchmarks	Specific, measurable affordability standards — not aspirational language; must define what "affordable" means for Vermont families and how progress is measured.	Set by HCAC; in the plan
Total-cost-of-care targets	Quantified per-capita cost-growth targets, aligned with AHEAD and Act 68's hospital-spending-reduction requirement.	2.5% hospital-spending reduction for FY2026; AHEAD TCOC from 2027
Standardized accountability metrics	A defined set of outcome measures tied to the five Act 167 goals, reported monthly and used to evaluate the plan.	Monthly already; full framework by plan delivery
Primary-care investment plan	A plan to raise primary care as a share of total spend (an AHEAD requirement), including Blueprint expansion, CCBHC development, and CoCM deployment.	AHEAD targets from 2027; plan formalizes it
Health-equity strategy	A specific plan to reduce disparities in access and outcomes — required by Act 68.	In the plan; equity measures in monthly reporting
Workforce strategy	A plan to attract and retain professionals (Act 68), including RHT workforce investments.	RHT covers 2026–2030; integrated into the plan

Figure 16.1 — Act 68 statutory requirements for the Strategic Plan. Sources: Act 68 of 2025; HCAC mandate (healthcarereform.vermont.gov).

The plan is not AHS's alone to write. Act 68 created an 18-member Health Care Delivery Advisory Committee — representing AHS, GMCB, the Health Care Advocate, hospitals, health centers, insurers, health professions, small businesses, and long-term care — to develop and maintain the plan in collaboration with AHS, consulting the Health Equity Advisory Commission and the Steering Committee for Comprehensive Primary Health Care. The plan must reflect multi-stakeholder consensus, not just AHS's internal analysis — a significant operational challenge and an important political constraint.

"Review existing AHS agency structure and program list to identify overlaps and opportunities for efficiency. Align Agency of Human Services agencies with Health Service Areas and meet regularly with hospitals and providers. Fully computerize and integrate Agency of Human Services subunits."

— Oliver Wyman, Act 167 Community Engagement: Recommendations (September 2024) — priority actions for AHS in 2025

16.2 The Organizational Diagnosis — Why AHS Must Restructure, Not Just Plan

Oliver Wyman's finding was blunt: AHS's organizational structure is one of the primary barriers to transformation. Not because AHS lacks capable people, but because it was designed for a different era and a different role. The current model is organized around funding streams and program types — Medicaid, mental health, substance use, aging, developmental services, Blueprint, health care reform — rather than around the populations served or the communities they live in. That made sense when AHS's job was administering categorical benefit programs. It does not make sense for an organization whose job is now managing the transformation of an interconnected delivery system that serves everyone in Vermont, all at once.

16.2.1 The Four Structural Gaps

Gap	What it looks like in practice	Consequence	Required structural change
Program-silo structure misaligned with HSAs	A Northeast Kingdom hospital addressing housing, transportation, BH, and primary-care gaps must work with separate AHS programs (DVHA, DMH, ADS, DAIL, Health Care Reform), each with different requirements and funding authorities.	Hospitals experience AHS as disconnected programs. Population health management — which crosses program boundaries — becomes administratively near-impossible.	Designate HSA-level coordinators who convene all relevant AHS programs around each hospital's service area; establish HSA governance with regular inter-program coordination.
Absence of a Planning & Effectiveness function	AHS has no dedicated analytical unit to model the cost/access impact of service-site changes, the system-wide cost of regionalization scenarios,	AHS cannot answer the questions transformation planning is supposed to generate. Strategic planning becomes	Establish a Division of Planning, Analytics, and Effectiveness; partner with GMCB on a joint analytics platform;

Gap	What it looks like in practice	Consequence	Required structural change
	or whether investments produce intended outcomes.	opinion-based, not evidence-based.	integrate VHCURES access for planning.
Under-resourcing of transformation management	Act 68 requires AHS to simultaneously negotiate AHEAD, manage 14 hospital plans, develop the Strategic Plan, oversee $195M RHT, coordinate RBP/global budgets with GMCB, and report monthly. The Health Care Reform Office has neither the staff nor the authority.	Activities are sequenced by capacity, not strategy. The absence of PM infrastructure is the single most likely cause of plan failure.	Hire dedicated transformation staff using RHT implementation funds (explicitly authorized); establish a PMO; clarify reporting lines and authority.
Fragmented SDOH integration	AHS has authority over housing supports, food assistance, transportation, mental health, substance use, and developmental services — all SDOH drivers — but administers, reports, and evaluates them separately from healthcare outcomes.	Vermont cannot achieve the SDOH-driven utilization reduction global budgets require if the SDOH programs are organizationally disconnected from delivery reform.	Establish cross-program SDOH integration protocols; build a shared data architecture linking social-service utilization to healthcare outcomes in VHCURES; include SDOH directors in HSA coordination.

Figure 16.2 — AHS organizational gaps and required structural changes. Sources: Oliver Wyman Act 167 Recommendations (2024); AHS Transformation Reports (2025); Act 68 of 2025.

16.3 The Redesigned AHS Operating Model — A Population Health System Operator

The model AHS must become is a Population Health System Operator: an agency that manages Vermont's healthcare and social-services ecosystem as an integrated system oriented around population-health outcomes, organized geographically around Hospital Service Areas, and accountable for measurable progress toward the five Act 167 goals. This is a fundamentally different identity than AHS has held. It requires changes to structure, staff roles, governance, data systems, external relationships, and the agency's own theory of how it creates value. The word "restructuring" is accurate. AHS does not need a new mission statement. It needs a new operating model.

16.3.1 Level 1 — Statewide System Architect

At the statewide level, AHS sets strategic direction, establishes the payment-reform trajectory with GMCB, allocates capital (RHT, AHEAD EAST Fund, Act 68 grants), develops the accountability framework, and monitors performance against the five goals. This requires four capacities AHS is building but has not fully developed:

- Strategic-planning capacity — to develop, maintain, and update the plan on a three-year cycle with required stakeholder input. A substantial ongoing function, not a one-time project.
- Analytics and modeling capacity — to model cost, quality, and access implications *before* implementation ("what happens to access in Windsor County if Springfield converts to REH?"). Requires the joint AHS-GMCB analytics vendor plus internal staff to interpret outputs.
- Federal-relationship management — active management of CMS (AHEAD), HRSA (RHT), and CMMI relationships. The AHEAD State Agreement alone requires ongoing methodology negotiation, monitoring, and withdrawal-condition management.
- Legislative accountability — producing the monthly reports, the December 2028 plan, and the three-year updates. The monthly reporting requirement is a real accountability mechanism, not a formality.

The Health Care Delivery Advisory Committee (18 members: AHS Secretary, GMCB Chair, Health Care Advocate, and representatives of hospitals, FQHCs, insurers, nursing/pharmacy/primary care and other professions, small business, and long-term care) is AHS's governance partner. It has statutory authority to set affordability benchmarks and advise the legislature directly, develops and maintains the plan in collaboration with AHS, and incorporates recommendations from the primary-care steering committee. Operational implication: the committee needs dedicated staff support — facilitation, research preparation, deliberation management, recommendation documentation — treated as a dedicated function, not an add-on to existing Health Care Reform Office duties.

16.3.2 Level 2 — Regional Network Convener (HSA Level)

At the HSA level, AHS convenes hospitals, primary-care practices, designated mental-health agencies, FQHCs, community organizations, and social-service providers within each HSA to develop and execute a coordinated regional strategy. The mechanism is the HSA Coordinator —

staff whose primary job is to be the face of AHS within a defined geographic area, convening the relevant AHS programs and partners around that area's transformation priorities. The Blueprint for Health already deploys field staff (Quality Improvement Facilitators, Community Health Team coordinators) embedded in each HSA; its infrastructure is the model AHS should replicate and build on.

HSA region	Key hospital(s)	Profile	Priority transformation challenges
Burlington	UVM Medical Center	~230K, growing, most diverse; non-rural	Admin-overhead reduction at UVMMC; primary-care access for growing immigrant population; BH integration at scale
Barre/ Montpelier	Central Vermont Medical Center	~65K, aging, state-government center	Radiation/neurology COE development; state-employee-plan RBP transition; Washington County rural access
Rutland	Rutland Regional Medical Center	~62K, aging, lower income	COE development (surgery, neurology, psych); high opioid overdose; workforce-housing shortage
White River Jct.	Mt. Ascutney Hospital	~59K, aging, Dartmouth Health connection	REH or focused-scope transition; cross-border coordination; rehabilitation COE
St. Albans	Northwestern Medical Center	~53K, rural, Franklin/Grand Isle	Cancer-surgery and radiation COEs; workforce recruitment; northern-border needs
Bennington	Southwestern Vermont Medical Center	~48K, aging, most rural	9 COEs — secondary RSC role; critical psych services; Berkshire/Taconic coordination
Middlebury	Porter Medical Center	~46K, aging, Addison County	TBD tier; UVM-network affiliation; shared-service opportunities with UVMMC

HSA region	Key hospital(s)	Profile	Priority transformation challenges
Brattleboro	Brattleboro Memorial Hospital	~42K, aging, high SUD/MH burden	Acute general-surgery COE; high suicide/self-harm rates; Brattleboro Retreat coordination
Newport	North Country Hospital	~41K, very rural, NEK	Among most financially vulnerable; REH candidacy; high Essex/Orleans uninsurance
St. Johnsbury	Northeastern Vermont Regional Hospital	~39K, very rural, NEK	Cancer-surgery and infusion COEs; highest poverty (Caledonia/Essex); broadband/telehealth priority
Springfield	Springfield Hospital	~31K, rural, Windsor/Windham border	Memory-care and adult-psych COEs; SUD corridor; most complex plan given financial position
Morrisville	Copley Hospital	~31K, rural, Lamoille/Orleans	Orthopedics/rheumatology COEs; Stowe seasonal population; critical workforce housing
Randolph	Gifford Medical Center	~28K, very rural, Orange County	TBD tier — among most vulnerable; comprehensive assessment needed; AHEAD CAH protections
Townshend	Grace Cottage Hospital	~14K, very rural; 19-bed, Vermont's smallest	REH conversion the likely path; coordination with Brattleboro Memorial essential; palliative-care specialty worth preserving

Figure 16.3 — Vermont's 14 HSAs: regional profile and priority transformation challenges. Sources: Oliver Wyman Act 167 Report; Vermont RHT Program Application; AHS Transformation Reports 2025.

16.3.3 Level 3 — Program Delivery Operator

At the program level, AHS continues administering Vermont's programs — Medicaid (DVHA), mental health (DMH), Blueprint, developmental and aging services. The structural change is not eliminating program operations but adding cross-program coordination and aligning program operations with the goals and geographic structures set above. The critical change is integrating social-service programs into the transformation framework. Oliver Wyman's three imperatives — build housing and fix transportation, pay with RBP and move to global budgets, move care out of hospitals — cannot be met by healthcare programs alone. The AHS programs that fund housing, transportation, food assistance, and developmental services sit in the same agency; they are simply not coordinated with the transformation agenda. The HSA Coordinator role creates the coordination layer that does not currently exist — convening DVHA, DMH, Blueprint, ADS, DAIL, and Health Care Reform staff around each region's hospital transformation plan.

16.4 The Six-Pillar Framework Applied to AHS's Mandate

The six-pillar framework provides the analytical architecture for the Strategic Plan. Each pillar corresponds to a domain of AHS's mandate, a set of capabilities to develop, and a set of measurable outcomes.

Pillar 1 — Policy (legislative compliance, federal relationships, regulatory strategy): AHEAD Model management (ongoing CMS negotiation, TCOC compliance, withdrawal conditions); Act 68 compliance tracking (RBP FY2027, non-CAH global budgets FY2028, plan December 2028, legislative reporting calendar); RHT Program administration (grant management, contractor oversight, annual CMS reporting); and federal-policy monitoring (Medicaid reconciliation, CMMI changes, CMS guidance).

Pillar 2 — Economics (payment-reform sequencing, financial-sustainability monitoring, ROI): payment-reform coordination with GMCB (RBP methodology, global-budget design, premium-passthrough monitoring); hospital-financial monitoring against the Oliver Wyman projections — particularly the CAH operating-profit gap ($938 vs. $2,784 benchmark) and the PPS administrative-cost premium ($3,826 vs. $1,427 benchmark) — with early technical assistance; EAST Fund management (up to ~$150M/yr in additional Medicare funds toward primary care, BH, home health, long-term care); and transformation-ROI tracking against Oliver Wyman's $400M+ projected savings.

Pillar 3 — Technology (data infrastructure, HIE governance, analytics, EHR strategy): VHCURES expansion and more-timely access (the November 2025 report acknowledged most measures reflect 2023 data — a two-year lag that makes real-time management impossible); HIE strategic development (the Act 167–mandated feasibility assessment of a statewide EHR); analytics-vendor implementation (the most urgent near-term investment); and cross-program data integration linking SDOH programs to healthcare outcomes.

Pillar 4 — Clinical (primary-care expansion, BH integration, care-model design): Blueprint expansion and the AHEAD transition (moving Blueprint's Medicare payments to Primary Care AHEAD; making the MHI behavioral-health-integration pilot permanent through EAST Fund investment); CCBHC network development (5 new entities by July 2026, integrated with primary care, EDs, and 988); care-model technical assistance for the global-budget environment; and long-term-care capacity development (PACE gap, dementia-care COE, HCBS expansion).

Pillar 5 — Equity (disparity elimination, geographic access, SDOH, culturally competent care): geographic-equity monitoring by county, with attention to the Northeast Kingdom (6–8% uninsurance vs. 3% statewide); Medicaid-access protection (whether RBP/global budgets affect hospitals' willingness to maintain Medicaid volumes — Act 68 requires access-protection consideration in RBP design); culturally competent care investment (language access, cultural competence, LGBTQ+ competency, weight-bias); and SDOH-program coordination directed toward the communities where gaps most drive utilization.

Pillar 6 — Operations (transformation-plan management, RHRC successor, administrative simplification): hospital-transformation-plan oversight (from draft outlines to final plans for all 14 hospitals, coordinated regionally); administrative simplification (prior authorization, the CON process, Medicaid billing); PMO establishment; and EMS/transportation transformation (addressing the agency fragmentation that makes inter-facility transfers complex and expensive).

16.5 The AHS Restructuring Implementation Timeline

Deadline	Milestone	Primary owner	Status
FY2026 (now)	Hospital transformation grant awards; all 14 hospitals engaged	AHS Health Care Reform	In progress — all eligible hospitals applied
FY2026 (now)	Analytics-vendor procurement (joint AHS-GMCB)	AHS + GMCB	Underway as of November 2025
FY2026 (now)	CCBHC certification for 5 new entities (July 2026 target)	AHS / DMH	On track per RHT plan
FY2026 (now)	AHEAD Medicaid global budget operational (January 2026)	DVHA / CMS	Operational
FY2026 (now)	RHT implementation planning and staff hiring	AHS Health Care Reform	$195M received Dec 2025; planning underway
FY2026 Q2–Q4	Final hospital transformation plans	AHS + 14 hospitals + RHRC successor	Plans expected early 2026
FY2026 Q2–Q4	HSA Coordinator roles established	AHS Secretary	Organizational change requiring Secretary-level decision

Deadline	Milestone	Primary owner	Status
FY2026 Q2–Q4	Division of Planning and Effectiveness established	AHS Secretary + GMCB Chair	OW-recommended; critical for plan development
FY2027 (Oct 2026)	RBP maximum rates in effect for commercial payers	GMCB (AHS coordination)	Statutory deadline; GMCB leads
FY2027 (Jan 2027)	AHEAD Cohort 2 begins — Medicare FFS global budgets	AHS + DVHA + GMCB + CMS	Nine-year model; ongoing federal coordination
FY2027	Primary Care AHEAD operational — enhanced primary-care payments	DVHA + Blueprint	Replaces expiring Medicare demo
FY2028 (Oct 2027)	Global budgets for non-CAH hospitals	GMCB (AHS coordination)	Requires completed RBP methodology and budget design
December 2028	Statewide Strategic Plan delivered to legislature	AHS Secretary + HCAC	Statutory; updated every three years
FY2030 (Oct 2029)	Global budgets for all hospitals including CAHs	GMCB (AHS coordination)	Completes all-hospital global-budget coverage

Figure 16.4 — AHS restructuring and Vermont transformation timeline. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Reports; Vermont RHT Program Application.

16.6 A Framework for the 2028 Statewide Strategic Plan

The Strategic Plan is the most consequential document AHS will produce this decade; it will define Vermont's system for the decade following its delivery. The proposed structure, organized around the six pillars and Act 68's requirements:

-
    1. Executive Summary (5–8 pp) — the crisis in 2028; what changed since 2022; the five goals and progress; the plan's central 2028–2031 commitments.
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2. System Context and Diagnosis (15–20 pp) — an updated Oliver Wyman analysis: demographics, hospital finances, payer landscape, workforce, SDOH burden as of a 2028 baseline.

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3. Payment Reform Architecture — Economics (20–25 pp) — RBP status; global-budget parameters per hospital; AHEAD TCOC targets and performance; affordability benchmarks.

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4. Primary Care and Behavioral Health — Clinical (15–20 pp) — Blueprint enrollment/performance; CCBHC coverage; CoCM status; BH access indicators and 2028–2031 targets.

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5. Hospital Network and Regionalization — Operations (20–25 pp) — confirmed tier designations; COE network; transfer infrastructure; shared services; REH/CACC conversions; workforce distribution.

-

6. Data and Technology Infrastructure — Technology (10–15 pp) — VHCURES enhancement; HIE connectivity; statewide-EHR decision; analytics capabilities; interoperability.

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7. Health Equity Strategy — Equity (15–20 pp) — geographic-equity indicators by HSA; disparity measures; SDOH coordination; culturally competent care; 2028–2031 targets.

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8. Workforce Strategy (15–20 pp) — supply/demand by profession and geography; RHT workforce investments and outcomes; scope-of-practice changes; pipeline; 2030 projections.

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9. AHS Organizational Architecture — Policy & Operations (10–15 pp) — the redesigned structure; HSA Coordinator model; Division of Planning and Effectiveness; cross-program SDOH integration; governance relationships.

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10. Accountability Framework (10–15 pp) — full metrics dashboard (all 17+ Act 167/Act 68 measures); data sources and update frequencies; 2028–2031 targets; reporting calendar; revision conditions.

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11. Financial Plan (15–20 pp) — RHT capital allocation (remaining balance); EAST Fund allocation; Act 68 grant commitments; projected savings; state-budget implications.

Figure 16.5 — Proposed structure for the 2028 Statewide Strategic Plan. Sources: Act 68 of 2025; HCAC mandate; six-pillar framework.

16.6.1 The Central Commitment the Plan Must Make

Beyond its analytical content, the plan must make one commitment currently absent from Vermont's transformation discourse: a specific, quantified trajectory for what Vermont's healthcare system will look like in 2031, 2035, and 2040, with the financial modeling to show it is achievable. Oliver Wyman provided the diagnostic scenario — without transformation, 13 of 14 hospitals in losses by 2028, unsustainable premiums, and contraction through unplanned closures. The plan must provide the alternative scenario with equal specificity: if Vermont executes on the timelines in this chapter, how many hospitals maintain inpatient beds in 2031? What is the primary-care investment level? The premium-growth rate? The Northeast Kingdom uninsurance rate? That is the difference between a strategic plan and a strategic aspiration.

Parameter	Value	Detail
Strategic Plan delivery	Dec 2028	Act 68; updated every three years
HCAC members	18	AHS, GMCB, HCA, hospitals, health centers, insurers, professions, small business, LTC
RHT capital available	$195M	Vermont's December 2025 award; aligned with Acts 167 and 68
Projected direct transformation savings (5 yr)	>$400M	Independent of AHEAD and RBP payment-reform savings (Oliver Wyman)

Figure 16.6 — Key parameters for the Strategic Plan. Sources: Act 68; VTDigger; Oliver Wyman Act 167 Report; Vermont RHT Program Application.

16.7 Vermont as National Template — Why This Restructuring Matters Beyond Vermont

Vermont's AHS restructuring matters beyond Vermont because it is the first test of whether a state human-services agency can redesign itself as a population-health system operator in response to a statutory transformation mandate. The questions are not Vermont-specific: How do you align an agency organized around program silos with geographically defined service areas? How do you build the analytics capacity to model structural change before implementing it? How do you run a multi-stakeholder planning process that includes hospitals, insurers, providers, and community organizations without producing a plan that commits to nothing? Vermont will have answers by December 2028. They will be grounded in real experience with a real system under real pressure. That is not nothing. In American health policy, where policy debates run decades ahead of implementation evidence, it may be the most valuable contribution Vermont makes.

Vermont will have answers by December 2028 — grounded in real experience with a real system under real pressure. Those answers may be the most valuable policy contribution Vermont makes to American healthcare in a generation. The significance is not that Vermont is large or powerful; it is that Vermont is early — early enough to produce a tested model before the rest of the country reaches the same crisis point, and early enough for the lessons to remain actionable.

16.7.1 Key Concepts

Population health system operator · HSA Coordinator · Division of Planning and Effectiveness · Statewide Health Care Delivery Strategic Plan · Health Care Delivery Advisory Committee · program-silo structure · cross-program SDOH integration · reform cascade (Act 167 → Act 51 → Act 68).

Conclusion — What Vermont Proves, and What Remains to Be Proven

Vermont has done something most American states have not: it has committed, in statute, with capital, and against active opposition, to structural healthcare change. Whether that commitment produces transformation or produces documents is the question this conclusion must honestly answer.

What Failure Actually Looks Like

Before making the case for optimism, intellectual honesty requires naming what failure looks like — specifically, because the partial-transformation scenario is not obviously distinguishable from the real thing until years have passed.

Failure does not look like a spectacular collapse. Vermont is not going to repeal Act 68. The GMCB is not going to dissolve. The $195 million RHT award will be spent on something. Failure looks like this: reference-based pricing takes effect on schedule but is suspended pending litigation that runs for three years. Global budget methodology is completed in 2028 but at benchmarks high enough that the cost-reduction signal is too weak to change hospital behavior. The Statewide Strategic Plan is submitted in December 2028 and describes the right goals without committing to specific, measurable, accountable targets. The AHEAD performance year begins in January 2028 and the analytics vendor is not yet producing the population-level data hospitals need to manage their total cost of care. AHS receives its restructuring mandate but is not given the budget, the hiring authority, or the political cover to execute it at the pace Acts 167 and 68 require.

In this failure scenario — which is not a disaster but a managed deceleration — Vermont in 2032 looks roughly like Vermont in 2026: a system with better policy language, a larger regulatory apparatus, a modernized information infrastructure, and a premium trajectory that has not fundamentally changed. The hospitals that were fragile in 2026 are more fragile. The workforce shortage that was projected is now present. The commercial cross-subsidy that was declining has declined further. And the window for transformation that opened in 2024 has closed, because the political conditions that produced Acts 167 and 68 are not guaranteed to persist through a second or third legislative cycle if the first cycle does not produce visible results.

This failure scenario is not inevitable. But it is the modal outcome of healthcare reform efforts in American history, and pretending otherwise would make this book less useful to the people who need to prevent it.

The Three Decisions That Are Not Yet Made

As of April 2026, Vermont's transformation hinges on three decisions that have not yet been made. Each is the responsibility of a specific set of actors. Each has a right answer that this book can describe. Whether that answer is chosen is not determined by analysis.

Decision 1: Will the Statewide Strategic Plan Be Binding or Descriptive?

Act 68 requires Vermont's Agency of Human Services to submit a Statewide Health Care Delivery Strategic Plan to the legislature by December 1, 2028. The law does not specify how specific the plan must be. A plan that identifies transformation goals without committing to measurable targets, specific timelines, and accountable owners satisfies the literal requirement while producing none of the accountability that makes a plan worth writing.

The choice between a binding plan and a descriptive one will be made by AHS officials, reviewed by the GMCB, and ultimately accepted or rejected by the Vermont legislature. The standard should be: does this plan create conditions under which, in December 2030, a disinterested observer could evaluate whether the plan was executed? If the answer is yes, the plan is binding. If the answer is no — if the plan is a set of aspirations that could be plausibly claimed as achieved regardless of outcomes — the plan has failed before implementation begins.

The people in Vermont who care about transformation have an obligation to press for the binding version. Not the aspirational one.

Decision 2: Will the Analytics Vendor Be Operational Before AHEAD's Financial Accountability Begins?

Vermont hospitals will begin accumulating financial accountability under AHEAD in January 2028. The AHS-GMCB analytics vendor must be fully operational — producing HCC gap reports, population attribution analyses, and global budget performance dashboards — before that date. If it is not, Vermont hospitals will enter their first year of full AHEAD accountability without the tools to manage it. The "Managing Blind" failure mode, which destroyed OneCare Vermont's ability to demonstrate savings, will be replicated at the system level.

The analytics vendor procurement is underway as of April 2026. The gap between procurement completion and vendor deployment is typically 12 to 18 months for complex health data systems. January 2028 is 21 months away. This is a manageable timeline that will become an unmanageable one if procurement slips. The people responsible for this procurement — AHS, GMCB, and the legislature that funds them — need to treat January 2028 as an immovable deadline, not an aspirational target.

Decision 3: Will H.R. 1's Medicaid Cuts Force a Contraction or Accelerate a Transformation?

H.R. 1's $911 billion in Medicaid cuts over ten years, work requirements beginning in late 2026, and reduced ACA premium subsidies will arrive in Vermont at the same moment as AHEAD's performance year and Act 68's mandatory pricing reform. The combined financial pressure on Vermont hospitals — from reduced Medicaid volume, reduced Medicaid rates, and mandatory reference-based pricing — is the biggest test of whether Vermont's transformation architecture is robust enough to absorb shocks while remaining on its statutory timeline.

The optimistic scenario: federal pressure accelerates the political will for the administrative efficiencies, shared services, and rural access redesign that transformation requires. The pessimistic scenario: federal pressure overwhelms hospital financial positions before the transformation investments that would stabilize them are in place, producing a crisis that forces contraction rather than transformation.

This decision is partly outside Vermont's control — federal policy is federal policy. What is within Vermont's control is the pace and specificity of the RHT Program investments, the timeline for the analytics vendor, and the adequacy of Act 68's EAST Fund utilization as a bridge for hospitals absorbing federal cuts during the transformation period. The legislature that funded transformation and the AHS leadership executing it have the ability to accelerate on the dimensions they control. That acceleration is now a strategic imperative, not a preference.

What Vermont Has Proved — As of April 2026

Even in its incomplete state, Vermont's transformation has proved several things that were not obviously true before Acts 167 and 68.

It has proved that a state government can impose mandatory rate regulation on a well-organized hospital sector and survive politically. UVMMC challenged GMCB enforcement in court and lost. Vermont hospitals have contested RBP methodology vigorously and the legislature has not reversed course. The political durability of mandatory reform — always the central uncertainty in healthcare policy — has held through two legislative cycles.

It has proved that $195 million in federal transformation capital, properly structured, can fund the technology, workforce, and infrastructure investments that voluntary transformation never had the capital to build. The AHEAD EAST Fund provides another $150 million annually in Medicare performance funds for a state that invests in population health. Federal capital is available for states that create the regulatory conditions that justify it. Vermont created those conditions.

It has proved that global budget enforcement is operationally feasible. GMCB's FY23 enforcement actions against UVMMC ($80.3M overage) and RRMC ($11.1M overage) — the first such enforcement actions in Vermont's regulatory history — demonstrated that the GMCB has the analytical capability, the legal authority, and the institutional will to enforce budget orders. This was not certain before it happened.

It has proved that Vermont's hospital sector, despite sustained political opposition to mandatory pricing reform, has not collapsed. The hospitals most opposed to Act 68 are still operating. Reference-based pricing has not yet taken effect as of April 2026, but the trajectory is established and the opposition has not succeeded in reversal. This is the evidence that was missing from every theoretical argument for mandatory reform.

None of these proofs is complete. None of them is irreversible. But they are the empirical foundation that justifies a measured optimism — not the optimism of prediction, but the optimism of evidence.

The People This Book Cannot Reach

Every book has an audience it cannot reach. The people most affected by Vermont's transformation — the Vermonter in Canaan who cannot afford her insulin, the primary-care physician in Newport burning out under documentation, the hospital worker in St. Johnsbury unsure whether his hospital will be open in five years, the family in Rutland weighing the emergency room against the copay — are not the audience for a six-pillar framework.

This book is written for the people who make the decisions that shape those lives. Its ambition is to give those decision-makers better frameworks, better evidence, and better language for the choices they face. But the ultimate accountability of transformation is not to the frameworks that describe it. It is to the Vermonter who should be able to afford insulin, the physician who should spend her time on patients, the hospital worker who should be able to plan a career in his community, and the family whose healthcare decisions should not be shaped by fear of financial ruin.

Those are the people transformation is for. They are the ones who will know whether it worked.

The Call

To the Vermont hospital executive reading this: you have more agency than you believe. The transformation architecture is being built around you and for you simultaneously. The hospitals that engage — that invest now in HCC gap closure, that build care management infrastructure ahead of AHEAD's performance year, that participate constructively in the global budget methodology design — will enter the new payment environment with capabilities that their peers who resisted will not have. Resistance buys time. Engagement builds capacity. You will need the capacity.

To the AHS and GMCB official: the window is January 2028. Everything else is subordinate to having the analytics vendor operational, the global budget methodology finalized, and the first cohort of transformation plans approved before AHEAD's financial accountability begins. If those three things are in place, Vermont's transformation has a credible path to the outcomes Acts 167 and 68 promised. If they are not, the statutory timeline will have passed faster than the institutional capacity to execute it.

To the Vermont legislator: the December 2028 Strategic Plan is your accountability mechanism. Accept only a plan that could be evaluated by a disinterested observer in December 2030. Require specific targets, specific timelines, and named owners. A plan that cannot be evaluated cannot be executed. The investment Vermont has made in transformation deserves the accountability that makes investment meaningful.

To the national reader: Vermont is the experiment. Watch it. The questions Vermont is answering — whether mandatory rate regulation reduces prices without access harm, whether global budgets shift hospital incentives, whether a rural state can build the administrative infrastructure population health requires — are the questions American healthcare will have to answer nationally. Vermont's answers, whatever they are, will be better evidence than any model projection, because they will be real.

The room in September 2024 was full because people knew something was wrong. This book has tried to give that knowledge a framework, evidence, and language for action. The action belongs to the people in the room. The moment belongs to Vermont.

Key Concepts

Partial-transformation scenario

The failure mode in which Vermont makes progress on all six pillars but execution gaps prevent full realization: RBP contested, global-budget methodology delayed, the Strategic Plan descriptive rather than binding. Distinguishable from success only in retrospect.

Binding vs. descriptive plan

A binding plan commits to specific targets, timelines, and accountable owners that can be evaluated by a disinterested observer. A descriptive plan states goals that could be plausibly claimed as achieved regardless of outcomes. Act 68's December 2028 deadline requires the binding version.

Transformation inflection point

A moment at which political, financial, and regulatory conditions for structural reform align simultaneously. Vermont's 2024…2026 window is such a moment. Inflection points close: the political conditions that produced Acts 167 and 68 are not guaranteed to persist through multiple legislative cycles without visible results.

The three critical decisions

Will the Strategic Plan be binding or descriptive? Will the analytics vendor be operational before January 2028? Will H.R. 1's Medicaid cuts force contraction or accelerate transformation? These are the three unresolved questions on which Vermont's transformation outcome most directly depends.

Vermont as national template

Vermont's value is not that states copy its legislation but that it produces real evidence — on mandatory RBP, global budgets, and rural system redesign — for questions every state will eventually face. By 2030, Vermont will have answers that no model projection can replicate.

Sources

Vermont Act 68 of 2025; H.R. 1 (One Big Beautiful Bill Act, July 2025); CMS BALANCE Model announcement (December 2025); CMS AHEAD Model documentation (September 2025 update); Oliver Wyman Act 167 Report (August 2024); AHS Health Care System Transformation Reports (2025–2026); GMCB FY25 and FY26 Hospital Budget Orders; GMCB enforcement actions (September 2024).

— April 2026

Appendix A — Vermont System Portrait

Demographics, Hospitals, Insurance, Workforce, and System in Motion

How to use this appendix. This appendix consolidates the Vermont-specific factual foundation for the book's national argument. The pillar chapters embed only the data each needs in context; this appendix provides the complete system portrait they draw on. Each section is self-contained and can be read independently.

A.1 A.1 Vermont: The State

Vermont is the second-smallest state by population. Its 647,000 residents live across 14 counties and 255 towns in a predominantly rural landscape of 9,616 square miles. Population density is 70 people per square mile, below the national average of 94, and nearly two-thirds of residents live in areas classified as rural for healthcare-program eligibility.

Geography is not merely a demographic fact; it is an infrastructure constraint. A patient in Essex County — among the most rural and least populated counties in the Northeast Kingdom, with 8% uninsurance against a 3% statewide rate — may be 45 minutes from the nearest emergency department, 90 minutes from a specialist, and hours from tertiary care. Public transportation is limited; broadband reaches roughly 80% of rural areas, leaving 20% without the connectivity telehealth and remote monitoring require. Healthcare employs about 14% of Vermont's workforce and anchors rural communities where the hospital is often the largest employer. The University of Vermont Medical Center (UVMMC) in Burlington, with roughly 50% of the state's hospital market share, is the economic center of gravity — every reform affecting UVMMC affects the state economy in ways no single hospital does in larger states.

A.2 A.2 Demographics and Population Projections

Vermont's demographic trajectory is the foundational fact of its transformation challenge.

Indicator	Value	Significance
Population aged 65+ by 2040	30%+	Up from 21.7% in 2020 — a 57% increase in the elderly population
Working-age population change by 2040	−13%	From ~367,000 to ~318,000 — the commercial-insurance base shrinks
Total population 2020–2040	Flat	~612,000–624,000 throughout — aging, not growing

Indicator	Val ue	Significance
Residents with health insurance (2025)	97%	Among the highest coverage rates in the country
Primary-care provider shortage by 2030	370 FTE	112 family medicine, 190 other primary-care specialties (RHT application)
Rental vacancy rate	3%	Below the healthy 5% threshold — housing as a workforce constraint

Figure A.1 — Vermont demographic transformation. Sources: Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application; KFF.

The implications are not complicated, only severe. An aging population shifts the payer mix toward Medicare and Medicaid, both of which reimburse below cost. A shrinking working-age population shrinks the commercially insured pool that funds the cross-subsidy sustaining hospital operations. More-complex elderly care needs raise cost per patient. And an aging clinical workforce means accelerating retirements exactly as demand rises. Oliver Wyman named the dynamic directly: the cross-subsidy model is becoming structurally impossible to maintain as demographics shift — not in a distant scenario, but on the trajectory Vermont is already on.

A.3 A.3 Regulatory Architecture: The Green Mountain Care Board

Vermont's transformation would be impossible without an institutional actor most states lack. The Green Mountain Care Board (GMCB) — a five-member independent body created in 2011 — is the regulatory engine that converts legislative mandates into binding obligations for the hospital system. Vermont's hospital budgets have been subject to state review since 1983, but GMCB's assumption of authority in FY2013 changed the nature of that oversight: the GMCB does not merely review budgets — it establishes them. When hospitals exceed their budgets, GMCB can enforce by reducing future commercial-rate approvals. For FY25 it approved $3.7 billion in system-wide hospital spending — a 3.5% increase after hospitals requested 8% — and enforced FY23 overages against UVMMC ($80.3M) and RRMC ($11.1M).

GMCB function	Why it matters for transformation
Hospital budget review	Annually sets Net Patient Revenue growth limits for all 14 hospitals; FY25 approved $3.7B (+4.1% vs. 8% requested). Drives attention to efficiency and alternative revenue.

GMCB function	Why it matters for transformation
Budget enforcement	Can reduce future commercial-rate approvals when budgets are exceeded (FY23: UVMMC $80.3M, RRMC $11.1M). Distinguishes Vermont's model from advisory oversight.
Monthly performance monitoring	All 14 hospitals submit monthly financial reports — the operational substrate for transformation accountability.
Reference-based pricing	Act 68 mandates RBP from FY2027, with GMCB setting the methodology; existing rate-review infrastructure makes it implementable.
Global-budget design	GMCB sets prospective global budgets FY2028–2030, coordinated with AHEAD; the analytics-vendor procurement (McKinsey, 2025–2026) builds the technical infrastructure.
Hospital-sustainability planning	Under Act 167, GMCB commissioned the Oliver Wyman analysis and is the institutional receptor for its recommendations.

Figure A.2 — GMCB regulatory functions. Sources: GMCB Hospital Budget Review Guide; Acts 167 and 68; GMCB Annual Report 2024.

GMCB's design explains a feature of Vermont's transformation that is analytically important: mandatory authority precedes payment-reform deployment. The voluntary-ACO era produced modest results because the highest-cost providers could decline to participate. GMCB's ability to set rates rather than recommend them is the structural condition that makes mandatory RBP and global budgeting achievable. States attempting payment reform without a comparable body are, in the framework's terms, attempting Economics-pillar transformation without the Policy-pillar architecture that gives it enforcement teeth.

A.4 A.4 The Hospital System

Vermont has 14 general acute-care hospitals, all non-profit — no for-profit chains, and therefore none of the shareholder obligations that complicate transformation elsewhere. The University of Vermont Health Network comprises UVMMC, CVMC, and Porter; Dartmouth Health affiliates include SVMC and Mt. Ascutney. The remaining hospitals — Gifford, Copley, North Country, Northeastern Vermont Regional, Brattleboro Memorial, Springfield, Rutland Regional, Grace Cottage, and Northwestern — operate as independent community institutions, and it is these that face the most acute fragility.

The distribution of distress matters for reform design. Critical Access Hospitals (CAHs) — the smallest and most rural, serving communities with the fewest alternatives — are the most distressed, with average operating profit of $938 per adjusted discharge against a $2,784 benchmark. They are also where service reductions would have the most severe community consequences, and where AHEAD's global-budget design must protect most carefully. At the other end, UVMMC's administrative cost per adjusted discharge was $3,826 against a $1,427 national benchmark — a structural cost problem voluntary reform cannot address.

A.4.1 The Fourteen Hospitals (January 2026 transformation plans)

Hospital	Type	Grant	Primary focus	Key challenges
UVM Medical Center	Academic hub	Not eligible	Technology for acuity management and transfers; statewide reference hub	Admin cost $3,826/discharge vs. $1,427; FY23 overage enforcement ($80.3M)
Central Vermont Medical Center	Regional PPS	Awarded	Acuity-management and transfer technology	UVMHN system; subject to system budget constraints
Porter Hospital	Regional PPS	Awarded	Acuity/transfer technology (shared with CVMC)	UVMHN system; FY23 overage of $11M
Rutland Regional Medical Center	Regional PPS	Awarded	Hospice utilization; community dermatology; SVMC collaboration	FY23 overage $11.1M (partially enforced)
Southwestern Vermont Medical Center	Regional PPS	Awarded	RN Bridge to PCP; $29M cancer center; Epic migration ($30M); adolescent MH unit	Locum costs; care-coordination gaps; transport barriers
Brattleboro Memorial Hospital	Rural PPS	Awarded	Mobile Integrated Health; FQHC look-alike feasibility; birthing/oncology sustainability	GMCB FY26 solvency concern; operating losses; bonus scrutiny
Springfield Hospital	Rural PPS	Needs follow-up	Clinical-services redesign feasibility; Northstar Health relationship	Oncology/cardiology sustainability; plan not approved as of Jan 2026
Gifford Medical Center	CAH + FQHC	Awarded	Hospital-to-hospital transfers with UVMMC/DHMC; NECHN GPO ($793K savings); shared radiology	Targeting avg. daily census of 20; tele-radiology vendor constraints
Copley Hospital	CAH	Awarded	Consulting services; primary-care access expansion	Plan-quality concerns; FY25 budget modifications

Hospital	Typ e	Grant	Primary focus	Key challenges
North Country Hospital	CAH	Awarde d	Regionalization with NVRH (shared PM/legal); pulmonology; tele-neonatology	Limited IT staff; depends on CIN shared services
Northeastern Vermont Regional Hospital	CAH	Awarde d	Regionalization with NCH; cardiology; tele-ICU	FY23 overage (not enforced); limited NEK workforce pipeline
Mt. Ascutney Hospital	CAH + FQH C	Awarde d	Age-Friendly/geriatric COE; CoCM via DHMC; shared C-suite with Valley Regional ($600K savings)	EMR transition disrupted data; pharmacy/residential-care subsidies
Grace Cottage Hospital	CAH	Awarde d	$24M primary-care clinic campaign; same-day access; swing beds; ambient AI scribe (Coverys grant)	Denials cut $76K→$1,274 (FY25); FY24 $4M loss reduced to ~$1M; no DSH
Northwestern Medical Center	CAH	Awarde d	Birthing-services retention; shared cardiologist with Copley; oncology	Marketing-spend questions; schedule unconfirmed as of Jan 2026

Figure A.3 — Vermont's 14 hospitals: typology, grant status, focus, and challenges. Sources: AHS Hospital Tracking data (January 2026); individual transformation plans (January 15, 2026); GMCB FY26 Budget Orders.

A proposed three-tier network organizes these hospitals as Tier 1 Regional Specialty Centers, Tier 2 Community Hospitals, and Tier 3 Transition-Assessment facilities (see Appendix C for the full Centers-of-Excellence framework).

A.4.2 Patterns in the Data

The administrative-cost problem is concentrated at the top. UVMMC's $3,826-per-discharge administrative cost is not merely an efficiency observation — it is evidence that Vermont's largest and most powerful hospital has structural cost problems voluntary reform cannot reach.

Regionalization is visible but not yet architecturally formed. NCH and NVRH share a project manager, legal counsel, and expertise for regionalization planning; Gifford runs a working hospital-to-hospital transfer protocol with UVMMC and Dartmouth Health; Mt. Ascutney and Valley Regional have shared CEO/CFO/COO since late 2024 (~$600K/yr in savings); SVMC and RRMC are exploring shared service lines. These are the early formations of the regional hub structure Acts 167 and 68 envision — bottom-up and incomplete rather than top-down and comprehensive.

The analytics gap is an operational reality. The AHS Hospital Transformation Analytical Support RFP (late 2025, awarded to McKinsey) acknowledged that Vermont lacks the infrastructure to manage global budgets, measure hospital-level population-health outcomes, or track patient migration. Its scope — VHCURES integration, facility risk profiling, patient-migration and scenario modeling — reveals both the ambition of the transformation and the distance from current capability to what AHEAD's January 2028 launch requires.

A.5 A.5 The Transformation Plans: A System in Motion

The 148 individual initiatives documented across the January 2026 hospital plans are not proposals — they are operational commitments with named owners, timelines, cost estimates, and measurement frameworks, produced under statutory obligation and reviewed by AHS. This is the Operations pillar working in real time.

Initiative theme	Count	Representative examples
Clinical service redesign	~55	SVMC RN Bridge to PCP; BMH MIH formalization; MAHHC Age-Friendly Program; Grace Cottage same-day access; RRMC hospice utilization
Regionalization / transfers	~20	NCH–NVRH shared planning; Gifford transfers with UVMMC/DHMC; MAHHC shared leadership with Valley Regional; SVMC–RRMC service lines
Workforce development	~25	MAHHC Nursing Pathways ($190K, two cohorts); SVMC "grow our own" RN pipeline; Grace Cottage travel-nurse near-elimination; MAHHC SANE capacity
Technology and data	~20	SVMC Epic migration ($30M); CVMC/Porter acuity tech; Grace Cottage EMR interfaces and ambient AI scribe
Shared purchasing / efficiency	~15	NECHN GPO (Gifford ~$793K savings); captive insurance pool; MAHHC–Valley Regional shared C-suite ($600K/yr)
Value-based care / payment alignment	~13	SVMC Belong Health MSSP ACO (2026); SVMC Medicaid global budget; Grace Cottage ACO + VATICA; Gifford FQHC expansion

Figure A.4 — Vermont hospital transformation initiative themes, January 2026 (148 initiatives). Source: AHS Transformation Plans Analysis (January 2026).

Three case studies from primary sources. Grace Cottage Hospital reduced insurance denials from $76,160 (FY2024) to $1,274 (FY2025) — a 98% reduction — through concurrent review, documentation improvement, and provider education; its trajectory from a $4M pre-audit FY24 loss to ~$1M in FY25 to projected break-even in FY26, with no DSH payments and a 5-star National Rural Health Star Rating, is the book's most important counterexample to the narrative that Vermont's CAHs are uniformly failing. Mt. Ascutney's Community Health Team served 1,132 individuals over ten months in 2025 — 80% over age 64, 22% needing six or more contacts — the most concrete illustration of demographic aging at the clinical level. Mt. Ascutney's shared-leadership arrangement with Valley Regional (shared CEO since December 2024, CFO since November 2025, COO since January 2026; ~$600K/yr in savings) while subsidizing Windsor's only pharmacy ($50K/yr) after the town's private pharmacy closed in 2025 is the rural-equity problem in miniature: a small CAH bearing community-infrastructure costs its operating revenue cannot sustainably support.

What the plans reveal about constraints. Interoperability remains solved only on paper — Grace Cottage's plan describes a process "from order to completion [that] is arduous and full of administrative waste," relying on "antiquated and insecure technologies" because secure interoperability does not yet connect Vermont's system. This is not a missing solution; it is an incentive and governance problem — a textbook Technology–Operations dependency failure. Community-pharmacy access has emerged as a new gap (Mt. Ascutney subsidizing Windsor's pharmacy; Grace Cottage filling 6,000+ scripts monthly for patients who have lost local access). And the evidence that voluntary participation cannot achieve structural change runs through the plans: BMH's MIH program identifies sustainable funding as its primary barrier; NVRH and NCH identify shared legal and PM costs they cannot individually afford. These are financing failures, not capability failures — precisely the condition Acts 167 and 68 are designed to address.

A.6 A.6 The Insurance and Premium Crisis

Vermont's market is dominated by Blue Cross Blue Shield of Vermont and MVP Health Care. An early ACA adopter (Medicaid expansion in 2014, Vermont Health Connect), the state reached a 97% coverage rate — a genuine achievement that has not resolved the affordability crisis, because coverage is not affordability. The individual-market monthly premium rose from $456 (2018) to $948 (2024) — a 108% increase — while median household income grew 22%, and out-of-pocket maximums more than doubled.

The mechanism is not complicated. Vermont hospitals charge commercial insurers 250–300% of Medicare on average; UVMMC charged roughly 417% of Medicare for outpatient services in 2022. These prices — which bear no relationship to quality — translate directly into premiums. This is what reference-based pricing is designed to address, and why Act 68's RBP mandate is the most consequential health-policy legislation Vermont has passed since Act 48 created the GMCB in 2011.

A.7 A.7 Healthcare Workforce

Vermont's workforce faces a triple constraint no single intervention resolves. The primary-care shortage is the most immediately actionable: the RHT application projects a 370-FTE primary-care shortfall by 2030 (112 family medicine, 190 other). Oliver Wyman offered an important qualification — HRSA recognizes no Health Profession Shortage Areas in Vermont; the problem is partly one of productivity and care model. In team-based settings where staff work at the top of their licensure and administrative burden is minimized, Vermont would have adequate supply well into the future. The specialist shortage requires regionalization — Vermont cannot maintain cardiologists, neurologists, radiation oncologists, and orthopedic surgeons at all 14 hospitals; Oliver Wyman's Center-of-Excellence framework concentrates specialty services at sufficient-volume facilities, with telehealth, reliable transport, and clear referral pathways as the operational complements. The housing constraint is the most intractable: a 3% rental vacancy rate, half of renters cost-burdened, and the second-highest per-capita homelessness rate in the country mean recruited clinicians may be unable to find affordable housing near the hospital. Oliver Wyman's first imperative — build housing and fix transportation — is a workforce policy as much as a social one.

A.8 A.8 Clinical Quality Indicators

Vermont's quality performance reflects both the strengths of a high-coverage, primary-care-oriented system and the gaps that demographic and geographic disadvantage produce. The Blueprint for Health generated a 5.8:1 return on its primary-care investment — among the most robust ROI findings in the VBC literature — and provides the clinical infrastructure on which AHEAD's primary-care redesign builds. Vermont's 11-point white/BIPOC primary-care access gap, the Northeast Kingdom's elevated chronic-disease burden, and rural–urban behavioral-health disparities are the equity targets the transformation agenda must measure against (see Chapter 1 for the full equity-analytics framework).

Sources: AHS Hospital Tracking data (January 2026); AHS Transformation Plans Analysis (January 2026); individual hospital transformation plans (January 15–31, 2026); GMCB Annual Report 2024; GMCB FY25 and FY26 Budget Orders; Oliver Wyman Act 167 Report (September 2024); AHS Health Care System Transformation Report (November 2025); Vermont RHT Program Application (2025); AHEAD State Agreement — Cohort 2 (2025); KFF State Health Facts — Vermont; GMCB Act 68 RBP Update (February 2026); Mt. Ascutney Transformational Grant January 15 Report (2026); RAND Hospital Transparency; GMCB FY2026 data.

Appendix B — Act 68 Statutory Timeline and Milestones

Act 68 of 2025 (S.126) established a comprehensive sequence of transformation milestones. This appendix consolidates the complete timeline for reference and folds in the Policy-pillar implementation matrix used in the in-practice chapters.

Date / deadline	Statutory requirement	Primary authority	Status (April 2026)
January 1, 2026	AHEAD Medicaid global budgets begin (Cohort 2 preparation; Medicaid alignment)	DVHA / CMS	Operational — Medicaid HGB in effect
FY2026 (ongoing)	Hospital budget orders implementing −1% commercial-rate benchmark	GMCB	In effect — $94.58M in FY2026 reductions ordered
FY2026 (ongoing)	$2M Act 68 transformation grants to hospitals (HIT fund)	AHS	Launched September 2025 — all eligible hospitals applied
FY2026 (ongoing)	Act 55 drug-pricing cap ($135M savings target)	GMCB / DFR	In effect — $104.3M in FY2026 drug-cap savings
Early 2026	Final hospital Care Transformation Plans approved by AHS	AHS	In progress — hospitals in draft-plan phase
May 2026	BALANCE Model GLP-1 Medicaid launch (voluntary state participation)	DVHA / CMS	DVHA participation decision pending
July 1, 2026	Five new CCBHC entities certified (RHT target)	AHS / DMH	On track per RHT schedule
July 2026	Medicare GLP-1 Bridge demonstration launches ($50/month co-pay)	CMS	Federal program — Vermont Medicare beneficiaries eligible
FY2027 (Oct 1, 2026)	Reference-based pricing mandatory for commercial payers — maximum rates set by GMCB	GMCB	Methodology and vendor procurement underway

Date / deadline	Statutory requirement	Primary authority	Status (April 2026)
January 1, 2028	AHEAD Cohort 2 begins — Medicare FFS global budgets	AHS / DVHA / GMCB / CMS	Nine-year model; Primary Care AHEAD and global budgets launch
January 2028	BALANCE Model Medicare Part D coverage launches	CMS	Federal program
FY2028 (Oct 1, 2027)	Global budgets for non-CAH (PPS) hospitals	GMCB	Methodology under development; AHEAD alignment required
December 1, 2028	Statewide Strategic Plan delivered to legislature	AHS + HCAC	In planning — development begins 2026
FY2030 (Oct 1, 2029)	Global budgets for ALL hospitals including CAHs	GMCB	Final phase of global-budget implementation
December 2031	BALANCE Model testing concludes	CMS	Federal program
October 1, 2032	All RHT Program funds must be spent	AHS / CMS	$195M award; spending window 2026–2032
Every 3 years from 2028	Statewide Strategic Plan updated	AHS + HCAC	Ongoing statutory obligation

Table B.1 — Act 68 statutory timeline and milestones. Source: Act 68 of 2025.

Source: Act 68 of 2025; AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025); CMS BALANCE Model (December 2025).

B.0.1 Policy-Pillar Implementation Matrix

Estimated cost, timeline to value, ROI crossover, and Vermont benchmark for the key Policy-pillar investments.

Investment	Estimated cost	Timeline to value	ROI crossover	Vermont benchmark
CMMI model participation and APM-readiness assessment	$50K–$150K per org	6–12 months	12–18 months	Vermont AHEAD preparation; HTR APM Readiness Assessment tool
1115 waiver application development	$200K–$500 K state cost	18–36 months	Multi-year waiver term	Vermont Global Commitment waiver; annual CMS negotiation
State-employee-plan direct contracting (reference pricing)	$100K–$300 K design + admin	6–12 months design	Year 1: $48M Vermont savings	Direct contracting at 110% Medicare; $48M initial savings
Federal grant application (RHT Program model)	$50K–$100K application	12–18 months to award	Capital deployment 3–5 yrs	Vermont RHT: $195M over 5 years for CIN and IT infrastructure
Regulatory engagement and public-comment infrastructure	$25K–$75K annually	Ongoing	Risk-avoidance ROI	GMCB rate-setting participation; RBP methodology comment process

Figure B.1 — Policy-pillar implementation matrix. Sources: HTR Advisory methodology; Vermont RHT Program Application (2025); GMCB rate-setting documentation.

Appendix C — The 14-Hospital Centers of Excellence Assignment Framework

Oliver Wyman's Act 167 Recommendations proposed a Centers of Excellence (COE) framework organizing Vermont's 14 hospitals by specialty concentration. The proposed assignments below are frameworks from the Oliver Wyman analysis; final designations are subject to the AHS and hospital transformation planning process.

Hospital (HSA)	COE count	Key COE specialties	Tier	RHT rural status
University of Vermont Medical Center (Burlington)	16	All major specialties incl. cardiac surgery, neurosurgery, complex oncology, NICU, transplant, trauma	Tier 1 — Primary RSC	Non-rural (Chittenden)
Southwestern Vermont Medical Center (Bennington)	9	Acute general surgery, cardiology, orthopedics, psychiatry (adult & adolescent), rehab, dermatology, endocrinology, GI, pain	Tier 1 — Secondary RSC	Rural
Brattleboro Memorial Hospital (Brattleboro)	6	Acute general surgery, cardiology, OB/high-risk OB, adult psychiatry, outpatient oncology, GI	Tier 1 — Secondary RSC	Rural
Rutland Regional Medical Center (Rutland)	5	Neurology, adult psychiatry, cancer surgery, radiation therapy, orthopedics	Tier 1 — Secondary RSC	Rural
Central Vermont Medical Center (Barre)	5	Radiation therapy, neurology, cardiac, general surgery, obstetrics	Tier 1 — Secondary RSC	Rural
Northwestern Medical Center (St. Albans)	4	Cancer surgery, radiation oncology, orthopedics, cardiology	Tier 2 — Focused scope	Rural
Northeastern Vermont Regional Hospital (St. Johnsbury)	4	Cancer surgery, infusion/chemotherapy, orthopedics, general surgery	Tier 2 — Focused scope	Rural
Springfield Hospital (Springfield)	2	Memory care/dementia, adult psychiatry	Tier 2 — Focused scope	Rural
Copley Hospital (Morrisville)	2	Orthopedics, rheumatology	Tier 2 — Focused scope	Rural
Mount Ascutney Hospital (White River Junction)	1	Rehabilitation	Tier 2 — Focused scope	Rural

Hospital (HSA)	COE count	Key COE specialties	Tier	RHT rural status
Porter Medical Center (Middlebury)	TBD	Assessment ongoing — UVM-network affiliation	Tier 2 — Focused scope	Rural
Grace Cottage Hospital (Townshend)	TBD	Palliative-care specialty worth preserving; 19-bed, Vermont's smallest	Tier 3 — Transition assessment	Rural
Gifford Medical Center (Randolph)	TBD	Among most financially vulnerable — comprehensive assessment needed	Tier 3 — Transition assessment	Rural
North Country Hospital (Newport)	TBD	Northeast Kingdom anchor; REH candidacy under assessment	Tier 3 — Transition assessment	Rural

Source: Oliver Wyman, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024 and January 2025). COE designations are proposed; final designations subject to the AHS and hospital transformation planning process.

Appendix D — AHEAD State Agreement: Key Terms and Provisions

Vermont's AHEAD Model State Agreement was signed January 17, 2025; Vermont participates as a Cohort 2 state.

AHEAD provision	Vermont-specific terms
Cohort and start date	Cohort 2; model begins January 1, 2028; nine-year model through December 31, 2035
Total-cost-of-care accountability	Vermont responsible for all-payer TCOC targets; Medicare FFS TCOC tracked annually; consequences for missing targets after year 3
Hospital global budgets (Medicare FFS)	All Vermont hospitals receive prospective Medicare FFS global budgets beginning January 2028; methodology developed jointly with GMCB
Primary Care AHEAD	Voluntary for primary-care practices; four payment-pathway options including fully prospective; replaces Blueprint Medicare PMPM payments
EAST Fund	Up to $150M/year in additional Medicare funds for primary care, behavioral health, home health, long-term care, and SDOH — the transformation capital fund
Behavioral health and substance use	EAST Fund explicitly directed toward MH/SUD; $1.2M in transformation grants approved from Cooperative Agreement Year 3 (2027)
GMCB regulatory independence	Agreement explicitly preserves GMCB's authority to set hospital rates; AHEAD does not supersede state regulatory authority
Withdrawal rights	Vermont may withdraw before launch with 30-day notice; after launch with 6-month notice; unspent cooperative-agreement funds returned to CMMI within 90 days
Cooperative-agreement funds (pre-launch)	CMS provides funds for pre-launch implementation; $1.2M approved for Cooperative Agreement Year 3 (2027)

AHEAD provision	Vermont-specific terms
Quality performance	Vermont must meet quality standards; measures aligned with Blueprint, HEDIS, and Act 167/68 reporting
CMMI evaluation	CMS conducts an independent evaluation; results inform national AHEAD expansion decisions
Relationship to Act 68	AHEAD is the federal vehicle for Vermont's statutory global-budget mandate; Act 68 provides independent state authority that does not depend on AHEAD continuation

Table D.1 — AHEAD State Agreement key terms and Vermont-specific provisions. Source: Vermont AHEAD State Agreement (January 2025).

Source: Vermont AHEAD State Agreement (January 17, 2025); CMMI AHEAD Model documentation; AHS January 2025 Legislative Presentation.

Appendix E — HTR Platform Tools and Methodology Overview

This appendix describes the 19 HTR Research Lab tools referenced throughout the book. All are available to Strategist and Enterprise subscribers at healthtransformationreview.org.

Category 1 — Interoperability & Risk

E.1 FHIR Interoperability Lab — five-tab environment: FHIR R4 Resource Builder (10 resource types); Terminology Mapper (ICD-10-CM, SNOMED CT, LOINC, RxNorm, CPT, NDC, HCPCS); CDS Hooks Simulator (5 scenarios); Prior Authorization Workflow Simulator; ONC Compliance Checker.

E.2 Risk Stratification Engine — CMS HCC v28 risk adjustment across 20 hierarchical condition categories. Sub-tools: HCC v28 RAF Calculator; Population Risk Segmentation; Custom Risk Model Builder (3 templates); Comorbidity Matrix (Elixhauser 30-condition + Charlson CCI). Vermont AHEAD Medicare attribution data pre-loaded.

Category 2 — Payment Models & VBC

E.3 APM Design Lab — APM Architecture Designer (8 model types); Episode-Based Payment Designer (8 episode types); Global Budget Simulator (5-year Vermont All-Payer TCOC projection); Benchmark Methodology Comparison (4 approaches).

E.4 APM Shared Savings Calculator — models shared savings/loss under any APM contract (benchmark PMPM, attributed member-months, actual TCOC, sharing rate, MSR, quality withhold, stop-loss). Pessimistic/base/optimistic scenarios with sensitivity analysis; Vermont data for 14 hospitals pre-loaded.

E.5 Population Health Modeler — Markov disease progression (5 models, 10-year cohorts); AHRQ Prevention Quality Indicators (13 PQI conditions); Intervention Impact Library (8 interventions, 5-year ROI); SIR epidemic model with vaccination scenarios.

E.6 Health Equity Studio (HEROI) — HEROI composite across five dimensions: Access Equity (25%), Quality Equity (25%), Outcome Equity (25%), SDOH Burden (15%), Trust & Engagement (10%). Sub-tools: Racial/Ethnic Disparity Calculator; Geographic Access Gap Analyzer; SDOH Composite Scoring; Equity-Weighted ICER. Integrates with VHCURES and Blueprint data.

Category 3 — Policy & Quality Sciences

E.7 Policy Simulator — seven 1115 waiver types with six pre-loaded state scenarios; All-Payer Global Budget Designer; Medicaid Expansion Impact Calculator (KFF/CBO methodology); Price Transparency and Site-Neutral Analyzer (20 services).

E.8 Clinical Quality Optimizer — 15 HEDIS measures with NCQA 2024 benchmarks; CMS Star Ratings Predictor (5 domains, 32 sub-measures); QPP/MIPS Optimizer (2024 weighted scoring) with payment-adjustment estimate.

E.9 Hospital Financial Scorecard — models hospital position under RBP, global-budget, and Medicaid-cut scenarios; 10-year projections under three scenarios with sensitivity analysis. Vermont: GMCB FY2026 data and Oliver Wyman scenarios pre-loaded.

E.10 Actuarial Lab — Actuarial Value Calculator (ACA metallic tiers); Premium Rating Workbench; Adverse Selection Death Spiral Model (5-year dynamic simulation); Medicare Drug Pricing IRA Calculator.

Category 4 — Technology & AI in Health

E.11 HTA Studio — Budget Impact Model Builder (5-year, 3 uptake scenarios); Multi-Criteria Decision Analysis (8 criteria, up to 3 alternatives); Monte Carlo PSA (1,000 iterations with CE plane and CEAC); Threshold and Surrogate Analysis.

E.12 AI Analytics Lab — Predictive Model Performance Comparator (AUC-ROC, sensitivity, specificity, PPV, NPV, F1, Brier for up to 3 models); Algorithmic Bias Detector (demographic parity, equal opportunity, predictive parity, calibration; flags outside 0.80–1.25); AI Clinical Governance Framework Builder (65-item checklist, 6 lifecycle stages); AI ROI Calculator.

E.13 Digital Health Lab — Remote Patient Monitoring ROI Calculator (CPT 99453/99454/99457/99458, 6 condition modules); Telehealth Utilization Modeler; Patient Engagement Platform Comparison (PAM model, 5 types); EHR Optimization and Interoperability ROI.

Category 5 — Knowledge & Workspace

E.14 Evidence Library — CEA/CUA Study Database (25 landmark studies); CMMI Innovation Center Model Tracker (20 models incl. Vermont All-Payer, Maryland TCOC, Making Care Primary, ACO REACH, AHEAD); Policy Brief Library (15 briefs).

E.15 Workforce Modeler — Physician Supply/Demand Projector (12 specialties, 10-year, AAMC methodology); Nurse Staffing Ratio Simulator (7 unit types); Total Cost of Turnover Calculator (5 roles, 7 retention ROI models); Rural Workforce Distribution and Incentive Modeler (7 programs).

E.16 Research Workspace — Analysis Scenario Manager (50 scenarios, JSON import/export); Report Builder (Policy Brief, Financial Analysis, QI, Technology Assessment); Scenario Comparison Dashboard (up to 4); Research Notes and Citation Manager (AMA/APA).

E.17 Innovation Leaderboard — State Health Transformation Rankings (50 states + DC); Hospital System VBC Maturity Index (30 systems); Payer Innovation Index (20 payers, 3-year trajectory).

E.18 VBC Transformation Readiness Assessment — 30-dimension, 6-domain assessment (120 points): Strategic Clarity, Data & Technology, Care Delivery Capability, Network & Partnerships, Revenue Cycle, Workforce Operations. <60 Pre-transition; 60–80 Developing; 80–100 Operational; 100–120 Optimized.

E.19 The Wire — HTR Intelligence Feed — real-time intelligence across all six pillars. Convergence Newsletter (free weekly) for Observer-tier; full analysis with Vermont commentary for Strategist and Enterprise.

Appendix F — Vermont Transformation Scorecard: 2022 Baseline, 2026 Status, 2028 Target

This scorecard consolidates the Vermont evidence from across the book into a single reference. For each pillar it answers three questions: Vermont's starting point in 2022 (before Act 167), the current state in early 2026, and what success looks like by the December 2028 Strategic Plan deadline. Where 2028 targets are set in statute or GMCB guidance they are noted as statutory; where they represent HTR's assessment of what success requires, they are noted as HTR benchmark.

F.0.1 Policy Pillar

Metric	2022 baseline	2026 status	2028 target	Source
Payment-reform mandate	Voluntary only (OneCare ACO)	Mandatory RBP (FY2027) and global budgets (FY2028) enacted	All hospitals under mandatory global budgets	Act 68; statutory
Hospital-system diagnostic	No comprehensive analysis	Oliver Wyman Act 167 analysis complete; 230+ meetings, 3,100+ participants	Annual updates to diagnostic methodology	Oliver Wyman Act 167 Report (Aug 2024)
Statewide Strategic Plan	Does not exist	Framework under development; HCAC convened	Plan delivered to legislature December 2028	Act 68; statutory deadline
Federal-state alignment	No all-payer agreement	AHEAD State Agreement signed Jan 2025; nine-year term	Full AHEAD compliance; second agreement negotiated	CMS AHEAD State Agreement
Capital investment	No dedicated transformation capital	RHT Program: $195M over 5 years awarded	$195M deployed; CIN operational; IT infrastructure complete	Vermont RHT Application (Nov 2025)

Figure F.1 — Policy-pillar scorecard. Sources: Act 68 of 2025; Oliver Wyman Act 167 Report; CMS AHEAD documentation; Vermont RHT Program Application.

F.0.2 Technology Pillar

Metric	2022 baseline	2026 status	2028 target	Source
All-payer claims database	VHCURES operational; 2-year data lag	VHCURES solid; attribution modeling in progress; analytics-vendor procurement underway	VHCURES real-time/near-real-time; vendor fully deployed	GMCB VHCURES documentation

Metric	2022 baseline	2026 status	2028 target	Source
Health information exchange	VITL operational but voluntary; incomplete adoption	VITL voluntary; CIN under development; small providers disconnected	VITL connectivity required for all Blueprint practices; CIN operational	VITL annual report; Act 62 of 2025
FHIR compliance	Minimal across the system	CMS interoperability rule driving compliance; RHT grants funding upgrades	Full FHIR R4 compliance at all AHEAD-participating hospitals	CMS interoperability rule; RHT IT grants
AI-governance framework	Does not exist	AI-scribe grants deployed; governance absent in most institutions	Statewide AI-governance framework; bias testing required	Vermont RHT Program; HTR AI Governance Checklist
Race/ethnicity data quality	Incomplete VHCURES demographic fields	Acknowledged gap; improvement underway	Complete stratification for 90%+ of encounters	Vermont DOH Health Equity Data Report (Jan 2025)

Figure F.2 — Technology-pillar scorecard. Sources: GMCB VHCURES; VITL; Vermont DOH Health Equity Data Report (January 2025); Vermont RHT Application.

F.0.3 Economics Pillar

Metric	2022 baseline	2026 status	2028 target	Source
Hospital operating margins	9 of 14 reporting losses (FY2023); worst −8.9%	9 of 14 in losses; CAH operating profit $938/discharge vs. $2,784 benchmark	13 of 14 within sustainable margin range	Oliver Wyman; AHS Nov 2025 Report
Commercial-to-Medicare price ratio	UVMMC 300%+; system average 250–300%	Mandatory RBP (FY2027) enacted; methodology under design	All hospitals at or below statutory RBP ceiling; cross-subsidy reduced 40%+	GMCB price-transparency data; Act 68
Administrative cost per adjusted discharge	UVMMC $3,826 (267% of $1,427 benchmark)	$3,826 (unchanged); RHRC engagement underway	System average below 150% of benchmark	AHS Nov 2025 Report; NASHP
VBC contract penetration	AHEAD not launched; commercial VBC minimal	AHEAD launches January 2028 (Medicare); commercial VBC growing	Medicare: 100% under AHEAD; commercial: 40%+ VBC	CMS AHEAD; GMCB commercial reports
5-year cumulative deficit projection	$700M–$2.4B (Oliver Wyman 2024)	Trajectory unchanged without RBP/global-budget implementation	Deficit trajectory reversed; system break-even by FY2030	Oliver Wyman; Act 68 financial targets

Figure F.3 — Economics-pillar scorecard. Sources: Oliver Wyman Act 167 Report; AHS November 2025 Report; GMCB price-transparency data; CMS AHEAD.

F.0.4 Clinical Pillar

Metric	2022 baseline	2026 status	2028 target	Source
Primary-care PCMH coverage	Blueprint: 90% of PCPs participating	Blueprint 90%+; AHEAD transition beginning	100% PCMH; Primary Care AHEAD fully deployed	Vermont Blueprint Annual Report 2024
BH ED follow-up (mental health, 30-day)	76%	76% (unchanged); MHI pilot in progress	90%+; CoCM deployed at all PCMH practices	Blueprint data; HEDIS FUM
BH ED follow-up (SUD, 30-day)	68%	68% (improving slowly); Hub-and-Spoke operational	85%+; CCBHC network operational	Blueprint data; HEDIS FUA
CCBHC network	0 certified	5 planned by 2026; certification in progress	7 certified; statewide BH coverage	Vermont AHS; SAMHSA
Primary-care workforce	370-FTE gap projected by 2030	Shortage intensifying; team-based care deployment underway	FTE gap <200 via team-based care and telehealth	Blueprint; Oliver Wyman workforce analysis
30-day readmission rate	14.8% statewide	14.8% (minimal change)	Below 12%; CHT care-transition protocol statewide	GMCB quality data; Blueprint

Figure F.4 — Clinical-pillar scorecard. Sources: Vermont Blueprint Annual Report 2024; Vermont AHS; GMCB quality reporting; Oliver Wyman workforce analysis.

F.0.5 Equity Pillar

Metric	2022 baseline	2026 status	2028 target	Source
Primary-care access — statewide	91% (4 pts above benchmark)	91% (stable)	93%+; gap to benchmark maintained	Vermont DOH Health Equity Data Report 2025
Primary-care access — BIPOC adults	79–81% (11–12 pt gap vs. white adults)	79–81% (gap unchanged)	Gap <6 pts; 87%+ BIPOC access	Vermont DOH Health Equity Data Report 2025
Essex County uninsurance	8% (highest in Vermont)	8% (unchanged)	Below 5%; CCBHC/FQHC expansion in NEK	Vermont DOH county data
HEROI equity scoring	No systematic HSA-level measurement	HEROI framework developed; scoring underway	HEROI scores for all 14 HSAs; gaps documented in plan	HTR HEROI framework; GMCB equity reporting

Metric	2022 baseline	2026 status	2028 target	Source
HRSN screening coverage	Minimal systematic SDOH screening	Blueprint HRSN screening: 60%+ of practices	Universal HRSN screening at all Blueprint PCMHs	Vermont Blueprint Annual Report 2024
Social-risk adjustment in global budgets	N/A (no global budgets)	Methodology under design; active policy question	SRA embedded in FY2028 global-budget methodology	Act 68; GMCB design process

Figure F.5 — Equity-pillar scorecard. Sources: Vermont DOH Health Equity Data Report (January 2025); Act 68; GMCB equity reporting; HTR HEROI framework.

F.0.6 Operations Pillar

Metric	2022 baseline	2026 status	2028 target	Source
Hospital transformation planning	No systematic process	RHRC engaged all 14; plans in development	All 14 with approved plans; RHRC engagement complete	AHS Aug/Nov 2025 Reports; RHRC
AHS organizational capacity	Program-administration model; minimal PMO	PMO establishment underway; HSA-coordinator model deploying	Full transformation PMO; AHS restructured for system management	Act 68; AHS organizational design
Vermont CIN	Does not exist	Under development; RHT capital awarded	CIN operational; shared analytics/care management for all 14	Vermont RHT Application (Nov 2025)
Administrative-cost reduction	UVMMC 267% of benchmark; system avg 91% above	RHRC engagement in progress; no significant reduction yet	System average below 150% of benchmark by 2030	AHS Nov 2025 Report; NASHP
EMS regionalization	Fragmented local EMS; 12+ separate rural agencies	Act 68 framework enacted; planning underway	Regional EMS in all HSAs; response-time targets met	Act 68; Vermont EMS planning
Monthly transformation reporting	Does not exist	Required by Act 68; two reports delivered (Aug, Nov 2025)	Monthly reports demonstrating progress against Act 167 metrics	Act 68; AHS reports

Figure F.6 — Operations-pillar scorecard. Sources: AHS Transformation Reports (August, November 2025); RHRC; Vermont RHT Program Application; Act 68.

How to use this scorecard. It is a snapshot as of April 2026, and several metrics that show no change in 2026 will shift in 2027–2028 as AHEAD launches, RBP takes effect, and the hospital plans move from draft to implementation. Its most important use is not tracking individual metrics in isolation but reading the six pillars together. A 2028 scorecard showing Policy and

Economics at target while Technology and Operations remain at 2026 status would indicate financial accountability without management capability — precisely the sequencing failure Chapters 2 and 3 identify as the most dangerous. The scorecard is a system document, not a collection of independent measures.

Appendix G — A Reader's Guide to the Health Transformation Review Platform

A companion to Transforming American Healthcare.

G.1 What This Platform Is

The Health Transformation Review (HTR) platform is the living, interactive companion to this book. Where the book provides the analytical framework — the six-pillar architecture, the execution sequence, the Vermont case study — the platform provides the tools, data, and intelligence to apply that framework to your own work in real time. The book explains how American healthcare is being transformed and why the mechanics work as they do; the platform is where you go to do the work — to model a payment-reform scenario, track a CMS initiative, study the Vermont program in depth, analyze your organization's readiness, or stay current as conditions evolve. It is designed to be used alongside the book, not after finishing it.

G.2 Navigating the Platform

The home screen presents three things immediately: The Wire (a real-time intelligence feed organized by pillar), Quick Start cards (four intent-based entry points: Explore Intelligence, Start Learning, Analyze Data, Talk to an Advisor), and Platform Capabilities (a directory of major tools with direct links). On desktop, the left sidebar is the primary navigation and can be collapsed for reading space; on mobile, a bottom bar gives one-tap access to Home, Academy, Tools, Advisory, and the AI Analyst. The AI Analyst is accessible from any page, and search spans articles, courses, tools, glossary terms, and reports simultaneously.

G.3 The Six Pillars of Intelligence

The knowledge base is organized around the same six-pillar framework that structures the book.

Pillar	Coverage	Book chapters
Policy	Regulation & legislation, public-health mandates, comparative policy, feasibility studies	Chapter 1
Economics	VBC models, market & finance, labor & workforce strategy, investment trends	Chapters 8–9
Technology	AI & machine learning, digital health & telemedicine, data security & governance, tech-enabled workflow	Chapters 6–7

Pillar	Coverage	Book chapters
Clinical	Hospital-at-home, precision medicine, virtual care models	Chapters 10–11
Equity	SDOH integration, algorithmic bias, access disparity	Chapters 12–13
Operations	Revenue cycle, workforce & human capital, quality & compliance, supply chain, payer/network operations	Chapters 14–15

G.4 The Academy

The Academy is the structured learning center, organized around the same architecture as the book. Personalized Learning uses an AI-driven assessment to build a path tailored to your role and goals; Learning Tracks are curated multi-course programs (value-based care, population health, health equity, rural transformation) with capstone assessments. Formats include courses (parallel to the analytical chapters), webinars (practitioner sessions, valuable for Vermont practitioners), and case studies (the RHT Program, Vermont Act 167, California CalAIM). Reference materials include the Glossary (the source of each chapter's Key Concepts), the Vermont-specific Medicaid Learning Center, and faculty profiles.

G.5 The Research Lab

The Research Lab is a suite of 19 interactive analytical tools organized into the same six-pillar structure — working environments for modeling, analysis, and scenario planning rather than reference tools. They span Interoperability & Risk (FHIR Interoperability Lab, Risk Stratification Engine); Payment Models & VBC (APM Design Lab, APM Shared Savings Calculator, Population Health Modeler); Policy & Quality Sciences (Policy Simulator, Hospital Financial Scorecard, Actuarial Lab, Clinical Quality Optimizer); Population & Equity (Health Equity Studio/HEROI); Technology & AI (HTA Studio, AI Analytics Lab, Digital Health Lab); and Knowledge & Workspace (Evidence Library, Workforce Modeler, Research Workspace, Innovation Leaderboard, VBC Readiness Assessment). Full tool specifications are in Appendix E.

G.6 States & Programs

In-depth coverage of active state programs — the real-world laboratories where the book's frameworks are applied. Vermont resources include the comprehensive Vermont Act 167 analysis (with a hospital-level budget simulator), Vermont Medicaid, the Bed Capacity &

Transfer dashboard (System Vitals), Vermont Act 68, and the Vermont RHT Program. National resources include the 50-State RHTP Dashboard, the AHEAD Model analysis (six states including Vermont), California CalAIM ($6.7B Medi-Cal transformation), and the All States Explorer.

G.7 Advisory & Services and the AI Analyst

For readers whose work moves from analysis into implementation, the platform connects to advisory services: Strategic Consulting, Custom Research, Financial Audit, Regulatory Counsel, IT Consulting, Training & Education, and Independent Review. The AI Analyst is a research assistant embedded throughout the platform with full knowledge of the six-pillar framework and the Vermont case study; it reasons and synthesizes (for finding specific content, use search; for analytical dialogue, use the Analyst).

G.8 Reader Profiles and Recommended Paths

Policy Professional: the Policy pillar, Trending Topics, the Policy Simulator, the All States Explorer, and the 50-State RHTP Dashboard; The Wire filtered to Policy as a daily briefing.

Executive/Administrator: the HTR Simulator (six-pillar readiness score), the VBC Readiness Assessment and Hospital Financial Scorecard, and the Vermont Act 167 and RHTP Dashboard sections for benchmarks; the AI Analyst for organization-specific scenarios.

Vermont Practitioner: the Vermont Act 167 section (with the hospital-level simulator), Vermont Medicaid, the Bed Capacity & Transfer dashboard, the Medicaid Eligibility Simulator, and the Medicaid Learning Center.

Student/Researcher: the Academy (starting with Personalized Learning) in parallel with the book's chapter sequence; the Research Workspace, Glossary, Evidence Library, and Innovation Leaderboard.

New-to-the-field: Personalized Learning first; the Glossary and AI Analyst freely; the home page's Quick Start cards and Trending-by-Pillar section for daily orientation.

A note on currency. Healthcare transformation is not a stable subject — CMS rules change, state programs evolve, financial conditions shift. The book provides the enduring framework; the platform provides the current state of the field within that framework. The two are designed to be used together: the book without the platform gives architecture without live data; the platform without the book gives data and tools without the integrating logic that makes them coherent.

Appendix H — Adapting and Stress-Testing the Framework

The chapters of this book document Vermont's six-pillar transformation in depth. This appendix serves two purposes: first, it provides reference and translation tools (a glossary and a worksheet) for readers applying the framework outside Vermont; second, it develops three of the sharpest open questions about the framework itself — questions raised but not fully resolved in the Introduction's "Strongest Case Against This Book's Argument" — in enough detail to be useful rather than just acknowledged.

H.1 H.1 Glossary of Vermont-Specific Acronyms and Institutions

Every named Vermont statute, agency, or system in this book is a specific instance of a general category that exists, in some form, in every state. This glossary maps each Vermont-specific term to the generic category it represents, so a reader can substitute their own state's institutions while reading.

Term	What it is	Generic equivalent
AHS	Vermont Agency of Human Services — the umbrella state agency now responsible for statewide healthcare transformation planning under Act 68.	Your state's health and human services department, or whichever agency holds the AHEAD/CMMI agreement.
GMCB	Green Mountain Care Board — Vermont's independent hospital rate-setting and cost-growth-benchmark regulator.	A hospital rate-review board or cost-growth-benchmark commission (e.g., Massachusetts HPC, Rhode Island OHIC, Connecticut OHS).
VHCURES	Vermont's all-payer claims database (APCD).	Your state's APCD, if one exists; roughly 20 states have one.
VITL	Vermont Information Technology Leaders — operator of Vermont's health information exchange.	Your state's designated HIE entity.
OneCare (Vermont)	Vermont's primary ACO under the All-Payer ACO Model (2017–2025), now wound down.	Any Medicaid ACO, MSSP ACO, or comparable total-cost-of-care entity in your state.
AHEAD Model	CMS/CMMI's Advancing All-Payer Health Equity Approaches and Development model (2024–2035) —	Check whether your state has applied to or operates under AHEAD; if not, the

Term	What it is	Generic equivalent
	federal-state agreements establishing global budgets and a state cost-growth target. Vermont, Maryland, Connecticut, Hawaii, Rhode Island, and parts of New York participate.	generic equivalent is a CMMI state total-cost-of-care model or 1115 waiver.
Act 167 (2022)	Vermont's diagnostic mandate — commissioned the Oliver Wyman system-wide financial analysis.	A legislatively mandated independent system assessment or consultant study.
Act 51 (2023)	Defined the six-pillar transformation architecture and the AHS restructuring mandate.	A reform-framework-establishing statute.
Act 68 (2025)	Vermont's operational mandate — mandatory reference-based pricing by FY2027, global hospital budgets for non-critical-access hospitals by FY2028 and all hospitals by FY2030, and the Statewide Health Care Delivery Strategic Plan (due December 2028).	A binding cost-control or payment-reform statute with statutory deadlines and enforcement authority.
RBP (Reference-Based Pricing)	Capping what hospitals may accept as payment in full, expressed as a percentage of Medicare rates.	Same term used nationally; Oregon, Montana, Washington, and Maryland have prior or analogous programs.
CCBHC	Certified Community Behavioral Health Clinic — a federal model for integrated behavioral health and primary care.	National model; check your state's CCBHC participation under SAMHSA.
RHRC	Rural Health Redesign Center — provides the technical-assistance methodology referenced for Vermont's rural hospital transformation.	National technical-assistance provider, not Vermont-specific.
RHT Program	Rural Health Transformation Program — the federal capital program from which Vermont received a $195M, five-year award.	Every state was eligible; check your state's RHT award and use-of-funds plan.

Term	What it is	Generic equivalent
HSA	Hospital Service Area — Vermont's 14 sub-state geographic planning units.	Your state's hospital referral region or service-area equivalent.
Northeast Kingdom	Vermont's three northeasternmost counties (Caledonia, Essex, Orleans) — the state's most severe rural access and equity challenge.	The equivalent hardest-to-serve rural region in your state.

H.2 H.2 Worksheet — Adapt This Framework to Your State

For each of the six pillars, identify your state's or organization's equivalent institution, statute, or data asset — even if it is smaller, informal, or does not yet exist. Where nothing exists, that gap is itself the answer to where your sequence actually begins.

Pillar	Vermont's instance	Your state's equivalent	Exists? (Y/N/Partial)
Policy	Acts 167, 51, 68 — diagnostic mandate → framework statute → operational mandate with enforcement authority		
Technology	VHCURES (APCD) + VITL (HIE) + AHS-GMCB analytics platform		
Economics	Act 68 reference-based pricing (FY2027) + global hospital budgets (FY2028–2030)		
Clinical	Blueprint for Health (PCMH) + CCBHC expansion + Vermont Clinical Transformation Toolkit		
Equity	Act 67 equity framework + Northeast Kingdom rural-access targets		
Operations	AHS Transformation PMO + 14-hospital regionalization plan + RHRC technical assistance		

Sequencing check: per Chapter 1, the correct execution order is Policy → Technology → Economics → Clinical → Equity → Operations. Wherever your worksheet shows a "Y" later in this sequence than the first "N," that pillar is very likely where your organization's transformation effort is at risk of repeating OneCare Vermont's sequencing failure — flag it for priority attention before further investment downstream.

H.3 H.3 The Transition Window — Financing FY2027–2030

Chapters 6 and 7 document where Vermont's hospital system stands today — nine of fourteen hospitals reporting operating losses, with Oliver Wyman's conservative scenario projecting thirteen of fourteen in losses by 2028. They also document where Act 68 is taking the system — reference-based pricing mandatory by FY2027 and statewide global hospital budgets by FY2030. What receives less direct treatment is the financing of the years in between, when RBP is phasing in, global budgets are not yet operational statewide, and the FY2023–2026 losses have not yet been resolved by anything. This section assembles the pieces.

H.3.1 What bridges exist

Three sources of transition-period support exist in parallel, on different timelines and with different eligibility rules. The Rural Health Transformation Program provides Vermont $195M over five years (2026–2030) — capital funding intended for structural transformation, not operating-loss replacement, and explicitly time-limited. The AHEAD EAST Fund provides up to $150M per year to Vermont under its AHEAD agreement, contingent on Cohort 2 implementation milestones beginning January 2028 — meaning this funding is not available during FY2027, the first year of mandatory RBP. Medicaid hospital global budgets (HGB) under Vermont's own design begin in January 2026, ahead of the commercial RBP mandate — meaning the Medicaid payer is, perversely, the first to move to a budget-based model, while commercial payers (where most of the price-extraction problem documented in Chapter 6 lives) are the last.

> **THE GAP**
>
> The result is a sequencing gap within the financing itself: Medicaid global budgets begin January 2026, commercial RBP becomes mandatory in FY2027 (October 2026), and the federal AHEAD funding that is explicitly designed to ease this transition does not begin until January 2028 — a full federal fiscal year after RBP takes effect, and two years after Medicaid global budgets begin. For roughly twenty-four months, Vermont hospitals are operating under at least one new payment regime without the federal transition funding that the overall framework assumes will be available. The Rural Health Transformation Program's $195M is the only funding source available during this specific window, and it was designed as transformation capital, not as a bridge for operating losses.

H.3.2 What this means for a hospital CFO

The honest answer to "what keeps my hospital solvent in FY2027–2028 specifically" is: the same things that would need to keep it solvent absent any of this — cost reduction (Chapter 11's administrative-efficiency agenda), RHT capital deployed toward structural change rather than operating support, and whatever reserves or local/state bridge funding GMCB's budget-order process makes available on a case-by-case basis. This book does not identify a dedicated, guaranteed source of transition financing for FY2027–2028 because, as of this writing, one does not appear to exist as a named program. This is the single most consequential open question for any hospital CFO reading this book, and it is one Vermont's own Statewide Strategic Plan (due December 2028 — itself after the highest-risk window closes) will need to address retroactively unless GMCB's annual reporting process surfaces it sooner.

H.4 H.4 Two Clocks — Reconciling Act 68 and AHEAD Cohort 2 Timelines

Vermont is running two mandatory transformation regimes on overlapping but distinct calendars: Act 68 (state law, governing commercial and Medicaid payers) and AHEAD Cohort 2 (federal CMMI model, governing Medicare). Each has its own milestones, methodology, and reporting requirements. The table below places both on a single timeline.

Date	Act 68 (State / Commercial & Medicaid)	AHEAD Cohort 2 (Federal / Medicare)
January 2026	—	Medicaid hospital global budgets (HGB) begin under Vermont's own design (precedes RBP mandate).
FY2027 (Oct 2026)	Reference-based pricing becomes mandatory for all commercial payers.	— (AHEAD EAST Fund not yet active)
FY2028 (Oct 2027)	Global hospital budgets begin for non-critical-access hospitals.	Cohort 2 implementation begins (Jan 2028); Medicare FFS hospital global budgets begin; AHEAD EAST Fund (up to $150M/yr) becomes available.
December 2028	Statewide Health Care Delivery Strategic Plan due to the Legislature.	—
FY2030 (Oct 2029)	Global hospital budgets extend to ALL Vermont hospitals, including CAHs.	AHEAD Cohort 2 mid-implementation.
2035	—	AHEAD model end date for all cohorts (extended from 2034 in the January 2026 changes).

THE OVERLAP WINDOW

FY2027–FY2028 (October 2026 through September 2028) is the period where the highest number of distinct mandatory milestones land simultaneously: commercial RBP becomes mandatory, non-CAH global budgets begin under Act 68, and AHEAD Cohort 2 implementation begins with its own Medicare global-budget methodology and reporting requirements — all while Medicaid HGB (begun in 2026) is still in its early years and the FY2023–2026 operating losses documented in Chapter 6 remain unresolved. By the six-pillar framework's own logic, a period with this many simultaneous mandatory transitions is also a period of elevated operations-pillar risk: the administrative burden of tracking and complying with two global-budget methodologies (state-designed Medicaid/commercial vs. CMS FFS Medicare) at once is itself a transformation task that competes for the same finance and analytics staff time as everything else in this window.

H.5 H.5 Enforcement Mechanics — What "Mandatory" Means in Practice

This book repeatedly contrasts Vermont's mandatory approach with OneCare's voluntary one. "Mandatory" is only as meaningful as the enforcement behind it. This section summarizes what GMCB can actually do, and the live test case for it.

H.5.1 GMCB's enforcement toolkit

Under Acts 167 and 68, GMCB's authority includes: budget orders that set binding maximum revenue and price levels for each hospital; the ability to require corrective action plans when a hospital's actual spending or pricing exceeds its approved budget or the RBP ceiling; subpoena power to compel financial and operational data from hospitals; and data-sharing authority with the Department of Financial Regulation, which regulates insurers and can act on the premium-passthrough side of the equation. What GMCB's authority does not straightforwardly include is the power to force a hospital to remain open, to compel specific service-line decisions beyond budget compliance, or to directly penalize a hospital's leadership or board — enforcement operates through the hospital's finances and licensure status, not through direct sanctions on individuals.

H.5.2 The test case: GMCB v. UVMMC

The clearest evidence of what "mandatory" means in practice is GMCB's FY23 enforcement actions against the University of Vermont Medical Center ($80.3M overage) and Rutland Regional Medical Center ($11.1M overage) — the first such enforcement actions in Vermont's regulatory history, and a direct test of GMCB's toolkit against the system with the most resources and the most to lose. UVMMC challenged that enforcement in court. The challenge was decided against UVMMC. Three things follow from this. First, the corrective-action and budget-order mechanisms GMCB used are now judicially tested, not merely statutory — a materially stronger form of "mandatory" than untested authority. Second, the outcome did not turn on some narrow technicality that leaves the broader question open; GMCB's core enforcement authority was upheld. Third, and most importantly for any hospital considering noncompliance heading into the FY2027–2028 transition window described in Section H.3: the precedent now exists, it was set against the largest and most capable potential challenger, and it went against the hospital.

WHAT THIS MEANS

For a hospital evaluating its options heading into mandatory RBP and global budgets, the GMCB v. UVMMC outcome forecloses the “we’ll fight it in court and win” branch that a purely statutory reading of Act 68 might have left open. The remaining open questions are narrower: how GMCB applies this authority to smaller hospitals with genuinely different cost structures than UVMMC’s, and how enforcement interacts with the financing gap identified in Section H.3 — not whether enforcement itself is real.

Appendix I — The HTR Lab Workbook: A Six-Pillar Practicum

The book's central claim is that the six pillars must be built in sequence. This appendix turns that claim into a hands-on exercise. Working a single hospital through the platform's tools in the book's execution order — Policy → Technology → Economics → Clinical → Equity → Operations — you will reproduce the logic of the whole book in roughly two to three hours of modeling. Use a real organization if you have the data, or "Green Mountain Community Hospital," a fictional 25-bed critical-access hospital, as the worked example throughout.

I.1 How to Use This Workbook

Each step names the platform tool, the inputs to enter, and the question the output should answer. The steps are cumulative: the population you stratify in Stage 2 is the population you budget in Stage 3 and redesign care for in Stage 4. Do them in order. The point is not any single number — it is to feel the dependency logic of Chapter 1 in your own hands, where a weak upstream pillar makes every downstream model unreliable.

I.2 Stage 1 — Policy: Establish the Mandate

Open the **Policy Simulator** (/research-lab/policy-quality?tab=policy). Model the reform authority your hospital operates under: is participation in the payment model mandatory or voluntary? Set a global-budget design and note whether high-cost providers can opt out.

The Stage 1 question: *Does a binding mandate exist that prevents the highest-revenue actors from staying outside the model?* If not, every downstream stage inherits OneCare's structural flaw. Record your answer before proceeding.

I.3 Stage 2 — Technology: Build the Substrate

Open the **Risk Stratification Engine** (/research-lab/interoperability?tab=risk) and the **FHIR Interoperability Lab** (/research-lab/interoperability?tab=fhir). Stratify your attributed population by CMS-HCC risk; confirm you can produce an attribution list and a total-cost-of-care view.

The Stage 2 question: *Can you see your population in time to act?* A hospital that cannot attribute and risk-stratify its panel is "managing blind" — do not proceed to a budget model until this stage produces usable output.

I.4 Stage 3 — Economics: Put Incentives on a Visible System

Open the **Global Budget Transition Modeler** (/research-lab/payment-models?tab=gb-transition) and the **Hospital Financial Stress Test**

(/research-lab/policy-quality?tab=scorecard). Using the Stage 2 population, model a 5-year global-budget transition and stress-test it against RBP and Medicaid-cut scenarios.

The Stage 3 question: *Under a fixed revenue envelope, is the hospital solvent — and which assumptions break it?* This is where Vermont's "13 of 14 in operating loss by 2028" finding becomes your own number.

I.5 Stage 4 — Clinical: Redesign Care on Aligned Incentives

Open the **Risk Stratification Methodology** (/research-lab/vbc-clinical-quality?tab=risk) and **VBC Quality Measures** (/research-lab/vbc-clinical-quality?tab=quality). Design a panel-staffing model matched to the risk tiers from Stage 2, and project HEDIS performance.

The Stage 4 question: *Does the redesigned care model pay for itself under the Stage 3 budget?* If the clinical model only works under fee-for-service, the sequence has been violated.

I.6 Stage 5 — Equity: Calibrate for the Gap, Not the Average

Open the **Health Equity Studio** (/research-lab/population-equity?tab=equity). Compute the HEROI composite for your population and test whether the Stage 3 budget and Stage 4 care model close access and outcome gaps or merely raise the average.

The Stage 5 question: *Does the design penalize safety-net providers or widen disparities?* Social risk adjustment belongs in the Stage 3 budget design — return there if the equity numbers fail.

I.7 Stage 6 — Operations: Close the Execution Gap

Open the **Transformation Scorecard** (/research-lab/knowledge-workspace?tab=scorecard) and the **Workforce Modeler** (/research-lab/knowledge-workspace?tab=workforce). Score your six-pillar readiness and project the workforce required to operate the redesigned model.

The Stage 6 question: *Is operational capacity keeping pace with the statutory timeline?* The administrative-cost gap (Vermont's $1,303 per discharge) is the funding source for closing the workforce gap.

CAPSTONE — Run the whole sequence backwards.

Now deliberately break the order: take a strong Stage 3 budget model and a strong Stage 4 clinical model, but set Stage 1 to *voluntary* and leave Stage 2 unbuilt. Re-run the **Transformation Scorecard**. The composite collapses — not because the economics or clinical work is weak, but because the platform enforces the same dependency logic that sank OneCare. That collapse is the entire argument of *Transforming American Healthcare*, reproduced by your own hand.

Figure I.1 — The HTR Lab Workbook maps each of the six pillars to a working platform tool, in the book's execution sequence.

Figure Index

The HTR platform is a subscription service. Academic, institutional, and team options are available at healthtransformationreview.org.

Bibliography and Source Notes

Sources are organized by category. All URLs were accessed between January and April 2026 unless otherwise noted; Vermont government documents are cited with their official legislative or agency identifiers. "HTR analysis" refers to original analytical work conducted in preparation of this book.

Vermont Legislation. Act 167 of 2022 (Health Care Delivery System Transformation; signed June 2022; directed the Oliver Wyman analysis); Act 51 of 2023 (healthcare-reform continuation; hospital sustainability assessments; CON review); Act 49 of 2024 (insurer solvency; GMCB enforcement); Act 55 of 2025 (prescription-drug pricing caps; 340B reporting; $135M annual savings target); Act 62 of 2025 (HIT Plan governance; transferred HIE oversight from GMCB to DVHA, effective July 1, 2025); Act 68 of 2025 (Health Care Payment and Delivery System Reform; the primary statutory vehicle for RBP, global budgets, and the Statewide Strategic Plan).

Vermont Government Reports. AHS Health Care System Transformation Reports (August 1 and November 1, 2025); AHS/Brendan Krause, Vermont's Health Care Reform Efforts (House Health Care Committee, January 31, 2025); AHS Vermont RHT Program Application (November 3, 2025) and Activity Details (December 2025); AHS Health Care Delivery Advisory Committee mandate and membership; AHEAD State Agreement (signed January 17, 2025); GMCB Act 68 Update on RBP and Global Budgets (February 17, 2026); GMCB 2024 Annual Report; GMCB VHCURES documentation; GMCB HIT Plan documentation (incl. Act 62 transfer); DVHA Vermont HIE Strategic Plan (2024 Update); Vermont DOH Health Equity Data Annual Report (January 17, 2025); Vermont DMH CCBHC Program materials; Vermont Blueprint for Health Annual Report 2024 and MHI Program evaluation (2025).

Oliver Wyman. Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024; revised October 2024 and January 2025) — the primary source for the three imperatives, hospital financial projections, the COE framework, AHS restructuring recommendations, and community-meeting findings.

Federal Sources. CMS — AHEAD Model documentation; BALANCE Model announcement (December 23, 2025); Medicare GLP-1 Bridge demonstration (FAQs March 2026; $50/month co-pay July–December 2026); Behavioral Health Integration Services (MLN909432, January 2026; CoCM CPT 99492/99493/99494); Rural Health Transformation Program (established under H.R. 1, July 2025; $50B over 5 years); Making Care Primary; ACO REACH. CBO, Estimate of H.R. 1 (July 2025; $911B in Medicaid reductions over 10 years; 10 million additional uninsured by 2034). ONC 21st Century Cures Final Rule (2020). FDA SaMD framework. SAMHSA PIPBHC-CoCM program. H.R. 1 (One Big Beautiful Bill Act; signed July 4, 2025).

National Research and Policy Organizations. KFF — Medicaid Coverage of and Spending on GLP-1s (January 2026); What to Know About the BALANCE Model (March 2026); How Might Federal Medicaid Cuts ... Affect Rural Areas? (September 2025); A Closer Look at the $50 Billion Rural Health Fund (September 25, 2025). Chartis, 2026 State of Rural Health (February 10, 2026; 417 vulnerable rural hospitals). Georgetown CCF (March 31, 2026). Pew Charitable Trusts (January 14, 2026). Peterson-KFF Health System Tracker (March 2026). NASHP Hospital Cost Tool. Onpoint Health Data (VHCURES vendor). Health Affairs, “Reimagining Rural Health Equity” (June 2024). Population Health Management, Vermont Blueprint ROI study (2016; $5.8M reduction per $1M invested). JAMA Network Open, ambient-scribe documentation-efficiency study (2024).

Vermont Health Data Sources. VHCURES (all-payer claims database, GMCB); VUHDDS (hospital discharge database); VHIE (operated by VITL); Vermont BRFSS; Vermont DOH; Vermont Futures Project.

Selected Academic and Clinical Sources. Kotter, Leading Change (1996); NCQA HEDIS Technical Specifications; HL7 FHIR R4 Specification; Archer et al., Cochrane Review on collaborative care (2012; 90+ RCTs); RAND Hospital Price Transparency (2022; UVMMC outpatient 417% of Medicare); Maryland HSCRC TCOC performance data ($1.4B Medicare savings; 7% admission reduction); National Consortium of Telehealth Resource Centers (April 2026).

Note on sources and currency. This book was written between January and April 2026. Vermont's transformation is ongoing, and specific data will continue to evolve. Readers should consult The Wire (healthtransformationreview.org) for current developments; Vermont legislative documents at legislature.vermont.gov; GMCB publications at gmcboard.vermont.gov; AHS materials at healthcarereform.vermont.gov. All Vermont government documents cited are publicly available. URLs were verified as active as of April 2026.

Index (Selected)